Harmonia

Enterprise Architecture Capability Map

Harmonia-EA-Capability-Map
Version 0.1

Definitive RAD-only LaTeX baseline

Current governed capability architecture
15 October 2026
`Harmonia-GovernedAccess`

Canonical architecture, explicit uncertainty and stable publication identities.

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1. Authority, identity and use

This publication answers: “What capability architecture does the governed RAD currently contain?” Version 0.1 represents the current governed Markdown architecture without extending, completing or reconciling it. The Markdown RAD remains architectural authority; this publication is its definitive LaTeX representation at the recorded baseline. The Architectural Axioms remain the highest design authority.

AX-01 and AX-04 preserve the business and health-information meaning of the model. AX-14 preserves distinctions between capability, behaviour, exposure, progression, relevance and status. AX-17 requires established, candidate and unresolved assertions to remain distinguishable. AX-18 prevents integration/coordination from conferring domain authority. This publication applies those constraints to representation; it makes no architectural change. The selected axiom text, metamodel and K9 decision are reproduced in the semantic-detail part.

1.1. Reading the model

Business Capabilities use only the explicitly established CT1 → CT2 → CT3 hierarchy. Regions are contextual groupings, not ancestors. Business Enabling Capabilities require CT1 → CT2 → CT3 → FT for a fully reconciled Feature, but the current RAD does not establish those BEC tier allocations or complete ancestry. This publication preserves established owners and the explicit partial Client Administration → Person Identity responsibility chain without manufacturing missing tiers. It reports zero *established* BEC CT1, CT2 or CT3 allocations; this does not mean that the 76 established BEC contexts do not exist.

Functions, Services and Processes retain their evidenced owners. They form a graph, not a mandatory Feature → Function → Service/Process tree. A Function need not have a Feature, Service or Process association. Enterprise Capabilities retain the fourteen governed entries and their actual descriptive responsibilities; no additional Enterprise decomposition is introduced.

Each element has a stable label, metadata and a link to exact RAD source detail. The semantic-detail part preserves the source definitions, boundaries, state-change statements, illustrative activities, examples, exclusions, notes, classifications, relationship tables and unresolved matters. Its source headings and retained diagram notation are not new containment assertions. The complete `manifest.json` records exact source lines and fingerprints; `source-coverage.json` records the transcribed blocks. Source references such as “business:152” mean source-key and Markdown line, not a new architectural identifier.

1.2. Definitive publication identity

Existing Feature identifiers (for example `FEAT-EM-01`) and Enterprise identifiers (`EC-01` through `EC-14`) are preserved exactly. Catalogue ordinals, BC-18-style source shorthand and CT syntax examples are not allocated structural Canonical IDs. No structural Canonical ID is allocated here.

For elements without an allocated identifier, the frozen Reference Id uses `HCM-`*`type-semantic-name-token`*. Type tokens are `BC`, `BEC`, `FN`, `SV` and `PR`. The initial token is the lowercase name with runs of non-alphanumeric characters replaced by a hyphen. This is a deterministic initial allocation rule only: **after v0.1, the manifest value must be reused verbatim, including after rename or relocation**. It is not regenerated from a future name, parent, tier, ordinal, chapter or build. A collision requires an explicitly recorded allocation, never a position-based suffix; this baseline has no collisions.

The Reference Id is a definitive publication identity, distinct from the metamodel’s contextual structural Canonical ID. The metamodel requires a structural ID to change when responsibility

context changes, retaining the former value as an alias. Keeping these two fields separate satisfies both that rule and publication identity stability. Unallocated or ancestry-dependent structural IDs remain unresolved.

Stable LaTeX label namespaces are `cap:bc-`, `cap:bec-`, `cap:ec-`, `feature:`, `function:`, `service:` and `process:`. The exact values are frozen in the manifest and are also preserved across relocation or rename. Source-detail labels are locators, not architectural element identities. Internal reference-candidate record keys are retained for navigation continuity only; they are neither Canonical IDs nor aliases.

1.3. Architectural status and uncertainty

Canonical means that the element's identity/responsibility is established by the cited governed RAD. It does not establish every possible property or relationship of that element. **Candidate** preserves a named provisional concern without approving its placement or decomposition. **Unresolved** qualifies a particular unestablished dimension or relationship. It is not a fifth Harmonia relevance classification. Source-specific provisional proposals remain provisional in their source prose; occurrence in a canonical document does not promote them to Canonical.

Health Service Governance is a Canonical CT1 with unresolved relevance and enabling derivation. The three assurance BECs are Canonical responsibilities with unresolved tiers, ancestry, view placement, relevance and Feature decomposition. All 137 Features are established FT identities with unresolved complete capability ancestry. The 69 enabling catalogue descriptions with unestablished standalone element identity are not counted as new Capabilities.

1.4. Reconciled publication counts

Architectural category	RAD / publication count
Business CT1 / CT2 / CT3	18 / 39 / 133
Canonical Business Capabilities	190
Named candidate BC concerns with unresolved placement	3 (separate from 190)
Established BEC CT1 / CT2 / CT3 assignments	0 / 0 / 0; tiers unresolved
Canonical BEC responsibility contexts	76
Features / Functions / Services / Processes	137 / 147 / 133 / 19
Enterprise Capabilities	14
Named enabling decomposition descriptions	73; 4 independently established, 69 unresolved identities
Definitive architectural-element publication records	719; 716 Canonical, 3 Candidate

Counts are of independently established identities or explicitly retained candidates, not source occurrences, descriptive bullets or possible future peers. The reference candidate capture was used to locate current Markdown evidence; its candidate standing supplies no architectural approval. Counts and identifiers are checked against the actual Markdown inventories and explicit parent assertions.

1.5. Preserved issues requiring later architectural review

The full source qualifications remain controlling. In particular:

- BEC tier/ancestry reconciliation, 137 Feature frames and 69 description identities remain unresolved under K9; the five views do not resolve them.
- Health Information Exchange identifies distinct BC and BEC responsibilities. Their naming collision remains unresolved; they are not merged or renamed.

- Medication compounding/preparation (OI-BC-001), operational-readiness placement (OI-BC-002), maintenance membership and sharing-programme placement remain unadjudicated. Clinical Intervention's proposed three-child taxonomy remains provisional; Clinical Determination and Individual Care Planning have no adjudicated CT3 set.
- Operational Credential Management remains distinct from Clinical Qualification Management and its historical name alias. Name similarity does not establish Feature association.
- The existing care-team collaboration Function description mentions managing clinical handovers while the metamodel excludes that authority. Both source assertions are retained for review; this publication does not resolve the conflict.
- Process illustrations retain their stated scope and unresolved transitions, including Encounter ON_LEAVE and the disconnected stalled/failed Synthetic Task outcome. Assurance lifecycle detail and internal Service/Function mappings remain unestablished; no additional state or edge is inferred.
- The assurance Strategy source's opening reconciliation qualifies older BC-18 tier/relevance wording. Its Strategy stop boundary does not erase the later approved Domain03 assurance catalogue. B2.1's retained L1/L2 terminology does not allocate or override current CT classifications.

No legacy documentation or architectural material from the earlier candidate LaTeX publications was used. Version 0.2 derivation and reconciliation are outside this baseline.

2. Business Capabilities

The hierarchy below contains only explicit governed BC tier and parent assertions. Full definitions, decomposition assertions, boundaries, illustrative activities, care contexts, exclusions and stopping points are reproduced in the BC source detail. Relevance is not inherited. Regions are contextual groupings.

2.1. Individual Care Delivery

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-individual-care-delivery`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT1	Explicit source assertion.
Ancestry	CT1 root	Explicitly established.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.
Source	business:148 (p. 323); business:152 (p. 323); business:120 (p. 322)	Linked RAD detail; exact locators in manifest.
Enabled By	Health Information Access	Bounded support; not decomposition or runtime dependency.

Definition: Assess, clinically determine, plan, intervene, monitor and review individual care independently of setting, mode, time pattern or organisational arrangement.

Contained capabilities

Capability	Reference Id	Tier
Individual Health Assessment (p. 4)	`HCM-BC-individual-health-assessment`	CT2
Clinical Determination (p. 6)	`HCM-BC-clinical-determination`	CT2
Individual Care Planning (p. 6)	`HCM-BC-individual-care-planning`	CT2
Clinical Intervention (p. 7)	`HCM-BC-clinical-intervention`	CT2
Individual Health Monitoring (p. 7)	`HCM-BC-individual-health-monitoring`	CT2
Individual Care Review (p. 8)	`HCM-BC-individual-care-review`	CT2

2.1.1. Individual Health Assessment

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-individual-health-assessment`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	Individual Care Delivery (p. 4)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.

Name	Value	Comment / Description
Source	business:160 (p. 323); business:162 (p. 323); business:316 (p. 325); business:320 (p. 325); business:330 (p. 325)	Linked RAD detail; exact locators in manifest.
Enabled By	Health Information Access	Bounded support; not decomposition or runtime dependency.

Definition: The capability to establish an understanding of an individual's current health state through the collection, consideration and evaluation of relevant clinical, health and contextual information.

Contained capabilities

Capability	Reference Id	Tier
Health History Assessment (p. 5)	`HCM-BC-health-history-assessment`	CT3
Clinical Examination (p. 5)	`HCM-BC-clinical-examination`	CT3
Health Status Measurement (p. 6)	`HCM-BC-health-status-measurement`	CT3

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.1.1.1. Health History Assessment

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-history-assessment`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Individual Health Assessment (p. 4)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:326 (p. 325); business:322 (p. 325); business:330 (p. 325)	Linked RAD detail; exact locators in manifest.

Establish relevant health knowledge from the individual's history and applicable historical/ contextual information.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.1.1.2. Clinical Examination

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-examination`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Individual Health Assessment (p. 4)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:327 (p. 325); business:322 (p. 325); business:330 (p. 325)	Linked RAD detail; exact locators in manifest.

Establish health findings through clinical examination.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.1.1.3. Health Status Measurement

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-status-measurement`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Individual Health Assessment (p. 4)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:328 (p. 325); business:322 (p. 325); business:330 (p. 325)	Linked RAD detail; exact locators in manifest.

Establish individual health observations or measurements.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.1.2. Clinical Determination

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-determination`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	Individual Care Delivery (p. 4)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:186 (p. 323); business:188 (p. 323)	Linked RAD detail; exact locators in manifest.
Enabled By	Health Information Access	Bounded support; not decomposition or runtime dependency.

Definition: The capability to interpret assessed health information and establish clinically meaningful conclusions about an individual's health state, condition, risk or prognosis.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.1.3. Individual Care Planning

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-individual-care-planning`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	Individual Care Delivery (p. 4)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:211 (p. 324); business:213 (p. 324)	Linked RAD detail; exact locators in manifest.
Enabled By	Health Information Access	Bounded support; not decomposition or runtime dependency.

Definition: The capability to establish the intended clinical response to an individual's assessed and clinically interpreted health state, including the objectives, interventions and monitoring required to progress their care.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.1.4. Clinical Intervention

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-intervention`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	Individual Care Delivery (p. 4)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:237 (p. 324); business:239 (p. 324)	Linked RAD detail; exact locators in manifest.
Enabled By	Health Information Access	Bounded support; not decomposition or runtime dependency.

Definition: The capability to perform clinical actions intended to alter, maintain or otherwise manage an individual's health state in accordance with an established care intent.

Unresolved dimensions: relevance; intervention taxonomy. See the linked RAD detail and uncertainty register.

2.1.5. Individual Health Monitoring

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-individual-health-monitoring`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	Individual Care Delivery (p. 4)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:263 (p. 324); business:265 (p. 324); business:332 (p. 325); business:336 (p. 325); business:345 (p. 326)	Linked RAD detail; exact locators in manifest.
Enabled By	Health Information Access	Bounded support; not decomposition or runtime dependency.

Definition: The capability to observe and evaluate an individual's health state over time in order to identify clinically meaningful change, stability or deviation from an expected state or trajectory.

Contained capabilities

Capability	Reference Id	Tier
Health State Monitoring (p. 8)	`HCM-BC-health-state-monitoring`	CT3
Care Response Monitoring (p. 8)	`HCM-BC-care-response-monitoring`	CT3

Unresolved dimensions: relevance; semantic relationship representation. See the linked RAD detail and uncertainty register.

2.1.5.1. Health State Monitoring

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-state-monitoring`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Individual Health Monitoring (p. 7)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:342 (p. 326); business:338 (p. 326); business:345 (p. 326)	Linked RAD detail; exact locators in manifest.

Establish an aggregate, temporally situated view of the individual's health from applicable monitored data.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.1.5.2. Care Response Monitoring

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-care-response-monitoring`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Individual Health Monitoring (p. 7)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:343 (p. 326); business:338 (p. 326); business:345 (p. 326)	Linked RAD detail; exact locators in manifest.

Scope that health-state view to a particular problem/condition and the treatment applied to it.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.1.6. Individual Care Review

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-individual-care-review`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	Individual Care Delivery (p. 4)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:289 (p. 325); business:291 (p. 325); business:347 (p. 326); business:351 (p. 326); business:360 (p. 326)	Linked RAD detail; exact locators in manifest.
Enabled By	Health Information Access	Bounded support; not decomposition or runtime dependency.

Definition: The capability to evaluate the progress and effectiveness of an individual's care against its intended objectives and establish the continuing appropriateness of the current care intent.

Contained capabilities

Capability	Reference Id	Tier
Care Appropriateness Review (p. 9)	`HCM-BC-care-appropriateness-review`	CT3
Care Effectiveness Review (p. 9)	`HCM-BC-care-effectiveness-review`	CT3

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.1.6.1. Care Appropriateness Review

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-care-appropriateness-review`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Individual Care Review (p. 8)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:357 (p. 326); business:353 (p. 326); business:360 (p. 326)	Linked RAD detail; exact locators in manifest.

Evaluate whether the current intended care remains suitable in the individual's current context.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.1.6.2. Care Effectiveness Review

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-care-effectiveness-review`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Individual Care Review (p. 8)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:358 (p. 326); business:353 (p. 326); business:360 (p. 326)	Linked RAD detail; exact locators in manifest.

Evaluate whether delivered care is achieving or has achieved its intended goals and outcomes.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.2. Care Access & Coordination

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-care-access-coordination`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT1	Explicit source assertion.
Ancestry	CT1 root	Explicitly established.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.
Source	business:362 (p. 326); business:366 (p. 326); business:120 (p. 322)	Linked RAD detail; exact locators in manifest.

Definition: Managing patient intake, referral triage, appointment scheduling, multidisciplinary care team coordination, cross-facility patient transfers, and care continuity across organisational boundaries.

Contained capabilities

Capability	Reference Id	Tier
Care Access Management (p. 10)	`HCM-BC-care-access-management`	CT2
Care Scheduling & Capacity Management (p. 11)	`HCM-BC-care-scheduling-capacity-management`	CT2
Care Coordination & Transition (p. 14)	`HCM-BC-care-coordination-transition`	CT2
Care Support Logistics Management (p. 15)	`HCM-BC-care-support-logistics-management`	CT2

2.2.1. Care Access Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-care-access-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	Care Access & Coordination (p. 9)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:370 (p. 326); business:372 (p. 326); business:374 (p. 326); business:384 (p. 327)	Linked RAD detail; exact locators in manifest.

Definition: Establish and manage access to applicable care.

Contained capabilities

Capability	Reference Id	Tier
Care Intake Management (p. 10)	`HCM-BC-care-intake-management`	CT3
Care Referral Management (p. 11)	`HCM-BC-care-referral-management`	CT3
Care Eligibility Determination (p. 11)	`HCM-BC-care-eligibility-determination`	CT3

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.2.1.1. Care Intake Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-care-intake-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Care Access Management (p. 10)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:380 (p. 326); business:376 (p. 326); business:384 (p. 327)	Linked RAD detail; exact locators in manifest.

Establish and manage the intake context through which an individual seeks or enters care.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.2.1.2. Care Referral Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-care-referral-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Care Access Management (p. 10)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:381 (p. 326); business:376 (p. 326); business:384 (p. 327)	Linked RAD detail; exact locators in manifest.

Manage a request for another participant/service to assess, manage or assume applicable care responsibility.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.2.1.3. Care Eligibility Determination

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-care-eligibility-determination`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Care Access Management (p. 10)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:382 (p. 326); business:376 (p. 326); business:384 (p. 327)	Linked RAD detail; exact locators in manifest.

Determine whether applicable eligibility conditions for care access are satisfied.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.2.2. Care Scheduling & Capacity Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-care-scheduling-capacity-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	Care Access & Coordination (p. 9)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:386 (p. 327); business:388 (p. 327); business:390 (p. 327); business:403 (p. 327)	Linked RAD detail; exact locators in manifest.

Definition: Manage scheduled opportunities, care places, usable operational capacity and care-demand resourcing.

Contained capabilities

Capability	Reference Id	Tier
Care Appointment Management (p. 12)	`HCM-BC-care-appointment-management`	CT3
Care-Place Allocation (p. 12)	`HCM-BC-care-place-allocation`	CT3
Care-Place Readiness Management (p. 13)	`HCM-BC-care-place-readiness-management`	CT3
Care Resource Deployment (p. 13)	`HCM-BC-care-resource-deployment`	CT3
Operational Care Flow Coordination (p. 13)	`HCM-BC-operational-care-flow-coordination`	CT3
Operational Care Capacity Management (p. 14)	`HCM-BC-operational-care-capacity-management`	CT3

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.2.2.1. Care Appointment Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-care-appointment-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Care Scheduling & Capacity Management (p. 11)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:396 (p. 327); business:392 (p. 327); business:403 (p. 327)	Linked RAD detail; exact locators in manifest.

Establish and manage a scheduled opportunity for care.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.2.2.2. Care-Place Allocation

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-care-place-allocation`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Care Scheduling & Capacity Management (p. 11)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:397 (p. 327); business:392 (p. 327); business:403 (p. 327)	Linked RAD detail; exact locators in manifest.

Allocate an applicable care place to a care requirement.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.2.2.3. Care-Place Readiness Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-care-place-readiness-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Care Scheduling & Capacity Management (p. 11)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:398 (p. 327); business:392 (p. 327); business:403 (p. 327)	Linked RAD detail; exact locators in manifest.

Establish and manage whether a particular care place is ready for allocation or use.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.2.2.4. Care Resource Deployment

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-care-resource-deployment`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Care Scheduling & Capacity Management (p. 11)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:399 (p. 327); business:392 (p. 327); business:403 (p. 327)	Linked RAD detail; exact locators in manifest.

Apply legitimately available and appropriately authorised resources to a current or planned operational care requirement, particularly using roster/on-call and service-coverage context.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.2.2.5. Operational Care Flow Coordination

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-operational-care-flow-coordination`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Care Scheduling & Capacity Management (p. 11)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:400 (p. 327); business:392 (p. 327); business:403 (p. 327)	Linked RAD detail; exact locators in manifest.

Coordinate operational care flow and its progression within the care-delivery environment.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.2.2.6. Operational Care Capacity Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-operational-care-capacity-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Care Scheduling & Capacity Management (p. 11)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:401 (p. 327); business:392 (p. 327); business:403 (p. 327)	Linked RAD detail; exact locators in manifest.

Understand and manage the service's usable capacity to accommodate care in its current operational context.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.2.3. Care Coordination & Transition

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-care-coordination-transition`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	Care Access & Coordination (p. 9)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:405 (p. 327); business:407 (p. 327); business:409 (p. 327); business:419 (p. 327)	Linked RAD detail; exact locators in manifest.

Definition: Coordinate care participation, actual responsibility transition and discharge.

Contained capabilities

Capability	Reference Id	Tier
Care-Team Coordination (p. 14)	`HCM-BC-care-team-coordination`	CT3
Care Responsibility Transition (p. 15)	`HCM-BC-care-responsibility-transition`	CT3
Care Discharge Coordination (p. 15)	`HCM-BC-care-discharge-coordination`	CT3

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.2.3.1. Care-Team Coordination

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-care-team-coordination`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Care Coordination & Transition (p. 14)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:415 (p. 327); business:411 (p. 327); business:419 (p. 327)	Linked RAD detail; exact locators in manifest.

Coordinate participating care-team context and collaboration around care.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.2.3.2. Care Responsibility Transition

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-care-responsibility-transition`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Care Coordination & Transition (p. 14)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:416 (p. 327); business:411 (p. 327); business:419 (p. 327)	Linked RAD detail; exact locators in manifest.

Establish and coordinate who has actually assumed applicable care responsibility.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.2.3.3. Care Discharge Coordination

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-care-discharge-coordination`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Care Coordination & Transition (p. 14)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:417 (p. 327); business:411 (p. 327); business:419 (p. 327)	Linked RAD detail; exact locators in manifest.

Coordinate applicable discharge requirements and progression.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.2.4. Care Support Logistics Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-care-support-logistics-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	Care Access & Coordination (p. 9)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:421 (p. 328); business:423 (p. 328); business:425 (p. 328); business:435 (p. 328)	Linked RAD detail; exact locators in manifest.

Definition: Coordinate bounded operational support needed for care access and fulfilment.

Contained capabilities

Capability	Reference Id	Tier
Patient Movement Coordination (p. 16)	`HCM-BC-patient-movement-coordination`	CT3
Clinical Material Logistics Coordination (p. 16)	`HCM-BC-clinical-material-logistics-coordination`	CT3
Care Support Work Coordination (p. 16)	`HCM-BC-care-support-work-coordination`	CT3

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.2.4.1. Patient Movement Coordination

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-patient-movement-coordination`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Care Support Logistics Management (p. 15)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:431 (p. 328); business:427 (p. 328); business:435 (p. 328)	Linked RAD detail; exact locators in manifest.

Coordinate physical patient transit and applicable mobility constraints.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.2.4.2. Clinical Material Logistics Coordination

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-material-logistics-coordination`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Care Support Logistics Management (p. 15)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:432 (p. 328); business:427 (p. 328); business:435 (p. 328)	Linked RAD detail; exact locators in manifest.

Coordinate clinically relevant material movement, custody, availability or disposition needed for care or diagnostic fulfilment.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.2.4.3. Care Support Work Coordination

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-care-support-work-coordination`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Care Support Logistics Management (p. 15)	Immediate BC parent.

Name	Value	Comment / Description
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:433 (p. 328); business:427 (p. 328); business:435 (p. 328)	Linked RAD detail; exact locators in manifest.

Coordinate authorised operational support work: what needs doing, who performs it and how progression, completion or escalation is coordinated.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.3. Health Rights, Advocacy & Participation

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-rights-advocacy-participation`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT1	Explicit source assertion.
Ancestry	CT1 root	Explicitly established.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Adjacent	Independent of Architectural Status.
Source	business:437 (p. 328); business:441 (p. 328); business:120 (p. 322)	Linked RAD detail; exact locators in manifest.

Definition: Protecting patient rights, facilitating informed consent, managing advance care directives, supporting patient advocacy, and enabling consumer participation in health decisions.

2.4. Diagnostic, Therapeutic & Clinical Support Services

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-diagnostic-therapeutic-clinical-support-services`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT1	Explicit source assertion.
Ancestry	CT1 root	Explicitly established.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.
Source	business:447 (p. 328); business:451 (p. 328); business:120 (p. 322)	Linked RAD detail; exact locators in manifest.

Definition: The specialised clinical services that support direct patient care, including pathology and laboratory testing, diagnostic imaging (radiology), pharmacy and medication dispensing, and allied health therapies.

Contained capabilities

Capability	Reference Id	Tier
Diagnostic Support Provision (p. 18)	`HCM-BC-diagnostic-support-provision`	CT2
Medication & Pharmacy Services (p. 21)	`HCM-BC-medication-pharmacy-services`	CT2

2.4.1. Diagnostic Support Provision

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-diagnostic-support-provision`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	Diagnostic, Therapeutic & Clinical Support Services (p. 17)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:457 (p. 328); business:459 (p. 328); business:461 (p. 328); business:475 (p. 329)	Linked RAD detail; exact locators in manifest.

Definition: Provide diagnostic services and manage their evidence, interpretation and governed reporting.

Contained capabilities

Capability	Reference Id	Tier
Diagnostic Service Appointment Management (p. 18)	`HCM-BC-diagnostic-service-appointment-management`	CT3
Diagnostic Service Delivery (p. 19)	`HCM-BC-diagnostic-service-delivery`	CT3
Diagnostic Specimen Management (p. 19)	`HCM-BC-diagnostic-specimen-management`	CT3
Diagnostic Data Management (p. 19)	`HCM-BC-diagnostic-data-management`	CT3
Diagnostic Interpretation (p. 20)	`HCM-BC-diagnostic-interpretation`	CT3
Diagnostic Report Management (p. 20)	`HCM-BC-diagnostic-report-management`	CT3
Diagnostic Report Distribution (p. 20)	`HCM-BC-diagnostic-report-distribution`	CT3

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.4.1.1. Diagnostic Service Appointment Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-diagnostic-service-appointment-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Diagnostic Support Provision (p. 18)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:467 (p. 329); business:463 (p. 328); business:475 (p. 329)	Linked RAD detail; exact locators in manifest.

Manage the diagnostic-service-specific scheduled opportunity, including preparation, modality, urgency, duration and other service-specific constraints.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.4.1.2. Diagnostic Service Delivery

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-diagnostic-service-delivery`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Diagnostic Support Provision (p. 18)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:468 (p. 329); business:463 (p. 328); business:475 (p. 329)	Linked RAD detail; exact locators in manifest.

Perform the diagnostic activity that obtains or produces diagnostic evidence.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.4.1.3. Diagnostic Specimen Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-diagnostic-specimen-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Diagnostic Support Provision (p. 18)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:469 (p. 329); business:463 (p. 328); business:475 (p. 329)	Linked RAD detail; exact locators in manifest.

Maintain a specimen’s identity, integrity, suitability, custody and applicable lifecycle context.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.4.1.4. Diagnostic Data Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-diagnostic-data-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Diagnostic Support Provision (p. 18)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:470 (p. 329); business:463 (p. 328); business:475 (p. 329)	Linked RAD detail; exact locators in manifest.

Manage acquired or produced diagnostic evidence as information, including identity, association, integrity, organisation, availability and lifecycle.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.4.1.5. Diagnostic Interpretation

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-diagnostic-interpretation`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Diagnostic Support Provision (p. 18)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:471 (p. 329); business:463 (p. 328); business:475 (p. 329)	Linked RAD detail; exact locators in manifest.

Establish what diagnostic evidence demonstrates within the diagnostic service's professional scope.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.4.1.6. Diagnostic Report Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-diagnostic-report-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Diagnostic Support Provision (p. 18)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:472 (p. 329); business:463 (p. 328); business:475 (p. 329)	Linked RAD detail; exact locators in manifest.

Establish and govern the authoritative expression of diagnostic findings, including applicable creation, review, amendment, verification, finalisation and authorisation/signing.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.4.1.7. Diagnostic Report Distribution

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-diagnostic-report-distribution`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Diagnostic Support Provision (p. 18)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:473 (p. 329); business:463 (p. 328); business:475 (p. 329)	Linked RAD detail; exact locators in manifest.

Provide an established report to applicable authorised recipients or consuming contexts under applicable access, privacy, security and communication controls.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.4.2. Medication & Pharmacy Services

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-medication-pharmacy-services`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	Diagnostic, Therapeutic & Clinical Support Services (p. 17)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:477 (p. 329); business:479 (p. 329); business:481 (p. 329); business:492 (p. 329); business:1184 (p. 348)	Linked RAD detail; exact locators in manifest.

Definition: Provide professional medication/pharmacy services under applicable controls.

Contained capabilities

Capability	Reference Id	Tier
Prescription Medication Dispensing (p. 21)	`HCM-BC-prescription-medication-dispensing`	CT3
Non-Prescription Medication Provision (p. 22)	`HCM-BC-non-prescription-medication-provision`	CT3
Medication Advice (p. 22)	`HCM-BC-medication-advice`	CT3
Medication Dose Administration Aid Provision (p. 22)	`HCM-BC-medication-dose-administration-aid-provision`	CT3

Unresolved dimensions: relevance; medication compounding. See the linked RAD detail and uncertainty register.

2.4.2.1. Prescription Medication Dispensing

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-prescription-medication-dispensing`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Medication & Pharmacy Services (p. 21)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:487 (p. 329); business:483 (p. 329); business:492 (p. 329)	Linked RAD detail; exact locators in manifest.

Professionally prepare and provide medication according to an applicable authorised prescription/order and dispensing requirements.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.4.2.2. Non-Prescription Medication Provision

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-non-prescription-medication-provision`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Medication & Pharmacy Services (p. 21)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:488 (p. 329); business:483 (p. 329); business:492 (p. 329)	Linked RAD detail; exact locators in manifest.

Provide medication not requiring an applicable prescription, subject to professional, regulatory and safety controls.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.4.2.3. Medication Advice

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-medication-advice`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Medication & Pharmacy Services (p. 21)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:489 (p. 329); business:483 (p. 329); business:492 (p. 329)	Linked RAD detail; exact locators in manifest.

Provide professional medication-specific advice about appropriate use, precautions, interactions, administration, adherence or safe handling.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.4.2.4. Medication Dose Administration Aid Provision

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-medication-dose-administration-aid-provision`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Medication & Pharmacy Services (p. 21)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:490 (p. 329); business:483 (p. 329); business:492 (p. 329)	Linked RAD detail; exact locators in manifest.

Prepare and provide organised medication doses supporting administration according to an already established regimen.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.5. Health Products & Clinical Technology

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-products-clinical-technology`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT1	Explicit source assertion.
Ancestry	CT1 root	Explicitly established.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.
Source	business:494 (p. 329); business:498 (p. 330); business:120 (p. 322)	Linked RAD detail; exact locators in manifest.

Definition: Manage health products and clinical technology in the care environment, with distinct device inventory, functional/release, deployment, utilisation, integration and lifecycle responsibilities. Product procurement and supply context is retained without inferring further Health Product Management decomposition.

Contained capabilities

Capability	Reference Id	Tier
Clinical Technology Management (p. 23)	`HCM-BC-clinical-technology-management`	CT2

2.5.1. Clinical Technology Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-technology-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	Health Products & Clinical Technology (p. 23)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:502 (p. 330); business:504 (p. 330); business:506 (p. 330); business:520 (p. 330)	Linked RAD detail; exact locators in manifest.

Definition: Manage clinical technology as a coherent business concern rather than only the device information Harmonia happens to use.

Contained capabilities

Capability	Reference Id	Tier
Clinical Device Inventory Management (p. 24)	`HCM-BC-clinical-device-inventory-management`	CT3
Clinical Device Feature & Release Management (p. 24)	`HCM-BC-clinical-device-feature-release-management`	CT3
Clinical Device Loan & Deployment Management (p. 24)	`HCM-BC-clinical-device-loan-deployment-management`	CT3
Clinical Device Utilisation Support (p. 25)	`HCM-BC-clinical-device-utilisation-support`	CT3
Clinical Device Integration Provision (p. 25)	`HCM-BC-clinical-device-integration-provision`	CT3

Capability	Reference Id	Tier
Clinical Device Data Integration (p. 25)	`HCM-BC-clinical-device-data-integration`	CT3
Clinical Device Lifecycle Management (p. 26)	`HCM-BC-clinical-device-lifecycle-management`	CT3

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.5.1.1. Clinical Device Inventory Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-device-inventory-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Clinical Technology Management (p. 23)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:512 (p. 330); business:508 (p. 330); business:520 (p. 330)	Linked RAD detail; exact locators in manifest.

Establish what devices exist and their managed asset identity, specification, ownership/custody and inventory context.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.5.1.2. Clinical Device Feature & Release Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-device-feature-release-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Clinical Technology Management (p. 23)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:513 (p. 330); business:508 (p. 330); business:520 (p. 330)	Linked RAD detail; exact locators in manifest.

Manage functional/release state, including applicable software, firmware, configuration and enabled device characteristics.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.5.1.3. Clinical Device Loan & Deployment Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-device-loan-deployment-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Clinical Technology Management (p. 23)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.

Name	Value	Comment / Description
Source	business:514 (p. 330); business:508 (p. 330); business:520 (p. 330)	Linked RAD detail; exact locators in manifest.

Manage device loan, issue, assignment, placement, return and deployment.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.5.1.4. Clinical Device Utilisation Support

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-device-utilisation-support`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Clinical Technology Management (p. 23)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:515 (p. 330); business:508 (p. 330); business:520 (p. 330)	Linked RAD detail; exact locators in manifest.

Enable and support otherwise appropriately authorised users to use the particular device safely and effectively.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.5.1.5. Clinical Device Integration Provision

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-device-integration-provision`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Clinical Technology Management (p. 23)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:516 (p. 330); business:508 (p. 330); business:520 (p. 330)	Linked RAD detail; exact locators in manifest.

Establish and maintain the device’s capability and configuration to participate in applicable information exchanges.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.5.1.6. Clinical Device Data Integration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-device-data-integration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Clinical Technology Management (p. 23)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:517 (p. 330); business:508 (p. 330); business:520 (p. 330)	Linked RAD detail; exact locators in manifest.

Acquire, exchange or convey device data through established integration capability.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.5.1.7. Clinical Device Lifecycle Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-device-lifecycle-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Clinical Technology Management (p. 23)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:518 (p. 330); business:508 (p. 330); business:520 (p. 330)	Linked RAD detail; exact locators in manifest.

Govern introduction, continued service, review, replacement, withdrawal, decommissioning and retirement within the overall device estate.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.6. Clinical Quality, Safety & Improvement

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-quality-safety-improvement`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT1	Explicit source assertion.
Ancestry	CT1 root	Explicitly established.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.
Source	business:522 (p. 330); business:526 (p. 330); business:120 (p. 322)	Linked RAD detail; exact locators in manifest.

Definition: Operational clinical quality, safety and improvement: monitoring clinical outcomes; identifying quality and safety concerns; adverse event and incident management, reporting and response; infection control; operational quality and safety monitoring; continuous quality improvement; and coordination of operational responses to identified concerns.

Contained capabilities

Capability	Reference Id	Tier
Clinical Safety Management (p. 27)	`HCM-BC-clinical-safety-management`	CT2
Clinical Quality Management (p. 28)	`HCM-BC-clinical-quality-management`	CT2

2.6.1. Clinical Safety Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-safety-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	Clinical Quality, Safety & Improvement (p. 26)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:530 (p. 330); business:532 (p. 330); business:534 (p. 330); business:544 (p. 331)	Linked RAD detail; exact locators in manifest.

Definition: Manage the clinical-risk distinctions materially required for R1/R2.

Contained capabilities

Capability	Reference Id	Tier
Clinical Risk Identification (p. 27)	`HCM-BC-clinical-risk-identification`	CT3
Clinical Risk Mitigation Management (p. 27)	`HCM-BC-clinical-risk-mitigation-management`	CT3
Clinical Risk Response Management (p. 28)	`HCM-BC-clinical-risk-response-management`	CT3

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.6.1.1. Clinical Risk Identification

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-risk-identification`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Clinical Safety Management (p. 27)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:540 (p. 331); business:536 (p. 331); business:544 (p. 331)	Linked RAD detail; exact locators in manifest.

Recognise and characterise a clinical safety risk from applicable evidence.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.6.1.2. Clinical Risk Mitigation Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-risk-mitigation-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Clinical Safety Management (p. 27)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:541 (p. 331); business:536 (p. 331); business:544 (p. 331)	Linked RAD detail; exact locators in manifest.

Manage prospective/preventative measures reducing identified risk likelihood, exposure or consequence.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.6.1.3. Clinical Risk Response Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-risk-response-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Clinical Safety Management (p. 27)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:542 (p. 331); business:536 (p. 331); business:544 (p. 331)	Linked RAD detail; exact locators in manifest.

Manage active response, containment and remediation where risk has materialised, is emerging or requires active intervention.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.6.2. Clinical Quality Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-quality-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	Clinical Quality, Safety & Improvement (p. 26)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:546 (p. 331); business:548 (p. 331); business:550 (p. 331); business:561 (p. 331)	Linked RAD detail; exact locators in manifest.

Definition: Manage the clinical-quality distinctions materially required for R1/R2.

Contained capabilities

Capability	Reference Id	Tier
Clinical Quality Criteria Management (p. 29)	`HCM-BC-clinical-quality-criteria-management`	CT3
Clinical Quality Evidence Management (p. 29)	`HCM-BC-clinical-quality-evidence-management`	CT3
Clinical Quality Review & Assessment Management (p. 29)	`HCM-BC-clinical-quality-review-assessment-management`	CT3
Clinical Quality Reporting (p. 30)	`HCM-BC-clinical-quality-reporting`	CT3

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.6.2.1. Clinical Quality Criteria Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-quality-criteria-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Clinical Quality Management (p. 28)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:556 (p. 331); business:552 (p. 331); business:561 (p. 331)	Linked RAD detail; exact locators in manifest.

Establish and manage measures, indicators, expectations and criteria against which clinical quality is considered.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.6.2.2. Clinical Quality Evidence Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-quality-evidence-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Clinical Quality Management (p. 28)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:557 (p. 331); business:552 (p. 331); business:561 (p. 331)	Linked RAD detail; exact locators in manifest.

Manage the body of applicable evidence used for clinical-quality consideration without owning its originating activity.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.6.2.3. Clinical Quality Review & Assessment Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-quality-review-assessment-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Clinical Quality Management (p. 28)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:558 (p. 331); business:552 (p. 331); business:561 (p. 331)	Linked RAD detail; exact locators in manifest.

Review evidence against applicable criteria and establish a clinical-quality assessment, finding or conclusion.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.6.2.4. Clinical Quality Reporting

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-quality-reporting`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Clinical Quality Management (p. 28)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:559 (p. 331); business:552 (p. 331); business:561 (p. 331)	Linked RAD detail; exact locators in manifest.

Communicate established clinical-quality measures, assessments, findings and conclusions to authorised consumers.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.7. Community Health & Wellbeing

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-community-health-wellbeing`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT1	Explicit source assertion.
Ancestry	CT1 root	Explicitly established.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.
Source	business:563 (p. 331); business:567 (p. 331); business:120 (p. 322)	Linked RAD detail; exact locators in manifest.

Definition: Support community health and wellbeing, including the bounded preventative-health abilities required for R1/R2. Wider environmental health, outreach and social determinants remain enterprise context without inferred additional decomposition.

Contained capabilities

Capability	Reference Id	Tier
Preventative Health (p. 30)	`HCM-BC-preventative-health`	CT2

2.7.1. Preventative Health

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-preventative-health`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	Community Health & Wellbeing (p. 30)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:571 (p. 331); business:573 (p. 331); business:575 (p. 331); business:588 (p. 332)	Linked RAD detail; exact locators in manifest.

Definition: Address preventative-health needs through organised activity with sufficient R1/R2 distinctions.

Contained capabilities

Capability	Reference Id	Tier
Preventative Health Problem Identification (p. 31)	`HCM-BC-preventative-health-problem-identification`	CT3
Preventative Health Programme Management (p. 31)	`HCM-BC-preventative-health-programme-management`	CT3
Preventative Health Education Services (p. 32)	`HCM-BC-preventative-health-education-services`	CT3
Preventative Health Programme Monitoring (p. 32)	`HCM-BC-preventative-health-programme-monitoring`	CT3
Preventative Health Reporting (p. 32)	`HCM-BC-preventative-health-reporting`	CT3
Preventative Health Cohort Management (p. 33)	`HCM-BC-preventative-health-cohort-management`	CT3

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.7.1.1. Preventative Health Problem Identification

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-preventative-health-problem-identification`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Preventative Health (p. 30)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:581 (p. 332); business:577 (p. 332); business:588 (p. 332)	Linked RAD detail; exact locators in manifest.

Identify and characterise a preventative-health problem warranting organised attention.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.7.1.2. Preventative Health Programme Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-preventative-health-programme-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Preventative Health (p. 30)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:582 (p. 332); business:577 (p. 332); business:588 (p. 332)	Linked RAD detail; exact locators in manifest.

Establish and manage an organised response to a preventative-health need/objective without absorbing its delivery capabilities.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.7.1.3. Preventative Health Education Services

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-preventative-health-education-services`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Preventative Health (p. 30)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:583 (p. 332); business:577 (p. 332); business:588 (p. 332)	Linked RAD detail; exact locators in manifest.

Provide education intended to prevent adverse health outcomes and influence preventative knowledge or behaviour.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.7.1.4. Preventative Health Programme Monitoring

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-preventative-health-programme-monitoring`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Preventative Health (p. 30)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:584 (p. 332); business:577 (p. 332); business:588 (p. 332)	Linked RAD detail; exact locators in manifest.

Observe programme operation, reach, participation, coverage, progress and applicable performance.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.7.1.5. Preventative Health Reporting

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-preventative-health-reporting`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Preventative Health (p. 30)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:585 (p. 332); business:577 (p. 332); business:588 (p. 332)	Linked RAD detail; exact locators in manifest.

Report established preventative-health problems, programmes, cohorts, activity, performance and outcomes.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.7.1.6. Preventative Health Cohort Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-preventative-health-cohort-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Preventative Health (p. 30)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:586 (p. 332); business:577 (p. 332); business:588 (p. 332)	Linked RAD detail; exact locators in manifest.

Establish and manage the target population, inclusion/exclusion criteria, membership, stratification and applicable entry/exit context.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.8. Population Health & Health-System Planning

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-population-health-health-system-planning`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT1	Explicit source assertion.
Ancestry	CT1 root	Explicitly established.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.
Source	business:590 (p. 332); business:594 (p. 332); business:120 (p. 322)	Linked RAD detail; exact locators in manifest.

Definition: Understand population health through assessment/surveillance and applicable event/notification management; assess health-service demand and plan required capacity and its distribution.

Contained capabilities

Capability	Reference Id	Tier
Population Health Surveillance (p. 33)	`HCM-BC-population-health-surveillance`	CT2
Health-System Planning (p. 36)	`HCM-BC-health-system-planning`	CT2

2.8.1. Population Health Surveillance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-population-health-surveillance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	Population Health & Health-System Planning (p. 33)	Immediate BC parent.

Name	Value	Comment / Description
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:598 (p. 332); business:600 (p. 332); business:602 (p. 332); business:614 (p. 333)	Linked RAD detail; exact locators in manifest.

Definition: Understand population-health state and manage organised surveillance, significant events and applicable notification.

Contained capabilities

Capability	Reference Id	Tier
Population Health Longitudinal Assessment (p. 34)	`HCM-BC-population-health-longitudinal-assessment`	CT3
Population Health Surveillance Management (p. 34)	`HCM-BC-population-health-surveillance-management`	CT3
Population Health Event Identification (p. 35)	`HCM-BC-population-health-event-identification`	CT3
Notifiable Health Event Management (p. 35)	`HCM-BC-notifiable-health-event-management`	CT3
Population Health Reporting (p. 35)	`HCM-BC-population-health-reporting`	CT3

Unresolved dimensions: relevance; semantic relationship representation. See the linked RAD detail and uncertainty register.

2.8.1.1. Population Health Longitudinal Assessment

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-population-health-longitudinal-assessment`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Population Health Surveillance (p. 33)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:608 (p. 332); business:604 (p. 332); business:614 (p. 333)	Linked RAD detail; exact locators in manifest.

Establish population-health state, patterns and change over time from applicable evidence.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.8.1.2. Population Health Surveillance Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-population-health-surveillance-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Population Health Surveillance (p. 33)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:609 (p. 332); business:604 (p. 332); business:614 (p. 333)	Linked RAD detail; exact locators in manifest.

Establish and manage surveillance purposes, populations, concerns and regimes.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.8.1.3. Population Health Event Identification

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-population-health-event-identification`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Population Health Surveillance (p. 33)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:610 (p. 332); business:604 (p. 332); business:614 (p. 333)	Linked RAD detail; exact locators in manifest.

Identify and characterise significant population-health events/patterns from applicable evidence.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.8.1.4. Notifiable Health Event Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-notifiable-health-event-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Population Health Surveillance (p. 33)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:611 (p. 332); business:604 (p. 332); business:614 (p. 333)	Linked RAD detail; exact locators in manifest.

Manage formal notification applicability, required information, recipient, status, submission, correction/supplementation and applicable follow-up.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.8.1.5. Population Health Reporting

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-population-health-reporting`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Population Health Surveillance (p. 33)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:612 (p. 332); business:604 (p. 332); business:614 (p. 333)	Linked RAD detail; exact locators in manifest.

Communicate established population-health assessments, surveillance, events, trends and findings.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.8.2. Health-System Planning

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-system-planning`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	Population Health & Health-System Planning (p. 33)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:616 (p. 333); business:618 (p. 333); business:620 (p. 333); business:630 (p. 333)	Linked RAD detail; exact locators in manifest.

Definition: Assess health-service demand and plan the capacity and footprint required to meet it.

Contained capabilities

Capability	Reference Id	Tier
Health Service Demand Assessment (p. 36)	`HCM-BC-health-service-demand-assessment`	CT3
Health Service Capacity Planning (p. 37)	`HCM-BC-health-service-capacity-planning`	CT3
Health Service Distribution Planning (p. 37)	`HCM-BC-health-service-distribution-planning`	CT3

Unresolved dimensions: relevance; semantic relationship representation. See the linked RAD detail and uncertainty register.

2.8.2.1. Health Service Demand Assessment

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-service-demand-assessment`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Health-System Planning (p. 36)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:626 (p. 333); business:622 (p. 333); business:630 (p. 333)	Linked RAD detail; exact locators in manifest.

Assess current/expected health-service demand across a population, geography, service domain or health-system context.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.8.2.2. Health Service Capacity Planning

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-service-capacity-planning`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Health-System Planning (p. 36)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:627 (p. 333); business:622 (p. 333); business:630 (p. 333)	Linked RAD detail; exact locators in manifest.

Determine required strategic/tactical capacity to address assessed demand.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.8.2.3. Health Service Distribution Planning

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-service-distribution-planning`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Health-System Planning (p. 36)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:628 (p. 333); business:622 (p. 333); business:630 (p. 333)	Linked RAD detail; exact locators in manifest.

Determine the distribution of planned capacity across a geographical, organisational, site or service-network footprint.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.9. Health Information & Knowledge Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-information-knowledge-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT1	Explicit source assertion.
Ancestry	CT1 root	Explicitly established.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Core	Independent of Architectural Status.
Source	business:636 (p. 333); business:640 (p. 333); business:120 (p. 322)	Linked RAD detail; exact locators in manifest.

Definition: Manage governed health-information content, structures, lifecycle, catalogues, compartments, correlation and retrieval, alongside reusable clinical knowledge and its governed standing.

Contained capabilities

Capability	Reference Id	Tier
Health Information Management (p. 38)	`HCM-BC-health-information-management`	CT2
Clinical Knowledge Management (p. 41)	`HCM-BC-clinical-knowledge-management`	CT2

2.9.1. Health Information Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-information-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	Health Information & Knowledge Management (p. 37)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:644 (p. 333); business:646 (p. 333); business:648 (p. 333); business:662 (p. 334)	Linked RAD detail; exact locators in manifest.

Definition: Manage health information as governed, semantically meaningful content.

Contained capabilities

Capability	Reference Id	Tier
Health Information Data Management (p. 38)	`HCM-BC-health-information-data-management`	CT3
Health Information Structure Management (p. 39)	`HCM-BC-health-information-structure-management`	CT3
Health Information Lifecycle Management (p. 39)	`HCM-BC-health-information-lifecycle-management`	CT3
Health Information Catalogue and Index Management (p. 39)	`HCM-BC-health-information-catalogue-and-index-management`	CT3
Health Information Compartmentalisation (p. 40)	`HCM-BC-health-information-compartmentalisation`	CT3
Health Information Data Correlation (p. 40)	`HCM-BC-health-information-data-correlation`	CT3
Health Information Data Retrieval (p. 40)	`HCM-BC-health-information-data-retrieval`	CT3

Unresolved dimensions: relevance; semantic relationship representation. See the linked RAD detail and uncertainty register.

2.9.1.1. Health Information Data Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-information-data-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Health Information Management (p. 38)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:654 (p. 333); business:650 (p. 333); business:662 (p. 334)	Linked RAD detail; exact locators in manifest.

Manage actual health-information content within applicable governed structures.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.9.1.2. Health Information Structure Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-information-structure-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Health Information Management (p. 38)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:655 (p. 333); business:650 (p. 333); business:662 (p. 334)	Linked RAD detail; exact locators in manifest.

Manage structures, element/reference definitions, constraints and semantic bindings giving content unambiguous meaning.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.9.1.3. Health Information Lifecycle Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-information-lifecycle-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Health Information Management (p. 38)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:656 (p. 333); business:650 (p. 333); business:662 (p. 334)	Linked RAD detail; exact locators in manifest.

Govern state, amendment, correction, supersession, currency, retention and disposition while preserving provenance and historical meaning.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.9.1.4. Health Information Catalogue and Index Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-information-catalogue-and-index-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Health Information Management (p. 38)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:657 (p. 333); business:650 (p. 333); business:662 (p. 334)	Linked RAD detail; exact locators in manifest.

Manage descriptive catalogues/indexes through which information existence and location can be established.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.9.1.5. Health Information Compartmentalisation

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-information-compartmentalisation`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Health Information Management (p. 38)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:658 (p. 333); business:650 (p. 333); business:662 (p. 334)	Linked RAD detail; exact locators in manifest.

Manage contextual association, partitioning and bounded information groupings.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.9.1.6. Health Information Data Correlation

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-information-data-correlation`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Health Information Management (p. 38)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:659 (p. 333); business:650 (p. 333); business:662 (p. 334)	Linked RAD detail; exact locators in manifest.

Establish meaningful relationships between independently managed information items/sets.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.9.1.7. Health Information Data Retrieval

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-information-data-retrieval`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Health Information Management (p. 38)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:660 (p. 333); business:650 (p. 333); business:662 (p. 334)	Linked RAD detail; exact locators in manifest.

Obtain identified information under an applicable authorised request/context.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.9.2. Clinical Knowledge Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-knowledge-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	Health Information & Knowledge Management (p. 37)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:664 (p. 334); business:666 (p. 334); business:668 (p. 334); business:681 (p. 334)	Linked RAD detail; exact locators in manifest.

Definition: Identify, extract, formulate, associate, govern and share reusable clinical knowledge.

Contained capabilities

Capability	Reference Id	Tier
Clinical Knowledge Context Identification (p. 41)	`HCM-BC-clinical-knowledge-context-identification`	CT3
Clinical Knowledge Context Extraction (p. 42)	`HCM-BC-clinical-knowledge-context-extraction`	CT3
Clinical Knowledge Information Formulation (p. 42)	`HCM-BC-clinical-knowledge-information-formulation`	CT3
Clinical Knowledge Concept Association (p. 42)	`HCM-BC-clinical-knowledge-concept-association`	CT3
Clinical Knowledge Review & Approval Management (p. 43)	`HCM-BC-clinical-knowledge-review-approval-management`	CT3
Clinical Knowledge Publication & Sharing (p. 43)	`HCM-BC-clinical-knowledge-publication-sharing`	CT3

Unresolved dimensions: relevance; semantic relationship representation. See the linked RAD detail and uncertainty register.

2.9.2.1. Clinical Knowledge Context Identification

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-knowledge-context-identification`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Clinical Knowledge Management (p. 41)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:674 (p. 334); business:670 (p. 334); business:681 (p. 334)	Linked RAD detail; exact locators in manifest.

Identify sources/contexts potentially containing reusable clinical knowledge.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.9.2.2. Clinical Knowledge Context Extraction

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-knowledge-context-extraction`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Clinical Knowledge Management (p. 41)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:675 (p. 334); business:670 (p. 334); business:681 (p. 334)	Linked RAD detail; exact locators in manifest.

Extract relevant material while preserving its source/context relationship.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.9.2.3. Clinical Knowledge Information Formulation

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-knowledge-information-formulation`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Clinical Knowledge Management (p. 41)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:676 (p. 334); business:670 (p. 334); business:681 (p. 334)	Linked RAD detail; exact locators in manifest.

Formulate explicit reusable knowledge, including applicability, qualifications, rationale and evidence relationships.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.9.2.4. Clinical Knowledge Concept Association

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-knowledge-concept-association`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Clinical Knowledge Management (p. 41)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:677 (p. 334); business:670 (p. 334); business:681 (p. 334)	Linked RAD detail; exact locators in manifest.

Associate knowledge with applicable governed clinical concepts without acquiring concept authority.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.9.2.5. Clinical Knowledge Review & Approval Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-knowledge-review-approval-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Clinical Knowledge Management (p. 41)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:678 (p. 334); business:670 (p. 334); business:681 (p. 334)	Linked RAD detail; exact locators in manifest.

Manage review, endorsement, approval/rejection, conditions and applicable history for intended governed use.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.9.2.6. Clinical Knowledge Publication & Sharing

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-knowledge-publication-sharing`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Clinical Knowledge Management (p. 41)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:679 (p. 334); business:670 (p. 334); business:681 (p. 334)	Linked RAD detail; exact locators in manifest.

Make approved knowledge available to applicable authorised consumers and use contexts.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.10. Standards, Semantics & Reference Governance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-standards-semantics-reference-governance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT1	Explicit source assertion.
Ancestry	CT1 root	Explicitly established.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Core	Independent of Architectural Status.
Source	business:683 (p. 334); business:687 (p. 335); business:120 (p. 322)	Linked RAD detail; exact locators in manifest.

Definition: Govern organisational adoption, oversight and appropriate use of health-information models, standards, terminology, classifications and semantic mappings.

Contained capabilities

Capability	Reference Id	Tier
Health Information Standards Governance (p. 44)	`HCM-BC-health-information-standards-governance`	CT2
Clinical Semantics Governance (p. 47)	`HCM-BC-clinical-semantics-governance`	CT2

2.10.1. Health Information Standards Governance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-information-standards-governance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	Standards, Semantics & Reference Governance (p. 43)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:691 (p. 335); business:693 (p. 335); business:695 (p. 335); business:709 (p. 335)	Linked RAD detail; exact locators in manifest.

Definition: Govern organisational adoption and use of health-information models and applicable standards.

Contained capabilities

Capability	Reference Id	Tier
Information Model Discovery (p. 44)	`HCM-BC-information-model-discovery`	CT3
Information Model Adoption (p. 45)	`HCM-BC-information-model-adoption`	CT3
Information Model Education (p. 45)	`HCM-BC-information-model-education`	CT3
Information Model Compliance (p. 45)	`HCM-BC-information-model-compliance`	CT3
Information Model Governance Oversight (p. 46)	`HCM-BC-information-model-governance-oversight`	CT3
Standards Adoption (p. 46)	`HCM-BC-standards-adoption`	CT3
Standards Impact Identification (p. 46)	`HCM-BC-standards-impact-identification`	CT3

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.10.1.1. Information Model Discovery

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-information-model-discovery`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Health Information Standards Governance (p. 44)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:701 (p. 335); business:697 (p. 335); business:709 (p. 335)	Linked RAD detail; exact locators in manifest.

Identify information models relevant to the organisation’s health-information needs.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.10.1.2. Information Model Adoption

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-information-model-adoption`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Health Information Standards Governance (p. 44)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:702 (p. 335); business:697 (p. 335); business:709 (p. 335)	Linked RAD detail; exact locators in manifest.

Adopt applicable information models, including intrinsic assessment of adoption impacts.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.10.1.3. Information Model Education

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-information-model-education`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Health Information Standards Governance (p. 44)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:703 (p. 335); business:697 (p. 335); business:709 (p. 335)	Linked RAD detail; exact locators in manifest.

Support organisational understanding and appropriate use of adopted information models.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.10.1.4. Information Model Compliance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-information-model-compliance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Health Information Standards Governance (p. 44)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:704 (p. 335); business:697 (p. 335); business:709 (p. 335)	Linked RAD detail; exact locators in manifest.

Address organisational conformity to applicable information-model requirements.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.10.1.5. Information Model Governance Oversight

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-information-model-governance-oversight`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Health Information Standards Governance (p. 44)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:705 (p. 335); business:697 (p. 335); business:709 (p. 335)	Linked RAD detail; exact locators in manifest.

Oversee and evaluate effectiveness, consistency and appropriate operation of information-model governance.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.10.1.6. Standards Adoption

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-standards-adoption`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Health Information Standards Governance (p. 44)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:706 (p. 335); business:697 (p. 335); business:709 (p. 335)	Linked RAD detail; exact locators in manifest.

Adopt applicable standards within the organisation.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.10.1.7. Standards Impact Identification

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-standards-impact-identification`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Health Information Standards Governance (p. 44)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:707 (p. 335); business:697 (p. 335); business:709 (p. 335)	Linked RAD detail; exact locators in manifest.

Identify standards impacts across clinical/work practice, permitted medications, procedures, policy, safety, obligations and reporting as applicable.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.10.2. Clinical Semantics Governance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-semantics-governance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	Standards, Semantics & Reference Governance (p. 43)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:711 (p. 335); business:713 (p. 335); business:715 (p. 335); business:728 (p. 336)	Linked RAD detail; exact locators in manifest.

Definition: Govern clinical meaning and appropriate organisational use of semantic artefacts.

Contained capabilities

Capability	Reference Id	Tier
Terminology Governance (p. 47)	`HCM-BC-terminology-governance`	CT3
Terminology Utilisation Oversight (p. 47)	`HCM-BC-terminology-utilisation-oversight`	CT3
Terminology Localisation (p. 48)	`HCM-BC-terminology-localisation`	CT3
Classification Governance (p. 48)	`HCM-BC-classification-governance`	CT3
Classification Utilisation Oversight (p. 48)	`HCM-BC-classification-utilisation-oversight`	CT3
Semantic Mapping Governance (p. 49)	`HCM-BC-semantic-mapping-governance`	CT3

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.10.2.1. Terminology Governance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-terminology-governance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Clinical Semantics Governance (p. 47)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:721 (p. 335); business:717 (p. 335); business:728 (p. 336)	Linked RAD detail; exact locators in manifest.

Manage governed terminology/reference-set lifecycle and standing, including acceptance, release, update, supersession/retirement and authority/provider coordination.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.10.2.2. Terminology Utilisation Oversight

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-terminology-utilisation-oversight`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Clinical Semantics Governance (p. 47)	Immediate BC parent.

Name	Value	Comment / Description
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:722 (p. 335); business:717 (p. 335); business:728 (p. 336)	Linked RAD detail; exact locators in manifest.

Oversee actual terminology/reference-set use and identify inappropriate, inconsistent or drifting usage.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.10.2.3. Terminology Localisation

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-terminology-localisation`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Clinical Semantics Governance (p. 47)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:723 (p. 335); business:717 (p. 335); business:728 (p. 336)	Linked RAD detail; exact locators in manifest.

Manage controlled local responses where adopted terminology does not meet legitimate organisational semantic requirements.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.10.2.4. Classification Governance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-classification-governance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Clinical Semantics Governance (p. 47)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:724 (p. 335); business:717 (p. 335); business:728 (p. 336)	Linked RAD detail; exact locators in manifest.

Govern applicable classification definitions and their organisational standing.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.10.2.5. Classification Utilisation Oversight

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-classification-utilisation-oversight`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Clinical Semantics Governance (p. 47)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.

Name	Value	Comment / Description
Source	business:725 (p. 335); business:717 (p. 335); business:728 (p. 336)	Linked RAD detail; exact locators in manifest.

Oversee appropriate organisational application of classifications.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.10.2.6. Semantic Mapping Governance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-semantic-mapping-governance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Clinical Semantics Governance (p. 47)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:726 (p. 335); business:717 (p. 335); business:728 (p. 336)	Linked RAD detail; exact locators in manifest.

Govern mappings between applicable semantic artefacts without collapsing their source meanings or authorities.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.11. Connected Health Services

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-connected-health-services`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT1	Explicit source assertion.
Ancestry	CT1 root	Explicitly established.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Core	Independent of Architectural Status.
Source	business:730 (p. 336); business:734 (p. 336); business:120 (p. 322)	Linked RAD detail; exact locators in manifest.

Definition: Exchange health information appropriately and provide digitally mediated communication, notifications and collaborative interaction between health-service participants.

Contained capabilities

Capability	Reference Id	Tier
Health Information Exchange (p. 50)	`HCM-BC-health-information-exchange`	CT2
Digital Communication Channel (p. 50)	`HCM-BC-digital-communication-channel`	CT2

2.11.1. Health Information Exchange

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-information-exchange`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	Connected Health Services (p. 49)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:738 (p. 336); business:740 (p. 336); business:742 (p. 336); business:746 (p. 336); business:1225 (p. 348)	Linked RAD detail; exact locators in manifest.

Definition: Exchange health information appropriately between participating parties, systems and services.

Unresolved dimensions: relevance; name uniqueness. See the linked RAD detail and uncertainty register.

2.11.2. Digital Communication Channel

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-digital-communication-channel`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	Connected Health Services (p. 49)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:748 (p. 336); business:750 (p. 336); business:752 (p. 336); business:756 (p. 336)	Linked RAD detail; exact locators in manifest.

Definition: Provide digitally mediated communication, notification and collaborative interaction between patients, practitioners and other health-service participants.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.12. Security, Privacy & Digital Trust

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-security-privacy-digital-trust`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT1	Explicit source assertion.
Ancestry	CT1 root	Explicitly established.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Core	Independent of Architectural Status.
Source	business:758 (p. 336); business:762 (p. 336); business:120 (p. 322)	Linked RAD detail; exact locators in manifest.

Definition: Establish adequate identity confidence, coherent access controls, privacy/protection safeguards, governed activity attribution and accountability for security-relevant activity and information use.

Contained capabilities

Capability	Reference Id	Tier
Digital Identity & Access Governance (p. 51)	`HCM-BC-digital-identity-access-governance`	CT2
Health Information Protection (p. 54)	`HCM-BC-health-information-protection`	CT2
Digital Accountability & Trust (p. 56)	`HCM-BC-digital-accountability-trust`	CT2

2.12.1. Digital Identity & Access Governance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-digital-identity-access-governance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	Security, Privacy & Digital Trust (p. 50)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:766 (p. 336); business:768 (p. 337); business:770 (p. 337); business:785 (p. 337)	Linked RAD detail; exact locators in manifest.

Definition: Govern sufficient identity confidence and coherent access controls across the organisation.

Contained capabilities

Capability	Reference Id	Tier
Identity Assurance (p. 51)	`HCM-BC-identity-assurance`	CT3
Identity Modelling & Interoperability (p. 52)	`HCM-BC-identity-modelling-interoperability`	CT3
Identity Qualification Control (p. 52)	`HCM-BC-identity-qualification-control`	CT3
Identity Identification & Authentication (p. 52)	`HCM-BC-identity-identification-authentication`	CT3
Access Control Context Management (p. 53)	`HCM-BC-access-control-context-management`	CT3
Access Control Assurance & Reporting (p. 53)	`HCM-BC-access-control-assurance-reporting`	CT3
Access Control Collaboration (p. 53)	`HCM-BC-access-control-collaboration`	CT3
Access Control Enforcement (p. 54)	`HCM-BC-access-control-enforcement`	CT3

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.12.1.1. Identity Assurance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-identity-assurance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Digital Identity & Access Governance (p. 51)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:776 (p. 337); business:772 (p. 337); business:785 (p. 337)	Linked RAD detail; exact locators in manifest.

Establish sufficient confidence that a person is who they claim to be through appropriate evidence and processes.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.12.1.2. Identity Modelling & Interoperability

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-identity-modelling-interoperability`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Digital Identity & Access Governance (p. 51)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:777 (p. 337); business:772 (p. 337); business:785 (p. 337)	Linked RAD detail; exact locators in manifest.

Establish identity models and interoperability arrangements appropriate to organisational identity use.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.12.1.3. Identity Qualification Control

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-identity-qualification-control`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Digital Identity & Access Governance (p. 51)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:778 (p. 337); business:772 (p. 337); business:785 (p. 337)	Linked RAD detail; exact locators in manifest.

Determine whether presented identity characteristics, provenance and assurance quality are sufficient for its intended context.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.12.1.4. Identity Identification & Authentication

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-identity-identification-authentication`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Digital Identity & Access Governance (p. 51)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:779 (p. 337); business:772 (p. 337); business:785 (p. 337)	Linked RAD detail; exact locators in manifest.

Identify an actor and authenticate the asserted identity.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.12.1.5. Access Control Context Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-access-control-context-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Digital Identity & Access Governance (p. 51)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:780 (p. 337); business:772 (p. 337); business:785 (p. 337)	Linked RAD detail; exact locators in manifest.

Determine what information, functions, activities or other concerns require control, under what circumstances and how requirements are modelled.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.12.1.6. Access Control Assurance & Reporting

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-access-control-assurance-reporting`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Digital Identity & Access Governance (p. 51)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:781 (p. 337); business:772 (p. 337); business:785 (p. 337)	Linked RAD detail; exact locators in manifest.

Establish organisational assurance/evidence of whether access-control arrangements are appropriately established and operating.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.12.1.7. Access Control Collaboration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-access-control-collaboration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Digital Identity & Access Governance (p. 51)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:782 (p. 337); business:772 (p. 337); business:785 (p. 337)	Linked RAD detail; exact locators in manifest.

Coordinate arrangements across independently controlled domains/parties while retaining their control ownership.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.12.1.8. Access Control Enforcement

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-access-control-enforcement`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Digital Identity & Access Governance (p. 51)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:783 (p. 337); business:772 (p. 337); business:785 (p. 337)	Linked RAD detail; exact locators in manifest.

Ensure physical, procedural and digital control mechanisms coherently impose intended controls.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.12.2. Health Information Protection

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-information-protection`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	Security, Privacy & Digital Trust (p. 50)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:787 (p. 337); business:789 (p. 337); business:791 (p. 337); business:803 (p. 338)	Linked RAD detail; exact locators in manifest.

Definition: Govern privacy consequences and adequate protection of the information estate and its environment.

Contained capabilities

Capability	Reference Id	Tier
Health Privacy Safeguard Management (p. 55)	`HCM-BC-health-privacy-safeguard-management`	CT3
Health Information Safeguarding (p. 55)	`HCM-BC-health-information-safeguarding`	CT3
Health Information Access Monitoring (p. 55)	`HCM-BC-health-information-access-monitoring`	CT3
Health Information Compartment Assurance (p. 56)	`HCM-BC-health-information-compartment-assurance`	CT3
Health Information Classification Assurance (p. 56)	`HCM-BC-health-information-classification-assurance`	CT3

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.12.2.1. Health Privacy Safeguard Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-privacy-safeguard-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Health Information Protection (p. 54)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:797 (p. 337); business:793 (p. 337); business:803 (p. 338)	Linked RAD detail; exact locators in manifest.

Establish organisational safeguards and assurance processes concerning privacy consequences of information supply, exposure or use.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.12.2.2. Health Information Safeguarding

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-information-safeguarding`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Health Information Protection (p. 54)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:798 (p. 337); business:793 (p. 337); business:803 (p. 338)	Linked RAD detail; exact locators in manifest.

Ensure information and its operational environment are protected and resilient against loss, compromise, inappropriate access or operational failure.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.12.2.3. Health Information Access Monitoring

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-information-access-monitoring`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Health Information Protection (p. 54)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:799 (p. 337); business:793 (p. 337); business:803 (p. 338)	Linked RAD detail; exact locators in manifest.

Evaluate actual organisational access/handling practice, including controls, accessibility, compartments, classification and physical/printed handling.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.12.2.4. Health Information Compartment Assurance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-information-compartment-assurance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Health Information Protection (p. 54)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:800 (p. 337); business:793 (p. 337); business:803 (p. 338)	Linked RAD detail; exact locators in manifest.

Assure appropriate establishment and application of compartmentalisation without establishing access decisions.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.12.2.5. Health Information Classification Assurance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-information-classification-assurance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Health Information Protection (p. 54)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:801 (p. 337); business:793 (p. 337); business:803 (p. 338)	Linked RAD detail; exact locators in manifest.

Assure appropriate classification according to sensitivity and potential consequences of exposure/mishandling.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.12.3. Digital Accountability & Trust

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-digital-accountability-trust`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	Security, Privacy & Digital Trust (p. 50)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:805 (p. 338); business:807 (p. 338); business:809 (p. 338); business:818 (p. 338)	Linked RAD detail; exact locators in manifest.

Definition: Establish governed attribution and accountability for activity and information use.

Contained capabilities

Capability	Reference Id	Tier
Governed Activity Attribution (p. 57)	HCM-BC-governed-activity-attribution	CT3
Security & Information-Use Accountability (p. 57)	HCM-BC-security-information-use-accountability	CT3

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.12.3.1. Governed Activity Attribution

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	HCM-BC-governed-activity-attribution	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Digital Accountability & Trust (p. 56)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:815 (p. 338); business:811 (p. 338); business:818 (p. 338)	Linked RAD detail; exact locators in manifest.

Preserve who participated, what was done, the attributable person/role/unit/organisation capacity and applicable permission/authority/constraint context.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.12.3.2. Security & Information-Use Accountability

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	HCM-BC-security-information-use-accountability	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Digital Accountability & Trust (p. 56)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:816 (p. 338); business:811 (p. 338); business:818 (p. 338)	Linked RAD detail; exact locators in manifest.

Evaluate whether attributed security-relevant activity/information use was appropriate within policy, obligation and authority constraints, and establish accountability.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.13. Health Research & Innovation

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	HCM-BC-health-research-innovation	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT1	Explicit source assertion.
Ancestry	CT1 root	Explicitly established.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.

Name	Value	Comment / Description
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.
Source	business:824 (p. 338); business:828 (p. 338); business:120 (p. 322)	Linked RAD detail; exact locators in manifest.

Definition: Provide existing health information for authorised research under applicable constraints and evaluate health innovation; deliberately under-decomposed for R1/R2.

Contained capabilities

Capability	Reference Id	Tier
Research Information Support (p. 58)	`HCM-BC-research-information-support`	CT2
Health Innovation (p. 59)	`HCM-BC-health-innovation`	CT2

2.13.1. Research Information Support

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-research-information-support`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	Health Research & Innovation (p. 57)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:832 (p. 338); business:834 (p. 338); business:836 (p. 338); business:844 (p. 339)	Linked RAD detail; exact locators in manifest.

Definition: Provide the existing health information needed for authorised research within the deliberately bounded R1/R2 responsibility.

Contained capabilities

Capability	Reference Id	Tier
Governed Research Information Provision (p. 58)	`HCM-BC-governed-research-information-provision`	CT3

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.13.1.1. Governed Research Information Provision

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-governed-research-information-provision`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Research Information Support (p. 58)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:842 (p. 338); business:838 (p. 338); business:844 (p. 339)	Linked RAD detail; exact locators in manifest.

Provide existing health information for an authorised research purpose under applicable ethics, governance, privacy, consent, security and information-use constraints.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.13.2. Health Innovation

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-innovation`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	Health Research & Innovation (p. 57)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:846 (p. 339); business:848 (p. 339); business:850 (p. 339); business:858 (p. 339)	Linked RAD detail; exact locators in manifest.

Definition: Consider health-service and clinical innovation within the R1/R2 business context.

Contained capabilities

Capability	Reference Id	Tier
Health Innovation Evaluation (p. 59)	`HCM-BC-health-innovation-evaluation`	CT3

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.13.2.1. Health Innovation Evaluation

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-innovation-evaluation`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Health Innovation (p. 59)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:856 (p. 339); business:852 (p. 339); business:858 (p. 339)	Linked RAD detail; exact locators in manifest.

Evaluate proposed/emerging innovation for suitability, value, implications and potential adoption.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.14. Health Service Direction & Stewardship

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-service-direction-stewardship`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT1	Explicit source assertion.
Ancestry	CT1 root	Explicitly established.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.

Name	Value	Comment / Description
Source	business:864 (p. 339); business:868 (p. 339); business:120 (p. 322)	Linked RAD detail; exact locators in manifest.

Definition: Model needed health services and their viability requirements, support establishment/improvement and align services with the wider ecosystem of dependencies, resources, financial arrangements and organisational change.

Contained capabilities

Capability	Reference Id	Tier
Health Service Modelling (p. 60)	`HCM-BC-health-service-modelling`	CT2
Health Service Alignment (p. 62)	`HCM-BC-health-service-alignment`	CT2

2.14.1. Health Service Modelling

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-service-modelling`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	Health Service Direction & Stewardship (p. 59)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:872 (p. 339); business:874 (p. 339); business:876 (p. 339); business:887 (p. 340)	Linked RAD detail; exact locators in manifest.

Definition: Establish what service should exist and what it needs to become viable.

Contained capabilities

Capability	Reference Id	Tier
Health Service Need Identification (p. 60)	`HCM-BC-health-service-need-identification`	CT3
Health Service Capability Identification (p. 61)	`HCM-BC-health-service-capability-identification`	CT3
Health Service Implementation Support (p. 61)	`HCM-BC-health-service-implementation-support`	CT3
Health Service Continuous Improvement (p. 61)	`HCM-BC-health-service-continuous-improvement`	CT3

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.14.1.1. Health Service Need Identification

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-service-need-identification`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Health Service Modelling (p. 60)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:882 (p. 339); business:878 (p. 339); business:887 (p. 340)	Linked RAD detail; exact locators in manifest.

Establish an unmet health-service need requiring response.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.14.1.2. Health Service Capability Identification

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-service-capability-identification`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Health Service Modelling (p. 60)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:883 (p. 339); business:878 (p. 339); business:887 (p. 340)	Linked RAD detail; exact locators in manifest.

Identify the ordinary business abilities needed to satisfy the service need across clinical, workforce, facility, technology, information and support concerns.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.14.1.3. Health Service Implementation Support

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-service-implementation-support`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Health Service Modelling (p. 60)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:884 (p. 339); business:878 (p. 339); business:887 (p. 340)	Linked RAD detail; exact locators in manifest.

Support establishment of the framed service through applicable organisational preparation and programmes of work.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.14.1.4. Health Service Continuous Improvement

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-service-continuous-improvement`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Health Service Modelling (p. 60)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:885 (p. 339); business:878 (p. 339); business:887 (p. 340)	Linked RAD detail; exact locators in manifest.

Evaluate the established service and identify correction/improvement needs that may feed back into modelling and implementation support.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.14.2. Health Service Alignment

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-service-alignment`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	Health Service Direction & Stewardship (p. 59)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:889 (p. 340); business:891 (p. 340); business:893 (p. 340); business:904 (p. 340)	Linked RAD detail; exact locators in manifest.

Definition: Position the service coherently in its wider health-service ecosystem.

Contained capabilities

Capability	Reference Id	Tier
Health Service Co-Dependency Management (p. 62)	`HCM-BC-health-service-co-dependency-management`	CT3
Health Service Resource Identification (p. 63)	`HCM-BC-health-service-resource-identification`	CT3
Health Service Financial Management (p. 63)	`HCM-BC-health-service-financial-management`	CT3
Health Service Implementation Planning (p. 63)	`HCM-BC-health-service-implementation-planning`	CT3

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.14.2.1. Health Service Co-Dependency Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-service-co-dependency-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Health Service Alignment (p. 62)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:899 (p. 340); business:895 (p. 340); business:904 (p. 340)	Linked RAD detail; exact locators in manifest.

Identify and manage what the service needs from others and what others need from it.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.14.2.2. Health Service Resource Identification

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-service-resource-identification`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Health Service Alignment (p. 62)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:900 (p. 340); business:895 (p. 340); business:904 (p. 340)	Linked RAD detail; exact locators in manifest.

Identify required workforce, facilities, equipment, clinical technology, ICT, administration and support resources without owning them.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.14.2.3. Health Service Financial Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-service-financial-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Health Service Alignment (p. 62)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:901 (p. 340); business:895 (p. 340); business:904 (p. 340)	Linked RAD detail; exact locators in manifest.

Establish the service's fit within funding, financial and charging arrangements without absorbing enterprise finance/accounting.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.14.2.4. Health Service Implementation Planning

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-service-implementation-planning`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Health Service Alignment (p. 62)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:902 (p. 340); business:895 (p. 340); business:904 (p. 340)	Linked RAD detail; exact locators in manifest.

Establish and coordinate surrounding organisational/service changes needed to introduce or materially change the service.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.15. Workforce & Organisational Capability

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-workforce-organisational-capability`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT1	Explicit source assertion.
Ancestry	CT1 root	Explicitly established.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.
Source	business:906 (p. 340); business:910 (p. 340); business:120 (p. 322)	Linked RAD detail; exact locators in manifest.

Definition: Govern practitioner qualification, engagement/privilege and enterprise service coverage, together with organisation identity/structure, recognised locations and identifiable healthcare service offerings.

Contained capabilities

Capability	Reference Id	Tier
Health Workforce Management (p. 64)	`HCM-BC-health-workforce-management`	CT2
Healthcare Organisation & Service Structure Management (p. 66)	`HCM-BC-healthcare-organisation-service-structure-management`	CT2

2.15.1. Health Workforce Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-workforce-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	Workforce & Organisational Capability (p. 64)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:916 (p. 340); business:918 (p. 340); business:920 (p. 340); business:930 (p. 341)	Linked RAD detail; exact locators in manifest.

Definition: Govern practitioners' professional standing, engagement/privilege and whole-enterprise service coverage.

Contained capabilities

Capability	Reference Id	Tier
Practitioner Qualification Management (p. 65)	`HCM-BC-practitioner-qualification-management`	CT3
Practitioner Engagement & Privilege Management (p. 65)	`HCM-BC-practitioner-engagement-privilege-management`	CT3
Workforce Service Coverage Management (p. 65)	`HCM-BC-workforce-service-coverage-management`	CT3

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.15.1.1. Practitioner Qualification Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-practitioner-qualification-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Health Workforce Management (p. 64)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:926 (p. 340); business:922 (p. 340); business:930 (p. 341)	Linked RAD detail; exact locators in manifest.

Govern professional/clinical standing and qualifications, including applicable review, escalation, complaints, penalties/issues, suspension and revocation.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.15.1.2. Practitioner Engagement & Privilege Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-practitioner-engagement-privilege-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Health Workforce Management (p. 64)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:927 (p. 340); business:922 (p. 340); business:930 (p. 341)	Linked RAD detail; exact locators in manifest.

Manage practitioners as service participants, their roles/capacities, applicable privileges and qualification context above ordinary HR employment administration.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.15.1.3. Workforce Service Coverage Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-workforce-service-coverage-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Health Workforce Management (p. 64)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:928 (p. 340); business:922 (p. 340); business:930 (p. 341)	Linked RAD detail; exact locators in manifest.

Observe whether required enterprise work/service obligations can be fulfilled, identify coverage gaps/risks and adjust coverage in response.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.15.2. Healthcare Organisation & Service Structure Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-healthcare-organisation-service-structure-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	Workforce & Organisational Capability (p. 64)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:932 (p. 341); business:934 (p. 341); business:936 (p. 341); business:946 (p. 341)	Linked RAD detail; exact locators in manifest.

Definition: Manage organisational identity/structure, recognised locations and identifiable organisational service offerings.

Contained capabilities

Capability	Reference Id	Tier
Healthcare Organisation Identity & Structure Management (p. 66)	`HCM-BC-healthcare-organisation-identity-structure-management`	CT3
Healthcare Location Definition Management (p. 66)	`HCM-BC-healthcare-location-definition-management`	CT3
Healthcare Service Offering Management (p. 67)	`HCM-BC-healthcare-service-offering-management`	CT3

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.15.2.1. Healthcare Organisation Identity & Structure Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-healthcare-organisation-identity-structure-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Healthcare Organisation & Service Structure Management (p. 66)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:942 (p. 341); business:938 (p. 341); business:946 (p. 341)	Linked RAD detail; exact locators in manifest.

Establish and manage identity and structural relationships of participating organisations and organisational units.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.15.2.2. Healthcare Location Definition Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-healthcare-location-definition-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.

Name	Value	Comment / Description
Capability Tier	CT3	Explicit source assertion.
Ancestry	Healthcare Organisation & Service Structure Management (p. 66)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:943 (p. 341); business:938 (p. 341); business:946 (p. 341)	Linked RAD detail; exact locators in manifest.

Establish and manage recognised locations and their applicable business characteristics and relationships.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.15.2.3. Healthcare Service Offering Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-healthcare-service-offering-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Healthcare Organisation & Service Structure Management (p. 66)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:944 (p. 341); business:938 (p. 341); business:946 (p. 341)	Linked RAD detail; exact locators in manifest.

Establish/manage what identifiable service is offered, by whom, where, when and under which availability conditions/constraints.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.16. Corporate Resources & Enterprise Services

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-corporate-resources-enterprise-services`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT1	Explicit source assertion.
Ancestry	CT1 root	Explicitly established.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.
Source	business:948 (p. 341); business:952 (p. 341); business:120 (p. 322)	Linked RAD detail; exact locators in manifest.

Definition: Provide the bounded clinical-activity charging information and enterprise support context required for R1/R2, without assigning enterprise finance, HR, facilities or ICT operating authority to Harmonia.

Contained capabilities

Capability	Reference Id	Tier
Financial & Commercial Services (p. 68)	`HCM-BC-financial-commercial-services`	CT2
Corporate Support Services (p. 68)	`HCM-BC-corporate-support-services`	CT2

2.16.1. Financial & Commercial Services

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-financial-commercial-services`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	Corporate Resources & Enterprise Services (p. 67)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:956 (p. 341); business:958 (p. 341); business:960 (p. 341); business:968 (p. 342)	Linked RAD detail; exact locators in manifest.

Definition: Provide the financial/commercial business context materially required for R1/R2.

Contained capabilities

Capability	Reference Id	Tier
Clinical Activity Charging Information Provision (p. 68)	`HCM-BC-clinical-activity-charging-information-provision`	CT3

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.16.1.1. Clinical Activity Charging Information Provision

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-activity-charging-information-provision`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Financial & Commercial Services (p. 68)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:966 (p. 342); business:962 (p. 341); business:968 (p. 342)	Linked RAD detail; exact locators in manifest.

Identify and provide information about chargeable or cost-relevant clinical/health-service activity where required for applicable funding, financial or charging purposes.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.16.2. Corporate Support Services

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-corporate-support-services`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.

Name	Value	Comment / Description
Ancestry	Corporate Resources & Enterprise Services (p. 67)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:970 (p. 342); business:972 (p. 342); business:974 (p. 342); business:987 (p. 342)	Linked RAD detail; exact locators in manifest.

Definition: Provide enterprise support whose resources, availability, state or constraints may materially affect service activity.

Contained capabilities

Capability	Reference Id	Tier
Back-Office Services (p. 69)	`HCM-BC-back-office-services`	CT3
Facilities Management (p. 69)	`HCM-BC-facilities-management`	CT3
IT Infrastructure Management (p. 70)	`HCM-BC-it-infrastructure-management`	CT3
Communications & Publicity (p. 70)	`HCM-BC-communications-publicity`	CT3
People Management (p. 70)	`HCM-BC-people-management`	CT3
Operational Oversight (p. 71)	`HCM-BC-operational-oversight`	CT3

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.16.2.1. Back-Office Services

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-back-office-services`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Corporate Support Services (p. 68)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:980 (p. 342); business:976 (p. 342); business:987 (p. 342)	Linked RAD detail; exact locators in manifest.

Provide administrative/support services on which health-service activity depends.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.16.2.2. Facilities Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-facilities-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Corporate Support Services (p. 68)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:981 (p. 342); business:976 (p. 342); business:987 (p. 342)	Linked RAD detail; exact locators in manifest.

Manage physical facilities/services and applicable resource, location, availability and facility-state context.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.16.2.3. IT Infrastructure Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-it-infrastructure-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Corporate Support Services (p. 68)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:982 (p. 342); business:976 (p. 342); business:987 (p. 342)	Linked RAD detail; exact locators in manifest.

Operate/manage underlying technology infrastructure and its availability.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.16.2.4. Communications & Publicity

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-communications-publicity`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Corporate Support Services (p. 68)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:983 (p. 342); business:976 (p. 342); business:987 (p. 342)	Linked RAD detail; exact locators in manifest.

Manage organisational communication/publication arrangements relevant to information availability/distribution.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.16.2.5. People Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-people-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Corporate Support Services (p. 68)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:984 (p. 342); business:976 (p. 342); business:987 (p. 342)	Linked RAD detail; exact locators in manifest.

Manage people across the organisation, distinct from practitioner-specific Health Workforce Management.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.16.2.6. Operational Oversight

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-operational-oversight`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Corporate Support Services (p. 68)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:985 (p. 342); business:976 (p. 342); business:987 (p. 342)	Linked RAD detail; exact locators in manifest.

Observe broader operational conditions affecting the organisation's ability to work or deliver services.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.17. Health Service Governance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-service-governance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT1	Explicit source assertion.
Ancestry	CT1 root	Explicitly established.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:993 (p. 342); business:997 (p. 342); business:120 (p. 322)	Linked RAD detail; exact locators in manifest.

Definition: Establish and maintain the obligations, authorities, constraints, policies and governance framework under which health-service activity must operate, and define how adherence will be assessed, checked and reported.

Contained capabilities

Capability	Reference Id	Tier
Governance Framework Management (p. 71)	`HCM-BC-governance-framework-management`	CT2
Compliance Governance (p. 73)	`HCM-BC-compliance-governance`	CT2

Unresolved dimensions: relevance; enablement. See the linked RAD detail and uncertainty register.

2.17.1. Governance Framework Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-governance-framework-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	Health Service Governance (p. 71)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.

Name	Value	Comment / Description
Source	business:1003 (p. 342); business:1005 (p. 343); business:1007 (p. 343); business:1016 (p. 343)	Linked RAD detail; exact locators in manifest.

Definition: Establish the authoritative WHAT and legitimate working parameters.

Contained capabilities

Capability	Reference Id	Tier
Policy / Obligation Governance (p. 72)	`HCM-BC-policy-obligation-governance`	CT3
Authority & Constraint Governance (p. 72)	`HCM-BC-authority-constraint-governance`	CT3

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.17.1.1. Policy / Obligation Governance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-policy-obligation-governance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Governance Framework Management (p. 71)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:1013 (p. 343); business:1009 (p. 343); business:1016 (p. 343)	Linked RAD detail; exact locators in manifest.

Identify, interpret, establish and maintain authoritative legislative, regulatory, policy, standards, guideline, contractual and other obligations, attributable and traceable to source authority.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.17.1.2. Authority & Constraint Governance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-authority-constraint-governance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Governance Framework Management (p. 71)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:1014 (p. 343); business:1009 (p. 343); business:1016 (p. 343)	Linked RAD detail; exact locators in manifest.

Establish who may make, approve or govern organisational decisions/artefacts and the bounds within which authority may be exercised.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.17.2. Compliance Governance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-compliance-governance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	Health Service Governance (p. 71)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:1018 (p. 343); business:1020 (p. 343); business:1022 (p. 343); business:1031 (p. 343)	Linked RAD detail; exact locators in manifest.

Definition: Define how adherence to the authoritative WHAT will be determined, checked and reported.

Contained capabilities

Capability	Reference Id	Tier
Compliance Assessment Definition (p. 73)	`HCM-BC-compliance-assessment-definition`	CT3
Governance Reporting & Checkpoint Definition (p. 73)	`HCM-BC-governance-reporting-checkpoint-definition`	CT3

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.17.2.1. Compliance Assessment Definition

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-compliance-assessment-definition`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Compliance Governance (p. 73)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:1028 (p. 343); business:1024 (p. 343); business:1031 (p. 343)	Linked RAD detail; exact locators in manifest.

Define the evidence, measures, criteria, controls and assessment arrangements sufficient to evaluate compliance with established obligations, authorities and constraints.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.17.2.2. Governance Reporting & Checkpoint Definition

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-governance-reporting-checkpoint-definition`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Compliance Governance (p. 73)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.

Name	Value	Comment / Description
Source	business:1029 (p. 343); business:1024 (p. 343); business:1031 (p. 343)	Linked RAD detail; exact locators in manifest.

Establish where/how adherence is checked and what governance/compliance information must be reported.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.18. Health Service Assurance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-service-assurance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT1	Explicit source assertion.
Ancestry	CT1 root	Explicitly established.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.
Source	business:1033 (p. 343); business:1037 (p. 343); business:120 (p. 322)	Linked RAD detail; exact locators in manifest.
Enabled By	Assurance Design; Assurance Criteria Management; Governed Assurance	Bounded support; not decomposition or runtime dependency.

Definition: Establish and maintain justified confidence that health services can be, and are being, delivered in accordance with applicable expectations, obligations, constraints and required levels of performance.

Contained capabilities

Capability	Reference Id	Tier
Health Service Assurance Specification (p. 74)	`HCM-BC-health-service-assurance-specification`	CT2
Assurance Evaluation (p. 75)	`HCM-BC-assurance-evaluation`	CT2

2.18.1. Health Service Assurance Specification

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-service-assurance-specification`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	Health Service Assurance (p. 74)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:1043 (p. 343); business:1045 (p. 344); business:1047 (p. 344); business:1056 (p. 344)	Linked RAD detail; exact locators in manifest.

Definition: Specify how macro-level service-delivery assurance will be undertaken and what sufficiently supports confidence.

Contained capabilities

Capability	Reference Id	Tier
Assurance Approach Definition (p. 75)	`HCM-BC-assurance-approach-definition`	CT3
Assurance Evaluation Criteria Governance (p. 75)	`HCM-BC-assurance-evaluation-criteria-governance`	CT3

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.18.1.1. Assurance Approach Definition

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-assurance-approach-definition`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Health Service Assurance Specification (p. 74)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:1053 (p. 344); business:1049 (p. 344); business:1056 (p. 344)	Linked RAD detail; exact locators in manifest.

Define scope, evidence expectations, responsibilities, methods, applicable degree of independence and timing/frequency for service-delivery assurance.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.18.1.2. Assurance Evaluation Criteria Governance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-assurance-evaluation-criteria-governance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Health Service Assurance Specification (p. 74)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:1054 (p. 344); business:1049 (p. 344); business:1056 (p. 344)	Linked RAD detail; exact locators in manifest.

Establish and maintain criteria against which sufficient assurance can be determined.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.18.2. Assurance Evaluation

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-assurance-evaluation`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	Health Service Assurance (p. 74)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.

Name	Value	Comment / Description
Source	business:1058 (p. 344); business:1060 (p. 344); business:1062 (p. 344); business:1071 (p. 344)	Linked RAD detail; exact locators in manifest.

Definition: Determine what justified confidence the applicable evidence supports.

Contained capabilities

Capability	Reference Id	Tier
Assurance Evidence Assessment (p. 76)	`HCM-BC-assurance-evidence-assessment`	CT3
Assurance Conclusion Determination (p. 76)	`HCM-BC-assurance-conclusion-determination`	CT3

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.18.2.1. Assurance Evidence Assessment

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-assurance-evidence-assessment`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Assurance Evaluation (p. 75)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:1068 (p. 344); business:1064 (p. 344); business:1071 (p. 344)	Linked RAD detail; exact locators in manifest.

Evaluate available evidence against the established assurance approach and criteria.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.18.2.2. Assurance Conclusion Determination

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-assurance-conclusion-determination`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	Assurance Evaluation (p. 75)	Immediate BC parent.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:1069 (p. 344); business:1064 (p. 344); business:1071 (p. 344)	Linked RAD detail; exact locators in manifest.

Establish the legitimately supported conclusion, including insufficiency, qualification and unresolved concerns where applicable.

Unresolved dimensions: relevance. See the linked RAD detail and uncertainty register.

2.19. Candidate concerns with unresolved placement

These three named concerns remain candidates; no tier or parent is allocated. Other provisional proposals remain in the linked RAD prose and are not promoted to independently established elements.

2.19.1. Health Service Operational Readiness

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-service-operational-readiness`	Stable publication identity.
Architectural Status	Candidate	Not approved hierarchy; retain candidate standing.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:1201 (p. 348)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; ancestry; root status; identified readiness placement. See the linked RAD detail and uncertainty register.

2.19.2. Clinical Device Maintenance Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-device-maintenance-management`	Stable publication identity.
Architectural Status	Candidate	Not approved hierarchy; retain candidate standing.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:1216 (p. 348)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; ancestry; root status; maintenance membership. See the linked RAD detail and uncertainty register.

2.19.3. Health Information Sharing Programme Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-information-sharing-programme-management`	Stable publication identity.
Architectural Status	Candidate	Not approved hierarchy; retain candidate standing.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	business:1222 (p. 348)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; ancestry; root status; sharing programme placement. See the linked RAD detail and uncertainty register.

3. Business Enabling Capabilities and owned behaviour

All 76 established BEC contexts have unresolved CT tiers and complete ancestry. This chapter groups them by documented contextual view and responsibility; its heading depth is navigation, not a CT assertion. Features, Functions, Services and Processes below retain their actual owning context. Function/Service/Process and Feature associations form a graph, recorded in the relationship register. A Feature listing is not a claim of complete CT1 → CT2 → CT3 → FT reconciliation.

3.1. Entity Management

Capability	Reference Id
Client Administration (p. 78)	HCM-BEC-client-administration
Provider Administration (p. 79)	HCM-BEC-provider-administration
Organisation Administration (p. 86)	HCM-BEC-organisation-administration
Location Administration (p. 90)	HCM-BEC-location-administration
Health Service Administration (p. 94)	HCM-BEC-health-service-administration
Health Product Administration (p. 98)	HCM-BEC-health-product-administration
Clinical Device Administration (p. 99)	HCM-BEC-clinical-device-administration
Person Identity (p. 103)	HCM-BEC-person-identity
Healthcare Subject (p. 108)	HCM-BEC-healthcare-subject
Client Relationship (p. 111)	HCM-BEC-client-relationship
Client Privacy (p. 113)	HCM-BEC-client-privacy

3.1.1. Client Administration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	HCM-BEC-client-administration	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:76 (p. 355); enabling:112 (p. 356)	Linked RAD detail; exact locators in manifest.

Established partial responsibility relationship

Capability	Reference Id	Tier
Person Identity (p. 103)	HCM-BEC-person-identity	Unresolved

Unresolved dimensions: tier; root status; ancestry; relevance. See the linked RAD detail and uncertainty register.

3.1.2. Provider Administration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-provider-administration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:81 (p. 355); enabling:137 (p. 356); em:86 (p. 427)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry; relevance. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
National Practitioner Verification (p. 79)	Feature	`FEAT-EM-11`
Practitioner Profile Governance (p. 80)	Feature	`FEAT-EM-12`
Role & Specialty Modeling (p. 80)	Feature	`FEAT-EM-13`
Electronic Endpoint Binding (p. 80)	Feature	`FEAT-EM-14`
Practitioner-Organisation Affiliation (p. 81)	Feature	`FEAT-EM-15`
Clinical Privileging Record (p. 81)	Feature	`FEAT-EM-16`
Verify Practitioner Registration (p. 81)	Function	`HCM-FN-verify-practitioner-registration`
Practitioner Verification Service (p. 82)	Service	`HCM-SV-practitioner-verification-service`
Govern Practitioner Profile (p. 82)	Function	`HCM-FN-govern-practitioner-profile`
Practitioner Directory Query (p. 82)	Service	`HCM-SV-practitioner-directory-query`
Maintain Practitioner Roles (p. 83)	Function	`HCM-FN-maintain-practitioner-roles`
Practitioner Role Resolution (p. 83)	Service	`HCM-SV-practitioner-role-resolution`
Bind Practitioner Electronic Endpoints (p. 83)	Function	`HCM-FN-bind-practitioner-electronic-endpoints`
Practitioner Endpoint Lookup (p. 84)	Service	`HCM-SV-practitioner-endpoint-lookup`
Maintain Practitioner Affiliations (p. 84)	Function	`HCM-FN-maintain-practitioner-affiliations`
Practitioner Affiliation Resolution (p. 84)	Service	`HCM-SV-practitioner-affiliation-resolution`
Maintain Clinical Privileges (p. 85)	Function	`HCM-FN-maintain-clinical-privileges`
Clinical Privilege Verification (p. 85)	Service	`HCM-SV-clinical-privilege-verification`
Practitioner Verification Process (p. 85)	Process	`HCM-PR-practitioner-verification-process`

3.1.2.1. National Practitioner Verification

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-11`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Provider Administration (p. 79)	Responsibility; no invented intermediate element.

Name	Value	Comment / Description
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:142 (p. 356)	Linked RAD detail; exact locators in manifest.

Ingest, validate, and verify national practitioner identifiers (e.g., HPI-I) and professional registration status.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.2.2. Practitioner Profile Governance

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-12`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Provider Administration (p. 79)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:143 (p. 356)	Linked RAD detail; exact locators in manifest.

Maintain core professional demographic details, qualifications, and active practice status.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.2.3. Role & Specialty Modeling

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-13`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Provider Administration (p. 79)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:147 (p. 356)	Linked RAD detail; exact locators in manifest.

Define and associate clinical specialties, practice roles, and service capacities for a practitioner.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.2.4. Electronic Endpoint Binding

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-14`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Provider Administration (p. 79)	Responsibility; no invented intermediate element.

Name	Value	Comment / Description
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:148 (p. 356)	Linked RAD detail; exact locators in manifest.

Associate validated communication endpoints (secure messaging addresses, direct routing queues) with specific practitioner roles.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.2.5. Practitioner-Organisation Affiliation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-15`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Provider Administration (p. 79)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:152 (p. 356)	Linked RAD detail; exact locators in manifest.

Maintain temporal affiliations between practitioners and healthcare organizations.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.2.6. Clinical Privileging Record

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-16`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Provider Administration (p. 79)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:156 (p. 356)	Linked RAD detail; exact locators in manifest.

Track active clinical admitting and practice privileges within healthcare facilities.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.2.7. Verify Practitioner Registration

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-verify-practitioner-registration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Provider Administration (p. 79)	Responsibility; no invented intermediate element.

Name	Value	Comment / Description
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:92 (p. 427)	Linked RAD detail; exact locators in manifest.

Validates national registration, specialty endorsements, and disciplinary sanctions against regulatory registries (e.g., AHPRA).

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.2.8. Practitioner Verification Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-practitioner-verification-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Provider Administration (p. 79)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:93 (p. 427)	Linked RAD detail; exact locators in manifest.

Discloses verified registration and status to credentialing and clinical systems.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.2.9. Govern Practitioner Profile

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-govern-practitioner-profile`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Provider Administration (p. 79)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:96 (p. 427)	Linked RAD detail; exact locators in manifest.

Maintains official names, professional identifiers (e.g., HPI-I, Prescriber Number), and clinical qualifications.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.2.10. Practitioner Directory Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-practitioner-directory-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Provider Administration (p. 79)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.

Name	Value	Comment / Description
Source	em:97 (p. 427)	Linked RAD detail; exact locators in manifest.

Provides provider directory lookups across the healthcare network.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.2.11. Maintain Practitioner Roles

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-practitioner-roles`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Provider Administration (p. 79)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:99 (p. 427)	Linked RAD detail; exact locators in manifest.

Assigns and maintains clinical roles (e.g., Specialist Physician, Registrar, General Practitioner) fulfilled by a practitioner.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.2.12. Practitioner Role Resolution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-practitioner-role-resolution`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Provider Administration (p. 79)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:100 (p. 427)	Linked RAD detail; exact locators in manifest.

Resolves the specific clinical capacities of a practitioner.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.2.13. Bind Practitioner Electronic Endpoints

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-bind-practitioner-electronic-endpoints`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Provider Administration (p. 79)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:102 (p. 427)	Linked RAD detail; exact locators in manifest.

Associates secure messaging addresses, direct communication endpoints, and notification preferences to practitioner roles.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.2.14. Practitioner Endpoint Lookup

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-practitioner-endpoint-lookup`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Provider Administration (p. 79)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:103 (p. 427)	Linked RAD detail; exact locators in manifest.

Provides secure message routing destinations for clinical communication.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.2.15. Maintain Practitioner Affiliations

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-practitioner-affiliations`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Provider Administration (p. 79)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:105 (p. 427)	Linked RAD detail; exact locators in manifest.

Tracks employment, admitting rights, and consulting appointments at specific healthcare facilities.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.2.16. Practitioner Affiliation Resolution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-practitioner-affiliation-resolution`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Provider Administration (p. 79)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:106 (p. 427)	Linked RAD detail; exact locators in manifest.

Discloses organizational ties and practice locations.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.2.17. Maintain Clinical Privileges

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-clinical-privileges`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Provider Administration (p. 79)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:108 (p. 427)	Linked RAD detail; exact locators in manifest.

Records approved clinical scopes of practice, procedural authorizations, and prescribing privileges.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.2.18. Clinical Privilege Verification

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-clinical-privilege-verification`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Provider Administration (p. 79)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:109 (p. 427)	Linked RAD detail; exact locators in manifest.

Verifies procedural and prescribing authority for clinical ordering systems.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.2.19. Practitioner Verification Process

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-PR-practitioner-verification-process`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Provider Administration (p. 79)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	processes:85 (p. 406); processes:86 (p. 406)	Linked RAD detail; exact locators in manifest.

- **Process Purpose**: Governs the lifecycle of validating, certifying, and periodically re-verifying a healthcare practitioner's professional credentials, registration status, and clinical scope of practice.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.1.3. Organisation Administration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-organisation-administration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:86 (p. 355); enabling:158 (p. 357); em:118 (p. 428)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry; relevance. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
National Organisation Verification (p. 86)	Feature	`FEAT-EM-17`
Organisation Profile Management (p. 87)	Feature	`FEAT-EM-18`
Hierarchical Structure Navigation (p. 87)	Feature	`FEAT-EM-19`
Directory Contact Management (p. 87)	Feature	`FEAT-EM-20`
Verify Healthcare Organisation (p. 88)	Function	`HCM-FN-verify-healthcare-organisation`
Organisation Verification Service (p. 88)	Service	`HCM-SV-organisation-verification-service`
Govern Organisation Profile (p. 88)	Function	`HCM-FN-govern-organisation-profile`
Organisation Directory Query (p. 89)	Service	`HCM-SV-organisation-directory-query`
Maintain Organisational Structure (p. 89)	Function	`HCM-FN-maintain-organisational-structure`
Organisational Hierarchy Resolution (p. 89)	Service	`HCM-SV-organisational-hierarchy-resolution`
Maintain Organisation Contacts (p. 90)	Function	`HCM-FN-maintain-organisation-contacts`
Organisation Contact Lookup (p. 90)	Service	`HCM-SV-organisation-contact-lookup`

3.1.3.1. National Organisation Verification

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-17`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Organisation Administration (p. 86)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:163 (p. 357)	Linked RAD detail; exact locators in manifest.

Validate national organization identifiers (e.g., HPI-O) and legal entity classifications.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.3.2. Organisation Profile Management

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-18`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Organisation Administration (p. 86)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:164 (p. 357)	Linked RAD detail; exact locators in manifest.

Govern directory profiles, operational status, and published service descriptions.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.3.3. Hierarchical Structure Navigation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-19`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Organisation Administration (p. 86)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:168 (p. 357)	Linked RAD detail; exact locators in manifest.

Model, traverse, and resolve nested departmental, divisional, and regional healthcare networks.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.3.4. Directory Contact Management

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-20`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Organisation Administration (p. 86)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:172 (p. 357)	Linked RAD detail; exact locators in manifest.

Manage verified phone, email, and administrative contact channels for healthcare facilities.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.3.5. Verify Healthcare Organisation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-verify-healthcare-organisation`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Organisation Administration (p. 86)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:121 (p. 428)	Linked RAD detail; exact locators in manifest.

Validates enterprise legal identity, national facility identifiers (e.g., HPI-O), and accreditation status.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.3.6. Organisation Verification Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-organisation-verification-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Organisation Administration (p. 86)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:122 (p. 428)	Linked RAD detail; exact locators in manifest.

Provides verified organisation credentials to external and internal partners.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.3.7. Govern Organisation Profile

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-govern-organisation-profile`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Organisation Administration (p. 86)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:124 (p. 428)	Linked RAD detail; exact locators in manifest.

Maintains official trading names, enterprise registration numbers, and governing bodies.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.3.8. Organisation Directory Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-organisation-directory-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Organisation Administration (p. 86)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:125 (p. 428)	Linked RAD detail; exact locators in manifest.

Discloses healthcare organisation profiles across the region.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.3.9. Maintain Organisational Structure

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-organisational-structure`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Organisation Administration (p. 86)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:127 (p. 428)	Linked RAD detail; exact locators in manifest.

Models parent-subsidiary relationships, clinical directorates, departments, and operational divisions.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.3.10. Organisational Hierarchy Resolution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-organisational-hierarchy-resolution`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Organisation Administration (p. 86)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:128 (p. 428)	Linked RAD detail; exact locators in manifest.

Resolves department nesting and reporting hierarchies.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.3.11. Maintain Organisation Contacts

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-organisation-contacts`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Organisation Administration (p. 86)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:130 (p. 428)	Linked RAD detail; exact locators in manifest.

Manages administrative, clinical, and emergency contact details for facilities.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.3.12. Organisation Contact Lookup

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-organisation-contact-lookup`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Organisation Administration (p. 86)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:131 (p. 428)	Linked RAD detail; exact locators in manifest.

Provides official operational contact details.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.4. Location Administration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-location-administration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:90 (p. 355); enabling:174 (p. 357); em:140 (p. 429)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry; relevance. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Location Geospatial & Address Resolution (p. 91)	Feature	`FEAT-EM-21`
Physical Hierarchy Modeling (p. 91)	Feature	`FEAT-EM-22`
Atomic Care-Place Modeling (p. 91)	Feature	`FEAT-EM-23`

Element	Type	Reference Id
Maintain Healthcare Location (p. 92)	Function	`HCM-FN-maintain-healthcare-location`
Location Directory Query (p. 92)	Service	`HCM-SV-location-directory-query`
Maintain Physical Location Hierarchy (p. 92)	Function	`HCM-FN-maintain-physical-location-hierarchy`
Location Hierarchy Resolution (p. 93)	Service	`HCM-SV-location-hierarchy-resolution`
Maintain Care-Place Definition (p. 93)	Function	`HCM-FN-maintain-care-place-definition`
Care-Place Specification Lookup (p. 93)	Service	`HCM-SV-care-place-specification-lookup`

3.1.4.1. Location Geospatial & Address Resolution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-21`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Location Administration (p. 90)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:179 (p. 357)	Linked RAD detail; exact locators in manifest.

Validate physical addresses, geospatial coordinates, and location operational status.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.4.2. Physical Hierarchy Modeling

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-22`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Location Administration (p. 90)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:183 (p. 357)	Linked RAD detail; exact locators in manifest.

Represent campus, building, wing, floor, and ward architectural relationships.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.4.3. Atomic Care-Place Modeling

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-23`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.

Name	Value	Comment / Description
Owning Context	Location Administration (p. 90)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:187 (p. 357)	Linked RAD detail; exact locators in manifest.

Define specific beds, care-bays, and examination rooms with their physical capabilities (e.g., telemetry-capable, negative-pressure).

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.4.4. Maintain Healthcare Location

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-healthcare-location`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Location Administration (p. 90)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:145 (p. 429)	Linked RAD detail; exact locators in manifest.

Records physical sites, campuses, outpatient clinics, and community health centres.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.4.5. Location Directory Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-location-directory-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Location Administration (p. 90)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:146 (p. 429)	Linked RAD detail; exact locators in manifest.

Exposes location profiles and geospatial addresses.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.4.6. Maintain Physical Location Hierarchy

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-physical-location-hierarchy`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Location Administration (p. 90)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.

Name	Value	Comment / Description
Source	em:148 (p. 429)	Linked RAD detail; exact locators in manifest.

Models the nested structural containment tree: `Campus` → `Building` → `Floor` → `Ward/Suite` → `Room/Bay` → `BedCare-Place`.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.4.7. Location Hierarchy Resolution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-location-hierarchy-resolution`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Location Administration (p. 90)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:149 (p. 429)	Linked RAD detail; exact locators in manifest.

Discloses physical containment and navigation paths for clinical facilities.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.4.8. Maintain Care-Place Definition

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-care-place-definition`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Location Administration (p. 90)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:151 (p. 429)	Linked RAD detail; exact locators in manifest.

Defines static bed bays, treatment chairs, surgical tables, and emergency resuscitation cubicles, including equipment capabilities and oxygen/telemetry infrastructure.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.4.9. Care-Place Specification Lookup

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-care-place-specification-lookup`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Location Administration (p. 90)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:152 (p. 429)	Linked RAD detail; exact locators in manifest.

Discloses care-place physical capabilities to Bed & Care-Place Management.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.5. Health Service Administration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-health-service-administration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:94 (p. 355); enabling:189 (p. 358); em:161 (p. 429)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry; relevance. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Service Catalogue Publishing (p. 94)	Feature	`FEAT-EM-24`
Service-Location-Provider Binding (p. 95)	Feature	`FEAT-EM-25`
Operating Schedule Specification (p. 95)	Feature	`FEAT-EM-26`
Eligibility & Catchment Specification (p. 95)	Feature	`FEAT-EM-27`
Maintain Healthcare Service Definition (p. 96)	Function	`HCM-FN-maintain-healthcare-service-definition`
Healthcare Service Directory Query (p. 96)	Service	`HCM-SV-healthcare-service-directory-query`
Maintain Service / Location / Provider Map (p. 96)	Function	`HCM-FN-maintain-service-location-provider-map`
Service Provision Resolution (p. 97)	Service	`HCM-SV-service-provision-resolution`
Maintain Service Availability (p. 97)	Function	`HCM-FN-maintain-service-availability`
Service Availability Query (p. 97)	Service	`HCM-SV-service-availability-query`
Maintain Service Eligibility Rules (p. 98)	Function	`HCM-FN-maintain-service-eligibility-rules`
Service Eligibility Evaluation (p. 98)	Service	`HCM-SV-service-eligibility-evaluation`

3.1.5.1. Service Catalogue Publishing

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-24`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Health Service Administration (p. 94)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.

Name	Value	Comment / Description
Source	enabling:194 (p. 358)	Linked RAD detail; exact locators in manifest.

Publish structured healthcare service definitions, clinical specialties, and intake criteria to regional directories.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.5.2. Service-Location-Provider Binding

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-EM-25	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Health Service Administration (p. 94)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:198 (p. 358)	Linked RAD detail; exact locators in manifest.

Correlate which organization provides a named health service at which physical location with which practitioner roles.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.5.3. Operating Schedule Specification

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-EM-26	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Health Service Administration (p. 94)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:202 (p. 358)	Linked RAD detail; exact locators in manifest.

Define temporal availability, clinic operating hours, and on-call service windows.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.5.4. Eligibility & Catchment Specification

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-EM-27	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Health Service Administration (p. 94)	Responsibility; no invented intermediate element.

Name	Value	Comment / Description
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:206 (p. 358)	Linked RAD detail; exact locators in manifest.

Define clinical referral thresholds and geographic catchment boundaries.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.5.5. Maintain Healthcare Service Definition

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-healthcare-service-definition`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Health Service Administration (p. 94)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:174 (p. 429)	Linked RAD detail; exact locators in manifest.

Defines clinical service offerings (e.g., Cardiology Outpatient Clinic, MRI Imaging, Acute Stroke Care), specialty classifications, and clinical scope.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.5.6. Healthcare Service Directory Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-healthcare-service-directory-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Health Service Administration (p. 94)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:175 (p. 429)	Linked RAD detail; exact locators in manifest.

Exposes published clinical service offerings.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.5.7. Maintain Service / Location / Provider Map

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-service-location-provider-map`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Health Service Administration (p. 94)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.

Name	Value	Comment / Description
Source	em:177 (p. 429)	Linked RAD detail; exact locators in manifest.

Binds health service definitions to delivering organisations, physical clinic locations, and participating practitioner roles.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.5.8. Service Provision Resolution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-service-provision-resolution`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Health Service Administration (p. 94)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:178 (p. 429)	Linked RAD detail; exact locators in manifest.

Exposes multi-dimensional resolution queries answering who, where, and when healthcare services are delivered.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.5.9. Maintain Service Availability

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-service-availability`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Health Service Administration (p. 94)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:180 (p. 429)	Linked RAD detail; exact locators in manifest.

Governs regular operating hours, emergency intake availability, and seasonal clinic schedules.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.5.10. Service Availability Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-service-availability-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Health Service Administration (p. 94)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:181 (p. 429)	Linked RAD detail; exact locators in manifest.

Discloses operational intake hours and open status.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.5.11. Maintain Service Eligibility Rules

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-service-eligibility-rules`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Health Service Administration (p. 94)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:183 (p. 429)	Linked RAD detail; exact locators in manifest.

Configures catchment criteria, age thresholds, referral requirements, and clinical indications for service access.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.5.12. Service Eligibility Evaluation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-service-eligibility-evaluation`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Health Service Administration (p. 94)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:184 (p. 429)	Linked RAD detail; exact locators in manifest.

Evaluates referral eligibility for intake coordinators.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.6. Health Product Administration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-health-product-administration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:99 (p. 355); enabling:208 (p. 358)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry; relevance. See the linked RAD detail and uncertainty register.

3.1.7. Clinical Device Administration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-clinical-device-administration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:103 (p. 355); enabling:215 (p. 358); em:193 (p. 430)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry; relevance. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Device Type Specification (p. 99)	Feature	`FEAT-EM-28`
Device Serial Tracking (p. 100)	Feature	`FEAT-EM-29`
Device-Patient/Location Assignment (p. 100)	Feature	`FEAT-EM-30`
Device Endpoint Registration (p. 100)	Feature	`FEAT-EM-31`
Maintain Device Definition (p. 101)	Function	`HCM-FN-maintain-device-definition`
Device Definition Query (p. 101)	Service	`HCM-SV-device-definition-query`
Maintain Device Instance Identity (p. 101)	Function	`HCM-FN-maintain-device-instance-identity`
Device Instance Resolution (p. 102)	Service	`HCM-SV-device-instance-resolution`
Maintain Device Association (p. 102)	Function	`HCM-FN-maintain-device-association`
Device Association Query (p. 102)	Service	`HCM-SV-device-association-query`
Maintain Device Communication Endpoints (p. 103)	Function	`HCM-FN-maintain-device-communication-endpoints`
Device Endpoint Resolution (p. 103)	Service	`HCM-SV-device-endpoint-resolution`

3.1.7.1. Device Type Specification

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-28`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Clinical Device Administration (p. 99)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:220 (p. 358)	Linked RAD detail; exact locators in manifest.

Catalog medical device models, regulatory approvals, and capabilities.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.7.2. Device Serial Tracking

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-29`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Clinical Device Administration (p. 99)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:224 (p. 358)	Linked RAD detail; exact locators in manifest.

Ingest and maintain unique device identifier (UDI) records and serial numbers.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.7.3. Device-Patient/Location Assignment

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-30`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Clinical Device Administration (p. 99)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:228 (p. 358)	Linked RAD detail; exact locators in manifest.

Track dynamic association between a telemetry monitor, patient, and physical bed.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.7.4. Device Endpoint Registration

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-31`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Clinical Device Administration (p. 99)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:232 (p. 358)	Linked RAD detail; exact locators in manifest.

Register communication interfaces and security credentials for diagnostic telemetry feeds.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.7.5. Maintain Device Definition

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-device-definition`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinical Device Administration (p. 99)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:196 (p. 430)	Linked RAD detail; exact locators in manifest.

Models medical device models, manufacturers, regulatory classifications (e.g., TGA/FDA classes), and calibration specifications.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.7.6. Device Definition Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-device-definition-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinical Device Administration (p. 99)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:197 (p. 430)	Linked RAD detail; exact locators in manifest.

Discloses approved medical device models and technical parameters.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.7.7. Maintain Device Instance Identity

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-device-instance-identity`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinical Device Administration (p. 99)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:199 (p. 430)	Linked RAD detail; exact locators in manifest.

Tracks physical serial numbers, Unique Device Identifiers (UDI), asset tags, and firmware versions.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.7.8. Device Instance Resolution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-device-instance-resolution`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinical Device Administration (p. 99)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:200 (p. 430)	Linked RAD detail; exact locators in manifest.

Resolves physical device instances from scanned barcodes or UDIs.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.7.9. Maintain Device Association

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-device-association`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinical Device Administration (p. 99)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:202 (p. 430)	Linked RAD detail; exact locators in manifest.

Records active associations between clinical devices, patient encounters, and specific care-places (e.g., Bed 4 Vital Signs Monitor).

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.7.10. Device Association Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-device-association-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinical Device Administration (p. 99)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:203 (p. 430)	Linked RAD detail; exact locators in manifest.

Discloses active device-to-patient and device-to-location bindings.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.7.11. Maintain Device Communication Endpoints

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-device-communication-endpoints`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinical Device Administration (p. 99)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:205 (p. 430)	Linked RAD detail; exact locators in manifest.

Manages network telemetry addresses, gateway ports, and protocol bindings for medical devices.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.7.12. Device Endpoint Resolution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-device-endpoint-resolution`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinical Device Administration (p. 99)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:206 (p. 430)	Linked RAD detail; exact locators in manifest.

Exposes network addresses for telemetry routing.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.8. Person Identity

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-person-identity`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Client Administration (p. 78)	Partial responsibility parent; tiers unresolved.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Core	Independent of Architectural Status.
Source	enabling:115 (p. 356); em:16 (p. 425)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Identifier Resolution (p. 104)	Feature	`FEAT-EM-01`
Cross-Authority Identifier Correlation (p. 104)	Feature	`FEAT-EM-02`
Identity Alias Association (p. 105)	Feature	`FEAT-EM-03`
Governed Identity Correction (p. 105)	Feature	`FEAT-EM-04`
Resolve Person Identifier (p. 105)	Function	`HCM-FN-resolve-person-identifier`
Person Identifier Resolution (p. 106)	Service	`HCM-SV-person-identifier-resolution`
Correlate Person Identifiers (p. 106)	Function	`HCM-FN-correlate-person-identifiers`
Person Identifier Correlation (p. 106)	Service	`HCM-SV-person-identifier-correlation`
Maintain Person Identity Aliases (p. 107)	Function	`HCM-FN-maintain-person-identity-aliases`
Apply Governed Person Identity Correction (p. 107)	Function	`HCM-FN-apply-governed-person-identity-correction`
Person Identity Correction (p. 107)	Service	`HCM-SV-person-identity-correction`
Governed Person Identity Correction Process (p. 108)	Process	`HCM-PR-governed-person-identity-correction-process`

3.1.8.1. Identifier Resolution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-01`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Person Identity (p. 103)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:117 (p. 356)	Linked RAD detail; exact locators in manifest.

Ingest and resolve external identifiers against registered issuing authorities and namespace systems.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.8.2. Cross-Authority Identifier Correlation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-02`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Person Identity (p. 103)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:118 (p. 356)	Linked RAD detail; exact locators in manifest.

Correlate disparate local identifiers (e.g., hospital MRN, clinic client ID, national IHI) belonging to the same individual.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.8.3. Identity Alias Association

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-03`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Person Identity (p. 103)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:119 (p. 356)	Linked RAD detail; exact locators in manifest.

Maintain explicit associations between active identifiers and historical or demographic aliases without destructive overwriting.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.8.4. Governed Identity Correction

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-04`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Person Identity (p. 103)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:120 (p. 356)	Linked RAD detail; exact locators in manifest.

Apply administrative identity updates, linked-record corrections, and split/merge notifications propagated from authoritative sources.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.8.5. Resolve Person Identifier

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-resolve-person-identifier`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Person Identity (p. 103)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:24 (p. 425)	Linked RAD detail; exact locators in manifest.

Validates and retrieves the authoritative demographic record bound to a specific identifier.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.8.6. Person Identifier Resolution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-person-identifier-resolution`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Person Identity (p. 103)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:25 (p. 425)	Linked RAD detail; exact locators in manifest.

Exposed to Service Administration and Service Delivery to disambiguate client identities.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.8.7. Correlate Person Identifiers

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-correlate-person-identifiers`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Person Identity (p. 103)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:27 (p. 425)	Linked RAD detail; exact locators in manifest.

Correlates local identifiers (e.g., Hospital MRN, National IHI, Pathology ID) using relationships established by authoritative information, retaining their source and namespace.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.8.8. Person Identifier Correlation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-person-identifier-correlation`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Person Identity (p. 103)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:28 (p. 425)	Linked RAD detail; exact locators in manifest.

Exposed across the HIE to establish patient cross-reference mappings.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.8.9. Maintain Person Identity Aliases

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-person-identity-aliases`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Person Identity (p. 103)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:30 (p. 425)	Linked RAD detail; exact locators in manifest.

Records and qualifies alias names, titles, and emergency aliases for a governed person identity.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.8.10. Apply Governed Person Identity Correction

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-apply-governed-person-identity-correction`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Person Identity (p. 103)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:33 (p. 425)	Linked RAD detail; exact locators in manifest.

Processes externally authoritative corrections and merge/unlink outcomes, updates governed identity associations and propagates resulting managed-state consequences with audit history. Harmonia does not adjudicate the originating person-merge decision.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.8.11. Person Identity Correction

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-person-identity-correction`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Person Identity (p. 103)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:34 (p. 425)	Linked RAD detail; exact locators in manifest.

Exposed to authorized HIM/Medical Records administrators.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.8.12. Governed Person Identity Correction Process

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-PR-governed-person-identity-correction-process`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Person Identity (p. 103)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	processes:69 (p. 405); processes:70 (p. 405)	Linked RAD detail; exact locators in manifest.

- **Process Purpose**: Processes externally authoritative identity corrections and merge/unlink outcomes, applies their consequences to Harmonia-managed identity information and identifier associations, and communicates resulting changes with an audit history.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.1.9. Healthcare Subject

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-healthcare-subject`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Core	Independent of Architectural Status.
Source	enabling:122 (p. 356); em:41 (p. 426)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Subject Context Binding (p. 109)	Feature	`FEAT-EM-05`
Demographic Context Governance (p. 109)	Feature	`FEAT-EM-06`
Establish Healthcare Subject Context (p. 109)	Function	`HCM-FN-establish-healthcare-subject-context`
Healthcare Subject Context Resolution (p. 110)	Service	`HCM-SV-healthcare-subject-context-resolution`
Govern Subject Demographic Context (p. 110)	Function	`HCM-FN-govern-subject-demographic-context`
Demographic Change Distribution (p. 110)	Service	`HCM-SV-demographic-change-distribution`

3.1.9.1. Subject Context Binding

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-05`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Healthcare Subject (p. 108)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:124 (p. 356)	Linked RAD detail; exact locators in manifest.

Bind authenticated person identity to clinical encounters, orders, and observations as the enduring subject of care with verifiable association.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.9.2. Demographic Context Governance

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-06`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Healthcare Subject (p. 108)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:125 (p. 356)	Linked RAD detail; exact locators in manifest.

Manage essential demographic attributes (date of birth, sex, vital status) with full provenance and timestamping.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.9.3. Establish Healthcare Subject Context

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-establish-healthcare-subject-context`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Healthcare Subject (p. 108)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:45 (p. 426)	Linked RAD detail; exact locators in manifest.

Binds demographic, cultural, and communication requirements to a patient record.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.9.4. Healthcare Subject Context Resolution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-healthcare-subject-context-resolution`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Healthcare Subject (p. 108)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:46 (p. 426)	Linked RAD detail; exact locators in manifest.

Provides comprehensive demographic, language, and indigenous status context to clinical applications.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.9.5. Govern Subject Demographic Context

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-govern-subject-demographic-context`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Healthcare Subject (p. 108)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:48 (p. 426)	Linked RAD detail; exact locators in manifest.

Applies validation rules to address changes, date of birth verification, and death notifications.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.9.6. Demographic Change Distribution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-demographic-change-distribution`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Healthcare Subject (p. 108)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:49 (p. 426)	Linked RAD detail; exact locators in manifest.

Emits governed subject demographic updates to subscribed clinical systems.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.10. Client Relationship

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-client-relationship`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.
Source	enabling:127 (p. 356); em:55 (p. 426)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Next-of-Kin & Guardian Association (p. 111)	Feature	`FEAT-EM-07`
Relationship Validity Tracking (p. 112)	Feature	`FEAT-EM-08`
Maintain Client Relationships (p. 112)	Function	`HCM-FN-maintain-client-relationships`
Client Relationship Query (p. 112)	Service	`HCM-SV-client-relationship-query`
Govern Client Relationship Validity (p. 113)	Function	`HCM-FN-govern-client-relationship-validity`
Representative Authority Verification (p. 113)	Service	`HCM-SV-representative-authority-verification`

3.1.10.1. Next-of-Kin & Guardian Association

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-07`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Client Relationship (p. 111)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:129 (p. 356)	Linked RAD detail; exact locators in manifest.

Associate nominated emergency contacts, legal guardians, and care agents with the client record.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.10.2. Relationship Validity Tracking

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-08`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Client Relationship (p. 111)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:130 (p. 356)	Linked RAD detail; exact locators in manifest.

Maintain effective date intervals and authority scopes for nominated client relationships.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.10.3. Maintain Client Relationships

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-client-relationships`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Client Relationship (p. 111)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:59 (p. 426)	Linked RAD detail; exact locators in manifest.

Records links between patients and carers, next of kin, nominated representatives, and legal guardians.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.10.4. Client Relationship Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-client-relationship-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Client Relationship (p. 111)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:60 (p. 426)	Linked RAD detail; exact locators in manifest.

Discloses verified support network members and contact details to clinical teams.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.10.5. Govern Client Relationship Validity

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-govern-client-relationship-validity`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Client Relationship (p. 111)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:62 (p. 426)	Linked RAD detail; exact locators in manifest.

Verifies power-of-attorney documents, guardianship orders, and proxy validity periods.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.10.6. Representative Authority Verification

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-representative-authority-verification`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Client Relationship (p. 111)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:63 (p. 426)	Linked RAD detail; exact locators in manifest.

Confirms legal decision-making authority for substitute consent.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.11. Client Privacy

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-client-privacy`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Core	Independent of Architectural Status.
Source	enabling:132 (p. 356); em:69 (p. 427)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Consent Directive Evaluation (p. 114)	Feature	`FEAT-EM-09`

Element	Type	Reference Id
Confidentiality Flag Enforcement (p. 114)	Feature	`FEAT-EM-10`
Evaluate Client Consent (p. 114)	Function	`HCM-FN-evaluate-client-consent`
Consent Decision Service (p. 115)	Service	`HCM-SV-consent-decision-service`
Evaluate Client Confidentiality Restrictions (p. 115)	Function	`HCM-FN-evaluate-client-confidentiality-restrictions`
Confidentiality Restriction Evaluation (p. 115)	Service	`HCM-SV-confidentiality-restriction-evaluation`

3.1.11.1. Consent Directive Evaluation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-09`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Client Privacy (p. 113)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:134 (p. 356)	Linked RAD detail; exact locators in manifest.

Evaluate client consent directives against requested information disclosure and exchange actions.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.11.2. Confidentiality Flag Enforcement

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-10`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Client Privacy (p. 113)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:135 (p. 356)	Linked RAD detail; exact locators in manifest.

Enforce restricted access flags (e.g., VIP, sensitive diagnosis, restricted record) across query and presentation interfaces.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.11.3. Evaluate Client Consent

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-evaluate-client-consent`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Client Privacy (p. 113)	Responsibility; no invented intermediate element.

Name	Value	Comment / Description
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:73 (p. 427)	Linked RAD detail; exact locators in manifest.

Evaluates patient opt-in, opt-out, and general information sharing preferences against proposed exchanges.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.11.4. Consent Decision Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-consent-decision-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Client Privacy (p. 113)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:74 (p. 427)	Linked RAD detail; exact locators in manifest.

Provides real-time consent evaluation decisions to Health Information Control.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.11.5. Evaluate Client Confidentiality Restrictions

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-evaluate-client-confidentiality-restrictions`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Client Privacy (p. 113)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	em:76 (p. 427)	Linked RAD detail; exact locators in manifest.

Evaluates sensitive record flags (e.g., VIP masking, restricted department exclusions).

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.1.11.6. Confidentiality Restriction Evaluation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-confidentiality-restriction-evaluation`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Client Privacy (p. 113)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.

Name	Value	Comment / Description
Source	em:77 (p. 427)	Linked RAD detail; exact locators in manifest.

Enforces record-level masking directives across the HIE.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2. Service Administration

Capability	Reference Id
Client-Centred Administration (p. 116)	`HCM-BEC-client-centred-administration`
Referral Administration (p. 119)	`HCM-BEC-referral-administration`
Scheduling Administration (p. 123)	`HCM-BEC-scheduling-administration`
Episode & Encounter Administration (p. 125)	`HCM-BEC-episode-encounter-administration`
Order Administration (p. 129)	`HCM-BEC-order-administration`
Diagnostic Administration (p. 135)	`HCM-BEC-diagnostic-administration`
Medication Administration (p. 139)	`HCM-BEC-medication-administration`
Procedure Administration (p. 142)	`HCM-BEC-procedure-administration`
Clinical Record Administration (p. 145)	`HCM-BEC-clinical-record-administration`
Care Plan Administration (p. 149)	`HCM-BEC-care-plan-administration`
Clinical Communication Administration (p. 152)	`HCM-BEC-clinical-communication-administration`
Funding & Coverage Administration (p. 155)	`HCM-BEC-funding-coverage-administration`
Clinical Charging & Claims Administration (p. 156)	`HCM-BEC-clinical-charging-claims-administration`

3.2.1. Client-Centred Administration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-client-centred-administration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.
Source	enabling:241 (p. 359); enabling:279 (p. 360); sa:12 (p. 431)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Demographic Update Ingestion (p. 117)	Feature	`FEAT-SA-01`
Demographic Change Distribution (p. 117)	Feature	`FEAT-SA-02`

Element	Type	Reference Id
Receive Client Demographic Change (p. 117)	Function	`HCM-FN-receive-client-demographic-change`
Demographic Intake Service (p. 118)	Service	`HCM-SV-demographic-intake-service`
Distribute Governed Demographic Change (p. 118)	Function	`HCM-FN-distribute-governed-demographic-change`
Demographic Update Notification (p. 118)	Service	`HCM-SV-demographic-update-notification`

3.2.1.1. Demographic Update Ingestion

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SA-01`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Client-Centred Administration (p. 116)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:281 (p. 360)	Linked RAD detail; exact locators in manifest.

Ingest and validate patient admission and registration updates from external PAS systems.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.1.2. Demographic Change Distribution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SA-02`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Client-Centred Administration (p. 116)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:282 (p. 360)	Linked RAD detail; exact locators in manifest.

Fan out verified client demographic updates to downstream clinical subsystems.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.1.3. Receive Client Demographic Change

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-receive-client-demographic-change`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Client-Centred Administration (p. 116)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.

Name	Value	Comment / Description
Source	sa:18 (p. 431)	Linked RAD detail; exact locators in manifest.

Ingests administrative demographic updates from PAS/registration feeds.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.1.4. Demographic Intake Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-demographic-intake-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Client-Centred Administration (p. 116)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:19 (p. 431)	Linked RAD detail; exact locators in manifest.

Exposed to departmental registration systems.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.1.5. Distribute Governed Demographic Change

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-distribute-governed-demographic-change`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Client-Centred Administration (p. 116)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:21 (p. 431)	Linked RAD detail; exact locators in manifest.

Propagates verified demographic modifications to subscribed clinical subsystems.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.1.6. Demographic Update Notification

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-demographic-update-notification`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Client-Centred Administration (p. 116)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:22 (p. 431)	Linked RAD detail; exact locators in manifest.

Emits change notifications across the HIE.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.2. Referral Administration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-referral-administration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.
Source	enabling:242 (p. 359); enabling:284 (p. 360); sa:28 (p. 431)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Referral Document Ingestion (p. 119)	Feature	`FEAT-SA-03`
Referral Supporting Information (p. 120)	Feature	`FEAT-SA-04`
Referral Status & Outcome Tracking (p. 120)	Feature	`FEAT-SA-05`
Receive Referral (p. 120)	Function	`HCM-FN-receive-referral`
Referral Submission Service (p. 121)	Service	`HCM-SV-referral-submission-service`
Assemble Referral Context (p. 121)	Function	`HCM-FN-assemble-referral-context`
Referral Context Query (p. 121)	Service	`HCM-SV-referral-context-query`
Manage Referral Progression (p. 122)	Function	`HCM-FN-manage-referral-progression`
Referral Status & Outcome Service (p. 122)	Service	`HCM-SV-referral-status-outcome-service`
Referral Progression Process (p. 122)	Process	`HCM-PR-referral-progression-process`

3.2.2.1. Referral Document Ingestion

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SA-03`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Referral Administration (p. 119)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:286 (p. 360)	Linked RAD detail; exact locators in manifest.

Ingest structured clinical referral messages across integration boundaries.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.2.2. Referral Supporting Information

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SA-04`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Referral Administration (p. 119)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:287 (p. 360)	Linked RAD detail; exact locators in manifest.

Collate and associate supporting clinical documentation, diagnostic history, and patient context with referral requests.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.2.3. Referral Status & Outcome Tracking

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SA-05`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Referral Administration (p. 119)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:288 (p. 360)	Linked RAD detail; exact locators in manifest.

Track referral operational progression and communicate referral outcomes (acceptance, decline, or redirection) to referring providers.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.2.4. Receive Referral

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-receive-referral`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Referral Administration (p. 119)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:33 (p. 431)	Linked RAD detail; exact locators in manifest.

Validates and ingests structured electronic referrals, clinical indications, and priority categories.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.2.5. Referral Submission Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-referral-submission-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Referral Administration (p. 119)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:34 (p. 431)	Linked RAD detail; exact locators in manifest.

Exposed to external GP practices and referring health facilities.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.2.6. Assemble Referral Context

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-assemble-referral-context`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Referral Administration (p. 119)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:36 (p. 431)	Linked RAD detail; exact locators in manifest.

Gathers relevant clinical history, medications, and diagnostic investigations into the referral package.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.2.7. Referral Context Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-referral-context-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Referral Administration (p. 119)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:37 (p. 431)	Linked RAD detail; exact locators in manifest.

Provides comprehensive referral dossiers to intake triage clinicians.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.2.8. Manage Referral Progression

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-manage-referral-progression`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Referral Administration (p. 119)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:39 (p. 431)	Linked RAD detail; exact locators in manifest.

Tracks referral disposition in an **illustrative specialist pathway; dispositions vary**: *Submitted → Triaged → Accepted/Waitlisted → Scheduled → Discharged*.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.2.9. Referral Status & Outcome Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-referral-status-outcome-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Referral Administration (p. 119)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:40 (p. 431)	Linked RAD detail; exact locators in manifest.

Exposes progression milestones to referrers and patients.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.2.10. Referral Progression Process

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-PR-referral-progression-process`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Referral Administration (p. 119)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	processes:102 (p. 406); processes:103 (p. 406)	Linked RAD detail; exact locators in manifest.

- **Process Purpose**: Governs the end-to-end operational progression of an incoming clinical referral from receipt through specialist clinical triage, booking, and final service acceptance.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.2.3. Scheduling Administration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-scheduling-administration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.
Source	enabling:243 (p. 359); enabling:290 (p. 360); sa:48 (p. 432)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Appointment Notification Ingestion (p. 123)	Feature	`FEAT-SA-06`
Appointment Status Synchronization (p. 124)	Feature	`FEAT-SA-07`
Receive Appointment Notification (p. 124)	Function	`HCM-FN-receive-appointment-notification`
Appointment Notification Ingress (p. 124)	Service	`HCM-SV-appointment-notification-ingress`
Synchronise Appointment Status (p. 125)	Function	`HCM-FN-synchronise-appointment-status`
Appointment Schedule Query (p. 125)	Service	`HCM-SV-appointment-schedule-query`

3.2.3.1. Appointment Notification Ingestion

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SA-06`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Scheduling Administration (p. 123)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:292 (p. 360)	Linked RAD detail; exact locators in manifest.

Ingest scheduled appointment events from departmental scheduling systems.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.3.2. Appointment Status Synchronization

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SA-07`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Scheduling Administration (p. 123)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:293 (p. 360)	Linked RAD detail; exact locators in manifest.

Propagate booking cancellations, reschedules, and attendances across care coordination systems.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.3.3. Receive Appointment Notification

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-receive-appointment-notification`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Scheduling Administration (p. 123)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:53 (p. 432)	Linked RAD detail; exact locators in manifest.

Ingests appointment creation, rescheduling, cancellation, and check-in events from host PAS engines.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.3.4. Appointment Notification Ingress

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-appointment-notification-ingress`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Scheduling Administration (p. 123)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:54 (p. 432)	Linked RAD detail; exact locators in manifest.

Ingests booking events from departmental schedulers.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.3.5. Synchronise Appointment Status

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-synchronise-appointment-status`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Scheduling Administration (p. 123)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:56 (p. 432)	Linked RAD detail; exact locators in manifest.

Projects synchronised appointment timelines to clinical dashboards and care coordinators.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.3.6. Appointment Schedule Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-appointment-schedule-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Scheduling Administration (p. 123)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:57 (p. 432)	Linked RAD detail; exact locators in manifest.

Exposes consolidated patient appointment schedules across facilities.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.4. Episode & Encounter Administration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-episode-encounter-administration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Core	Independent of Architectural Status.
Source	enabling:244 (p. 359); enabling:295 (p. 360); sa:63 (p. 432)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Encounter Context Creation (p. 126)	Feature	`FEAT-SA-08`

Element	Type	Reference Id
Encounter State Progression (p. 126)	Feature	`FEAT-SA-09`
Encounter Bed/Location Association (p. 127)	Feature	`FEAT-SA-10`
Establish Encounter Context (p. 127)	Function	`HCM-FN-establish-encounter-context`
Encounter Context Resolution (p. 127)	Service	`HCM-SV-encounter-context-resolution`
Progress Encounter State (p. 128)	Function	`HCM-FN-progress-encounter-state`
Encounter Lifecycle Event Notification (p. 128)	Service	`HCM-SV-encounter-lifecycle-event-notification`
Maintain Encounter Care-Place Association (p. 128)	Function	`HCM-FN-maintain-encounter-care-place-association`
Encounter Location History Query (p. 129)	Service	`HCM-SV-encounter-location-history-query`
Encounter Lifecycle Process (p. 129)	Process	`HCM-PR-encounter-lifecycle-process`

3.2.4.1. Encounter Context Creation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SA-08`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Episode & Encounter Administration (p. 125)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:297 (p. 360)	Linked RAD detail; exact locators in manifest.

Create and maintain authoritative encounter tracking contexts for hospital admissions, outpatient visits, and community contacts.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.4.2. Encounter State Progression

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SA-09`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Episode & Encounter Administration (p. 125)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:298 (p. 360)	Linked RAD detail; exact locators in manifest.

Track encounter status transitions (`PLANNED` → `ARRIVED` → `IN_PROGRESS` → `ON_LEAVE` → `DISCHARGED`).

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.4.3. Encounter Bed/Location Association

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SA-10`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Episode & Encounter Administration (p. 125)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:299 (p. 360)	Linked RAD detail; exact locators in manifest.

Dynamically maintain the association between an encounter, care team, and physical bed or ward location.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.4.4. Establish Encounter Context

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-establish-encounter-context`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Episode & Encounter Administration (p. 125)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:68 (p. 432)	Linked RAD detail; exact locators in manifest.

Binds patient, attending practitioner, clinical class (e.g., Inpatient, Emergency, Ambulatory), and admitting diagnosis to a unique encounter identifier.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.4.5. Encounter Context Resolution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-encounter-context-resolution`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Episode & Encounter Administration (p. 125)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:69 (p. 432)	Linked RAD detail; exact locators in manifest.

Discloses active encounter details to clinical applications.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.4.6. Progress Encounter State

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-progress-encounter-state`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Episode & Encounter Administration (p. 125)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:71 (p. 432)	Linked RAD detail; exact locators in manifest.

Coordinates encounter progression (**scoped illustration; ON_LEAVE relationship unresolved**): *Planned → Arrived → In-Progress → Discharged → Completed*.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.4.7. Encounter Lifecycle Event Notification

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-encounter-lifecycle-event-notification`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Episode & Encounter Administration (p. 125)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:72 (p. 432)	Linked RAD detail; exact locators in manifest.

Emits encounter transition events across the HIE.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.4.8. Maintain Encounter Care-Place Association

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-encounter-care-place-association`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Episode & Encounter Administration (p. 125)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:76 (p. 432)	Linked RAD detail; exact locators in manifest.

Binds the encounter to physical wards, rooms, and beds over time.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.4.9. Encounter Location History Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-encounter-location-history-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Episode & Encounter Administration (p. 125)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:77 (p. 432)	Linked RAD detail; exact locators in manifest.

Discloses patient location tracking history.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.4.10. Encounter Lifecycle Process

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-PR-encounter-lifecycle-process`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Episode & Encounter Administration (p. 125)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	processes:119 (p. 407); processes:120 (p. 407)	Linked RAD detail; exact locators in manifest.

- **Process Purpose**: Governs the clinical and administrative lifecycle of an acute, emergency, inpatient, or outpatient encounter between a patient and healthcare services.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.2.5. Order Administration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-order-administration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Core	Independent of Architectural Status.
Source	enabling:245 (p. 359); enabling:301 (p. 361); sa:83 (p. 433)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Order Request Ingestion (p. 130)	Feature	FEAT-SA-11
Order Destination Resolution & Routing (p. 130)	Feature	FEAT-SA-12
Order Closed-Loop Progression Tracking (p. 131)	Feature	FEAT-SA-13
Order Cancellation & Modification Coordination (p. 131)	Feature	FEAT-SA-14
Receive Order Request (p. 131)	Function	HCM-FN-receive-order-request
Order Requisition Ingress (p. 132)	Service	HCM-SV-order-requisition-ingress
Resolve Order Destination (p. 132)	Function	HCM-FN-resolve-order-destination
Order Dispatch Service (p. 132)	Service	HCM-SV-order-dispatch-service
Manage Order Progression (p. 133)	Function	HCM-FN-manage-order-progression
Order Status & Tracking Query (p. 133)	Service	HCM-SV-order-status-tracking-query
Coordinate Order Modification / Cancellation (p. 133)	Function	HCM-FN-coordinate-order-modification-cancellation
Order Cancellation Service (p. 134)	Service	HCM-SV-order-cancellation-service
Associate Order Outcome (p. 134)	Function	HCM-FN-associate-order-outcome
Order Outcome Notification (p. 134)	Service	HCM-SV-order-outcome-notification
Closed-Loop Order Progression Process (p. 135)	Process	HCM-PR-closed-loop-order-progression-process

3.2.5.1. Order Request Ingestion

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-SA-11	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Order Administration (p. 129)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:303 (p. 361)	Linked RAD detail; exact locators in manifest.

Ingest clinical diagnostic, medication, and procedure order requests with validated ordering clinician context.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.5.2. Order Destination Resolution & Routing

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-SA-12	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Order Administration (p. 129)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:304 (p. 361)	Linked RAD detail; exact locators in manifest.

Resolve fulfillment endpoints and reliably route orders to performing service systems.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.5.3. Order Closed-Loop Progression Tracking

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SA-13`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Order Administration (p. 129)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:305 (p. 361)	Linked RAD detail; exact locators in manifest.

Track order progression across destination determination, routing, delivery, receipt acknowledgement (distinguishing technical delivery acknowledgement from business acknowledgement), execution progress, completion, and outcome association.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.5.4. Order Cancellation & Modification Coordination

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SA-14`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Order Administration (p. 129)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:306 (p. 361)	Linked RAD detail; exact locators in manifest.

Coordinate order cancellation and amendment requests, ensuring synchronization with performing systems.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.5.5. Receive Order Request

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-receive-order-request`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Order Administration (p. 129)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:89 (p. 433)	Linked RAD detail; exact locators in manifest.

Ingests structured orders containing clinical indication, requesting clinician, and test specifications.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.5.6. Order Requisition Ingress

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-order-requisition-ingress`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Order Administration (p. 129)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:90 (p. 433)	Linked RAD detail; exact locators in manifest.

Ingests electronic orders from clinical ordering systems.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.5.7. Resolve Order Destination

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-resolve-order-destination`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Order Administration (p. 129)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:92 (p. 433)	Linked RAD detail; exact locators in manifest.

Evaluates order routing rules to determine target laboratory, imaging centre, or procedural unit.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.5.8. Order Dispatch Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-order-dispatch-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Order Administration (p. 129)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:93 (p. 433)	Linked RAD detail; exact locators in manifest.

Transmits orders to performing diagnostic systems.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.5.9. Manage Order Progression

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-manage-order-progression`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Order Administration (p. 129)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:95 (p. 433)	Linked RAD detail; exact locators in manifest.

Tracks order state (**scoped diagnostic/procedural illustration**): *Placed → Received → Specimen Collected → In-Progress → Preliminary Result → Final Result → Closed*.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.5.10. Order Status & Tracking Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-order-status-tracking-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Order Administration (p. 129)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:96 (p. 433)	Linked RAD detail; exact locators in manifest.

Provides real-time order tracking to ordering clinicians.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.5.11. Coordinate Order Modification / Cancellation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-coordinate-order-modification-cancellation`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Order Administration (p. 129)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:100 (p. 433)	Linked RAD detail; exact locators in manifest.

Handles requests to amend clinical details or cancel unexecuted orders.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.5.12. Order Cancellation Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-order-cancellation-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Order Administration (p. 129)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:101 (p. 433)	Linked RAD detail; exact locators in manifest.

Dispatches cancel directives to performing systems.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.5.13. Associate Order Outcome

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-associate-order-outcome`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Order Administration (p. 129)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:103 (p. 433)	Linked RAD detail; exact locators in manifest.

Correlates returning diagnostic reports and observations back to the originating order requisition.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.2.5.14. Order Outcome Notification

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-order-outcome-notification`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Order Administration (p. 129)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:104 (p. 433)	Linked RAD detail; exact locators in manifest.

Emits completion notices to ordering practitioners.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.2.5.15. Closed-Loop Order Progression Process

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-PR-closed-loop-order-progression-process`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Order Administration (p. 129)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	processes:135 (p. 407); processes:136 (p. 407)	Linked RAD detail; exact locators in manifest.

- **Process Purpose**: Governs the rigorous, closed-loop tracking of diagnostic pathology, radiology, and procedural orders from requisition to result correlation, preventing dropped or unfulfilled investigations.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.2.6. Diagnostic Administration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-diagnostic-administration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Core	Independent of Architectural Status.
Source	enabling:246 (p. 359); enabling:308 (p. 361); sa:110 (p. 434)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Diagnostic Request Correlation (p. 136)	Feature	`FEAT-SA-15`
Diagnostic Report Ingestion & Binding (p. 136)	Feature	`FEAT-SA-16`
Diagnostic Report Distribution (p. 136)	Feature	`FEAT-SA-17`
Correlate Diagnostic Request (p. 137)	Function	`HCM-FN-correlate-diagnostic-request`
Diagnostic Correlation Resolution (p. 137)	Service	`HCM-SV-diagnostic-correlation-resolution`
Bind Diagnostic Report (p. 137)	Function	`HCM-FN-bind-diagnostic-report`
Diagnostic Report Ingress (p. 138)	Service	`HCM-SV-diagnostic-report-ingress`
Distribute Diagnostic Report (p. 138)	Function	`HCM-FN-distribute-diagnostic-report`
Diagnostic Result Delivery Service (p. 138)	Service	`HCM-SV-diagnostic-result-delivery-service`

3.2.6.1. Diagnostic Request Correlation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-SA-15	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Diagnostic Administration (p. 135)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:310 (p. 361)	Linked RAD detail; exact locators in manifest.

Correlate diagnostic test orders with collected clinical specimens and patient encounters.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.6.2. Diagnostic Report Ingestion & Binding

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-SA-16	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Diagnostic Administration (p. 135)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:311 (p. 361)	Linked RAD detail; exact locators in manifest.

Ingest finalized laboratory and radiology reports, verifying content integrity and binding them to the parent order.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.6.3. Diagnostic Report Distribution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-SA-17	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Diagnostic Administration (p. 135)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:312 (p. 361)	Linked RAD detail; exact locators in manifest.

Distribute finalized laboratory, pathology, and imaging reports to ordering clinicians and designated care teams.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.6.4. Correlate Diagnostic Request

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-correlate-diagnostic-request`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Diagnostic Administration (p. 135)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:116 (p. 434)	Linked RAD detail; exact locators in manifest.

Matches incoming lab/radiology results to active patient encounters and order requisitions.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.6.5. Diagnostic Correlation Resolution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-diagnostic-correlation-resolution`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Diagnostic Administration (p. 135)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:117 (p. 434)	Linked RAD detail; exact locators in manifest.

Discloses correlated order/result linkages.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.6.6. Bind Diagnostic Report

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-bind-diagnostic-report`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Diagnostic Administration (p. 135)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:119 (p. 434)	Linked RAD detail; exact locators in manifest.

Associates structured observations, reference ranges, and abnormal flags with patient health records.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.6.7. Diagnostic Report Ingress

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-diagnostic-report-ingress`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Diagnostic Administration (p. 135)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:120 (p. 434)	Linked RAD detail; exact locators in manifest.

Ingests results from external LIS/RIS systems.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.6.8. Distribute Diagnostic Report

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-distribute-diagnostic-report`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Diagnostic Administration (p. 135)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:122 (p. 434)	Linked RAD detail; exact locators in manifest.

Delivers preliminary, final, and corrected reports to ordering and copied practitioners.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.6.9. Diagnostic Result Delivery Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-diagnostic-result-delivery-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Diagnostic Administration (p. 135)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:124 (p. 434)	Linked RAD detail; exact locators in manifest.

Emits secure result notifications.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.7. Medication Administration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-medication-administration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.
Source	enabling:247 (p. 359); enabling:316 (p. 361); sa:130 (p. 434)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Medication Order Ingestion (p. 139)	Feature	`FEAT-SA-18`
Dispense Event Tracking (p. 140)	Feature	`FEAT-SA-19`
Medication Administration Record (MAR) Ingestion (p. 140)	Feature	`FEAT-SA-20`
Receive Medication Order (p. 140)	Function	`HCM-FN-receive-medication-order`
Medication Order Ingress (p. 141)	Service	`HCM-SV-medication-order-ingress`
Track Dispense Event (p. 141)	Function	`HCM-FN-track-dispense-event`
Dispense Event Ingress (p. 141)	Service	`HCM-SV-dispense-event-ingress`
Receive Medication Administration Event (p. 142)	Function	`HCM-FN-receive-medication-administration-event`
Medication Administration Ingress (p. 142)	Service	`HCM-SV-medication-administration-ingress`

3.2.7.1. Medication Order Ingestion

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SA-18`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Medication Administration (p. 139)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:318 (p. 361)	Linked RAD detail; exact locators in manifest.

Ingest inpatient and discharge prescription orders.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.7.2. Dispense Event Tracking

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SA-19`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Medication Administration (p. 139)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:319 (p. 361)	Linked RAD detail; exact locators in manifest.

Ingest medication dispense verification records from pharmacy management systems.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.7.3. Medication Administration Record (MAR) Ingestion

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SA-20`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Medication Administration (p. 139)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:320 (p. 361)	Linked RAD detail; exact locators in manifest.

Ingest point-of-care medication administration events.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.7.4. Receive Medication Order

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-receive-medication-order`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Medication Administration (p. 139)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:136 (p. 434)	Linked RAD detail; exact locators in manifest.

Ingests inpatient and outpatient drug prescriptions.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.7.5. Medication Order Ingress

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-medication-order-ingress`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Medication Administration (p. 139)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:137 (p. 434)	Linked RAD detail; exact locators in manifest.

Ingests prescription events.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.7.6. Track Dispense Event

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-track-dispense-event`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Medication Administration (p. 139)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:139 (p. 434)	Linked RAD detail; exact locators in manifest.

Ingests community and hospital pharmacy dispensing records.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.7.7. Dispense Event Ingress

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-dispense-event-ingress`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Medication Administration (p. 139)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:140 (p. 434)	Linked RAD detail; exact locators in manifest.

Ingests pharmacy supply notifications.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.7.8. Receive Medication Administration Event

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-receive-medication-administration-event`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Medication Administration (p. 139)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:142 (p. 434)	Linked RAD detail; exact locators in manifest.

Records medication administration-event information, including nurse-administered doses, e-MAR documentation and infusion telemetry as non-exhaustive examples; scope is not limited to nursing, bedside or inpatient settings.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.7.9. Medication Administration Ingress

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-medication-administration-ingress`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Medication Administration (p. 139)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:143 (p. 434)	Linked RAD detail; exact locators in manifest.

Ingests administration records.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.8. Procedure Administration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-procedure-administration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.
Source	enabling:248 (p. 359); enabling:322 (p. 361); sa:149 (p. 435)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Procedure Booking Ingestion (p. 143)	Feature	FEAT-SA-21
Procedural Documentation Ingestion (p. 143)	Feature	FEAT-SA-22
Receive Procedure Booking / Request (p. 143)	Function	HCM-FN-receive-procedure-booking-request
Procedure Booking Ingress (p. 144)	Service	HCM-SV-procedure-booking-ingress
Receive Procedural Documentation (p. 144)	Function	HCM-FN-receive-procedural-documentation
Procedural Documentation Ingress (p. 144)	Service	HCM-SV-procedural-documentation-ingress

3.2.8.1. Procedure Booking Ingestion

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-SA-21	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Procedure Administration (p. 142)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:324 (p. 361)	Linked RAD detail; exact locators in manifest.

Ingest scheduled surgical case bookings and procedural requests.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.8.2. Procedural Documentation Ingestion

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-SA-22	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Procedure Administration (p. 142)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:325 (p. 361)	Linked RAD detail; exact locators in manifest.

Ingest operative reports, anesthesia records, and post-procedure summaries.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.8.3. Receive Procedure Booking / Request

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	HCM-FN-receive-procedure-booking-request	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Procedure Administration (p. 142)	Responsibility; no invented intermediate element.

Name	Value	Comment / Description
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:153 (p. 435)	Linked RAD detail; exact locators in manifest.

Ingests procedural and surgical requisitions.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.8.4. Procedure Booking Ingress

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-procedure-booking-ingress`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Procedure Administration (p. 142)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:154 (p. 435)	Linked RAD detail; exact locators in manifest.

Receives surgical/procedural requests.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.8.5. Receive Procedural Documentation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-receive-procedural-documentation`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Procedure Administration (p. 142)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:156 (p. 435)	Linked RAD detail; exact locators in manifest.

Ingests operation reports, anaesthetic records, and post-procedure notes.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.8.6. Procedural Documentation Ingress

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-procedural-documentation-ingress`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Procedure Administration (p. 142)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:157 (p. 435)	Linked RAD detail; exact locators in manifest.

Ingests procedural documentation, including surgical notes, into patient record streams.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.9. Clinical Record Administration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-clinical-record-administration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Core	Independent of Architectural Status.
Source	enabling:249 (p. 359); enabling:327 (p. 361); sa:163 (p. 435)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Clinical Document Ingestion (p. 145)	Feature	`FEAT-SA-23`
Document Versioning & Supersession (p. 146)	Feature	`FEAT-SA-24`
Document Metadata Indexing (p. 146)	Feature	`FEAT-SA-25`
Receive Clinical Document (p. 146)	Function	`HCM-FN-receive-clinical-document`
Clinical Document Ingress (p. 147)	Service	`HCM-SV-clinical-document-ingress`
Govern Clinical Document Lifecycle (p. 147)	Function	`HCM-FN-govern-clinical-document-lifecycle`
Clinical Document Lifecycle Service (p. 147)	Service	`HCM-SV-clinical-document-lifecycle-service`
Index Clinical Document Metadata (p. 148)	Function	`HCM-FN-index-clinical-document-metadata`
Document Metadata Registry Query (p. 148)	Service	`HCM-SV-document-metadata-registry-query`
Clinical Document Lifecycle Process (p. 148)	Process	`HCM-PR-clinical-document-lifecycle-process`

3.2.9.1. Clinical Document Ingestion

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SA-23`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Clinical Record Administration (p. 145)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.

Name	Value	Comment / Description
Source	enabling:329 (p. 362)	Linked RAD detail; exact locators in manifest.

Ingest structured and unstructured clinical documents (discharge summaries, consult notes, letters).

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.9.2. Document Versioning & Supersession

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SA-24`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Clinical Record Administration (p. 145)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:330 (p. 362)	Linked RAD detail; exact locators in manifest.

Manage document classification, author attribution, amendment attribution, versioning, and supersession or withdrawal without destructive data loss.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.9.3. Document Metadata Indexing

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SA-25`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Clinical Record Administration (p. 145)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:331 (p. 362)	Linked RAD detail; exact locators in manifest.

Index clinical document metadata (author, specialty, encounter, date, confidentiality) for rapid discovery.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.9.4. Receive Clinical Document

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-receive-clinical-document`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinical Record Administration (p. 145)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.

Name	Value	Comment / Description
Source	sa:168 (p. 435)	Linked RAD detail; exact locators in manifest.

Receives clinical documents from information suppliers.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.9.5. Clinical Document Ingress

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-clinical-document-ingress`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinical Record Administration (p. 145)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:169 (p. 435)	Linked RAD detail; exact locators in manifest.

Exposes document submission endpoints across the HIE.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.9.6. Govern Clinical Document Lifecycle

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-govern-clinical-document-lifecycle`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinical Record Administration (p. 145)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:171 (p. 435)	Linked RAD detail; exact locators in manifest.

Manages document states (**scoped illustration, not a universal transition model**): *Draft → Preliminary → Final → Amended → Superseded → Entered-in-Error*.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.9.7. Clinical Document Lifecycle Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-clinical-document-lifecycle-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinical Record Administration (p. 145)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:172 (p. 435)	Linked RAD detail; exact locators in manifest.

Allows authoring clinicians to publish addenda or corrections.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.9.8. Index Clinical Document Metadata

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-index-clinical-document-metadata`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinical Record Administration (p. 145)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:176 (p. 435)	Linked RAD detail; exact locators in manifest.

Indexes author, specialty, encounter, date, and document type (LOINC/SNOMED).

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.9.9. Document Metadata Registry Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-document-metadata-registry-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinical Record Administration (p. 145)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:177 (p. 435)	Linked RAD detail; exact locators in manifest.

Discloses document registry metadata to clinical viewers.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.9.10. Clinical Document Lifecycle Process

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-PR-clinical-document-lifecycle-process`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinical Record Administration (p. 145)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	processes:152 (p. 407); processes:153 (p. 408)	Linked RAD detail; exact locators in manifest.

- **Process Purpose**: Governs the versioning, clinical sign-off, addenda, superseding, and legal status of clinical documents (discharge summaries, specialist letters, advance care directives).

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.2.10. Care Plan Administration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-care-plan-administration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.
Source	enabling:250 (p. 359); enabling:333 (p. 362); sa:183 (p. 436)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Care Plan Ingestion & Distribution (p. 149)	Feature	`FEAT-SA-26`
Receive Care Plan (p. 150)	Function	`HCM-FN-receive-care-plan`
Care Plan Ingress (p. 150)	Service	`HCM-SV-care-plan-ingress`
Associate Care Plan Context (p. 150)	Function	`HCM-FN-associate-care-plan-context`
Care Plan Context Query (p. 151)	Service	`HCM-SV-care-plan-context-query`
Distribute Care Plan Change (p. 151)	Function	`HCM-FN-distribute-care-plan-change`
Care Plan Change Notification (p. 151)	Service	`HCM-SV-care-plan-change-notification`

3.2.10.1. Care Plan Ingestion & Distribution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SA-26`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Care Plan Administration (p. 149)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:335 (p. 362)	Linked RAD detail; exact locators in manifest.

Ingest multidisciplinary care plans and syndicate updates to authorized care team members.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.10.2. Receive Care Plan

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-receive-care-plan`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Care Plan Administration (p. 149)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:186 (p. 436)	Linked RAD detail; exact locators in manifest.

Ingests multidisciplinary care plans, clinical goals, and patient action steps.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.10.3. Care Plan Ingress

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-care-plan-ingress`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Care Plan Administration (p. 149)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:187 (p. 436)	Linked RAD detail; exact locators in manifest.

Ingests shared care plans.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.10.4. Associate Care Plan Context

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-associate-care-plan-context`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Care Plan Administration (p. 149)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:189 (p. 436)	Linked RAD detail; exact locators in manifest.

Links care plans to active diagnoses, health problems, and care team members.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.2.10.5. Care Plan Context Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-care-plan-context-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Care Plan Administration (p. 149)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:190 (p. 436)	Linked RAD detail; exact locators in manifest.

Discloses active care plans to treating clinicians.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.2.10.6. Distribute Care Plan Change

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-distribute-care-plan-change`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Care Plan Administration (p. 149)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:192 (p. 436)	Linked RAD detail; exact locators in manifest.

Broadcasts care plan updates to all enrolled care team participants.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.2.10.7. Care Plan Change Notification

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-care-plan-change-notification`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Care Plan Administration (p. 149)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:193 (p. 436)	Linked RAD detail; exact locators in manifest.

Emits care plan revision notices.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.2.11. Clinical Communication Administration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-clinical-communication-administration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Core	Independent of Architectural Status.
Source	enabling:251 (p. 359); enabling:337 (p. 362); sa:199 (p. 436)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Clinical Communication Distribution (p. 152)	Feature	`FEAT-SA-27`
Delivery Acknowledgement Tracking (p. 153)	Feature	`FEAT-SA-28`
Communication Audit Logging (p. 153)	Feature	`FEAT-SA-29`
Distribute Clinical Communication (p. 153)	Function	`HCM-FN-distribute-clinical-communication`
Secure Clinical Message Dispatch (p. 154)	Service	`HCM-SV-secure-clinical-message-dispatch`
Track Business Delivery Acknowledgement (p. 154)	Function	`HCM-FN-track-business-delivery-acknowledgement`
Communication Delivery Status Query (p. 154)	Service	`HCM-SV-communication-delivery-status-query`
Record Communication Transaction (p. 155)	Function	`HCM-FN-record-communication-transaction`
Communication Audit Query (p. 155)	Service	`HCM-SV-communication-audit-query`

3.2.11.1. Clinical Communication Distribution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SA-27`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Clinical Communication Administration (p. 152)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:339 (p. 362)	Linked RAD detail; exact locators in manifest.

Distribute secure clinical communications between verified healthcare providers and designated care team endpoints.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.11.2. Delivery Acknowledgement Tracking

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SA-28`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Clinical Communication Administration (p. 152)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:340 (p. 362)	Linked RAD detail; exact locators in manifest.

Correlate recipient delivery and read receipts with outbound clinical communications, distinguishing transport delivery from recipient acknowledgement. Receipt/read evidence does not by itself establish business acceptance, clinical review, incorporation, approval, finality or originating authority.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.11.3. Communication Audit Logging

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SA-29`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Clinical Communication Administration (p. 152)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:341 (p. 362)	Linked RAD detail; exact locators in manifest.

Maintain an enduring, non-PHI, tamper-evident log of all clinical communication transactions.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.11.4. Distribute Clinical Communication

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-distribute-clinical-communication`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinical Communication Administration (p. 152)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:206 (p. 436)	Linked RAD detail; exact locators in manifest.

Routes secure messages, referral letters, and critical alerts to practitioner endpoints.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.11.5. Secure Clinical Message Dispatch

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-secure-clinical-message-dispatch`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinical Communication Administration (p. 152)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:207 (p. 436)	Linked RAD detail; exact locators in manifest.

Exposes message dispatch to clinical authors.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.11.6. Track Business Delivery Acknowledgement

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-track-business-delivery-acknowledgement`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinical Communication Administration (p. 152)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:209 (p. 436)	Linked RAD detail; exact locators in manifest.

Tracks end-to-end delivery state and records business-level delivery/rejection acknowledgements.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.11.7. Communication Delivery Status Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-communication-delivery-status-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinical Communication Administration (p. 152)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:211 (p. 436)	Linked RAD detail; exact locators in manifest.

Discloses delivery confirmation to message senders.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.11.8. Record Communication Transaction

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-record-communication-transaction`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinical Communication Administration (p. 152)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:213 (p. 436)	Linked RAD detail; exact locators in manifest.

Maintains an immutable audit trail of clinical message dispatch and receipt.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.11.9. Communication Audit Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-communication-audit-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinical Communication Administration (p. 152)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sa:214 (p. 436)	Linked RAD detail; exact locators in manifest.

Discloses transmission history for medico-legal verification.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.2.12. Funding & Coverage Administration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-funding-coverage-administration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Adjacent	Independent of Architectural Status.
Source	enabling:252 (p. 359); enabling:343 (p. 362)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

3.2.13. Clinical Charging & Claims Administration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-clinical-charging-claims-administration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Adjacent	Independent of Architectural Status.
Source	enabling:253 (p. 359); enabling:346 (p. 362)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

3.3. Service Delivery

Capability	Reference Id
Clinical Assessment (p. 156)	`HCM-BEC-clinical-assessment`
Primary Care (p. 157)	`HCM-BEC-primary-care`
Acute Care (p. 159)	`HCM-BEC-acute-care`
Emergency Care (p. 162)	`HCM-BEC-emergency-care`
Inpatient Care (p. 165)	`HCM-BEC-inpatient-care`
Ambulatory Care (p. 167)	`HCM-BEC-ambulatory-care`
Surgical & Procedural Care (p. 168)	`HCM-BEC-surgical-procedural-care`
Diagnostic Services (p. 168)	`HCM-BEC-diagnostic-services`
Therapeutic Services (p. 171)	`HCM-BEC-therapeutic-services`
Medication Therapy (p. 171)	`HCM-BEC-medication-therapy`
Allied Health (p. 174)	`HCM-BEC-allied-health`
Preventive Care (p. 174)	`HCM-BEC-preventive-care`
Rehabilitation (p. 177)	`HCM-BEC-rehabilitation`
Community Care (p. 177)	`HCM-BEC-community-care`
Outreach Care (p. 180)	`HCM-BEC-outreach-care`
Remote & Virtual Care (p. 182)	`HCM-BEC-remote-virtual-care`

3.3.1. Clinical Assessment

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-clinical-assessment`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Adjacent	Independent of Architectural Status.

Name	Value	Comment / Description
Source	enabling:361 (p. 362)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

3.3.2. Primary Care

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-primary-care`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.
Source	enabling:362 (p. 362); enabling:389 (p. 363); sd:51 (p. 439)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
General Practice Shared Record Ingestion (p. 157)	Feature	`FEAT-SD-01`
Primary Care Notification Dispatch (p. 158)	Feature	`FEAT-SD-02`
Correlate Primary Care Consultation Context (p. 158)	Function	`HCM-FN-correlate-primary-care-consultation-context`
Primary Care Summary Ingress (p. 158)	Service	`HCM-SV-primary-care-summary-ingress`
Coordinate Multi-Agency Chronic Care Plan (p. 159)	Function	`HCM-FN-coordinate-multi-agency-chronic-care-plan`
Chronic Care Timeline Query (p. 159)	Service	`HCM-SV-chronic-care-timeline-query`

3.3.2.1. General Practice Shared Record Ingestion

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SD-01`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Primary Care (p. 157)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:391 (p. 363)	Linked RAD detail; exact locators in manifest.

Ingest shared health summaries, encounter notes, and immunization records from primary care practice management systems.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.2.2. Primary Care Notification Dispatch

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SD-02`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Primary Care (p. 157)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:392 (p. 363)	Linked RAD detail; exact locators in manifest.

Dispatch emergency department arrival, acute admission, and discharge notifications to the patient's registered general practitioner.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.2.3. Correlate Primary Care Consultation Context

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-correlate-primary-care-consultation-context`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Primary Care (p. 157)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sd:55 (p. 439)	Linked RAD detail; exact locators in manifest.

Binds GP consultation notes, vital signs, and encounter episodes to the regional Longitudinal Health Record (LHR).

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.2.4. Primary Care Summary Ingress

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-primary-care-summary-ingress`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Primary Care (p. 157)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sd:56 (p. 439)	Linked RAD detail; exact locators in manifest.

Ingests GP event summaries into the HIE.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.2.5. Coordinate Multi-Agency Chronic Care Plan

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-coordinate-multi-agency-chronic-care-plan`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Primary Care (p. 157)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sd:59 (p. 439)	Linked RAD detail; exact locators in manifest.

Enables GPs to track multi-provider disease management milestones (e.g., Diabetes or Cardiovascular Care Plans).

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.3.2.6. Chronic Care Timeline Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-chronic-care-timeline-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Primary Care (p. 157)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sd:60 (p. 439)	Linked RAD detail; exact locators in manifest.

Exposes aggregated multidisciplinary care timelines.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.3.3. Acute Care

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-acute-care`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.
Source	enabling:363 (p. 362); enabling:394 (p. 363); sd:71 (p. 440)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Acute Clinical State Event Capture (p. 160)	Feature	`FEAT-SD-03`
Acute Care Clinical Context Provision (p. 160)	Feature	`FEAT-SD-04`
Assemble Acute Clinical Baseline (p. 161)	Function	`HCM-FN-assemble-acute-clinical-baseline`
Acute Admission Dossier Query (p. 161)	Service	`HCM-SV-acute-admission-dossier-query`
Route Acute Critical Result Alert (p. 161)	Function	`HCM-FN-route-acute-critical-result-alert`
Acute Clinical Alert Dispatch (p. 162)	Service	`HCM-SV-acute-clinical-alert-dispatch`

3.3.3.1. Acute Clinical State Event Capture

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SD-03`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Acute Care (p. 159)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:396 (p. 363)	Linked RAD detail; exact locators in manifest.

Ingest clinical deterioration notifications and acute clinical status updates.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.3.2. Acute Care Clinical Context Provision

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SD-04`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Acute Care (p. 159)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:397 (p. 363)	Linked RAD detail; exact locators in manifest.

Provide consolidated longitudinal history and active medication lists to acute care clinicians at point of triage.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.3.3. Assemble Acute Clinical Baseline

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-assemble-acute-clinical-baseline`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Acute Care (p. 159)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sd:75 (p. 440)	Linked RAD detail; exact locators in manifest.

Rapidly aggregates previous discharge summaries, active medications, allergies, and diagnostic alerts upon acute hospital presentation.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.3.4. Acute Admission Dossier Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-acute-admission-dossier-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Acute Care (p. 159)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sd:76 (p. 440)	Linked RAD detail; exact locators in manifest.

Delivers a high-priority clinical briefing to admitting teams.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.3.5. Route Acute Critical Result Alert

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-route-acute-critical-result-alert`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Acute Care (p. 159)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sd:79 (p. 440)	Linked RAD detail; exact locators in manifest.

Immediately directs urgent lab/radiology alerts (e.g., critical troponin, intracranial bleed) to active acute attending clinicians.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.3.3.6. Acute Clinical Alert Dispatch

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-acute-clinical-alert-dispatch`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Acute Care (p. 159)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sd:80 (p. 440)	Linked RAD detail; exact locators in manifest.

Dispatches urgent interruptive alerts to on-duty ward teams.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.3.4. Emergency Care

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-emergency-care`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.
Source	enabling:364 (p. 362); enabling:399 (p. 363); sd:91 (p. 440)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Emergency Department Arrival & Triage Ingestion (p. 163)	Feature	`FEAT-SD-05`
Emergency Care Cross-Facility Correlation (p. 163)	Feature	`FEAT-SD-06`
Bind ED Triage Assessment (p. 163)	Function	`HCM-FN-bind-ed-triage-assessment`
ED Triage Stream Ingress (p. 164)	Service	`HCM-SV-ed-triage-stream-ingress`
Provide Emergency LHR Summary (p. 164)	Function	`HCM-FN-provide-emergency-lhr-summary`
Emergency Fast-Path LHR Query (p. 164)	Service	`HCM-SV-emergency-fast-path-lhr-query`

3.3.4.1. Emergency Department Arrival & Triage Ingestion

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SD-05`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Emergency Care (p. 162)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:401 (p. 364)	Linked RAD detail; exact locators in manifest.

Ingest emergency presentation, triage category, and chief complaint records.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.4.2. Emergency Care Cross-Facility Correlation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SD-06`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Emergency Care (p. 162)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:402 (p. 364)	Linked RAD detail; exact locators in manifest.

Correlate incoming emergency presentations with recent regional admissions, prior discharge summaries, and known allergy profiles.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.4.3. Bind ED Triage Assessment

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-bind-ed-triage-assessment`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Emergency Care (p. 162)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sd:95 (p. 440)	Linked RAD detail; exact locators in manifest.

Captures ATS (Australasian Triage Scale) category, presenting complaint, and ambulance paramedic handover data.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.4.4. ED Triage Stream Ingress

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-ed-triage-stream-ingress`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Emergency Care (p. 162)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sd:96 (p. 440)	Linked RAD detail; exact locators in manifest.

Ingests emergency triage presentations.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.4.5. Provide Emergency LHR Summary

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-provide-emergency-lhr-summary`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Emergency Care (p. 162)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sd:98 (p. 440)	Linked RAD detail; exact locators in manifest.

Delivers rapid, override-capable access to Advance Care Directives, allergy records, and recent hospital admissions.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.4.6. Emergency Fast-Path LHR Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-emergency-fast-path-lhr-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Emergency Care (p. 162)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sd:99 (p. 440)	Linked RAD detail; exact locators in manifest.

Provides low-latency emergency record summaries.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.5. Inpatient Care

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-inpatient-care`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.
Source	enabling:365 (p. 362); enabling:404 (p. 364); sd:110 (p. 441)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Inpatient Rounding Context Provision (p. 165)	Feature	`FEAT-SD-07`
Inpatient Clinical Progression Tracking (p. 166)	Feature	`FEAT-SD-08`
Consolidate Inpatient Clinical Round Feed (p. 166)	Function	`HCM-FN-consolidate-inpatient-clinical-round-feed`
Ward Round Clinical Briefing Query (p. 166)	Service	`HCM-SV-ward-round-clinical-briefing-query`
Assemble Inpatient Discharge Dossier (p. 167)	Function	`HCM-FN-assemble-inpatient-discharge-dossier`
Discharge Dossier Packaging Service (p. 167)	Service	`HCM-SV-discharge-dossier-packaging-service`

3.3.5.1. Inpatient Rounding Context Provision

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SD-07`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Inpatient Care (p. 165)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:406 (p. 364)	Linked RAD detail; exact locators in manifest.

Aggregate diagnostic results, current medication administration records, and nursing notes for multidisciplinary inpatient rounds.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.5.2. Inpatient Clinical Progression Tracking

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SD-08`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Inpatient Care (p. 165)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:407 (p. 364)	Linked RAD detail; exact locators in manifest.

Capture inpatient clinical milestone completions (e.g., medical stability achieved, allied health clearance).

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.5.3. Consolidate Inpatient Clinical Round Feed

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-consolidate-inpatient-clinical-round-feed`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Inpatient Care (p. 165)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sd:114 (p. 441)	Linked RAD detail; exact locators in manifest.

Aggregates daily vital signs, overnight pathology, allied health notes, and medication administration records.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.5.4. Ward Round Clinical Briefing Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-ward-round-clinical-briefing-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Inpatient Care (p. 165)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sd:115 (p. 441)	Linked RAD detail; exact locators in manifest.

Discloses consolidated 24-hour clinical briefing views.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.5.5. Assemble Inpatient Discharge Dossier

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-assemble-inpatient-discharge-dossier`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Inpatient Care (p. 165)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sd:118 (p. 441)	Linked RAD detail; exact locators in manifest.

Synthesises inpatient diagnoses, procedural summaries, reconciled discharge medications, and follow-up instructions into a canonical discharge summary package.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.3.5.6. Discharge Dossier Packaging Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-discharge-dossier-packaging-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Inpatient Care (p. 165)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sd:119 (p. 441)	Linked RAD detail; exact locators in manifest.

Emits canonical discharge summaries to GPs and community providers.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.3.6. Ambulatory Care

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-ambulatory-care`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Adjacent	Independent of Architectural Status.
Source	enabling:366 (p. 362)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

3.3.7. Surgical & Procedural Care

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-surgical-procedural-care`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Adjacent	Independent of Architectural Status.
Source	enabling:367 (p. 362)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

3.3.8. Diagnostic Services

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-diagnostic-services`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.
Source	enabling:368 (p. 362); enabling:409 (p. 364); sd:129 (p. 441)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Laboratory & Imaging Result Distribution (p. 169)	Feature	`FEAT-SD-09`
Diagnostic History Consolidation (p. 169)	Feature	`FEAT-SD-10`
Receive Diagnostic Requisition Feed (p. 169)	Function	`HCM-FN-receive-diagnostic-requisition-feed`
Modality Worklist Distribution (p. 170)	Service	`HCM-SV-modality-worklist-distribution`
Publish Governed Diagnostic Report (p. 170)	Function	`HCM-FN-publish-governed-diagnostic-report`
Diagnostic Publication Service (p. 170)	Service	`HCM-SV-diagnostic-publication-service`

3.3.8.1. Laboratory & Imaging Result Distribution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SD-09`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Diagnostic Services (p. 168)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:411 (p. 364)	Linked RAD detail; exact locators in manifest.

Reliably distribute structured diagnostic reports (pathology, radiology) to ordering clinicians and designated secondary recipients.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.8.2. Diagnostic History Consolidation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SD-10`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Diagnostic Services (p. 168)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:412 (p. 364)	Linked RAD detail; exact locators in manifest.

Consolidate historical diagnostic trends (e.g., cumulative laboratory results) across multiple regional providers into an integrated view.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.8.3. Receive Diagnostic Requisition Feed

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-receive-diagnostic-requisition-feed`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Diagnostic Services (p. 168)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sd:134 (p. 441)	Linked RAD detail; exact locators in manifest.

Normalises order requisitions from across the health network into modality worklists.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.3.8.4. Modality Worklist Distribution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-modality-worklist-distribution`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Diagnostic Services (p. 168)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sd:135 (p. 441)	Linked RAD detail; exact locators in manifest.

Supplies order worklists to diagnostic laboratory/imaging systems.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.3.8.5. Publish Governed Diagnostic Report

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-publish-governed-diagnostic-report`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Diagnostic Services (p. 168)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sd:138 (p. 441)	Linked RAD detail; exact locators in manifest.

Normalises and distributes structured diagnostic observations, reference ranges, and diagnostic image study links into the LHR.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.3.8.6. Diagnostic Publication Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-diagnostic-publication-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Diagnostic Services (p. 168)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sd:139 (p. 441)	Linked RAD detail; exact locators in manifest.

Publishes verified diagnostic reports across the enterprise.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.3.9. Therapeutic Services

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-therapeutic-services`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Adjacent	Independent of Architectural Status.
Source	enabling:369 (p. 362)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

3.3.10. Medication Therapy

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-medication-therapy`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.
Source	enabling:370 (p. 362); enabling:414 (p. 364); sd:147 (p. 442)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Medication History Reconciliation Ingestion (p. 172)	Feature	`FEAT-SD-11`
Medication Administration Tracking (p. 172)	Feature	`FEAT-SD-12`
Project Longitudinal Medication History (p. 172)	Function	`HCM-FN-project-longitudinal-medication-history`
Longitudinal Medication History Query (p. 173)	Service	`HCM-SV-longitudinal-medication-history-query`
Correlate Adverse Reaction & Allergy Profile (p. 173)	Function	`HCM-FN-correlate-adverse-reaction-allergy-profile`
Allergy & Adverse Reaction Query (p. 173)	Service	`HCM-SV-allergy-adverse-reaction-query`

3.3.10.1. Medication History Reconciliation Ingestion

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SD-11`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Medication Therapy (p. 171)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:416 (p. 364)	Linked RAD detail; exact locators in manifest.

Ingest reconciled medication histories compiled during clinical transitions of care.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.10.2. Medication Administration Tracking

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SD-12`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Medication Therapy (p. 171)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:417 (p. 364)	Linked RAD detail; exact locators in manifest.

Distribute medication therapy orders, dispensing updates, and administration records across clinical care boundaries.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.10.3. Project Longitudinal Medication History

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-project-longitudinal-medication-history`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Medication Therapy (p. 171)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sd:151 (p. 442)	Linked RAD detail; exact locators in manifest.

Aggregates community dispensing, hospital e-MAR administrations, and specialist prescriptions into a unified chronological medication profile.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.10.4. Longitudinal Medication History Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-longitudinal-medication-history-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Medication Therapy (p. 171)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sd:152 (p. 442)	Linked RAD detail; exact locators in manifest.

Discloses full medication timelines for medication reconciliation.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.10.5. Correlate Adverse Reaction & Allergy Profile

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-correlate-adverse-reaction-allergy-profile`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Medication Therapy (p. 171)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sd:155 (p. 442)	Linked RAD detail; exact locators in manifest.

Binds confirmed substance allergies, adverse drug reactions, and severity grades to the patient identity.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.3.10.6. Allergy & Adverse Reaction Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-allergy-adverse-reaction-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Medication Therapy (p. 171)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sd:156 (p. 442)	Linked RAD detail; exact locators in manifest.

Exposes active allergy profiles to clinical decision support systems.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.3.11. Allied Health

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-allied-health`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Adjacent	Independent of Architectural Status.
Source	enabling:371 (p. 362)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

3.3.12. Preventive Care

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-preventive-care`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.
Source	enabling:372 (p. 362); enabling:419 (p. 364); sd:164 (p. 442)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Immunisation Event Submission (p. 175)	Feature	`FEAT-SD-13`
Screening Recall Notification Distribution (p. 175)	Feature	`FEAT-SD-14`
Syndicate Immunisation Record (p. 175)	Function	`HCM-FN-syndicate-immunisation-record`
Immunisation History Query (p. 176)	Service	`HCM-SV-immunisation-history-query`
Project Screening Eligibility Timeline (p. 176)	Function	`HCM-FN-project-screening-eligibility-timeline`
Preventive Screening Status Query (p. 176)	Service	`HCM-SV-preventive-screening-status-query`

3.3.12.1. Immunisation Event Submission

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SD-13`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Preventive Care (p. 174)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:421 (p. 364)	Linked RAD detail; exact locators in manifest.

Ingest point-of-care immunisation administration records and syndicate to national immunization registers.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.12.2. Screening Recall Notification Distribution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SD-14`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Preventive Care (p. 174)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:422 (p. 364)	Linked RAD detail; exact locators in manifest.

Distribute clinical screening reminders and recall notices to designated healthcare providers.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.12.3. Syndicate Immunisation Record

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-syndicate-immunisation-record`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Preventive Care (p. 174)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sd:168 (p. 442)	Linked RAD detail; exact locators in manifest.

Records vaccine encounters and syndicates records to national immunisation registers (e.g., AIR).

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.12.4. Immunisation History Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-immunisation-history-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Preventive Care (p. 174)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sd:169 (p. 442)	Linked RAD detail; exact locators in manifest.

Discloses complete vaccination history.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.12.5. Project Screening Eligibility Timeline

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-project-screening-eligibility-timelin e`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Preventive Care (p. 174)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sd:172 (p. 442)	Linked RAD detail; exact locators in manifest.

Tracks age-based and risk-based screening schedules across primary care.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.3.12.6. Preventive Screening Status Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-preventive-screening-status-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Preventive Care (p. 174)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sd:173 (p. 442)	Linked RAD detail; exact locators in manifest.

Discloses screening due dates to primary care providers.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.3.13. Rehabilitation

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-rehabilitation`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Adjacent	Independent of Architectural Status.
Source	enabling:373 (p. 362)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

3.3.14. Community Care

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-community-care`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.
Source	enabling:374 (p. 362); enabling:424 (p. 364); sd:183 (p. 443)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Community Encounter Ingestion (p. 178)	Feature	`FEAT-SD-15`
Community Care Plan Synchronization (p. 178)	Feature	`FEAT-SD-16`
Synchronize Community Visit Record (p. 178)	Function	`HCM-FN-synchronize-community-visit-record`
Community Care Encounter Ingress (p. 179)	Service	`HCM-SV-community-care-encounter-ingress`
Bind Community Support Network Context (p. 179)	Function	`HCM-FN-bind-community-support-network-context`
Community Care Team Query (p. 179)	Service	`HCM-SV-community-care-team-query`

3.3.14.1. Community Encounter Ingestion

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SD-15`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Community Care (p. 177)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:426 (p. 364)	Linked RAD detail; exact locators in manifest.

Ingest home visit notes, community nursing observations, and social support assessments.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.14.2. Community Care Plan Synchronization

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SD-16`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Community Care (p. 177)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:427 (p. 364)	Linked RAD detail; exact locators in manifest.

Synchronize community care plan updates between hospital outreach teams and community health organizations.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.14.3. Synchronize Community Visit Record

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-synchronize-community-visit-record`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Community Care (p. 177)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sd:187 (p. 443)	Linked RAD detail; exact locators in manifest.

Ingests clinical notes, wound assessments, and vital signs recorded in community and home settings.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.14.4. Community Care Encounter Ingress

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-community-care-encounter-ingress`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Community Care (p. 177)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sd:188 (p. 443)	Linked RAD detail; exact locators in manifest.

Ingests community care documentation.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.14.5. Bind Community Support Network Context

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-bind-community-support-network-context`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Community Care (p. 177)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sd:191 (p. 443)	Linked RAD detail; exact locators in manifest.

Links community nursing teams with local non-clinical social care and home care packages.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.3.14.6. Community Care Team Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-community-care-team-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Community Care (p. 177)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sd:192 (p. 443)	Linked RAD detail; exact locators in manifest.

Exposes community provider contact rosters.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.3.15. Outreach Care

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-outreach-care`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.
Source	enabling:375 (p. 362); enabling:429 (p. 364); sd:202 (p. 443)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Outreach Visit Context Provision (p. 180)	Feature	`FEAT-SD-17`
Outreach Encounter Outcome Ingestion (p. 181)	Feature	`FEAT-SD-18`
Provide Outreach Visit Context (p. 181)	Function	`HCM-FN-provide-outreach-visit-context`
Reconcile Outreach Clinical Batch (p. 181)	Function	`HCM-FN-reconcile-outreach-clinical-batch`
Outreach Batch Reconciliation Service (p. 182)	Service	`HCM-SV-outreach-batch-reconciliation-service`

3.3.15.1. Outreach Visit Context Provision

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SD-17`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Outreach Care (p. 180)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:431 (p. 365)	Linked RAD detail; exact locators in manifest.

Package and deliver portable clinical summaries and offline-capable care summaries for remote outreach clinics.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.15.2. Outreach Encounter Outcome Ingestion

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SD-18`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Outreach Care (p. 180)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:432 (p. 365)	Linked RAD detail; exact locators in manifest.

Ingest and reconcile clinical documentation gathered during rural and remote outreach visits.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.15.3. Provide Outreach Visit Context

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-provide-outreach-visit-context`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Outreach Care (p. 180)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sd:206 (p. 443)	Linked RAD detail; exact locators in manifest.

Packages and delivers portable clinical summaries and offline-capable care summaries for remote outreach clinics.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.15.4. Reconcile Outreach Clinical Batch

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-reconcile-outreach-clinical-batch`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Outreach Care (p. 180)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sd:210 (p. 443)	Linked RAD detail; exact locators in manifest.

Reconciles and merges clinical documentation recorded during disconnected remote clinic operations.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.15.5. Outreach Batch Reconciliation Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-outreach-batch-reconciliation-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Outreach Care (p. 180)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sd:211 (p. 443)	Linked RAD detail; exact locators in manifest.

Reconciles batch clinical encounters upon network reconnection.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.16. Remote & Virtual Care

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-remote-virtual-care`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.
Source	enabling:376 (p. 362); enabling:434 (p. 365); sd:221 (p. 444)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Remote Telemetry Ingestion (p. 183)	Feature	`FEAT-SD-19`
Virtual Care Session Context Binding (p. 183)	Feature	`FEAT-SD-20`
Correlate Virtual Care Consultation Context (p. 183)	Function	`HCM-FN-correlate-virtual-care-consultation-context`
Virtual Encounter Context Resolution (p. 184)	Service	`HCM-SV-virtual-encounter-context-resolution`
Ingest Remote Patient Telemetry (p. 184)	Function	`HCM-FN-ingest-remote-patient-telemetry`
Remote Telemetry Ingress (p. 184)	Service	`HCM-SV-remote-telemetry-ingress`

3.3.16.1. Remote Telemetry Ingestion

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SD-19`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Remote & Virtual Care (p. 182)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:436 (p. 365)	Linked RAD detail; exact locators in manifest.

Ingest home patient monitoring data, biometric telemetry, and patient-reported outcome measures (PROMs).

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.16.2. Virtual Care Session Context Binding

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SD-20`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Remote & Virtual Care (p. 182)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:437 (p. 365)	Linked RAD detail; exact locators in manifest.

Associate synchronous virtual/telehealth consultations with the longitudinal health record and active care episode.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.16.3. Correlate Virtual Care Consultation Context

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-correlate-virtual-care-consultation-context`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Remote & Virtual Care (p. 182)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sd:225 (p. 444)	Linked RAD detail; exact locators in manifest.

Binds video consultation metadata, remote practitioner identity, and shared digital whiteboard artifacts to the encounter.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.16.4. Virtual Encounter Context Resolution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-virtual-encounter-context-resolution`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Remote & Virtual Care (p. 182)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sd:226 (p. 444)	Linked RAD detail; exact locators in manifest.

Binds virtual session identifiers to patient encounters.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.16.5. Ingest Remote Patient Telemetry

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-ingest-remote-patient-telemetry`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Remote & Virtual Care (p. 182)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sd:228 (p. 444)	Linked RAD detail; exact locators in manifest.

Ingests continuous or periodic physiological measurements (e.g., blood pressure, pulse oximetry, continuous glucose) from home monitoring devices.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.3.16.6. Remote Telemetry Ingress

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-remote-telemetry-ingress`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Remote & Virtual Care (p. 182)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	sd:229 (p. 444)	Linked RAD detail; exact locators in manifest.

Ingests remote patient data streams into the LHR.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4. Health Service Operations

Capability	Reference Id
Clinic & Practice Operations (p. 185)	`HCM-BEC-clinic-practice-operations`
Ward Operations (p. 188)	`HCM-BEC-ward-operations`
Theatre Operations (p. 191)	`HCM-BEC-theatre-operations`
Emergency Department Operations (p. 193)	`HCM-BEC-emergency-department-operations`
Outpatient Operations (p. 195)	`HCM-BEC-outpatient-operations`
Bed & Care-Place Management (p. 197)	`HCM-BEC-bed-care-place-management`
Clinical Resource Management (p. 201)	`HCM-BEC-clinical-resource-management`
Service Capacity Management (p. 202)	`HCM-BEC-service-capacity-management`
Clinical Workforce Management (p. 204)	`HCM-BEC-clinical-workforce-management`
Clinical Rostering (p. 204)	`HCM-BEC-clinical-rostering`
Mobile Staff Management (p. 204)	`HCM-BEC-mobile-staff-management`
On-Call Management (p. 207)	`HCM-BEC-on-call-management`
Work Allocation & Dispatch (p. 209)	`HCM-BEC-work-allocation-dispatch`
Clinical Training & Education (p. 212)	`HCM-BEC-clinical-training-education`
Clinical Competency Management (p. 213)	`HCM-BEC-clinical-competency-management`
Clinical Qualification Management (p. 213)	`HCM-BEC-clinical-qualification-management`
Clinical Supervision (p. 214)	`HCM-BEC-clinical-supervision`
Patient Transport (p. 215)	`HCM-BEC-patient-transport`
Clinical Logistics Coordination (p. 217)	`HCM-BEC-clinical-logistics-coordination`
Discharge Management (p. 219)	`HCM-BEC-discharge-management`
Credential Management (p. 221)	`HCM-BEC-credential-management`

3.4.1. Clinic & Practice Operations

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-clinic-practice-operations`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.
Source	enabling:448 (p. 365); enabling:481 (p. 366); hso:72 (p. 447)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Clinic Session Tracking (p. 186)	Feature	`FEAT-HSO-01`

Element	Type	Reference Id
Operational Queue Progression (p. 186)	Feature	`FEAT-HSO-02`
Manage Clinic & Practice Capacity (p. 187)	Function	`HCM-FN-manage-clinic-practice-capacity`
Track Clinic Operating Session (p. 187)	Function	`HCM-FN-track-clinic-operating-session`
Manage Clinic Queue Progression (p. 187)	Function	`HCM-FN-manage-clinic-queue-progression`
Clinic Queue Telemetry Service (p. 188)	Service	`HCM-SV-clinic-queue-telemetry-service`
Clinic / Operational Queue Progression Process (p. 188)	Process	`HCM-PR-clinic-operational-queue-progression-process`

3.4.1.1. Clinic Session Tracking

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-01`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Clinic & Practice Operations (p. 185)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:483 (p. 366)	Linked RAD detail; exact locators in manifest.

Ingest and track clinic operating session start, delay, and completion states.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.1.2. Operational Queue Progression

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-02`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Clinic & Practice Operations (p. 185)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:484 (p. 366)	Linked RAD detail; exact locators in manifest.

Track client queue progression through reception, waiting area, consultation, and billing.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.1.3. Manage Clinic & Practice Capacity

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-manage-clinic-practice-capacity`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinic & Practice Operations (p. 185)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:75 (p. 447)	Linked RAD detail; exact locators in manifest.

Understands and manages available, committed and utilised clinic/practice capacity in relation to practitioner availability, sessions, rooms, appointments and patient demand.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.4.1.4. Track Clinic Operating Session

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-track-clinic-operating-session`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinic & Practice Operations (p. 185)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:78 (p. 447)	Linked RAD detail; exact locators in manifest.

Ingests and tracks clinic operating-session start, delay and completion, distinct from an individual patient's queue progression.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.1.5. Manage Clinic Queue Progression

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-manage-clinic-queue-progression`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinic & Practice Operations (p. 185)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:82 (p. 447)	Linked RAD detail; exact locators in manifest.

Tracks patient arrival, rooming, consultation in-progress, and check-out states.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.1.6. Clinic Queue Telemetry Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-clinic-queue-telemetry-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinic & Practice Operations (p. 185)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:83 (p. 447)	Linked RAD detail; exact locators in manifest.

Discloses real-time clinic waiting times and queue states.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.1.7. Clinic / Operational Queue Progression Process

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-PR-clinic-operational-queue-progression-process`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinic & Practice Operations (p. 185)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	processes:187 (p. 408); processes:188 (p. 408)	Linked RAD detail; exact locators in manifest.

- **Process Purpose**: Governs the movement and state tracking of outpatients through an ambulatory specialty clinic session.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.4.2. Ward Operations

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-ward-operations`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.
Source	enabling:449 (p. 365); enabling:486 (p. 366); hso:90 (p. 447)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Ward Patient Flow Coordination (p. 189)	Feature	FEAT-HSO-03
Clinical Handover Context Assembly (p. 189)	Feature	FEAT-HSO-04
Manage Ward Capacity (p. 189)	Function	HCM-FN-manage-ward-capacity
Ward Operational Summary Query (p. 190)	Service	HCM-SV-ward-operational-summary-query
Assemble Ward Handover Context (p. 190)	Function	HCM-FN-assemble-ward-handover-context

3.4.2.1. Ward Patient Flow Coordination

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-HSO-03	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Ward Operations (p. 188)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:488 (p. 366)	Linked RAD detail; exact locators in manifest.

Coordinate inpatient ward occupancy, care-place allocation, and patient bed status across care facilities.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.2.2. Clinical Handover Context Assembly

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-HSO-04	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Ward Operations (p. 188)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:489 (p. 366)	Linked RAD detail; exact locators in manifest.

Collate current ward patient lists, active medical concerns, and pending tasks for nursing shift handovers.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.2.3. Manage Ward Capacity

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	HCM-FN-manage-ward-capacity	Stable publication identity.
Architectural Status	Canonical	Established by RAD.

Name	Value	Comment / Description
Owning Context	Ward Operations (p. 188)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:93 (p. 447)	Linked RAD detail; exact locators in manifest.
Name Aliases	Coordinate Ward Operational State	Same element only.

Coordinates available, committed and utilised ward capacity and its patient-flow implications, retaining acuity distribution, nurse-to-patient ratios and isolation cohorting alongside bed/care-place state, patient demand and operational constraints.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.2.4. Ward Operational Summary Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-ward-operational-summary-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Ward Operations (p. 188)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:95 (p. 447)	Linked RAD detail; exact locators in manifest.

Discloses ward-level operational telemetry.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.4.2.5. Assemble Ward Handover Context

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-assemble-ward-handover-context`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Ward Operations (p. 188)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:99 (p. 447)	Linked RAD detail; exact locators in manifest.

Collates current ward patient lists, active medical concerns and pending tasks for nursing shift handover.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.3. Theatre Operations

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-theatre-operations`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.
Source	enabling:450 (p. 365); enabling:491 (p. 366); hso:106 (p. 448)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Operating Theatre Case Progression (p. 191)	Feature	`FEAT-HSO-05`
Perioperative Resource Notification (p. 192)	Feature	`FEAT-HSO-06`
Manage Theatre Capacity (p. 192)	Function	`HCM-FN-manage-theatre-capacity`
Manage Theatre Case Progression (p. 192)	Function	`HCM-FN-manage-theatre-case-progression`
Theatre Case Progression Telemetry (p. 193)	Service	`HCM-SV-theatre-case-progression-telemetry`
Theatre Case Progression Process (p. 193)	Process	`HCM-PR-theatre-case-progression-process`

3.4.3.1. Operating Theatre Case Progression

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-05`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Theatre Operations (p. 191)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:493 (p. 366)	Linked RAD detail; exact locators in manifest.

Track surgical case state transitions (`CALLED` → `IN_THEATRE` → `ANAESTHETISED` → `INCISION` → `CLOSED` → `RECOVERY`).

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.3.2. Perioperative Resource Notification

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-06`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Theatre Operations (p. 191)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:494 (p. 366)	Linked RAD detail; exact locators in manifest.

Dispatch alerts to portering, sterilization, and recovery teams based on surgical case progression milestones.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.3.3. Manage Theatre Capacity

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-manage-theatre-capacity`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Theatre Operations (p. 191)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:109 (p. 448)	Linked RAD detail; exact locators in manifest.

Understands and manages available, committed and utilised theatre capacity in relation to theatre, team and equipment availability, scheduled cases and case progression.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.4.3.4. Manage Theatre Case Progression

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-manage-theatre-case-progression`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Theatre Operations (p. 191)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:112 (p. 448)	Linked RAD detail; exact locators in manifest.

Coordinates surgical case milestones: *Case Called → Patient in Anaesthetic Bay → Anaesthesia Commenced → Knife to Skin → Procedure Finished → In PACU/Recovery → Ward Handover Complete*.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.3.5. Theatre Case Progression Telemetry

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-theatre-case-progression-telemetry`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Theatre Operations (p. 191)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:113 (p. 448)	Linked RAD detail; exact locators in manifest.

Emits real-time surgical milestone updates.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.3.6. Theatre Case Progression Process

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-PR-theatre-case-progression-process`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Theatre Operations (p. 191)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	processes:170 (p. 408); processes:171 (p. 408)	Linked RAD detail; exact locators in manifest.

- **Process Purpose**: Governs the perioperative operational milestones of a surgical procedure within the operating theatre suite.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.4.4. Emergency Department Operations

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-emergency-department-operations`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.
Source	enabling:451 (p. 365); enabling:496 (p. 366); hso:121 (p. 448)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
ED Bed & Bay Tracking (p. 194)	Feature	`FEAT-HSO-07`
ED Length of Stay Tracking (p. 194)	Feature	`FEAT-HSO-08`
Coordinate ED Departmental Capacity (p. 194)	Function	`HCM-FN-coordinate-ed-departmental-capacity`
ED Operational Status Query (p. 195)	Service	`HCM-SV-ed-operational-status-query`

3.4.4.1. ED Bed & Bay Tracking

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-07`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Emergency Department Operations (p. 193)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:498 (p. 366)	Linked RAD detail; exact locators in manifest.

Track occupancy and status of emergency cubicles, resuscitation bays, and short-stay beds.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.4.2. ED Length of Stay Tracking

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-08`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Emergency Department Operations (p. 193)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:499 (p. 366)	Linked RAD detail; exact locators in manifest.

Monitor operational wait times and length-of-stay milestones, generating operational escalation notifications.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.4.3. Coordinate ED Departmental Capacity

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-coordinate-ed-departmental-capacity`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Emergency Department Operations (p. 193)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.

Name	Value	Comment / Description
Source	hso:124 (p. 448)	Linked RAD detail; exact locators in manifest.

Understands and manages available, committed and utilised ED capacity in relation to care-place availability, demand, patient presence/progression, acuity and operational constraints; retains waiting room occupancy, resuscitation bay availability and time-to-triage metrics.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.4.4. ED Operational Status Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-ed-operational-status-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Emergency Department Operations (p. 193)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:125 (p. 448)	Linked RAD detail; exact locators in manifest.

Discloses facility ED occupancy and surge level.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.4.5. Outpatient Operations

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-outpatient-operations`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.
Source	enabling:452 (p. 365); enabling:501 (p. 366); hso:134 (p. 449)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Outpatient Arrival Registration (p. 196)	Feature	`FEAT-HSO-09`
Post-Clinic Order Coordination (p. 196)	Feature	`FEAT-HSO-10`
Manage Outpatient Capacity (p. 196)	Function	`HCM-FN-manage-outpatient-capacity`
Ambulatory Session Status Query (p. 197)	Service	`HCM-SV-ambulatory-session-status-query`
Capture Post-Clinic Follow-up Requirements (p. 197)	Function	`HCM-FN-capture-post-clinic-follow-up-requirements`

3.4.5.1. Outpatient Arrival Registration

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-09`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Outpatient Operations (p. 195)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:503 (p. 367)	Linked RAD detail; exact locators in manifest.

Ingest self-service check-in and clinic arrival events, notifying treating clinicians.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.5.2. Post-Clinic Order Coordination

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-10`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Outpatient Operations (p. 195)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:504 (p. 367)	Linked RAD detail; exact locators in manifest.

Capture follow-up booking and diagnostic order requirements generated at clinic completion.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.5.3. Manage Outpatient Capacity

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-manage-outpatient-capacity`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Outpatient Operations (p. 195)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:137 (p. 449)	Linked RAD detail; exact locators in manifest.
Name Aliases	Coordinate Outpatient Session Utilisation	Same element only.

Understands and manages available, committed and utilised outpatient capacity through clinic/session, practitioner and room availability, appointment demand, patient arrival/waiting and operational progression. Retains specialty room allocation and clinician arrival tracking.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.5.4. Ambulatory Session Status Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-ambulatory-session-status-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Outpatient Operations (p. 195)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:140 (p. 449)	Linked RAD detail; exact locators in manifest.

Exposes clinic room occupancy.

Unresolved dimensions: ancestry; feature association; exposure consumer. See the linked RAD detail and uncertainty register.

3.4.5.5. Capture Post-Clinic Follow-up Requirements

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-capture-post-clinic-follow-up-requirements`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Outpatient Operations (p. 195)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:144 (p. 449)	Linked RAD detail; exact locators in manifest.

Captures follow-up booking and diagnostic-order requirements generated at clinic completion.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.6. Bed & Care-Place Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-bed-care-place-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Core	Independent of Architectural Status.
Source	enabling:453 (p. 365); enabling:506 (p. 367); hso:151 (p. 449)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Bed Availability Tracking (p. 198)	Feature	`FEAT-HSO-11`
Bed State Progression (p. 198)	Feature	`FEAT-HSO-12`
Bed Readiness Notification (p. 199)	Feature	`FEAT-HSO-13`
Maintain Operational Bed State (p. 199)	Function	`HCM-FN-maintain-operational-bed-state`
Bed Availability Query (p. 199)	Service	`HCM-SV-bed-availability-query`
Manage Bed Turnover Progression (p. 200)	Function	`HCM-FN-manage-bed-turnover-progression`
Bed Turnover Lifecycle Service (p. 200)	Service	`HCM-SV-bed-turnover-lifecycle-service`
Bed Turnover Process (p. 200)	Process	`HCM-PR-bed-turnover-process`

3.4.6.1. Bed Availability Tracking

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-11`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Bed & Care-Place Management (p. 197)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:508 (p. 367)	Linked RAD detail; exact locators in manifest.

Maintain operational status of all registered beds and care-places (`VACANT`, `OCCUPIED`, `BLOCKED`, `RESERVED`, `DIRTY`, `CLEANING`, `CLEAN`).

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.6.2. Bed State Progression

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-12`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Bed & Care-Place Management (p. 197)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:509 (p. 367)	Linked RAD detail; exact locators in manifest.

Progress bed states in response to clinical and environmental events (e.g., patient discharge automatically transitions bed to `DIRTY`).

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.6.3. Bed Readiness Notification

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-13`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Bed & Care-Place Management (p. 197)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:510 (p. 367)	Linked RAD detail; exact locators in manifest.

Notify bed allocation managers and clinical units when a cleaned bed achieves `READY` status.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.6.4. Maintain Operational Bed State

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-operational-bed-state`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Bed & Care-Place Management (p. 197)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:157 (p. 449)	Linked RAD detail; exact locators in manifest.

Tracks real-time bed status and environmental locks.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.6.5. Bed Availability Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-bed-availability-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Bed & Care-Place Management (p. 197)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:158 (p. 449)	Linked RAD detail; exact locators in manifest.

Discloses real-time bed availability across hospital directorates.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.6.6. Manage Bed Turnover Progression

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-manage-bed-turnover-progression`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Bed & Care-Place Management (p. 197)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:160 (p. 449)	Linked RAD detail; exact locators in manifest.

Coordinates cleaning, disinfection, and preparation: *Vacated → Cleaning Requested → Cleaning In-Progress → Inspected/Ready → Occupied*.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.6.7. Bed Turnover Lifecycle Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-bed-turnover-lifecycle-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Bed & Care-Place Management (p. 197)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:161 (p. 449)	Linked RAD detail; exact locators in manifest.

Coordinates turnover requests with environmental services.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.6.8. Bed Turnover Process

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-PR-bed-turnover-process`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Bed & Care-Place Management (p. 197)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	processes:203 (p. 409); processes:204 (p. 409)	Linked RAD detail; exact locators in manifest.

- **Process Purpose**: Governs the rapid physical turnover, cleaning, sanitisation, and re-allocation of inpatient and emergency care-places.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.4.7. Clinical Resource Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-clinical-resource-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.
Source	enabling:454 (p. 365); enabling:512 (p. 367); hso:169 (p. 450)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Mobile Clinical Equipment Tracking (p. 201)	Feature	`FEAT-HSO-14`
Track Mobile Clinical Asset Operational State (p. 201)	Function	`HCM-FN-track-mobile-clinical-asset-operational-state`
Clinical Resource Availability Query (p. 202)	Service	`HCM-SV-clinical-resource-availability-query`

3.4.7.1. Mobile Clinical Equipment Tracking

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-14`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Clinical Resource Management (p. 201)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:514 (p. 367)	Linked RAD detail; exact locators in manifest.

Track physical location and allocation status of shared clinical equipment (infusion pumps, ventilators, mobile telemetry).

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.7.2. Track Mobile Clinical Asset Operational State

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-track-mobile-clinical-asset-operational-state`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinical Resource Management (p. 201)	Responsibility; no invented intermediate element.

Name	Value	Comment / Description
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:172 (p. 450)	Linked RAD detail; exact locators in manifest.

Manages the real-time location and operational readiness of mobile ventilators, infusion pumps, and telemetry transmitters.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.7.3. Clinical Resource Availability Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-clinical-resource-availability-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinical Resource Management (p. 201)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:173 (p. 450)	Linked RAD detail; exact locators in manifest.

Exposes mobile equipment availability.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.8. Service Capacity Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-service-capacity-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.
Source	enabling:455 (p. 365); enabling:516 (p. 367); hso:179 (p. 450)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Operational Capacity Metric Aggregation (p. 203)	Feature	`FEAT-HSO-15`
Aggregate Service Capacity Telemetry (p. 203)	Function	`HCM-FN-aggregate-service-capacity-telemetry`
Regional Health System Capacity Query (p. 203)	Service	`HCM-SV-regional-health-system-capacity-query`

3.4.8.1. Operational Capacity Metric Aggregation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-15`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Service Capacity Management (p. 202)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:518 (p. 367)	Linked RAD detail; exact locators in manifest.

Aggregate operational capacity telemetry (occupancy rates, wait times, diversion status) across facilities.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.8.2. Aggregate Service Capacity Telemetry

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-aggregate-service-capacity-telemetry`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Service Capacity Management (p. 202)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:182 (p. 450)	Linked RAD detail; exact locators in manifest.

Consumes and coordinates context-specific service-capacity information for broader situational awareness, alongside bed occupancy, ICU surge capacity and ventilator availability across regional hospitals.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.8.3. Regional Health System Capacity Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-regional-health-system-capacity-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Service Capacity Management (p. 202)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:183 (p. 450)	Linked RAD detail; exact locators in manifest.

Provides executive operational situational awareness.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.9. Clinical Workforce Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-clinical-workforce-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Adjacent	Independent of Architectural Status.
Source	enabling:456 (p. 365)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

3.4.10. Clinical Rostering

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-clinical-rostering`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Adjacent	Independent of Architectural Status.
Source	enabling:457 (p. 365)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

3.4.11. Mobile Staff Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-mobile-staff-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.
Source	enabling:458 (p. 365); enabling:520 (p. 367); hso:190 (p. 450)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Mobile Worker Task Dispatch (p. 205)	Feature	`FEAT-HSO-16`
Mobile Visit Status Tracking (p. 205)	Feature	`FEAT-HSO-17`
Manage Mobile Service Capacity (p. 206)	Function	`HCM-FN-manage-mobile-service-capacity`
Track Operational Staff Presence (p. 206)	Function	`HCM-FN-track-operational-staff-presence`
Staff Presence Telemetry Query (p. 206)	Service	`HCM-SV-staff-presence-telemetry-query`
Receive Mobile Visit Progression (p. 207)	Function	`HCM-FN-receive-mobile-visit-progression`
Integrate Mobile Work Management (p. 207)	Function	`HCM-FN-integrate-mobile-work-management`

3.4.11.1. Mobile Worker Task Dispatch

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-16`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Mobile Staff Management (p. 204)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:522 (p. 367)	Linked RAD detail; exact locators in manifest.

Dispatch community care tasks and urgent visits to roving clinicians' mobile devices.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.11.2. Mobile Visit Status Tracking

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-17`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Mobile Staff Management (p. 204)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:524 (p. 367)	Linked RAD detail; exact locators in manifest.

Ingest arrival, visit progression, and safety check-in milestones from roving staff.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.11.3. Manage Mobile Service Capacity

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-manage-mobile-service-capacity`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Mobile Staff Management (p. 204)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:193 (p. 450)	Linked RAD detail; exact locators in manifest.

Understands and manages available, committed and utilised mobile-service capacity in relation to workforce availability, geography, time, workload and applicable service demand.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.4.11.4. Track Operational Staff Presence

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-track-operational-staff-presence`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Mobile Staff Management (p. 204)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:197 (p. 450)	Linked RAD detail; exact locators in manifest.

Tracks active on-duty presence and physical zone assignment for porters, phlebotomists, and roving nurses.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.4.11.5. Staff Presence Telemetry Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-staff-presence-telemetry-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Mobile Staff Management (p. 204)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:198 (p. 450)	Linked RAD detail; exact locators in manifest.

Discloses on-duty staff availability by hospital zone.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.4.11.6. Receive Mobile Visit Progression

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-receive-mobile-visit-progression`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Mobile Staff Management (p. 204)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:200 (p. 450)	Linked RAD detail; exact locators in manifest.

Ingests arrival, visit-progression and safety check-in milestones from roving staff.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.11.7. Integrate Mobile Work Management

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-integrate-mobile-work-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Mobile Staff Management (p. 204)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:203 (p. 450)	Linked RAD detail; exact locators in manifest.

Exchanges externally established work/assignment, dispatch and operational progression/status information, with applicable worker/context information, between mobile work-management mechanisms and Harmonia operational coordination.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.12. On-Call Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-on-call-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.
Source	enabling:459 (p. 365); enabling:526 (p. 367); hso:211 (p. 451)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Active On-Call Provider Resolution (p. 208)	Feature	FEAT-HSO-18
Resolve Active On-Call Coverage (p. 208)	Function	HCM-FN-resolve-active-on-call-coverage
On-Call Specialist Lookup (p. 208)	Service	HCM-SV-on-call-specialist-lookup

3.4.12.1. Active On-Call Provider Resolution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-HSO-18	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	On-Call Management (p. 207)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:528 (p. 367)	Linked RAD detail; exact locators in manifest.

Resolve currently active on-call specialists, registrars, and operational managers for a given specialty and hospital at any point in time.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.12.2. Resolve Active On-Call Coverage

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	HCM-FN-resolve-active-on-call-coverage	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	On-Call Management (p. 207)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:214 (p. 451)	Linked RAD detail; exact locators in manifest.

Resolves primary, secondary, and tertiary on-call clinical specialists for emergency escalation.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.12.3. On-Call Specialist Lookup

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	HCM-SV-on-call-specialist-lookup	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	On-Call Management (p. 207)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:215 (p. 451)	Linked RAD detail; exact locators in manifest.

Discloses active on-call contacts for switchboard and emergency teams.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.13. Work Allocation & Dispatch

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-work-allocation-dispatch`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Core	Independent of Architectural Status.
Source	enabling:460 (p. 365); enabling:530 (p. 368); hso:221 (p. 452)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Work Item Instantiation (p. 209)	Feature	`FEAT-HSO-19`
Worker Matching & Allocation (p. 210)	Feature	`FEAT-HSO-20`
Work Dispatch Delivery (p. 210)	Feature	`FEAT-HSO-21`
Work Progress Oversight & Escalation (p. 210)	Feature	`FEAT-HSO-22`
Dispatch Operational Work Order (p. 211)	Function	`HCM-FN-dispatch-operational-work-order`
Work Dispatch Service (p. 211)	Service	`HCM-SV-work-dispatch-service`
Manage Operational Work Progression (p. 211)	Function	`HCM-FN-manage-operational-work-progression`
Operational Work Status Query (p. 212)	Service	`HCM-SV-operational-work-status-query`
Operational Work Progression Process (p. 212)	Process	`HCM-PR-operational-work-progression-process`

3.4.13.1. Work Item Instantiation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-19`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Work Allocation & Dispatch (p. 209)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:532 (p. 368)	Linked RAD detail; exact locators in manifest.

Instantiate operational units of work from clinical triggers (e.g., bed cleaning required, specimen transport required).

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.13.2. Worker Matching & Allocation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-20`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Work Allocation & Dispatch (p. 209)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:533 (p. 368)	Linked RAD detail; exact locators in manifest.

Match work items against available staff roles, credentials, and locations based on allocation policies.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.13.3. Work Dispatch Delivery

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-21`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Work Allocation & Dispatch (p. 209)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:534 (p. 368)	Linked RAD detail; exact locators in manifest.

Deliver work orders to target workers, devices, or team queues with technical and operational acknowledgement capture.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.13.4. Work Progress Oversight & Escalation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-22`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Work Allocation & Dispatch (p. 209)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.

Name	Value	Comment / Description
Source	enabling:535 (p. 368)	Linked RAD detail; exact locators in manifest.

Monitor work progression against time-to-completion thresholds, escalating stalled or unacknowledged tasks.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.13.5. Dispatch Operational Work Order

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-dispatch-operational-work-order`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Work Allocation & Dispatch (p. 209)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:227 (p. 452)	Linked RAD detail; exact locators in manifest.

Allocates work requests to available wardspersons or support teams using applicable allocation policies, staff roles, credentials, location and skill, and delivers allocated work to the target worker, device or team queue.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.13.6. Work Dispatch Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-work-dispatch-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Work Allocation & Dispatch (p. 209)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:228 (p. 452)	Linked RAD detail; exact locators in manifest.

Delivers allocated work and captures distinct technical and operational acknowledgements; acknowledgement alone does not establish completion.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.13.7. Manage Operational Work Progression

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-manage-operational-work-progression`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Work Allocation & Dispatch (p. 209)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.

Name	Value	Comment / Description
Source	hso:231 (p. 452)	Linked RAD detail; exact locators in manifest.

Tracks execution states: *Queued → Assigned → Accepted → In-Progress → Completed / Aborted*.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.13.8. Operational Work Status Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-operational-work-status-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Work Allocation & Dispatch (p. 209)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:232 (p. 452)	Linked RAD detail; exact locators in manifest.

Discloses task progression to requesting wards.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.13.9. Operational Work Progression Process

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-PR-operational-work-progression-process`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Work Allocation & Dispatch (p. 209)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	processes:218 (p. 409); processes:219 (p. 409)	Linked RAD detail; exact locators in manifest.

- **Process Purpose**: Governs the dispatch, assignment, execution, and completion of facility operational jobs (e.g., equipment moves, waste removal, linen supply).

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.4.14. Clinical Training & Education

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-clinical-training-education`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.

Name	Value	Comment / Description
Harmonia Relevance	Reference	Independent of Architectural Status.
Source	enabling:461 (p. 365)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

3.4.15. Clinical Competency Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-clinical-competency-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Reference	Independent of Architectural Status.
Source	enabling:462 (p. 365)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

3.4.16. Clinical Qualification Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-clinical-qualification-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.
Source	enabling:463 (p. 365); enabling:539 (p. 368); hso:292 (p. 454); metamodel:397 (p. 386)	Linked RAD detail; exact locators in manifest.
Name Aliases	Clinical Credential Management	Same element only.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Operational Credential Check (p. 214)	Feature	`FEAT-HSO-23`
Verify Clinical Qualification for Work Dispatch (p. 214)	Function	`HCM-FN-verify-clinical-qualification-for-work-dispatch`

3.4.16.1. Operational Credential Check

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-23`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Clinical Qualification Management (p. 213)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:541 (p. 368)	Linked RAD detail; exact locators in manifest.

Validate that a practitioner possesses active credentials and required competency certs prior to dispatching specialized work.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.16.2. Verify Clinical Qualification for Work Dispatch

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-verify-clinical-qualification-for-work-dispatch`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinical Qualification Management (p. 213)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:295 (p. 454)	Linked RAD detail; exact locators in manifest.

Validates that a practitioner possesses active clinical credentials and required competency certificates before dispatch of specialised work.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.17. Clinical Supervision

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-clinical-supervision`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Reference	Independent of Architectural Status.
Source	enabling:464 (p. 365)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

3.4.18. Patient Transport

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-patient-transport`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Core	Independent of Architectural Status.
Source	enabling:465 (p. 365); enabling:543 (p. 368); hso:250 (p. 452)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Transport Request Ingestion (p. 215)	Feature	`FEAT-HSO-24`
Transport Dispatch & Progress Tracking (p. 216)	Feature	`FEAT-HSO-25`
Manage Patient Transport Progression (p. 216)	Function	`HCM-FN-manage-patient-transport-progression`
Patient Transport Tracking Service (p. 216)	Service	`HCM-SV-patient-transport-tracking-service`
Patient Transport Process (p. 217)	Process	`HCM-PR-patient-transport-process`

3.4.18.1. Transport Request Ingestion

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-24`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Patient Transport (p. 215)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:545 (p. 368)	Linked RAD detail; exact locators in manifest.

Ingest internal (portering) and external (ambulance, inter-hospital) patient transfer requests with clinical mobility requirements.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.18.2. Transport Dispatch & Progress Tracking

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-25`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Patient Transport (p. 215)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:546 (p. 368)	Linked RAD detail; exact locators in manifest.

Dispatch transport orders, tracking departure, transit, and destination arrival checkpoints.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.18.3. Manage Patient Transport Progression

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-manage-patient-transport-progression`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Patient Transport (p. 215)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:253 (p. 452)	Linked RAD detail; exact locators in manifest.

Coordinates intra-facility and inter-facility patient movements: *Transport Requested → Porter Dispatched → Patient Collected → In-Transit → Delivered at Destination → Handover Completed*.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.18.4. Patient Transport Tracking Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-patient-transport-tracking-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Patient Transport (p. 215)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:254 (p. 452)	Linked RAD detail; exact locators in manifest.

Provides real-time transit telemetry.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.18.5. Patient Transport Process

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-PR-patient-transport-process`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Patient Transport (p. 215)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	processes:233 (p. 409); processes:234 (p. 409)	Linked RAD detail; exact locators in manifest.

- **Process Purpose**: Governs the physical portering and transport of patients between hospital departments, wards, diagnostic suites, and external facilities.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.4.19. Clinical Logistics Coordination

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-clinical-logistics-coordination`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.
Source	enabling:466 (p. 365); enabling:548 (p. 368); hso:264 (p. 453)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Pathology Specimen Transport Tracking (p. 217)	Feature	`FEAT-HSO-26`
Manage Specimen Transport Progression (p. 218)	Function	`HCM-FN-manage-specimen-transport-progression`
Specimen Transit Tracking Service (p. 218)	Service	`HCM-SV-specimen-transit-tracking-service`
Specimen Transport Process (p. 218)	Process	`HCM-PR-specimen-transport-process`

3.4.19.1. Pathology Specimen Transport Tracking

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-26`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.

Name	Value	Comment / Description
Owning Context	Clinical Logistics Coordination (p. 217)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:550 (p. 368)	Linked RAD detail; exact locators in manifest.

Track pickup, custody chain, transport conditions, and laboratory delivery of clinical specimens.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.19.2. Manage Specimen Transport Progression

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-manage-specimen-transport-progression`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinical Logistics Coordination (p. 217)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:267 (p. 453)	Linked RAD detail; exact locators in manifest.

Tracks the physical transit of pathology bio-specimens, blood products, and surgical trays: *Collected → Courier Picked Up → In-Transit → Laboratory Ingress Received*.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.19.3. Specimen Transit Tracking Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-specimen-transit-tracking-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinical Logistics Coordination (p. 217)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:268 (p. 453)	Linked RAD detail; exact locators in manifest.

Exposes chain-of-custody tracking.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.19.4. Specimen Transport Process

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-PR-specimen-transport-process`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinical Logistics Coordination (p. 217)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.

Name	Value	Comment / Description
Source	processes:248 (p. 409); processes:249 (p. 409)	Linked RAD detail; exact locators in manifest.

- **Process Purpose**: Governs the physical courier chain-of-custody, transport, and delivery of pathology bio-specimens, blood products, and surgical biopsies.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.4.20. Discharge Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-discharge-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Core	Independent of Architectural Status.
Source	enabling:467 (p. 365); enabling:552 (p. 368); hso:275 (p. 453)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Discharge Milestone Tracking (p. 219)	Feature	`FEAT-HSO-27`
Discharge Readiness & Coordination Oversight (p. 220)	Feature	`FEAT-HSO-28`
Discharge Notification Dispatch (p. 220)	Feature	`FEAT-HSO-29`
Manage Discharge Coordination Progression (p. 220)	Function	`HCM-FN-manage-discharge-coordination-progression`
Discharge Readiness Telemetry (p. 221)	Service	`HCM-SV-discharge-readiness-telemetry`
Discharge Progression Process (p. 221)	Process	`HCM-PR-discharge-progression-process`

3.4.20.1. Discharge Milestone Tracking

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-27`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Discharge Management (p. 219)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:554 (p. 368)	Linked RAD detail; exact locators in manifest.

Track clinical criteria-led discharge milestones (medications dispensed, transportation arranged, home care confirmed).

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.20.2. Discharge Readiness & Coordination Oversight

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-28`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Discharge Management (p. 219)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:555 (p. 368)	Linked RAD detail; exact locators in manifest.

Track and aggregate multidisciplinary discharge milestones, home care confirmations, and transit readiness across care teams.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.20.3. Discharge Notification Dispatch

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-29`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Discharge Management (p. 219)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:556 (p. 368)	Linked RAD detail; exact locators in manifest.

Dispatch departure notifications, updated bed states, and finalized discharge summaries to external care providers upon physical patient exit.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.20.4. Manage Discharge Coordination Progression

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-manage-discharge-coordination-progression`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Discharge Management (p. 219)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.

Name	Value	Comment / Description
Source	hso:279 (p. 453)	Linked RAD detail; exact locators in manifest.

Synchronises multi-agency discharge readiness (**illustrative preparation; publication, authorisation and confirmed exit are distinct**): *Discharge Planning Initiated → Medications Reconciled → Transport Booked → Discharge Summary Finalised → Physically Departed*.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.20.5. Discharge Readiness Telemetry

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-discharge-readiness-telemetry`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Discharge Management (p. 219)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:283 (p. 453)	Linked RAD detail; exact locators in manifest.

Discloses discharge barrier checklists and planned departure times.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.4.20.6. Discharge Progression Process

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-PR-discharge-progression-process`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Discharge Management (p. 219)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	processes:263 (p. 410); processes:264 (p. 410)	Linked RAD detail; exact locators in manifest.

- **Process Purpose**: Governs the multidisciplinary coordination of patient discharge planning, pharmacy reconciliation, transport, and community handover.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.4.21. Credential Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-credential-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.

Name	Value	Comment / Description
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:239 (p. 452)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry; strategy catalogue correspondence; relevance. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Verify Operational Access Credential (p. 222)	Function	`HCM-FN-verify-operational-access-credential`
Operational Credential Checkpoint Service (p. 222)	Service	`HCM-SV-operational-credential-checkpoint-service`

3.4.21.1. Verify Operational Access Credential

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-verify-operational-access-credential`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Credential Management (p. 221)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:243 (p. 452)	Linked RAD detail; exact locators in manifest.

Verifies physical access badges, specialty ward entry authorizations, and restricted area permissions.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.4.21.2. Operational Credential Checkpoint Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-operational-credential-checkpoint-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Credential Management (p. 221)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	hso:244 (p. 452)	Linked RAD detail; exact locators in manifest.

Evaluates physical/operational access rights.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.5. Intrinsic / Shared Enablement

Capability	Reference Id
Patient Clinical Record (p. 223)	HCM-BEC-patient-clinical-record
Health Information Exchange (p. 227)	HCM-BEC-health-information-exchange
Health Information Access (p. 231)	HCM-BEC-health-information-access
Health Information Communication (p. 234)	HCM-BEC-health-information-communication
Health Information Control (p. 236)	HCM-BEC-health-information-control
Clinical Knowledge Services (p. 240)	HCM-BEC-clinical-knowledge-services
Clinical Collaboration (p. 243)	HCM-BEC-clinical-collaboration
General Collaboration (p. 246)	HCM-BEC-general-collaboration
Workflow & Activity Coordination (p. 248)	HCM-BEC-workflow-activity-coordination
Calendar Management (p. 254)	HCM-BEC-calendar-management
Presentation Services (p. 256)	HCM-BEC-presentation-services
Information Design Governance (p. 258)	HCM-BEC-information-design-governance

3.5.1. Patient Clinical Record

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	HCM-BEC-patient-clinical-record	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Core	Independent of Architectural Status.
Source	enabling:565 (p. 369); enabling:608 (p. 370); ise:35 (p. 456)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Longitudinal Health Record (LHR) Assembly (p. 224)	Feature	FEAT-ISE-01
Active Clinical Record Access (p. 224)	Feature	FEAT-ISE-02
Durable Clinical Record Preservation (p. 224)	Feature	FEAT-ISE-03
Assemble Longitudinal Clinical Record (p. 225)	Function	HCM-FN-assemble-longitudinal-clinical-record
Longitudinal Clinical Record Query (p. 225)	Service	HCM-SV-longitudinal-clinical-record-query
Maintain Active Clinical Record (p. 225)	Function	HCM-FN-maintain-active-clinical-record
Active Problem & Allergy Summary Query (p. 226)	Service	HCM-SV-active-problem-allergy-summary-query

Element	Type	Reference Id
Preserve Durable Clinical Record (p. 226)	Function	`HCM-FN-preserve-durable-clinical-record`
Historical Clinical Record Access (p. 226)	Service	`HCM-SV-historical-clinical-record-access`

3.5.1.1. Longitudinal Health Record (LHR) Assembly

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-01`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Patient Clinical Record (p. 223)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:610 (p. 370)	Linked RAD detail; exact locators in manifest.

Assemble disparate encounters, diagnostic reports, medications, and clinical documents into a unified, vendor-neutral longitudinal timeline.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.1.2. Active Clinical Record Access

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-02`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Patient Clinical Record (p. 223)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:611 (p. 370)	Linked RAD detail; exact locators in manifest.

Provide low-latency, active distributed access to consolidated patient clinical summaries.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.1.3. Durable Clinical Record Preservation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-03`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Patient Clinical Record (p. 223)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:612 (p. 370)	Linked RAD detail; exact locators in manifest.

Durably maintain governed, vendor-neutral longitudinal clinical representation and preservation with verifiable integrity controls.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.1.4. Assemble Longitudinal Clinical Record

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-assemble-longitudinal-clinical-record`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Patient Clinical Record (p. 223)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:39 (p. 456)	Linked RAD detail; exact locators in manifest.

Synthesises clinical documents, encounters, diagnostic reports, and medication histories across disparate source systems into a unified chronological clinical view.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.1.5. Longitudinal Clinical Record Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-longitudinal-clinical-record-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Patient Clinical Record (p. 223)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:40 (p. 456)	Linked RAD detail; exact locators in manifest.

Discloses the comprehensive clinical timeline to authorised clinicians.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.1.6. Maintain Active Clinical Record

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-active-clinical-record`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Patient Clinical Record (p. 223)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:42 (p. 456)	Linked RAD detail; exact locators in manifest.

Maintains active problem lists, current medication regimens, allergies, and open clinical alerts for active care coordination.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.1.7. Active Problem & Allergy Summary Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-active-problem-allergy-summary-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Patient Clinical Record (p. 223)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:43 (p. 456)	Linked RAD detail; exact locators in manifest.

Exposes immediate active clinical summaries.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.1.8. Preserve Durable Clinical Record

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-preserve-durable-clinical-record`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Patient Clinical Record (p. 223)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:45 (p. 456)	Linked RAD detail; exact locators in manifest.

Preserves the governed longitudinal representation and durable, immutable, append-only history of what Harmonia received, knew, asserted or managed, with provenance and versioning. Corrections/supersession establish changed knowledge without retrospectively altering that history or acquiring originating clinical authority.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.1.9. Historical Clinical Record Access

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-historical-clinical-record-access`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Patient Clinical Record (p. 223)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:46 (p. 456)	Linked RAD detail; exact locators in manifest.

Provides governed historical record retrieval for authorised clinical and legal review; preservation does not itself establish legal qualification or originating authority.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.2. Health Information Exchange

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-health-information-exchange`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Core	Independent of Architectural Status.
Source	enabling:566 (p. 369); enabling:614 (p. 370); ise:52 (p. 456); business:1225 (p. 348)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry; name uniqueness. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Direct Submission Ingestion & Delivery (p. 227)	Feature	`FEAT-ISE-04`
Clinical Record Retrieval Mediation (p. 228)	Feature	`FEAT-ISE-05`
Addressed Clinical Distribution (p. 228)	Feature	`FEAT-ISE-06`
Event-Driven Syndication (p. 228)	Feature	`FEAT-ISE-07`
Receive Information Submission (p. 229)	Function	`HCM-FN-receive-information-submission`
HIE Submission Gateway Service (p. 229)	Service	`HCM-SV-hie-submission-gateway-service`
Mediate Information Retrieval (p. 229)	Function	`HCM-FN-mediate-information-retrieval`
Federated Health Information Query (p. 230)	Service	`HCM-SV-federated-health-information-query`
Distribute Addressed Information (p. 230)	Function	`HCM-FN-distribute-addressed-information`
Addressed Document Delivery Service (p. 230)	Service	`HCM-SV-addressed-document-delivery-service`
Syndicate Information Change (p. 231)	Function	`HCM-FN-syndicate-information-change`
Clinical Event Syndication Stream (p. 231)	Service	`HCM-SV-clinical-event-syndication-stream`

3.5.2.1. Direct Submission Ingestion & Delivery

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-04`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Health Information Exchange (p. 227)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:616 (p. 370)	Linked RAD detail; exact locators in manifest.

Support push submissions of clinical documents and messages across boundaries.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.2.2. Clinical Record Retrieval Mediation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-05`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Health Information Exchange (p. 227)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:617 (p. 370)	Linked RAD detail; exact locators in manifest.

Mediate query-based retrieval requests across external and internal repositories.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.2.3. Addressed Clinical Distribution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-06`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Health Information Exchange (p. 227)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:618 (p. 370)	Linked RAD detail; exact locators in manifest.

Fan out clinical events and diagnostic reports to designated external and internal recipients with per-destination delivery tracking.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.2.4. Event-Driven Syndication

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-07`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Health Information Exchange (p. 227)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:619 (p. 370)	Linked RAD detail; exact locators in manifest.

Publish clinical and directory change events to subscribed systems matching interest filters.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.2.5. Receive Information Submission

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-receive-information-submission`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Health Information Exchange (p. 227)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:57 (p. 456)	Linked RAD detail; exact locators in manifest.

Validates and ingests standards-compliant clinical messages from external health providers.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.2.6. HIE Submission Gateway Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-hie-submission-gateway-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Health Information Exchange (p. 227)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:58 (p. 456)	Linked RAD detail; exact locators in manifest.

Ingress endpoint for regional data contributions.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.2.7. Mediate Information Retrieval

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-mediate-information-retrieval`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Health Information Exchange (p. 227)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:60 (p. 456)	Linked RAD detail; exact locators in manifest.

Federates queries across participating hospital and diagnostic node repositories.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.2.8. Federated Health Information Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-federated-health-information-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Health Information Exchange (p. 227)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:61 (p. 456)	Linked RAD detail; exact locators in manifest.

Resolves distributed record queries.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.2.9. Distribute Addressed Information

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-distribute-addressed-information`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Health Information Exchange (p. 227)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:63 (p. 456)	Linked RAD detail; exact locators in manifest.

Directs point-to-point clinical documents to specific recipient practitioner endpoints.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.2.10. Addressed Document Delivery Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-addressed-document-delivery-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Health Information Exchange (p. 227)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:64 (p. 456)	Linked RAD detail; exact locators in manifest.

Securely delivers documents to designated recipient mailboxes.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.2.11. Syndicate Information Change

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-syndicate-information-change`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Health Information Exchange (p. 227)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:66 (p. 456)	Linked RAD detail; exact locators in manifest.

Broadcasts event notifications and record updates to subscribed regional care network systems.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.2.12. Clinical Event Syndication Stream

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-clinical-event-syndication-stream`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Health Information Exchange (p. 227)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:67 (p. 456)	Linked RAD detail; exact locators in manifest.

Emits publish-subscribe clinical event feeds.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.3. Health Information Access

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-health-information-access`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Core	Independent of Architectural Status.
Source	enabling:567 (p. 369); enabling:621 (p. 370); ise:73 (p. 457)	Linked RAD detail; exact locators in manifest.
Supports	Individual Care Delivery; Individual Health Assessment; Clinical Determination; Individual Care Planning; Clinical Intervention; Individual Health Monitoring; Individual Care Review	Source-scoped; no inherited descendant edges.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Federated Clinical Query (p. 232)	Feature	`FEAT-ISE-08`
Access-Controlled Information Filtering (p. 232)	Feature	`FEAT-ISE-09`
Search Health Information (p. 233)	Function	`HCM-FN-search-health-information`
Clinical Information Search Service (p. 233)	Service	`HCM-SV-clinical-information-search-service`
Retrieve Health Information (p. 233)	Function	`HCM-FN-retrieve-health-information`
Clinical Resource Retrieval Service (p. 234)	Service	`HCM-SV-clinical-resource-retrieval-service`
Apply Access-Qualified Result Filtering (p. 234)	Function	`HCM-FN-apply-access-qualified-result-filtering`

3.5.3.1. Federated Clinical Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-08`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Health Information Access (p. 231)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:623 (p. 370)	Linked RAD detail; exact locators in manifest.

Execute bounded, indexed queries across distributed clinical resources and directory graphs.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.3.2. Access-Controlled Information Filtering

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-09`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Health Information Access (p. 231)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:624 (p. 370)	Linked RAD detail; exact locators in manifest.

Filter search results dynamically according to requesting user authorizations and patient consent directives.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.3.3. Search Health Information

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-search-health-information`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Health Information Access (p. 231)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:78 (p. 457)	Linked RAD detail; exact locators in manifest.

Executes structured semantic searches across diagnostic reports, clinical notes, and medication histories.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.3.4. Clinical Information Search Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-clinical-information-search-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Health Information Access (p. 231)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:79 (p. 457)	Linked RAD detail; exact locators in manifest.

Discloses search capabilities to clinical user interfaces.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.3.5. Retrieve Health Information

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-retrieve-health-information`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Health Information Access (p. 231)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:82 (p. 457)	Linked RAD detail; exact locators in manifest.

Fetches discrete clinical observations and original attachments.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.5.3.6. Clinical Resource Retrieval Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-clinical-resource-retrieval-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Health Information Access (p. 231)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:83 (p. 457)	Linked RAD detail; exact locators in manifest.

Delivers structured clinical information to requesting applications.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.5.3.7. Apply Access-Qualified Result Filtering

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-apply-access-qualified-result-filtering`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Health Information Access (p. 231)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:85 (p. 457)	Linked RAD detail; exact locators in manifest.

Redacts or masks search results and clinical content based on the caller's verified security context and patient consent directives.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.5.4. Health Information Communication

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-health-information-communication`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Core	Independent of Architectural Status.
Source	enabling:568 (p. 369); enabling:626 (p. 370); ise:92 (p. 457)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Standards-Based Boundary Exchange (p. 235)	Feature	`FEAT-ISE-10`
Mediate Standards-Based Health Information Communication (p. 235)	Function	`HCM-FN-mediate-standards-based-health-information-communication`
Standards Communication Gateway Service (p. 235)	Service	`HCM-SV-standards-communication-gateway-service`

3.5.4.1. Standards-Based Boundary Exchange

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-10`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Health Information Communication (p. 234)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:628 (p. 370)	Linked RAD detail; exact locators in manifest.

Provide standards-conformant boundary exchange for healthcare messaging and resource representations.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.4.2. Mediate Standards-Based Health Information Communication

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-mediate-standards-based-health-information-communication`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Health Information Communication (p. 234)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:97 (p. 457)	Linked RAD detail; exact locators in manifest.

Handles standards-based communication mediation, transaction coordination, and technical error handling.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.4.3. Standards Communication Gateway Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-standards-communication-gateway-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Health Information Communication (p. 234)	Responsibility; no invented intermediate element.

Name	Value	Comment / Description
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:98 (p. 457)	Linked RAD detail; exact locators in manifest.

Protocol interface for external client integrations.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.5. Health Information Control

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-health-information-control`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Core	Independent of Architectural Status.
Source	enabling:569 (p. 369); enabling:630 (p. 370); ise:104 (p. 458)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Health Information Access & Consent Control (p. 237)	Feature	`FEAT-ISE-11`
Security Context Propagation (p. 237)	Feature	`FEAT-ISE-12`
Non-PHI Compliance Auditing (p. 237)	Feature	`FEAT-ISE-13`
Evaluate Information Access Authority (p. 238)	Function	`HCM-FN-evaluate-information-access-authority`
Policy Evaluation & Authorisation Service (p. 238)	Service	`HCM-SV-policy-evaluation-authorisation-service`
Evaluate Consent Constraint (p. 238)	Function	`HCM-FN-evaluate-consent-constraint`
Consent Enforcement Service (p. 239)	Service	`HCM-SV-consent-enforcement-service`
Propagate Security Context (p. 239)	Function	`HCM-FN-propagate-security-context`
Security Context Validation Service (p. 239)	Service	`HCM-SV-security-context-validation-service`
Record Security-Significant Activity (p. 240)	Function	`HCM-FN-record-security-significant-activity`
Security Audit Ingress (p. 240)	Service	`HCM-SV-security-audit-ingress`

3.5.5.1. Health Information Access & Consent Control

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-ISE-11	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Health Information Control (p. 236)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:632 (p. 371)	Linked RAD detail; exact locators in manifest.

Evaluate patient consent preferences, practitioner authority boundaries, and access control policies for managed clinical records.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.5.2. Security Context Propagation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-ISE-12	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Health Information Control (p. 236)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:633 (p. 371)	Linked RAD detail; exact locators in manifest.

Propagate tamper-evident, attributable security context across asynchronous processing boundaries.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.5.3. Non-PHI Compliance Auditing

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-ISE-13	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Health Information Control (p. 236)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:634 (p. 371)	Linked RAD detail; exact locators in manifest.

Generate tamper-evident, verifiable audit records of security-significant events without emitting unmasked PHI.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.5.4. Evaluate Information Access Authority

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-evaluate-information-access-authority`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Health Information Control (p. 236)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:108 (p. 458)	Linked RAD detail; exact locators in manifest.

Evaluates user role, organisation, patient relationship, purpose of use, and break-glass overrides against default-deny policies.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.5.5. Policy Evaluation & Authorisation Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-policy-evaluation-authorisation-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Health Information Control (p. 236)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:109 (p. 458)	Linked RAD detail; exact locators in manifest.

Evaluates access requests across the platform.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.5.6. Evaluate Consent Constraint

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-evaluate-consent-constraint`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Health Information Control (p. 236)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:112 (p. 458)	Linked RAD detail; exact locators in manifest.

Cross-references access requests against client consent directives and sensitive health category restrictions.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.5.5.7. Consent Enforcement Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-consent-enforcement-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Health Information Control (p. 236)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:113 (p. 458)	Linked RAD detail; exact locators in manifest.

Returns masking and redaction instructions.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.5.5.8. Propagate Security Context

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-propagate-security-context`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Health Information Control (p. 236)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:115 (p. 458)	Linked RAD detail; exact locators in manifest.

Generates and binds canonical, immutable security contexts to all intra-platform transactions.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.5.5.9. Security Context Validation Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-security-context-validation-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Health Information Control (p. 236)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:116 (p. 458)	Linked RAD detail; exact locators in manifest.

Verifies transaction security credentials.

Unresolved dimensions: ancestry; feature association; function mapping. See the linked RAD detail and uncertainty register.

3.5.5.10. Record Security-Significant Activity

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-record-security-significant-activity`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Health Information Control (p. 236)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:119 (p. 458)	Linked RAD detail; exact locators in manifest.

Records tamper-evident audit evidence (who, what, when, why, patient MRN, policy outcome) without logging unmasked PHI.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.5.11. Security Audit Ingress

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-security-audit-ingress`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Health Information Control (p. 236)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:120 (p. 458)	Linked RAD detail; exact locators in manifest.

Ingests audit events across all capabilities.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.6. Clinical Knowledge Services

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-clinical-knowledge-services`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.
Source	enabling:570 (p. 369); enabling:636 (p. 371); ise:126 (p. 458)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Clinical Concept Resolution (p. 241)	Feature	`FEAT-ISE-14`
Bidirectional Terminology Mapping (p. 241)	Feature	`FEAT-ISE-15`
Resolve Clinical Concept (p. 241)	Function	`HCM-FN-resolve-clinical-concept`
Terminology Concept Resolution Service (p. 242)	Service	`HCM-SV-terminology-concept-resolution-service`
Map Clinical Concept (p. 242)	Function	`HCM-FN-map-clinical-concept`
Terminology Mapping Service (p. 242)	Service	`HCM-SV-terminology-mapping-service`

3.5.6.1. Clinical Concept Resolution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-14`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Clinical Knowledge Services (p. 240)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:638 (p. 371)	Linked RAD detail; exact locators in manifest.

Resolve local clinical codes against standard terminologies (SNOMED CT, LOINC, ICD-10).

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.6.2. Bidirectional Terminology Mapping

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-15`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Clinical Knowledge Services (p. 240)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:639 (p. 371)	Linked RAD detail; exact locators in manifest.

Translate between legacy local code systems and canonical national healthcare profiles.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.6.3. Resolve Clinical Concept

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-resolve-clinical-concept`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinical Knowledge Services (p. 240)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.

Name	Value	Comment / Description
Source	ise:130 (p. 458)	Linked RAD detail; exact locators in manifest.

Validates concept identifiers and retrieves preferred clinical rubrics and hierarchy paths.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.6.4. Terminology Concept Resolution Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-terminology-concept-resolution-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinical Knowledge Services (p. 240)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:131 (p. 458)	Linked RAD detail; exact locators in manifest.

Exposes concept validation lookups.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.6.5. Map Clinical Concept

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-map-clinical-concept`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinical Knowledge Services (p. 240)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:133 (p. 458)	Linked RAD detail; exact locators in manifest.

Translates local lab codes and legacy terms to canonical SNOMED/LOINC codes using governed translation maps.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.6.6. Terminology Mapping Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-terminology-mapping-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinical Knowledge Services (p. 240)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:134 (p. 458)	Linked RAD detail; exact locators in manifest.

Translates codes across terminology systems.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.7. Clinical Collaboration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-clinical-collaboration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Core	Independent of Architectural Status.
Source	enabling:571 (p. 369); enabling:641 (p. 371); ise:140 (p. 459)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Care-Team Clinical Collaboration Coordination (p. 243)	Feature	`FEAT-ISE-16`
In-Conversation LHR Query Resolution (p. 244)	Feature	`FEAT-ISE-17`
Zero-PHI Collaboration Metadata Governance (p. 244)	Feature	`FEAT-ISE-18`
Coordinate Care-Team Collaboration (p. 244)	Function	`HCM-FN-coordinate-care-team-collaboration`
Clinical Collaboration Space Service (p. 245)	Service	`HCM-SV-clinical-collaboration-space-service`
Resolve LHR Query within Collaboration (p. 245)	Function	`HCM-FN-resolve-lhr-query-within-collaboration`
Collaboration Clinical Summary Resolution (p. 245)	Service	`HCM-SV-collaboration-clinical-summary-resolution`
Govern Collaboration Metadata (p. 246)	Function	`HCM-FN-govern-collaboration-metadata`
Collaboration Space Governance Service (p. 246)	Service	`HCM-SV-collaboration-space-governance-service`

3.5.7.1. Care-Team Clinical Collaboration Coordination

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-16`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Clinical Collaboration (p. 243)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.

Name	Value	Comment / Description
Source	enabling:643 (p. 371)	Linked RAD detail; exact locators in manifest.

Coordinate secure multidisciplinary clinical communication and consultation exchange bound to patient encounters or clinical topics.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.7.2. In-Conversation LHR Query Resolution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-17`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Clinical Collaboration (p. 243)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:644 (p. 371)	Linked RAD detail; exact locators in manifest.

Resolve clinical data queries within an authorized collaboration context, presenting verified clinical summaries directly to care team members.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.7.3. Zero-PHI Collaboration Metadata Governance

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-18`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Clinical Collaboration (p. 243)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:645 (p. 371)	Linked RAD detail; exact locators in manifest.

Ensure collaboration channel titles, topics, and aliases use opaque identifiers free of patient names, MRNs, or sensitive details.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.7.4. Coordinate Care-Team Collaboration

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-coordinate-care-team-collaboration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinical Collaboration (p. 243)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.

Name	Value	Comment / Description
Source	ise:146 (p. 459); metamodel:175 (p. 380)	Linked RAD detail; exact locators in manifest.

Manages care-team communication spaces, participant memberships, and clinical handovers.

Unresolved dimensions: ancestry; semantic boundary conflict. See the linked RAD detail and uncertainty register.

3.5.7.5. Clinical Collaboration Space Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-clinical-collaboration-space-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinical Collaboration (p. 243)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:147 (p. 459)	Linked RAD detail; exact locators in manifest.

Manages team collaboration spaces and discussions.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.7.6. Resolve LHR Query within Collaboration

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-resolve-lhr-query-within-collaboration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinical Collaboration (p. 243)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:149 (p. 459)	Linked RAD detail; exact locators in manifest.

Contextually projects active patient summaries directly into collaborative clinical discussion channels.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.7.7. Collaboration Clinical Summary Resolution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-collaboration-clinical-summary-resolution`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinical Collaboration (p. 243)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.

Name	Value	Comment / Description
Source	ise:150 (p. 459)	Linked RAD detail; exact locators in manifest.

Embeds clinical summaries in collaboration feeds.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.7.8. Govern Collaboration Metadata

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-govern-collaboration-metadata`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinical Collaboration (p. 243)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:154 (p. 459)	Linked RAD detail; exact locators in manifest.

Enforces privacy policies, participant access gates, and zero-PHI space metadata rules.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.7.9. Collaboration Space Governance Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-collaboration-space-governance-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Clinical Collaboration (p. 243)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:155 (p. 459)	Linked RAD detail; exact locators in manifest.

Governs space lifecycles and membership.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.8. General Collaboration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-general-collaboration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Relevant	Independent of Architectural Status.

Name	Value	Comment / Description
Source	enabling:572 (p. 369); enabling:647 (p. 371); ise:161 (p. 459)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Operational Team Chat Enablement (p. 247)	Feature	`FEAT-ISE-19`
Coordinate Operational Collaboration (p. 247)	Function	`HCM-FN-coordinate-operational-collaboration`
Operational Collaboration Service (p. 248)	Service	`HCM-SV-operational-collaboration-service`

3.5.8.1. Operational Team Chat Enablement

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-19`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	General Collaboration (p. 246)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:649 (p. 371)	Linked RAD detail; exact locators in manifest.

Provide secure instant messaging channels for departmental, administrative, and facility support staff.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.8.2. Coordinate Operational Collaboration

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-coordinate-operational-collaboration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	General Collaboration (p. 246)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:164 (p. 459)	Linked RAD detail; exact locators in manifest.

Provides communication spaces for facility logistics, disaster management teams, and administrative committees.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.8.3. Operational Collaboration Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-operational-collaboration-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	General Collaboration (p. 246)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:165 (p. 459)	Linked RAD detail; exact locators in manifest.

Provides non-clinical operational messaging channels.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.9. Workflow & Activity Coordination

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-workflow-activity-coordination`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Core	Independent of Architectural Status.
Source	enabling:573 (p. 369); enabling:651 (p. 371); ise:171 (p. 459)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Work Order Progression (p. 249)	Feature	`FEAT-ISE-20`
To Do Decision & Approval Coordination (p. 249)	Feature	`FEAT-ISE-21`
Synthetic Task Orchestration (p. 249)	Feature	`FEAT-ISE-22`
Workflow Timeout & Escalation Oversight (p. 250)	Feature	`FEAT-ISE-23`
Coordinate Work Order (p. 250)	Function	`HCM-FN-coordinate-work-order`
Work Order Coordination Service (p. 250)	Service	`HCM-SV-work-order-coordination-service`
Coordinate To Do (p. 251)	Function	`HCM-FN-coordinate-to-do`
To Do Management Service (p. 251)	Service	`HCM-SV-to-do-management-service`
Coordinate Synthetic Task (p. 251)	Function	`HCM-FN-coordinate-synthetic-task`
Synthetic Task Execution Service (p. 252)	Service	`HCM-SV-synthetic-task-execution-service`
Supervise Activity Timeout / Escalation (p. 252)	Function	`HCM-FN-supervise-activity-timeout-escalation`
Activity Escalation Telemetry (p. 252)	Service	`HCM-SV-activity-escalation-telemetry`
Work Order Progression Process (p. 253)	Process	`HCM-PR-work-order-progression-process`

Element	Type	Reference Id
To Do Progression Process (p. 253)	Process	`HCM-PR-to-do-progression-process`
Synthetic Task Progression Process (p. 253)	Process	`HCM-PR-synthetic-task-progression-process`

3.5.9.1. Work Order Progression

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-20`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Workflow & Activity Coordination (p. 248)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:653 (p. 371)	Linked RAD detail; exact locators in manifest.

Manage execution states and dispatch of human operational work orders.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.9.2. To Do Decision & Approval Coordination

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-21`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Workflow & Activity Coordination (p. 248)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:654 (p. 371)	Linked RAD detail; exact locators in manifest.

Coordinate human clinical review items, sign-offs, and approval workflows.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.9.3. Synthetic Task Orchestration

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-22`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Workflow & Activity Coordination (p. 248)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:655 (p. 371)	Linked RAD detail; exact locators in manifest.

Execute non-human, automated platform activity units and sequence orchestration.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.9.4. Workflow Timeout & Escalation Oversight

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-23`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Workflow & Activity Coordination (p. 248)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:656 (p. 371)	Linked RAD detail; exact locators in manifest.

Supervise operational deadlines, executing automated timeout notifications and supervisory escalations.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.9.5. Coordinate Work Order

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-coordinate-work-order`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Workflow & Activity Coordination (p. 248)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:181 (p. 459)	Linked RAD detail; exact locators in manifest.

Coordinates operational task lifecycles and dispatches.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.9.6. Work Order Coordination Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-work-order-coordination-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Workflow & Activity Coordination (p. 248)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:182 (p. 459)	Linked RAD detail; exact locators in manifest.

Manages operational work progression.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.9.7. Coordinate To Do

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-coordinate-to-do`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Workflow & Activity Coordination (p. 248)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:185 (p. 459)	Linked RAD detail; exact locators in manifest.

Coordinates human review/update/approval items, reminders and sign-off queues within Harmonia's operational responsibility; clinical requirements and decisions remain with accountable clinical authority.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.9.8. To Do Management Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-to-do-management-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Workflow & Activity Coordination (p. 248)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:186 (p. 459)	Linked RAD detail; exact locators in manifest.

Provides To Do inbox and sign-off coordination without acquiring underlying clinical-work management.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.9.9. Coordinate Synthetic Task

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-coordinate-synthetic-task`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Workflow & Activity Coordination (p. 248)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:189 (p. 459)	Linked RAD detail; exact locators in manifest.

Orchestrates automated system workflows and operational progression.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.9.10. Synthetic Task Execution Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-synthetic-task-execution-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Workflow & Activity Coordination (p. 248)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:190 (p. 459)	Linked RAD detail; exact locators in manifest.

Coordinates automated tasks.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.9.11. Supervise Activity Timeout / Escalation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-supervise-activity-timeout-escalation`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Workflow & Activity Coordination (p. 248)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:193 (p. 459)	Linked RAD detail; exact locators in manifest.

Monitors SLAs and triggers escalations when clinical or operational deadlines elapse.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.9.12. Activity Escalation Telemetry

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-activity-escalation-telemetry`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Workflow & Activity Coordination (p. 248)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:194 (p. 459)	Linked RAD detail; exact locators in manifest.

Discloses SLA breaches.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.9.13. Work Order Progression Process

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-PR-work-order-progression-process`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Workflow & Activity Coordination (p. 248)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	processes:282 (p. 410); processes:283 (p. 410)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.5.9.14. To Do Progression Process

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-PR-to-do-progression-process`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Workflow & Activity Coordination (p. 248)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	processes:297 (p. 411); processes:298 (p. 411)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.5.9.15. Synthetic Task Progression Process

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-PR-synthetic-task-progression-process`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Workflow & Activity Coordination (p. 248)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	processes:322 (p. 411); processes:323 (p. 411)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.5.10. Calendar Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-calendar-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Core	Independent of Architectural Status.
Source	enabling:574 (p. 369); enabling:658 (p. 371); ise:200 (p. 460)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Schedule Projection Integration (p. 254)	Feature	`FEAT-ISE-24`
Temporal Event Correlation (p. 255)	Feature	`FEAT-ISE-25`
Project Operational Schedule (p. 255)	Function	`HCM-FN-project-operational-schedule`
Unified Calendar Projection Query (p. 255)	Service	`HCM-SV-unified-calendar-projection-query`
Correlate Activity with Temporal Context (p. 256)	Function	`HCM-FN-correlate-activity-with-temporal-context`
Temporal Context Resolution (p. 256)	Service	`HCM-SV-temporal-context-resolution`

3.5.10.1. Schedule Projection Integration

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-24`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Calendar Management (p. 254)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:660 (p. 372)	Linked RAD detail; exact locators in manifest.

Project clinician rosters, on-call schedules, and facility operating calendars into operational views.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.10.2. Temporal Event Correlation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-25`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Calendar Management (p. 254)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:661 (p. 372)	Linked RAD detail; exact locators in manifest.

Correlate incoming clinical events with scheduled appointments and operational calendar milestones.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.10.3. Project Operational Schedule

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-project-operational-schedule`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Calendar Management (p. 254)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:205 (p. 460)	Linked RAD detail; exact locators in manifest.

Aggregates and overlays multidisciplinary schedules onto a unified temporal view.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.10.4. Unified Calendar Projection Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-unified-calendar-projection-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Calendar Management (p. 254)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:206 (p. 460)	Linked RAD detail; exact locators in manifest.

Discloses consolidated temporal schedules.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.10.5. Correlate Activity with Temporal Context

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-correlate-activity-with-temporal-context`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Calendar Management (p. 254)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:208 (p. 460)	Linked RAD detail; exact locators in manifest.

Associates clinical events with shift boundaries and appointment slots.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.10.6. Temporal Context Resolution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-temporal-context-resolution`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Calendar Management (p. 254)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:209 (p. 460)	Linked RAD detail; exact locators in manifest.

Resolves operational shifts.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.11. Presentation Services

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-presentation-services`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Core	Independent of Architectural Status.
Source	enabling:575 (p. 369); enabling:663 (p. 372); ise:215 (p. 461)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Longitudinal Clinical & Operational Presentation (p. 257)	Feature	`FEAT-ISE-26`
Present Contextual Health and Operational Information (p. 257)	Function	`HCM-FN-present-contextual-health-and-operational-information`
Contextual Presentation Service (p. 257)	Service	`HCM-SV-contextual-presentation-service`

3.5.11.1. Longitudinal Clinical & Operational Presentation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-26`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Presentation Services (p. 256)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:665 (p. 372)	Linked RAD detail; exact locators in manifest.

Deliver contextual clinical timelines, directory administration views, and operational management dashboards.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.11.2. Present Contextual Health and Operational Information

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-present-contextual-health-and-operational-information`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Presentation Services (p. 256)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:220 (p. 461)	Linked RAD detail; exact locators in manifest.

Composes clinical summaries, work queues, and patient records into responsive role-based user interfaces.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.11.3. Contextual Presentation Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-contextual-presentation-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Presentation Services (p. 256)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.

Name	Value	Comment / Description
Source	ise:221 (p. 461)	Linked RAD detail; exact locators in manifest.

Delivers contextual health and operational information for presentation.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.12. Information Design Governance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-information-design-governance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Harmonia Relevance	Harmonia-Core	Independent of Architectural Status.
Source	enabling:576 (p. 369); enabling:667 (p. 372); ise:227 (p. 461)	Linked RAD detail; exact locators in manifest.

Unresolved dimensions: tier; root status; ancestry. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Information Standards & Semantic Governance (p. 259)	Feature	`FEAT-ISE-27`
Semantic Conformance Verification (p. 259)	Feature	`FEAT-ISE-28`
Govern Information Semantics (p. 259)	Function	`HCM-FN-govern-information-semantics`
Semantic Model Definition Service (p. 260)	Service	`HCM-SV-semantic-model-definition-service`
Govern Information Definition (p. 260)	Function	`HCM-FN-govern-information-definition`
Information Dictionary Query (p. 260)	Service	`HCM-SV-information-dictionary-query`
Govern Terminology Binding (p. 261)	Function	`HCM-FN-govern-terminology-binding`
Terminology Binding Specification Lookup (p. 261)	Service	`HCM-SV-terminology-binding-specification-lookup`
Govern Exchange Profile (p. 261)	Function	`HCM-FN-govern-exchange-profile`
Exchange Profile Specification Query (p. 262)	Service	`HCM-SV-exchange-profile-specification-query`
Verify Semantic Conformance (p. 262)	Function	`HCM-FN-verify-semantic-conformance`
Semantic Conformance Verification Service (p. 262)	Service	`HCM-SV-semantic-conformance-verification-service`

3.5.12.1. Information Standards & Semantic Governance

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-27`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Information Design Governance (p. 258)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:669 (p. 372)	Linked RAD detail; exact locators in manifest.

Govern canonical clinical data definitions, terminology bindings, and exchange profiles across regional systems.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.12.2. Semantic Conformance Verification

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-28`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	Information Design Governance (p. 258)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:670 (p. 372)	Linked RAD detail; exact locators in manifest.

Verify inbound and outbound representations against published semantic rules and conformance constraints.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.12.3. Govern Information Semantics

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-govern-information-semantics`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Information Design Governance (p. 258)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:231 (p. 461)	Linked RAD detail; exact locators in manifest.

Authors and validates canonical data concepts and entity relationships.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.12.4. Semantic Model Definition Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-semantic-model-definition-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Information Design Governance (p. 258)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:232 (p. 461)	Linked RAD detail; exact locators in manifest.

Discloses canonical model definitions.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.12.5. Govern Information Definition

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-govern-information-definition`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Information Design Governance (p. 258)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:234 (p. 461)	Linked RAD detail; exact locators in manifest.

Maintains governed data element dictionaries, data types, and structural rules.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.5.12.6. Information Dictionary Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-information-dictionary-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Information Design Governance (p. 258)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:235 (p. 461)	Linked RAD detail; exact locators in manifest.

Provides data dictionary specifications.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.5.12.7. Govern Terminology Binding

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-govern-terminology-binding`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Information Design Governance (p. 258)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:237 (p. 461)	Linked RAD detail; exact locators in manifest.

Binds canonical data elements to authoritative terminology value sets.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.5.12.8. Terminology Binding Specification Lookup

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-terminology-binding-specification-lookup`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Information Design Governance (p. 258)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:238 (p. 461)	Linked RAD detail; exact locators in manifest.

Discloses value set bindings.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.5.12.9. Govern Exchange Profile

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-govern-exchange-profile`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Information Design Governance (p. 258)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:240 (p. 461)	Linked RAD detail; exact locators in manifest.

Defines standards-based interchange profiles and message constraints.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.5.12.10. Exchange Profile Specification Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-exchange-profile-specification-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Information Design Governance (p. 258)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:241 (p. 461)	Linked RAD detail; exact locators in manifest.

Discloses interoperability schemas.

Unresolved dimensions: ancestry; feature association. See the linked RAD detail and uncertainty register.

3.5.12.11. Verify Semantic Conformance

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-verify-semantic-conformance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Information Design Governance (p. 258)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:243 (p. 461)	Linked RAD detail; exact locators in manifest.

Validates candidate messages against canonical governance schemas.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.5.12.12. Semantic Conformance Verification Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-semantic-conformance-verification-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Information Design Governance (p. 258)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	ise:244 (p. 461)	Linked RAD detail; exact locators in manifest.

Validates representations for compliance.

Unresolved dimensions: ancestry. See the linked RAD detail and uncertainty register.

3.6. Contextual view unestablished

Capability	Reference Id
Assurance Design (p. 263)	`HCM-BEC-assurance-design`
Assurance Criteria Management (p. 264)	`HCM-BEC-assurance-criteria-management`
Governed Assurance (p. 265)	`HCM-BEC-governed-assurance`

3.6.1. Assurance Design

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-assurance-design`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:680 (p. 372)	Linked RAD detail; exact locators in manifest.
Supports	Health Service Assurance	Source-scoped; no inherited descendant edges.

- **Definition**: The capability to determine and define how satisfaction of a governed requirement, constraint or expected behaviour will be assured, including its assurance disposition, applicable criteria, required evidence and evaluation expectations.

Unresolved dimensions: tier; root status; ancestry; contextual view; feature decomposition; relevance. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Design Service Assurance (p. 263)	Function	`HCM-FN-design-service-assurance`
Assurance Process Design (p. 264)	Process	`HCM-PR-assurance-process-design`

3.6.1.1. Design Service Assurance

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-design-service-assurance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Assurance Design (p. 263)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	assurance:71 (p. 416)	Linked RAD detail; exact locators in manifest.

- **Definition**: Defines how satisfaction of a governed requirement, constraint or expected behaviour is to be independently established, including the applicable assurance disposition, assurance criteria, required evidence and evaluation expectations.

Unresolved dimensions: ancestry; feature association; contextual view. See the linked RAD detail and uncertainty register.

3.6.1.2. Assurance Process Design

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-PR-assurance-process-design`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Assurance Design (p. 263)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	processes:355 (p. 412); processes:358 (p. 412)	Linked RAD detail; exact locators in manifest.

- **Process Purpose**: Establishes and maintains how satisfaction of a governed requirement, constraint or expected behaviour is to be assured.

Unresolved dimensions: ancestry; feature association; contextual view. See the linked RAD detail and uncertainty register.

3.6.2. Assurance Criteria Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-assurance-criteria-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:686 (p. 372)	Linked RAD detail; exact locators in manifest.
Supports	Health Service Assurance	Source-scoped; no inherited descendant edges.

- **Definition**: The capability to establish and manage reusable, governed and temporally identifiable assurance criteria through which subjects may be evaluated.

Unresolved dimensions: tier; root status; ancestry; contextual view; feature decomposition; relevance. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Manage Assurance Criteria (p. 264)	Function	`HCM-FN-manage-assurance-criteria`

3.6.2.1. Manage Assurance Criteria

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-manage-assurance-criteria`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Assurance Criteria Management (p. 264)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.

Name	Value	Comment / Description
Source	assurance:84 (p. 417)	Linked RAD detail; exact locators in manifest.

- **Definition**: Establishes and manages reusable, governed and temporally identifiable assurance criteria through which governed subjects may be evaluated.

Unresolved dimensions: ancestry; feature association; contextual view. See the linked RAD detail and uncertainty register.

3.6.3. Governed Assurance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-governed-assurance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	enabling:694 (p. 373)	Linked RAD detail; exact locators in manifest.
Supports	Health Service Assurance	Source-scoped; no inherited descendant edges.

- **Definition**: The capability to independently evaluate a governed subject against applicable assurance criteria through the assembly and evaluation of sufficient trustworthy evidence, establishing explicit findings, exceptions and assurance conclusions.

Unresolved dimensions: tier; root status; ancestry; contextual view; feature decomposition; relevance. See the linked RAD detail and uncertainty register.

Established owned elements

Element	Type	Reference Id
Establish Assurance Context (p. 265)	Function	`HCM-FN-establish-assurance-context`
Assess Assurance Evidence (p. 266)	Function	`HCM-FN-assess-assurance-evidence`
Adjudicate Assurance Assessment (p. 266)	Function	`HCM-FN-adjudicate-assurance-assessment`
Request Service Assurance (p. 267)	Service	`HCM-SV-request-service-assurance`
Request Service Assurance Status (p. 267)	Service	`HCM-SV-request-service-assurance-status`
Communicate Service Assurance Outcome (p. 267)	Service	`HCM-SV-communicate-service-assurance-outcome`
Assurance Process Execution (p. 268)	Process	`HCM-PR-assurance-process-execution`
Assurance Process Reporting / Communication (p. 268)	Process	`HCM-PR-assurance-process-reporting-communication`

3.6.3.1. Establish Assurance Context

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-establish-assurance-context`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Governed Assurance (p. 265)	Responsibility; no invented intermediate element.

Name	Value	Comment / Description
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	assurance:98 (p. 417)	Linked RAD detail; exact locators in manifest.

- **Definition**: Establishes the governed subject, applicable assurance purpose and criteria, relevant temporal context, and contextual association of sufficient trustworthy source information as evidence required for an assurance activity.

Unresolved dimensions: ancestry; feature association; contextual view. See the linked RAD detail and uncertainty register.

3.6.3.2. Assess Assurance Evidence

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-assess-assurance-evidence`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Governed Assurance (p. 265)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	assurance:108 (p. 417)	Linked RAD detail; exact locators in manifest.

- **Definition**: Evaluates the available trustworthy evidence against the applicable assurance criteria and establishes what that evidence demonstrates for each relevant criterion or assessment concern.

Unresolved dimensions: ancestry; feature association; contextual view. See the linked RAD detail and uncertainty register.

3.6.3.3. Adjudicate Assurance Assessment

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-adjudicate-assurance-assessment`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Governed Assurance (p. 265)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	assurance:116 (p. 418)	Linked RAD detail; exact locators in manifest.

- **Definition**: Determines the assurance finding or conclusion that follows from the assessed evidence and applicable assurance criteria.

Unresolved dimensions: ancestry; feature association; contextual view. See the linked RAD detail and uncertainty register.

3.6.3.4. Request Service Assurance

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-request-service-assurance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Governed Assurance (p. 265)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	assurance:164 (p. 419)	Linked RAD detail; exact locators in manifest.

- **Definition**: Enables an authorised consumer to request that Service Assurance be performed for an applicable governed subject in accordance with an established Assurance Definition.

Unresolved dimensions: ancestry; feature association; contextual view; function mapping. See the linked RAD detail and uncertainty register.

3.6.3.5. Request Service Assurance Status

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-request-service-assurance-status`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Governed Assurance (p. 265)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	assurance:175 (p. 420)	Linked RAD detail; exact locators in manifest.

- **Definition**: Enables the System Steward to obtain the current business progression state of an identifiable Service Assurance activity for stewardship purposes.

Unresolved dimensions: ancestry; feature association; contextual view; function mapping. See the linked RAD detail and uncertainty register.

3.6.3.6. Communicate Service Assurance Outcome

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-communicate-service-assurance-outcome`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Governed Assurance (p. 265)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	assurance:189 (p. 420)	Linked RAD detail; exact locators in manifest.

- **Definition**: Communicates applicable established Assurance Findings and Conclusions to authorised recipients according to their responsibilities, authority and information requirements.

Unresolved dimensions: ancestry; feature association; contextual view; function mapping. See the linked RAD detail and uncertainty register.

3.6.3.7. Assurance Process Execution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-PR-assurance-process-execution`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Governed Assurance (p. 265)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	processes:367 (p. 412); processes:370 (p. 412)	Linked RAD detail; exact locators in manifest.

- **Process Purpose**: Performs defined assurance for an identifiable governed subject in accordance with an applicable Assurance Definition and establishes an Assurance Finding / Conclusion from applicable trustworthy evidence.

Unresolved dimensions: ancestry; feature association; contextual view. See the linked RAD detail and uncertainty register.

3.6.3.8. Assurance Process Reporting / Communication

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-PR-assurance-process-reporting-communication`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	Governed Assurance (p. 265)	Responsibility; no invented intermediate element.
Structural Canonical ID	Unresolved	Unallocated in RAD; Reference Id is separate.
Source	processes:389 (p. 412); processes:392 (p. 413)	Linked RAD detail; exact locators in manifest.

- **Process Purpose**: Communicates applicable established Assurance Findings and Conclusions to legitimate recipients according to their responsibilities, authority and information requirements, without altering the established assurance outcome or assuming responsibility for the recipient's subsequent action.

Unresolved dimensions: ancestry; feature association; contextual view. See the linked RAD detail and uncertainty register.

4. Enterprise Capabilities

These are the fourteen governed reusable architectural capabilities. Functional responsibility bullets are descriptive responsibilities, not additional named Function or Service identities. No CT decomposition or additional Enterprise peers are inferred.

4.1. Managed Entity & Relationship

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	EC-01	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Source	enterprise:103 (p. 465)	Linked RAD detail; exact locators in manifest.

- **Architectural Scope**: Identify, classify, relate, and maintain the structural integrity of managed healthcare and administrative entities throughout the enterprise landscape.
- **Functional Responsibilities**:
  - Uniquely identifying core domain entities (Persons, Practitioners, Roles, Organisations, Locations, Services, Devices).
  - Resolving and maintaining multi-authority identifier systems and aliases without destructive overwriting.
  - Representing and traversing multi-dimensional relationships (affiliations, organizational hierarchies, physical location trees, care team memberships).
  - Maintaining referential integrity across interconnected entity graphs.
- **Anti-Responsibilities**: Does not own physical database relational tables or foreign key constraints (Domain 04/07).

4.2. Context Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	EC-02	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Source	enterprise:112 (p. 465)	Linked RAD detail; exact locators in manifest.

- **Architectural Scope**: Establish, validate, propagate, and preserve the execution, clinical, security, and administrative context within which information and activities have meaning.
- **Functional Responsibilities**:
  - Binding the subject of care, encounter, ordering provider, and healthcare service context to in-flight messages and transactions.
  - Propagating tamper-evident, attributable security context across asynchronous, multi-hop operational boundaries.
  - Ensuring context preservation during cross-protocol transformations across disparate healthcare exchange formats and protocols.
  - Detecting and rejecting requests where mandatory operational or clinical context is missing or inconsistent.
- **Collaborative Nature**: Bounded reusable context responsibility may be realised through participation across gateways, activity execution and information-access boundaries. Participation does not transfer semantic ownership.

4.3. Managed State & Lifecycle

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`EC-03`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Source	enterprise:121 (p. 466)	Linked RAD detail; exact locators in manifest.

- **Architectural Scope**: Govern the current and historical state, effective temporal periods, state transition validations, and lifecycle progression of managed integration artifacts and entities.
- **Functional Responsibilities**:
  - Defining and enforcing valid state transition state machines (e.g., `REQUESTED` → `VALIDATED` → `IN_PROGRESS` → `COMPLETED`).
  - Maintaining temporal validity intervals (effective start and end dates) for relationships, credentials, and states.
  - Supporting non-destructive state progression, preserving historical state versions for longitudinal review.
  - Rejecting illegal or out-of-order state transitions while accommodating idempotent replay.
- **Anti-Responsibilities**: Does not define thread-level mutexes or database transaction isolation levels.

4.4. Information Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`EC-04`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Source	enterprise:130 (p. 466)	Linked RAD detail; exact locators in manifest.

- **Architectural Scope**: Validate, represent, persist, retrieve, version, and manage governed clinical, administrative, and operational information assets.
- **Functional Responsibilities**:
  - Validating inbound information structures against canonical information constraints.
  - Providing authoritative durable persistence for historical health records and administrative registries.
  - Managing active distributed working memory representations for low-latency retrieval.
  - Governing document versioning, supersession, amendments, and retractions without data loss.
- **Architectural Principle**: Enforces the foundational separation between *active distributed state access* and *authoritative durable state preservation* (Axiom AX-05).

4.5. Search & Discovery

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`EC-05`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Source	enterprise:139 (p. 466)	Linked RAD detail; exact locators in manifest.

- **Architectural Scope**: Index, search, resolve, filter, and discover information assets, entities, directory endpoints, and healthcare services across the enterprise.
- **Functional Responsibilities**:
  - Providing multi-criteria indexed search across complex entity graphs (e.g., find active cardiologists at Location X accepting new referrals).
  - Resolving service and practitioner communication endpoints for addressed delivery.
  - Supporting bounded, paginated clinical queries across longitudinal patient records.
  - Applying dynamic security and consent filters to search results prior to egress.
- **Anti-Responsibilities**: Does not dictate specific search engine indexes, inverted index implementations, or SQL query syntaxes.

4.6. Policy & Control

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`EC-06`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Source	enterprise:148 (p. 467)	Linked RAD detail; exact locators in manifest.

- **Architectural Scope**: Evaluate and enforce security, consent, privacy, and regulatory policies including authority, access control, and information handling rules.
- **Functional Responsibilities**:
  - Executing impartial, default-deny policy evaluations for all integration transactions.
  - Enforcing Role-Based and Attribute-Based Access Control (RBAC/ABAC) policies.
  - Evaluating patient consent directives and privacy restrictions at boundary and query checkpoints.
  - Enforcing statutory information disclosure boundaries (e.g., mental health, VIP records).
- **Collaborative Nature**: Evaluated across boundary gateways, internal bus dispatch, storage interfaces, and presentation layers.

4.7. Provenance & Traceability

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`EC-07`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Source	enterprise:157 (p. 467)	Linked RAD detail; exact locators in manifest.

- **Architectural Scope**: Preserve required origin attribution, meaningful transformation history, significant execution checkpoints, tamper-evident audit evidence and end-to-end traceability. Harmonia SHALL preserve provenance, audit and operational evidence where required by governing obligations, architectural significance, accountability, reconstruction, security or assurance needs. Routine technical activity SHALL NOT automatically require durable evidentiary recording.
- **Functional Responsibilities**:
  - Recording attributable, verifiable origin metadata for information sources, authors, and timestamps.
  - Tracking information-significant transformation history and required mapping checkpoints as information crosses system seams.
  - Capturing non-PHI operational and security audit trails for regulatory compliance (HIPAA, GDPR).
  - Supporting end-to-end transaction tracing across asynchronous processing pipelines.

- **Collaborative Nature**: Participating elements preserve required provenance and audit context according to policy. Telemetry, operational facts, provenance, audit and assurance evidence remain distinct. Being evidence is contextual, not an intrinsic property of every recorded fact; preservation does not establish an assurance conclusion.

4.8. Interoperability & Exchange

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`EC-08`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Source	enterprise:166 (p. 467)	Linked RAD detail; exact locators in manifest.

- **Architectural Scope**: Resolve destinations, transform schemas, route, transport, deliver, acknowledge, and coordinate standards-based information exchange across enterprise boundaries.
- **Functional Responsibilities**:
  - Providing multi-protocol boundary adaptation for healthcare standards (HL7 v2, FHIR REST, DICOM, secure messaging).
  - Executing bidirectional structural and semantic transformations between legacy formats and canonical representations.
  - Ensuring reliable addressed delivery and fan-out distribution to external clinical endpoints.
  - Processing and distinguishing technical delivery acknowledgements from business acknowledgements.
- **Boundary Rule**: Egress terminates Harmonia management of the emitted representation (Axiom AX-13).

4.9. Event & Subscription

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`EC-09`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Source	enterprise:175 (p. 468)	Linked RAD detail; exact locators in manifest.

- **Architectural Scope**: Publish, ingest, correlate, filter, and syndicate events and subscription-driven changes across the enterprise landscape.
- **Functional Responsibilities**:
  - Ingesting operational and clinical event triggers from external systems and internal state changes.
  - Managing topic- and criteria-based event subscriptions for internal and external consumers.
  - Evaluating dynamic subscription filters to match event payloads with interested subscriber criteria.
  - Delivering asynchronous event notifications reliably to registered subscribers.
- **Decoupling Note**: Defined strictly in terms of event publishing, criteria matching, and notification delivery; entirely independent of broker topics, partitions, or queues.

4.10. Activity & Execution

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`EC-10`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Source	enterprise:184 (p. 468)	Linked RAD detail; exact locators in manifest.

- **Architectural Scope**: Define, instantiate, match, assign, dispatch, supervise, and progress coordinated operational and clinical activity units.
- **Functional Responsibilities**:
  - Instantiating operational work items (Work Orders, To Dos, synthetic platform Tasks) in response to triggers.
  - Matching and assigning work units to eligible human or automated performers and roles based on policy rules.
  - Dispatching work units to their assigned performers with acknowledgement according to the applicable interaction contract.
  - Supervising execution progress, tracking timeouts, managing escalations, and handling activity interruptions.
- **Decoupling Note**: Free of worker threads, thread pools, execution daemons, or specific workflow runtime engines.

4.11. Interaction & Experience

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`EC-11`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Source	enterprise:193 (p. 468)	Linked RAD detail; exact locators in manifest.

- **Architectural Scope**: Provide contextual interaction between human actors, Harmonia, and enterprise presentation services, including visualization, alerting, and collaboration integration.
- **Functional Responsibilities**:
  - Mediating presentation interactions via dedicated backend-for-frontend gateways decoupled from internal databases.
  - Rendering clinical timelines, provider directory management interfaces, and operational console dashboards.
  - Providing contextual notifications and alerts to human users across web and mobile channels.
  - Projecting longitudinal record context into secure multidisciplinary collaboration spaces.
- **Boundary Note**: Does not absorb the healthcare semantics of Clinical Collaboration or medical documentation. Consumes established identity and security context; does not own authentication, consent, or clinical truth.

4.12. Operational Assurance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	EC-12	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Source	enterprise:202 (p. 468)	Linked RAD detail; exact locators in manifest.

- **Architectural Scope**: Ensure concurrency integrity, system resilience, duplicate detection, exception handling, automated recovery, and operational telemetry across platform operations.
- **Functional Responsibilities**:
  - Enforcing sliding-window duplicate message detection and idempotent suppression.
  - Guaranteeing ingress dual-write safety (durable acceptance before emitting positive application ACK).
  - Managing structured failure recovery, exception escalation, and dead-letter isolation.
  - Emitting platform telemetry, operational health metrics, and queue processing metrics.
- **Collaborative Nature**: Realized collaboratively across boundary gateways, messaging infrastructure, execution workers, and storage layers.

EC-12 / EC-14 Boundary

EC-12 retains the established scope and responsibilities above. It may contribute directly to generic operational/processing assurance, limited to that contribution in the approved assurance matrix. It does not own the general semantics of assurance disposition, assurance-specific criteria applicability, evidentiary association, assessment against assurance criteria, adjudication, or independent assurance findings and conclusions. Those distinguishing responsibilities are introduced through EC-14 Service Guardian. Operational health and business-level processing assurance remain distinct; EC-12 is not expanded to absorb EC-14.

4.13. Semantic Governance & Conformance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	EC-13	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Source	enterprise:215 (p. 469)	Linked RAD detail; exact locators in manifest.

- **Architectural Scope**: Govern canonical models, schemas, terminologies, value sets, mapping tables, constraint rules, and semantic conformance validation across the enterprise.
- **Functional Responsibilities**:
  - Defining and publishing canonical healthcare data models and profile structures.
  - Governing clinical terminology mappings, value sets, and concept lookups (SNOMED CT, LOINC).
  - Validating inbound and outbound message representations against published semantic rules.
  - Governing semantic versioning, deprecation, and backward compatibility across schema revisions.
- **Component Relationship**: Informs the Calliope strategic logical component, but Calliope is the responsibility centre, not the capability definition.

4.14. Service Guardian

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	EC-14	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Source	enterprise:224 (p. 469)	Linked RAD detail; exact locators in manifest.

- **Architectural Scope**: Service Guardian provides the reusable capability to independently establish whether governed service behaviour, state or outcomes remain within defined assurance expectations by associating applicable criteria and trustworthy evidence with a governed subject, assessing that evidence, adjudicating the assessment, and establishing reportable assurance findings and conclusions.
- **Functional Responsibilities**:
  - Supplying assurance-specific semantics for assurance disposition and assurance criteria applicability.
  - Associating a governed subject/context with applicable criteria and sufficient trustworthy evidence for a particular assurance purpose.
  - Assessing evidence against applicable criteria and adjudicating assessed detail to establish reportable findings, exceptions and conclusions.
  - Preserving assurance independence, temporal/version-specific evidence and criteria applicability, and explicit inability to establish a conclusion where evidence is insufficient.
- **Collaborative Nature**: Supplies the assurance-specific semantics required to bind existing reusable capabilities into governed assurance. It contributes directly to Assurance Design, Assurance Criteria Management and Governed Assurance; direct contribution is not sole realisation. The approved matrix preserves the contributions of EC-01 through EC-13.
- **Anti-Responsibilities**: Does not own the underlying governed subject, authoritative governance requirements, or source information used as evidence. Does not perform, manage, remediate, assign, delegate or operationally escalate the subject activity. Does not replace System Operations or EC-12 Operational Assurance, and does not provide Clinical Services Delivery Assurance.
- **Allocation Boundary**: No strategic logical component, application component, runtime service or implementation construct is allocated EC-14. The catalogue defines a capability, not a software component or deployment boundary.

Conceptual Behavioural Sequence

RAD source listing (presentation only; no inferred ancestry).

Define Good Behaviour

↓

Gather Evidence

↓

Assess Evidence against Good Behaviour

↓

Adjudicate Assessed Detail

↓

Report

This sequence explains the capability; it is not an implementation workflow specification or a derived Business Process. “Define Good Behaviour” identifies assurance expectations and criteria; it does not transfer ownership of governing requirements to Service Guardian. Source information ownership remains with its responsible capability; assurance owns its contextual evidentiary association.

The Business Enabling assurance principles govern disposition, generic processing assurance, evidence/criteria temporality, assessment/adjudication, independence, failure/recovery and explicit evidence insufficiency. These semantics are not universally reducible to PASS / FAIL / UNKNOWN and do not imply an assurance information model or persistence mechanism.

5. Established and unresolved relationship register

Only source-evidenced assertions are listed. Unresolved associations are explicitly unestablished, not approved edges. The BC evidence/context table and the approved EC contribution matrix retain their exact semantics in the source detail; they are not coerced into these relationship types.

5.1. Capability Decomposition

From	To	Status / evidence
Individual Care Delivery (p. 4)	Individual Health Assessment (p. 4)	Canonical; business:162 (p. 323)
Individual Care Delivery (p. 4)	Clinical Determination (p. 6)	Canonical; business:188 (p. 323)
Individual Care Delivery (p. 4)	Individual Care Planning (p. 6)	Canonical; business:213 (p. 324)
Individual Care Delivery (p. 4)	Clinical Intervention (p. 7)	Canonical; business:239 (p. 324)
Individual Care Delivery (p. 4)	Individual Health Monitoring (p. 7)	Canonical; business:265 (p. 324)
Individual Care Delivery (p. 4)	Individual Care Review (p. 8)	Canonical; business:291 (p. 325)
Individual Health Assessment (p. 4)	Health History Assessment (p. 5)	Canonical; business:326 (p. 325); business:322 (p. 325)
Individual Health Assessment (p. 4)	Clinical Examination (p. 5)	Canonical; business:327 (p. 325); business:322 (p. 325)
Individual Health Assessment (p. 4)	Health Status Measurement (p. 6)	Canonical; business:328 (p. 325); business:322 (p. 325)
Individual Health Monitoring (p. 7)	Health State Monitoring (p. 8)	Canonical; business:342 (p. 326); business:338 (p. 326)
Individual Health Monitoring (p. 7)	Care Response Monitoring (p. 8)	Canonical; business:343 (p. 326); business:338 (p. 326)
Individual Care Review (p. 8)	Care Appropriateness Review (p. 9)	Canonical; business:357 (p. 326); business:353 (p. 326)
Individual Care Review (p. 8)	Care Effectiveness Review (p. 9)	Canonical; business:358 (p. 326); business:353 (p. 326)
Care Access & Coordination (p. 9)	Care Access Management (p. 10)	Canonical; business:372 (p. 326)
Care Access Management (p. 10)	Care Intake Management (p. 10)	Canonical; business:380 (p. 326); business:376 (p. 326)
Care Access Management (p. 10)	Care Referral Management (p. 11)	Canonical; business:381 (p. 326); business:376 (p. 326)
Care Access Management (p. 10)	Care Eligibility Determination (p. 11)	Canonical; business:382 (p. 326); business:376 (p. 326)
Care Access & Coordination (p. 9)	Care Scheduling & Capacity Management (p. 11)	Canonical; business:388 (p. 327)
Care Scheduling & Capacity Management (p. 11)	Care Appointment Management (p. 12)	Canonical; business:396 (p. 327); business:392 (p. 327)
Care Scheduling & Capacity Management (p. 11)	Care-Place Allocation (p. 12)	Canonical; business:397 (p. 327); business:392 (p. 327)
Care Scheduling & Capacity Management (p. 11)	Care-Place Readiness Management (p. 13)	Canonical; business:398 (p. 327); business:392 (p. 327)
Care Scheduling & Capacity Management (p. 11)	Care Resource Deployment (p. 13)	Canonical; business:399 (p. 327); business:392 (p. 327)
Care Scheduling & Capacity Management (p. 11)	Operational Care Flow Coordination (p. 13)	Canonical; business:400 (p. 327); business:392 (p. 327)

From	To	Status / evidence
Care Scheduling & Capacity Management (p. 11)	Operational Care Capacity Management (p. 14)	Canonical; business:401 (p. 327); business:392 (p. 327)
Care Access & Coordination (p. 9)	Care Coordination & Transition (p. 14)	Canonical; business:407 (p. 327)
Care Coordination & Transition (p. 14)	Care-Team Coordination (p. 14)	Canonical; business:415 (p. 327); business:411 (p. 327)
Care Coordination & Transition (p. 14)	Care Responsibility Transition (p. 15)	Canonical; business:416 (p. 327); business:411 (p. 327)
Care Coordination & Transition (p. 14)	Care Discharge Coordination (p. 15)	Canonical; business:417 (p. 327); business:411 (p. 327)
Care Access & Coordination (p. 9)	Care Support Logistics Management (p. 15)	Canonical; business:423 (p. 328)
Care Support Logistics Management (p. 15)	Patient Movement Coordination (p. 16)	Canonical; business:431 (p. 328); business:427 (p. 328)
Care Support Logistics Management (p. 15)	Clinical Material Logistics Coordination (p. 16)	Canonical; business:432 (p. 328); business:427 (p. 328)
Care Support Logistics Management (p. 15)	Care Support Work Coordination (p. 16)	Canonical; business:433 (p. 328); business:427 (p. 328)
Diagnostic, Therapeutic & Clinical Support Services (p. 17)	Diagnostic Support Provision (p. 18)	Canonical; business:459 (p. 328)
Diagnostic Support Provision (p. 18)	Diagnostic Service Appointment Management (p. 18)	Canonical; business:467 (p. 329); business:463 (p. 328)
Diagnostic Support Provision (p. 18)	Diagnostic Service Delivery (p. 19)	Canonical; business:468 (p. 329); business:463 (p. 328)
Diagnostic Support Provision (p. 18)	Diagnostic Specimen Management (p. 19)	Canonical; business:469 (p. 329); business:463 (p. 328)
Diagnostic Support Provision (p. 18)	Diagnostic Data Management (p. 19)	Canonical; business:470 (p. 329); business:463 (p. 328)
Diagnostic Support Provision (p. 18)	Diagnostic Interpretation (p. 20)	Canonical; business:471 (p. 329); business:463 (p. 328)
Diagnostic Support Provision (p. 18)	Diagnostic Report Management (p. 20)	Canonical; business:472 (p. 329); business:463 (p. 328)
Diagnostic Support Provision (p. 18)	Diagnostic Report Distribution (p. 20)	Canonical; business:473 (p. 329); business:463 (p. 328)
Diagnostic, Therapeutic & Clinical Support Services (p. 17)	Medication & Pharmacy Services (p. 21)	Canonical; business:479 (p. 329)
Medication & Pharmacy Services (p. 21)	Prescription Medication Dispensing (p. 21)	Canonical; business:487 (p. 329); business:483 (p. 329)
Medication & Pharmacy Services (p. 21)	Non-Prescription Medication Provision (p. 22)	Canonical; business:488 (p. 329); business:483 (p. 329)
Medication & Pharmacy Services (p. 21)	Medication Advice (p. 22)	Canonical; business:489 (p. 329); business:483 (p. 329)
Medication & Pharmacy Services (p. 21)	Medication Dose Administration Aid Provision (p. 22)	Canonical; business:490 (p. 329); business:483 (p. 329)
Health Products & Clinical Technology (p. 23)	Clinical Technology Management (p. 23)	Canonical; business:504 (p. 330)
Clinical Technology Management (p. 23)	Clinical Device Inventory Management (p. 24)	Canonical; business:512 (p. 330); business:508 (p. 330)
Clinical Technology Management (p. 23)	Clinical Device Feature & Release Management (p. 24)	Canonical; business:513 (p. 330); business:508 (p. 330)
Clinical Technology Management (p. 23)	Clinical Device Loan & Deployment Management (p. 24)	Canonical; business:514 (p. 330); business:508 (p. 330)
Clinical Technology Management (p. 23)	Clinical Device Utilisation Support (p. 25)	Canonical; business:515 (p. 330); business:508 (p. 330)
Clinical Technology Management (p. 23)	Clinical Device Integration Provision (p. 25)	Canonical; business:516 (p. 330); business:508 (p. 330)

From	To	Status / evidence
Clinical Technology Management (p. 23)	Clinical Device Data Integration (p. 25)	Canonical; business:517 (p. 330); business:508 (p. 330)
Clinical Technology Management (p. 23)	Clinical Device Lifecycle Management (p. 26)	Canonical; business:518 (p. 330); business:508 (p. 330)
Clinical Quality, Safety & Improvement (p. 26)	Clinical Safety Management (p. 27)	Canonical; business:532 (p. 330)
Clinical Safety Management (p. 27)	Clinical Risk Identification (p. 27)	Canonical; business:540 (p. 331); business:536 (p. 331)
Clinical Safety Management (p. 27)	Clinical Risk Mitigation Management (p. 27)	Canonical; business:541 (p. 331); business:536 (p. 331)
Clinical Safety Management (p. 27)	Clinical Risk Response Management (p. 28)	Canonical; business:542 (p. 331); business:536 (p. 331)
Clinical Quality, Safety & Improvement (p. 26)	Clinical Quality Management (p. 28)	Canonical; business:548 (p. 331)
Clinical Quality Management (p. 28)	Clinical Quality Criteria Management (p. 29)	Canonical; business:556 (p. 331); business:552 (p. 331)
Clinical Quality Management (p. 28)	Clinical Quality Evidence Management (p. 29)	Canonical; business:557 (p. 331); business:552 (p. 331)
Clinical Quality Management (p. 28)	Clinical Quality Review & Assessment Management (p. 29)	Canonical; business:558 (p. 331); business:552 (p. 331)
Clinical Quality Management (p. 28)	Clinical Quality Reporting (p. 30)	Canonical; business:559 (p. 331); business:552 (p. 331)
Community Health & Wellbeing (p. 30)	Preventative Health (p. 30)	Canonical; business:573 (p. 331)
Preventative Health (p. 30)	Preventative Health Problem Identification (p. 31)	Canonical; business:581 (p. 332); business:577 (p. 332)
Preventative Health (p. 30)	Preventative Health Programme Management (p. 31)	Canonical; business:582 (p. 332); business:577 (p. 332)
Preventative Health (p. 30)	Preventative Health Education Services (p. 32)	Canonical; business:583 (p. 332); business:577 (p. 332)
Preventative Health (p. 30)	Preventative Health Programme Monitoring (p. 32)	Canonical; business:584 (p. 332); business:577 (p. 332)
Preventative Health (p. 30)	Preventative Health Reporting (p. 32)	Canonical; business:585 (p. 332); business:577 (p. 332)
Preventative Health (p. 30)	Preventative Health Cohort Management (p. 33)	Canonical; business:586 (p. 332); business:577 (p. 332)
Population Health & Health-System Planning (p. 33)	Population Health Surveillance (p. 33)	Canonical; business:600 (p. 332)
Population Health Surveillance (p. 33)	Population Health Longitudinal Assessment (p. 34)	Canonical; business:608 (p. 332); business:604 (p. 332)
Population Health Surveillance (p. 33)	Population Health Surveillance Management (p. 34)	Canonical; business:609 (p. 332); business:604 (p. 332)
Population Health Surveillance (p. 33)	Population Health Event Identification (p. 35)	Canonical; business:610 (p. 332); business:604 (p. 332)
Population Health Surveillance (p. 33)	Notifiable Health Event Management (p. 35)	Canonical; business:611 (p. 332); business:604 (p. 332)
Population Health Surveillance (p. 33)	Population Health Reporting (p. 35)	Canonical; business:612 (p. 332); business:604 (p. 332)
Population Health & Health-System Planning (p. 33)	Health-System Planning (p. 36)	Canonical; business:618 (p. 333)
Health-System Planning (p. 36)	Health Service Demand Assessment (p. 36)	Canonical; business:626 (p. 333); business:622 (p. 333)
Health-System Planning (p. 36)	Health Service Capacity Planning (p. 37)	Canonical; business:627 (p. 333); business:622 (p. 333)
Health-System Planning (p. 36)	Health Service Distribution Planning (p. 37)	Canonical; business:628 (p. 333); business:622 (p. 333)

From	To	Status / evidence
Health Information & Knowledge Management (p. 37)	Health Information Management (p. 38)	Canonical; business:646 (p. 333)
Health Information Management (p. 38)	Health Information Data Management (p. 38)	Canonical; business:654 (p. 333); business:650 (p. 333)
Health Information Management (p. 38)	Health Information Structure Management (p. 39)	Canonical; business:655 (p. 333); business:650 (p. 333)
Health Information Management (p. 38)	Health Information Lifecycle Management (p. 39)	Canonical; business:656 (p. 333); business:650 (p. 333)
Health Information Management (p. 38)	Health Information Catalogue and Index Management (p. 39)	Canonical; business:657 (p. 333); business:650 (p. 333)
Health Information Management (p. 38)	Health Information Compartmentalisation (p. 40)	Canonical; business:658 (p. 333); business:650 (p. 333)
Health Information Management (p. 38)	Health Information Data Correlation (p. 40)	Canonical; business:659 (p. 333); business:650 (p. 333)
Health Information Management (p. 38)	Health Information Data Retrieval (p. 40)	Canonical; business:660 (p. 333); business:650 (p. 333)
Health Information & Knowledge Management (p. 37)	Clinical Knowledge Management (p. 41)	Canonical; business:666 (p. 334)
Clinical Knowledge Management (p. 41)	Clinical Knowledge Context Identification (p. 41)	Canonical; business:674 (p. 334); business:670 (p. 334)
Clinical Knowledge Management (p. 41)	Clinical Knowledge Context Extraction (p. 42)	Canonical; business:675 (p. 334); business:670 (p. 334)
Clinical Knowledge Management (p. 41)	Clinical Knowledge Information Formulation (p. 42)	Canonical; business:676 (p. 334); business:670 (p. 334)
Clinical Knowledge Management (p. 41)	Clinical Knowledge Concept Association (p. 42)	Canonical; business:677 (p. 334); business:670 (p. 334)
Clinical Knowledge Management (p. 41)	Clinical Knowledge Review & Approval Management (p. 43)	Canonical; business:678 (p. 334); business:670 (p. 334)
Clinical Knowledge Management (p. 41)	Clinical Knowledge Publication & Sharing (p. 43)	Canonical; business:679 (p. 334); business:670 (p. 334)
Standards, Semantics & Reference Governance (p. 43)	Health Information Standards Governance (p. 44)	Canonical; business:693 (p. 335)
Health Information Standards Governance (p. 44)	Information Model Discovery (p. 44)	Canonical; business:701 (p. 335); business:697 (p. 335)
Health Information Standards Governance (p. 44)	Information Model Adoption (p. 45)	Canonical; business:702 (p. 335); business:697 (p. 335)
Health Information Standards Governance (p. 44)	Information Model Education (p. 45)	Canonical; business:703 (p. 335); business:697 (p. 335)
Health Information Standards Governance (p. 44)	Information Model Compliance (p. 45)	Canonical; business:704 (p. 335); business:697 (p. 335)
Health Information Standards Governance (p. 44)	Information Model Governance Oversight (p. 46)	Canonical; business:705 (p. 335); business:697 (p. 335)
Health Information Standards Governance (p. 44)	Standards Adoption (p. 46)	Canonical; business:706 (p. 335); business:697 (p. 335)
Health Information Standards Governance (p. 44)	Standards Impact Identification (p. 46)	Canonical; business:707 (p. 335); business:697 (p. 335)
Standards, Semantics & Reference Governance (p. 43)	Clinical Semantics Governance (p. 47)	Canonical; business:713 (p. 335)
Clinical Semantics Governance (p. 47)	Terminology Governance (p. 47)	Canonical; business:721 (p. 335); business:717 (p. 335)
Clinical Semantics Governance (p. 47)	Terminology Utilisation Oversight (p. 47)	Canonical; business:722 (p. 335); business:717 (p. 335)
Clinical Semantics Governance (p. 47)	Terminology Localisation (p. 48)	Canonical; business:723 (p. 335); business:717 (p. 335)
Clinical Semantics Governance (p. 47)	Classification Governance (p. 48)	Canonical; business:724 (p. 335); business:717 (p. 335)
Clinical Semantics Governance (p. 47)	Classification Utilisation Oversight (p. 48)	Canonical; business:725 (p. 335); business:717 (p. 335)

From	To	Status / evidence
Clinical Semantics Governance (p. 47)	Semantic Mapping Governance (p. 49)	Canonical; business:726 (p. 335); business:717 (p. 335)
Connected Health Services (p. 49)	Health Information Exchange (p. 50)	Canonical; business:740 (p. 336)
Connected Health Services (p. 49)	Digital Communication Channel (p. 50)	Canonical; business:750 (p. 336)
Security, Privacy & Digital Trust (p. 50)	Digital Identity & Access Governance (p. 51)	Canonical; business:768 (p. 337)
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5.3. Associated Feature

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Confidentiality Restriction Evaluation (p. 115)	Confidentiality Flag Enforcement (p. 114)	Canonical; em:77 (p. 427)
Practitioner Verification Service (p. 82)	National Practitioner Verification (p. 79)	Canonical; em:93 (p. 427)
Practitioner Directory Query (p. 82)	Practitioner Profile Governance (p. 80)	Canonical; em:97 (p. 427)
Practitioner Role Resolution (p. 83)	Role & Specialty Modeling (p. 80)	Canonical; em:100 (p. 427)
Practitioner Endpoint Lookup (p. 84)	Electronic Endpoint Binding (p. 80)	Canonical; em:103 (p. 427)
Practitioner Affiliation Resolution (p. 84)	Practitioner-Organisation Affiliation (p. 81)	Canonical; em:106 (p. 427)
Clinical Privilege Verification (p. 85)	Clinical Privileging Record (p. 81)	Canonical; em:109 (p. 427)
Organisation Verification Service (p. 88)	National Organisation Verification (p. 86)	Canonical; em:122 (p. 428)
Organisation Directory Query (p. 89)	Organisation Profile Management (p. 87)	Canonical; em:125 (p. 428)
Organisational Hierarchy Resolution (p. 89)	Hierarchical Structure Navigation (p. 87)	Canonical; em:128 (p. 428)
Organisation Contact Lookup (p. 90)	Directory Contact Management (p. 87)	Canonical; em:131 (p. 428)

From	To	Status / evidence
Location Directory Query (p. 92)	Location Geospatial & Address Resolution (p. 91)	Canonical; em:146 (p. 429)
Location Hierarchy Resolution (p. 93)	Physical Hierarchy Modeling (p. 91)	Canonical; em:149 (p. 429)
Care-Place Specification Lookup (p. 93)	Atomic Care-Place Modeling (p. 91)	Canonical; em:152 (p. 429)
Healthcare Service Directory Query (p. 96)	Service Catalogue Publishing (p. 94)	Canonical; em:175 (p. 429)
Service Provision Resolution (p. 97)	Service-Location-Provider Binding (p. 95)	Canonical; em:178 (p. 429)
Service Availability Query (p. 97)	Operating Schedule Specification (p. 95)	Canonical; em:181 (p. 429)
Service Eligibility Evaluation (p. 98)	Eligibility & Catchment Specification (p. 95)	Canonical; em:184 (p. 429)
Device Definition Query (p. 101)	Device Type Specification (p. 99)	Canonical; em:197 (p. 430)
Device Instance Resolution (p. 102)	Device Serial Tracking (p. 100)	Canonical; em:200 (p. 430)
Device Association Query (p. 102)	Device-Patient/Location Assignment (p. 100)	Canonical; em:203 (p. 430)
Device Endpoint Resolution (p. 103)	Device Endpoint Registration (p. 100)	Canonical; em:206 (p. 430)
Demographic Intake Service (p. 118)	Demographic Update Ingestion (p. 117)	Canonical; sa:19 (p. 431)
Demographic Update Notification (p. 118)	Demographic Change Distribution (p. 117)	Canonical; sa:22 (p. 431)
Referral Submission Service (p. 121)	Referral Document Ingestion (p. 119)	Canonical; sa:34 (p. 431)
Referral Context Query (p. 121)	Referral Supporting Information (p. 120)	Canonical; sa:37 (p. 431)
Referral Status & Outcome Service (p. 122)	Referral Status & Outcome Tracking (p. 120)	Canonical; sa:40 (p. 431)
Appointment Notification Ingress (p. 124)	Appointment Notification Ingestion (p. 123)	Canonical; sa:54 (p. 432)
Appointment Schedule Query (p. 125)	Appointment Status Synchronization (p. 124)	Canonical; sa:57 (p. 432)
Encounter Context Resolution (p. 127)	Encounter Context Creation (p. 126)	Canonical; sa:69 (p. 432)
Encounter Lifecycle Event Notification (p. 128)	Encounter State Progression (p. 126)	Canonical; sa:72 (p. 432)
Encounter Location History Query (p. 129)	Encounter Bed/Location Association (p. 127)	Canonical; sa:77 (p. 432)
Order Requisition Ingress (p. 132)	Order Request Ingestion (p. 130)	Canonical; sa:90 (p. 433)
Order Dispatch Service (p. 132)	Order Destination Resolution & Routing (p. 130)	Canonical; sa:93 (p. 433)
Order Status & Tracking Query (p. 133)	Order Closed-Loop Progression Tracking (p. 131)	Canonical; sa:96 (p. 433)
Order Cancellation Service (p. 134)	Order Cancellation & Modification Coordination (p. 131)	Canonical; sa:101 (p. 433)
Diagnostic Correlation Resolution (p. 137)	Diagnostic Request Correlation (p. 136)	Canonical; sa:117 (p. 434)
Diagnostic Report Ingress (p. 138)	Diagnostic Report Ingestion & Binding (p. 136)	Canonical; sa:120 (p. 434)
Diagnostic Result Delivery Service (p. 138)	Diagnostic Report Distribution (p. 136)	Canonical; sa:124 (p. 434)

From	To	Status / evidence
Medication Order Ingress (p. 141)	Medication Order Ingestion (p. 139)	Canonical; sa:137 (p. 434)
Dispense Event Ingress (p. 141)	Dispense Event Tracking (p. 140)	Canonical; sa:140 (p. 434)
Medication Administration Ingress (p. 142)	Medication Administration Record (MAR) Ingestion (p. 140)	Canonical; sa:143 (p. 434)
Procedure Booking Ingress (p. 144)	Procedure Booking Ingestion (p. 143)	Canonical; sa:154 (p. 435)
Procedural Documentation Ingress (p. 144)	Procedural Documentation Ingestion (p. 143)	Canonical; sa:157 (p. 435)
Clinical Document Ingress (p. 147)	Clinical Document Ingestion (p. 145)	Canonical; sa:169 (p. 435)
Clinical Document Lifecycle Service (p. 147)	Document Versioning & Supersession (p. 146)	Canonical; sa:172 (p. 435)
Document Metadata Registry Query (p. 148)	Document Metadata Indexing (p. 146)	Canonical; sa:177 (p. 435)
Care Plan Ingress (p. 150)	Care Plan Ingestion & Distribution (p. 149)	Canonical; sa:187 (p. 436)
Secure Clinical Message Dispatch (p. 154)	Clinical Communication Distribution (p. 152)	Canonical; sa:207 (p. 436)
Communication Delivery Status Query (p. 154)	Delivery Acknowledgement Tracking (p. 153)	Canonical; sa:211 (p. 436)
Communication Audit Query (p. 155)	Communication Audit Logging (p. 153)	Canonical; sa:214 (p. 436)
Primary Care Summary Ingress (p. 158)	General Practice Shared Record Ingestion (p. 157)	Canonical; sd:56 (p. 439)
Acute Admission Dossier Query (p. 161)	Acute Care Clinical Context Provision (p. 160)	Canonical; sd:76 (p. 440)
ED Triage Stream Ingress (p. 164)	Emergency Department Arrival & Triage Ingestion (p. 163)	Canonical; sd:96 (p. 440)
Emergency Fast-Path LHR Query (p. 164)	Emergency Care Cross-Facility Correlation (p. 163)	Canonical; sd:99 (p. 440)
Ward Round Clinical Briefing Query (p. 166)	Inpatient Rounding Context Provision (p. 165)	Canonical; sd:115 (p. 441)
Longitudinal Medication History Query (p. 173)	Medication History Reconciliation Ingestion (p. 172)	Canonical; sd:152 (p. 442)
Immunisation History Query (p. 176)	Immunisation Event Submission (p. 175)	Canonical; sd:169 (p. 442)
Community Care Encounter Ingress (p. 179)	Community Encounter Ingestion (p. 178)	Canonical; sd:188 (p. 443)
Outreach Batch Reconciliation Service (p. 182)	Outreach Encounter Outcome Ingestion (p. 181)	Canonical; sd:211 (p. 443)
Virtual Encounter Context Resolution (p. 184)	Virtual Care Session Context Binding (p. 183)	Canonical; sd:226 (p. 444)
Remote Telemetry Ingress (p. 184)	Remote Telemetry Ingestion (p. 183)	Canonical; sd:229 (p. 444)
Clinic Queue Telemetry Service (p. 188)	Operational Queue Progression (p. 186)	Canonical; hso:83 (p. 447)
Theatre Case Progression Telemetry (p. 193)	Operating Theatre Case Progression (p. 191)	Canonical; hso:113 (p. 448)
Bed Availability Query (p. 199)	Bed Availability Tracking (p. 198)	Canonical; hso:158 (p. 449)
Bed Turnover Lifecycle Service (p. 200)	Bed State Progression (p. 198)	Canonical; hso:161 (p. 449)
Clinical Resource Availability Query (p. 202)	Mobile Clinical Equipment Tracking (p. 201)	Canonical; hso:173 (p. 450)

From	To	Status / evidence
Regional Health System Capacity Query (p. 203)	Operational Capacity Metric Aggregation (p. 203)	Canonical; hso:183 (p. 450)
On-Call Specialist Lookup (p. 208)	Active On-Call Provider Resolution (p. 208)	Canonical; hso:215 (p. 451)
Work Dispatch Service (p. 211)	Work Item Instantiation (p. 209)	Canonical; hso:228 (p. 452)
Operational Work Status Query (p. 212)	Work Progress Oversight & Escalation (p. 210)	Canonical; hso:232 (p. 452)
Patient Transport Tracking Service (p. 216)	Transport Dispatch & Progress Tracking (p. 216)	Canonical; hso:254 (p. 452)
Specimen Transit Tracking Service (p. 218)	Pathology Specimen Transport Tracking (p. 217)	Canonical; hso:268 (p. 453)
Discharge Readiness Telemetry (p. 221)	Discharge Readiness & Coordination Oversight (p. 220)	Canonical; hso:283 (p. 453)
Longitudinal Clinical Record Query (p. 225)	Longitudinal Health Record (LHR) Assembly (p. 224)	Canonical; ise:40 (p. 456)
Active Problem & Allergy Summary Query (p. 226)	Active Clinical Record Access (p. 224)	Canonical; ise:43 (p. 456)
Historical Clinical Record Access (p. 226)	Durable Clinical Record Preservation (p. 224)	Canonical; ise:46 (p. 456)
HIE Submission Gateway Service (p. 229)	Direct Submission Ingestion & Delivery (p. 227)	Canonical; ise:58 (p. 456)
Federated Health Information Query (p. 230)	Clinical Record Retrieval Mediation (p. 228)	Canonical; ise:61 (p. 456)
Addressed Document Delivery Service (p. 230)	Addressed Clinical Distribution (p. 228)	Canonical; ise:64 (p. 456)
Clinical Event Syndication Stream (p. 231)	Event-Driven Syndication (p. 228)	Canonical; ise:67 (p. 456)
Clinical Information Search Service (p. 233)	Federated Clinical Query (p. 232)	Canonical; ise:79 (p. 457)
Standards Communication Gateway Service (p. 235)	Standards-Based Boundary Exchange (p. 235)	Canonical; ise:98 (p. 457)
Policy Evaluation & Authorisation Service (p. 238)	Health Information Access & Consent Control (p. 237)	Canonical; ise:109 (p. 458)
Security Audit Ingress (p. 240)	Non-PHI Compliance Auditing (p. 237)	Canonical; ise:120 (p. 458)
Terminology Concept Resolution Service (p. 242)	Clinical Concept Resolution (p. 241)	Canonical; ise:131 (p. 458)
Terminology Mapping Service (p. 242)	Bidirectional Terminology Mapping (p. 241)	Canonical; ise:134 (p. 458)
Clinical Collaboration Space Service (p. 245)	Care-Team Clinical Collaboration Coordination (p. 243)	Canonical; ise:147 (p. 459)
Collaboration Clinical Summary Resolution (p. 245)	In-Conversation LHR Query Resolution (p. 244)	Canonical; ise:150 (p. 459)
Collaboration Space Governance Service (p. 246)	Zero-PHI Collaboration Metadata Governance (p. 244)	Canonical; ise:155 (p. 459)
Operational Collaboration Service (p. 248)	Operational Team Chat Enablement (p. 247)	Canonical; ise:165 (p. 459)
Work Order Coordination Service (p. 250)	Work Order Progression (p. 249)	Canonical; ise:182 (p. 459)
To Do Management Service (p. 251)	To Do Decision & Approval Coordination (p. 249)	Canonical; ise:186 (p. 459)
Synthetic Task Execution Service (p. 252)	Synthetic Task Orchestration (p. 249)	Canonical; ise:190 (p. 459)
Activity Escalation Telemetry (p. 252)	Workflow Timeout & Escalation Oversight (p. 250)	Canonical; ise:194 (p. 459)
Unified Calendar Projection Query (p. 255)	Schedule Projection Integration (p. 254)	Canonical; ise:206 (p. 460)

From	To	Status / evidence
Temporal Context Resolution (p. 256)	Temporal Event Correlation (p. 255)	Canonical; ise:209 (p. 460)
Contextual Presentation Service (p. 257)	Longitudinal Clinical & Operational Presentation (p. 257)	Canonical; ise:221 (p. 461)
Semantic Model Definition Service (p. 260)	Information Standards & Semantic Governance (p. 259)	Canonical; ise:232 (p. 461)
Semantic Conformance Verification Service (p. 262)	Semantic Conformance Verification (p. 259)	Canonical; ise:244 (p. 461)
Chronic Care Timeline Query (p. 159)	Primary Care Notification Dispatch (p. 158)	**Unresolved; association not established**; sd:58 (p. 439)
Acute Clinical Alert Dispatch (p. 162)	Acute Clinical State Event Capture (p. 160)	**Unresolved; association not established**; sd:78 (p. 440)
Discharge Dossier Packaging Service (p. 167)	Inpatient Clinical Progression Tracking (p. 166)	**Unresolved; association not established**; sd:117 (p. 441)
Modality Worklist Distribution (p. 170)	Laboratory & Imaging Result Distribution (p. 169)	**Unresolved; association not established**; sd:133 (p. 441)
Diagnostic Publication Service (p. 170)	Diagnostic History Consolidation (p. 169)	**Unresolved; association not established**; sd:137 (p. 441)
Allergy & Adverse Reaction Query (p. 173)	Medication Administration Tracking (p. 172)	**Unresolved; association not established**; sd:154 (p. 442)
Preventive Screening Status Query (p. 176)	Screening Recall Notification Distribution (p. 175)	**Unresolved; association not established**; sd:171 (p. 442)
Community Care Team Query (p. 179)	Community Care Plan Synchronization (p. 178)	**Unresolved; association not established**; sd:190 (p. 443)
Staff Presence Telemetry Query (p. 206)	Mobile Worker Task Dispatch (p. 205)	**Unresolved; association not established**; hso:196 (p. 450)
Clinical Resource Retrieval Service (p. 234)	Access-Controlled Information Filtering (p. 232)	**Unresolved; association not established**; ise:81 (p. 457)
Consent Enforcement Service (p. 239)	Security Context Propagation (p. 237)	**Unresolved; association not established**; ise:111 (p. 458)

5.4. Participating Function

From	To	Status / evidence
Assurance Process Design (p. 264)	Design Service Assurance (p. 263)	Canonical; processes:361 (p. 412)
Assurance Process Design (p. 264)	Manage Assurance Criteria (p. 264)	Canonical; processes:361 (p. 412)
Assurance Process Execution (p. 268)	Establish Assurance Context (p. 265)	Canonical; processes:373 (p. 412)
Assurance Process Execution (p. 268)	Assess Assurance Evidence (p. 266)	Canonical; processes:373 (p. 412)
Assurance Process Execution (p. 268)	Adjudicate Assurance Assessment (p. 266)	Canonical; processes:373 (p. 412)

5.5. Responsibility Parent

From	To	Status / evidence
Client Administration (p. 78)	Person Identity (p. 103)	Canonical; em:16 (p. 425)

5.6. Contributes Feature

From	To	Status / evidence
Manage Ward Capacity (p. 189)	Ward Patient Flow Coordination (p. 189)	Canonical; hso:96 (p. 447)

5.7. Enabled By

From	To	Status / evidence
Individual Care Delivery (p. 4)	Health Information Access (p. 231)	Canonical; business:314 (p. 325)
Individual Health Assessment (p. 4)	Health Information Access (p. 231)	Canonical; business:314 (p. 325)
Clinical Determination (p. 6)	Health Information Access (p. 231)	Canonical; business:314 (p. 325)
Individual Care Planning (p. 6)	Health Information Access (p. 231)	Canonical; business:314 (p. 325)
Clinical Intervention (p. 7)	Health Information Access (p. 231)	Canonical; business:314 (p. 325)
Individual Health Monitoring (p. 7)	Health Information Access (p. 231)	Canonical; business:314 (p. 325)
Individual Care Review (p. 8)	Health Information Access (p. 231)	Canonical; business:314 (p. 325)
Health Service Assurance (p. 74)	Assurance Design (p. 263)	Canonical; enabling:676 (p. 372)
Health Service Assurance (p. 74)	Assurance Criteria Management (p. 264)	Canonical; enabling:676 (p. 372)
Health Service Assurance (p. 74)	Governed Assurance (p. 265)	Canonical; enabling:676 (p. 372)

5.8. Governed Process

From	To	Status / evidence
Apply Governed Person Identity Correction (p. 107)	Governed Person Identity Correction Process (p. 108)	Canonical; em:35 (p. 425)
Verify Practitioner Registration (p. 81)	Practitioner Verification Process (p. 85)	Canonical; em:94 (p. 427)
Manage Referral Progression (p. 122)	Referral Progression Process (p. 122)	Canonical; sa:41 (p. 431)
Progress Encounter State (p. 128)	Encounter Lifecycle Process (p. 129)	Canonical; sa:73 (p. 432)
Manage Order Progression (p. 133)	Closed-Loop Order Progression Process (p. 135)	Canonical; sa:97 (p. 433)
Govern Clinical Document Lifecycle (p. 147)	Clinical Document Lifecycle Process (p. 148)	Canonical; sa:173 (p. 435)
Manage Clinic Queue Progression (p. 187)	Clinic / Operational Queue Progression Process (p. 188)	Canonical; hso:84 (p. 447)
Manage Theatre Case Progression (p. 192)	Theatre Case Progression Process (p. 193)	Canonical; hso:115 (p. 448)
Manage Bed Turnover Progression (p. 200)	Bed Turnover Process (p. 200)	Canonical; hso:163 (p. 449)
Manage Operational Work Progression (p. 211)	Operational Work Progression Process (p. 212)	Canonical; hso:233 (p. 452)
Manage Patient Transport Progression (p. 216)	Patient Transport Process (p. 217)	Canonical; hso:255 (p. 452)
Manage Specimen Transport Progression (p. 218)	Specimen Transport Process (p. 218)	Canonical; hso:269 (p. 453)

From	To	Status / evidence
Manage Discharge Coordination Progression (p. 220)	Discharge Progression Process (p. 221)	Canonical; hso:284 (p. 453)
Coordinate Work Order (p. 250)	Work Order Progression Process (p. 253)	Canonical; ise:183 (p. 459)
Coordinate To Do (p. 251)	To Do Progression Process (p. 253)	Canonical; ise:187 (p. 459)
Coordinate Synthetic Task (p. 251)	Synthetic Task Progression Process (p. 253)	Canonical; ise:191 (p. 459)

5.9. Uses Process

From	To	Status / evidence
Request Service Assurance (p. 267)	Assurance Process Execution (p. 268)	Canonical; assurance:170 (p. 420)

5.10. Exposes Process

From	To	Status / evidence
Communicate Service Assurance Outcome (p. 267)	Assurance Process Reporting / Communication (p. 268)	Canonical; assurance:195 (p. 420)

6. Catalogue descriptions with unresolved element identity

Of 73 named enabling decomposition descriptions, four independently identify established BEC contexts. The other 69 do not establish additional Capabilities or tiers. Their names and source wording remain in the full BEC transcription. The context keys below are navigation locators, not allocated architectural identities, ancestors or aliases.

Source description	Identity standing	Source / established context
Person Identity	Canonical Capability identity	enabling:115 (p. 356); Person Identity (p. 103)
Healthcare Subject	Canonical Capability identity	enabling:122 (p. 356); Healthcare Subject (p. 108)
Client Relationship	Canonical Capability identity	enabling:127 (p. 356); Client Relationship (p. 111)
Client Privacy	Canonical Capability identity	enabling:132 (p. 356); Client Privacy (p. 113)
Practitioner	Unresolved standalone element identity	enabling:140 (p. 356)
Practitioner Role	Unresolved standalone element identity	enabling:145 (p. 356)
Provider Relationship	Unresolved standalone element identity	enabling:150 (p. 356)
Provider Engagement	Unresolved standalone element identity	enabling:154 (p. 356)
Healthcare Organisation	Unresolved standalone element identity	enabling:161 (p. 357)
Organisational Structure	Unresolved standalone element identity	enabling:166 (p. 357)
Organisation Contact	Unresolved standalone element identity	enabling:170 (p. 357)
Healthcare Location	Unresolved standalone element identity	enabling:177 (p. 357)
Location Structure	Unresolved standalone element identity	enabling:181 (p. 357)
Care-Place Definition	Unresolved standalone element identity	enabling:185 (p. 357)
Healthcare Service Definition	Unresolved standalone element identity	enabling:192 (p. 358)
Service Provision Relationship	Unresolved standalone element identity	enabling:196 (p. 358)
Service Availability Definition	Unresolved standalone element identity	enabling:200 (p. 358)
Service Eligibility Definition	Unresolved standalone element identity	enabling:204 (p. 358)
Health Product Definition	Unresolved standalone element identity	enabling:210 (p. 358)
Product Formulary & Catalogue	Unresolved standalone element identity	enabling:211 (p. 358)
Product Batch & Serial Tracking	Unresolved standalone element identity	enabling:212 (p. 358)
Device Definition	Unresolved standalone element identity	enabling:218 (p. 358)
Device Instance	Unresolved standalone element identity	enabling:222 (p. 358)

Source description	Identity standing	Source / established context
Device Association	Unresolved standalone element identity	enabling:226 (p. 358)
Device Connectivity	Unresolved standalone element identity	enabling:230 (p. 358)
Client Intake & Demographic Update	Unresolved standalone element identity	enabling:280 (p. 360)
Referral Intake & Progression	Unresolved standalone element identity	enabling:285 (p. 360)
Appointment & Booking Administration	Unresolved standalone element identity	enabling:291 (p. 360)
Encounter Lifecycle Management	Unresolved standalone element identity	enabling:296 (p. 360)
Closed-Loop Order Progression	Unresolved standalone element identity	enabling:302 (p. 361)
Diagnostic Workflow Management	Unresolved standalone element identity	enabling:309 (p. 361)
Medication Order & Dispense Management	Unresolved standalone element identity	enabling:317 (p. 361)
Procedure & Surgical Administration	Unresolved standalone element identity	enabling:323 (p. 361)
Clinical Documentation Lifecycle	Unresolved standalone element identity	enabling:328 (p. 362)
Care Plan Coordination	Unresolved standalone element identity	enabling:334 (p. 362)
Secure Clinical Communication	Unresolved standalone element identity	enabling:338 (p. 362)
Primary Care Coordination	Unresolved standalone element identity	enabling:390 (p. 363)
Acute Care Monitoring & Event Coordination	Unresolved standalone element identity	enabling:395 (p. 363)
Emergency Care Progression	Unresolved standalone element identity	enabling:400 (p. 364)
Multidisciplinary Inpatient Care Coordination	Unresolved standalone element identity	enabling:405 (p. 364)
Diagnostic Service Integration	Unresolved standalone element identity	enabling:410 (p. 364)
Medication Therapy Provision and Monitoring	Unresolved standalone element identity	enabling:415 (p. 364)
Screening and Immunisation Enablement	Unresolved standalone element identity	enabling:420 (p. 364)
Community Care Coordination	Unresolved standalone element identity	enabling:425 (p. 364)
Outreach Care Continuity	Unresolved standalone element identity	enabling:430 (p. 365)
Remote Patient Monitoring	Unresolved standalone element identity	enabling:435 (p. 365)
Session & Queue Coordination	Unresolved standalone element identity	enabling:482 (p. 366)
Ward Logistics Coordination	Unresolved standalone element identity	enabling:487 (p. 366)
Perioperative Logistics	Unresolved standalone element identity	enabling:492 (p. 366)
ED Operational Coordination	Unresolved standalone element identity	enabling:497 (p. 366)
Outpatient Flow Enablement	Unresolved standalone element identity	enabling:502 (p. 367)
Bed State & Turnover Management	Unresolved standalone element identity	enabling:507 (p. 367)

Source description	Identity standing	Source / established context
Specialized Equipment Coordination	Unresolved standalone element identity	enabling:513 (p. 367)
Facility Demand & Capacity Telemetry	Unresolved standalone element identity	enabling:517 (p. 367)
Mobile Worker Dispatch & Safety	Unresolved standalone element identity	enabling:521 (p. 367)
On-Call Directory Resolution	Unresolved standalone element identity	enabling:527 (p. 367)
Operational Work Assignment & Supervision	Unresolved standalone element identity	enabling:531 (p. 368)
Credential Verification Checkpoint	Unresolved standalone element identity	enabling:540 (p. 368)
Patient Transfer Coordination	Unresolved standalone element identity	enabling:544 (p. 368)
Specimen & Supply Courier Tracking	Unresolved standalone element identity	enabling:549 (p. 368)
Discharge Progression & Coordination	Unresolved standalone element identity	enabling:553 (p. 368)
Longitudinal Clinical Record Governance	Unresolved standalone element identity	enabling:609 (p. 370)
Multi-Model Information Exchange	Unresolved standalone element identity	enabling:615 (p. 370)
Governed Query & Search	Unresolved standalone element identity	enabling:622 (p. 370)
Boundary Protocol Mediation	Unresolved standalone element identity	enabling:627 (p. 370)
Policy & Security Governance	Unresolved standalone element identity	enabling:631 (p. 371)
Terminology & Reference Mapping	Unresolved standalone element identity	enabling:637 (p. 371)
LHR-Aware Multidisciplinary Collaboration	Unresolved standalone element identity	enabling:642 (p. 371)
Operational Messaging	Unresolved standalone element identity	enabling:648 (p. 371)
Multi-Work-Unit Orchestration	Unresolved standalone element identity	enabling:652 (p. 371)
Operational & Clinical Calendar Sync	Unresolved standalone element identity	enabling:659 (p. 372)
Decoupled Human Interaction & Dashboards	Unresolved standalone element identity	enabling:664 (p. 372)
Canonical Semantics & Schema Governance	Unresolved standalone element identity	enabling:668 (p. 372)

7. Identifier register

Reference IDs and labels are frozen in manifest.json. The register is sorted by Reference Id and includes candidates with their actual status.

Reference Id	Element / page	Type / status
EC-01	Managed Entity & Relationship (p. 269)	enterprise capability / Canonical
EC-02	Context Management (p. 269)	enterprise capability / Canonical
EC-03	Managed State & Lifecycle (p. 270)	enterprise capability / Canonical
EC-04	Information Management (p. 270)	enterprise capability / Canonical
EC-05	Search & Discovery (p. 270)	enterprise capability / Canonical
EC-06	Policy & Control (p. 271)	enterprise capability / Canonical
EC-07	Provenance & Traceability (p. 271)	enterprise capability / Canonical
EC-08	Interoperability & Exchange (p. 272)	enterprise capability / Canonical
EC-09	Event & Subscription (p. 272)	enterprise capability / Canonical
EC-10	Activity & Execution (p. 273)	enterprise capability / Canonical
EC-11	Interaction & Experience (p. 273)	enterprise capability / Canonical
EC-12	Operational Assurance (p. 274)	enterprise capability / Canonical
EC-13	Semantic Governance & Conformance (p. 274)	enterprise capability / Canonical
EC-14	Service Guardian (p. 275)	enterprise capability / Canonical
FEAT-EM-01	Identifier Resolution (p. 104)	feature / Canonical
FEAT-EM-02	Cross-Authority Identifier Correlation (p. 104)	feature / Canonical
FEAT-EM-03	Identity Alias Association (p. 105)	feature / Canonical
FEAT-EM-04	Governed Identity Correction (p. 105)	feature / Canonical
FEAT-EM-05	Subject Context Binding (p. 109)	feature / Canonical
FEAT-EM-06	Demographic Context Governance (p. 109)	feature / Canonical
FEAT-EM-07	Next-of-Kin & Guardian Association (p. 111)	feature / Canonical
FEAT-EM-08	Relationship Validity Tracking (p. 112)	feature / Canonical
FEAT-EM-09	Consent Directive Evaluation (p. 114)	feature / Canonical
FEAT-EM-10	Confidentiality Flag Enforcement (p. 114)	feature / Canonical
FEAT-EM-11	National Practitioner Verification (p. 79)	feature / Canonical
FEAT-EM-12	Practitioner Profile Governance (p. 80)	feature / Canonical
FEAT-EM-13	Role & Specialty Modeling (p. 80)	feature / Canonical
FEAT-EM-14	Electronic Endpoint Binding (p. 80)	feature / Canonical

Reference Id	Element / page	Type / status
FEAT-EM-15	Practitioner-Organisation Affiliation (p. 81)	feature / Canonical
FEAT-EM-16	Clinical Privileging Record (p. 81)	feature / Canonical
FEAT-EM-17	National Organisation Verification (p. 86)	feature / Canonical
FEAT-EM-18	Organisation Profile Management (p. 87)	feature / Canonical
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HCM-PR-encounter-lifecycle-process	Encounter Lifecycle Process (p. 129)	process / Canonical
HCM-PR-governed-person-identity-correction-process	Governed Person Identity Correction Process (p. 108)	process / Canonical
HCM-PR-operational-work-progression-process	Operational Work Progression Process (p. 212)	process / Canonical
HCM-PR-patient-transport-process	Patient Transport Process (p. 217)	process / Canonical
HCM-PR-practitioner-verification-process	Practitioner Verification Process (p. 85)	process / Canonical
HCM-PR-referral-progression-process	Referral Progression Process (p. 122)	process / Canonical
HCM-PR-specimen-transport-process	Specimen Transport Process (p. 218)	process / Canonical
HCM-PR-synthetic-task-progression-process	Synthetic Task Progression Process (p. 253)	process / Canonical
HCM-PR-theatre-case-progression-process	Theatre Case Progression Process (p. 193)	process / Canonical
HCM-PR-to-do-progression-process	To Do Progression Process (p. 253)	process / Canonical
HCM-PR-work-order-progression-process	Work Order Progression Process (p. 253)	process / Canonical
HCM-SV-active-problem-allergy-summary-query	Active Problem & Allergy Summary Query (p. 226)	service / Canonical
HCM-SV-activity-escalation-telemetry	Activity Escalation Telemetry (p. 252)	service / Canonical
HCM-SV-acute-admission-dossier-query	Acute Admission Dossier Query (p. 161)	service / Canonical
HCM-SV-acute-clinical-alert-dispatch	Acute Clinical Alert Dispatch (p. 162)	service / Canonical
HCM-SV-addressed-document-delivery-service	Addressed Document Delivery Service (p. 230)	service / Canonical
HCM-SV-allergy-adverse-reaction-query	Allergy & Adverse Reaction Query (p. 173)	service / Canonical
HCM-SV-ambulatory-session-status-query	Ambulatory Session Status Query (p. 197)	service / Canonical
HCM-SV-appointment-notification-ingress	Appointment Notification Ingress (p. 124)	service / Canonical
HCM-SV-appointment-schedule-query	Appointment Schedule Query (p. 125)	service / Canonical
HCM-SV-bed-availability-query	Bed Availability Query (p. 199)	service / Canonical
HCM-SV-bed-turnover-lifecycle-service	Bed Turnover Lifecycle Service (p. 200)	service / Canonical
HCM-SV-care-place-specification-lookup	Care-Place Specification Lookup (p. 93)	service / Canonical
HCM-SV-care-plan-change-notification	Care Plan Change Notification (p. 151)	service / Canonical
HCM-SV-care-plan-context-query	Care Plan Context Query (p. 151)	service / Canonical

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HCM-SV-care-plan-ingress	Care Plan Ingress (p. 150)	service / Canonical
HCM-SV-chronic-care-timeline-query	Chronic Care Timeline Query (p. 159)	service / Canonical
HCM-SV-client-relationship-query	Client Relationship Query (p. 112)	service / Canonical
HCM-SV-clinic-queue-telemetry-service	Clinic Queue Telemetry Service (p. 188)	service / Canonical
HCM-SV-clinical-collaboration-space-service	Clinical Collaboration Space Service (p. 245)	service / Canonical
HCM-SV-clinical-document-ingress	Clinical Document Ingress (p. 147)	service / Canonical
HCM-SV-clinical-document-lifecycle-service	Clinical Document Lifecycle Service (p. 147)	service / Canonical
HCM-SV-clinical-event-syndication-stream	Clinical Event Syndication Stream (p. 231)	service / Canonical
HCM-SV-clinical-information-search-service	Clinical Information Search Service (p. 233)	service / Canonical
HCM-SV-clinical-privilege-verification	Clinical Privilege Verification (p. 85)	service / Canonical
HCM-SV-clinical-resource-availability-query	Clinical Resource Availability Query (p. 202)	service / Canonical
HCM-SV-clinical-resource-retrieval-service	Clinical Resource Retrieval Service (p. 234)	service / Canonical
HCM-SV-collaboration-clinical-summary-resolution	Collaboration Clinical Summary Resolution (p. 245)	service / Canonical
HCM-SV-collaboration-space-governance-service	Collaboration Space Governance Service (p. 246)	service / Canonical
HCM-SV-communicate-service-assurance-outcome	Communicate Service Assurance Outcome (p. 267)	service / Canonical
HCM-SV-communication-audit-query	Communication Audit Query (p. 155)	service / Canonical
HCM-SV-communication-delivery-status-query	Communication Delivery Status Query (p. 154)	service / Canonical
HCM-SV-community-care-encounter-ingress	Community Care Encounter Ingress (p. 179)	service / Canonical
HCM-SV-community-care-team-query	Community Care Team Query (p. 179)	service / Canonical
HCM-SV-confidentiality-restriction-evaluation	Confidentiality Restriction Evaluation (p. 115)	service / Canonical
HCM-SV-consent-decision-service	Consent Decision Service (p. 115)	service / Canonical
HCM-SV-consent-enforcement-service	Consent Enforcement Service (p. 239)	service / Canonical
HCM-SV-contextual-presentation-service	Contextual Presentation Service (p. 257)	service / Canonical
HCM-SV-demographic-change-distribution	Demographic Change Distribution (p. 110)	service / Canonical
HCM-SV-demographic-intake-service	Demographic Intake Service (p. 118)	service / Canonical
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HCM-SV-device-association-query	Device Association Query (p. 102)	service / Canonical
HCM-SV-device-definition-query	Device Definition Query (p. 101)	service / Canonical
HCM-SV-device-endpoint-resolution	Device Endpoint Resolution (p. 103)	service / Canonical
HCM-SV-device-instance-resolution	Device Instance Resolution (p. 102)	service / Canonical
HCM-SV-diagnostic-correlation-resolution	Diagnostic Correlation Resolution (p. 137)	service / Canonical
HCM-SV-diagnostic-publication-service	Diagnostic Publication Service (p. 170)	service / Canonical
HCM-SV-diagnostic-report-ingress	Diagnostic Report Ingress (p. 138)	service / Canonical
HCM-SV-diagnostic-result-delivery-service	Diagnostic Result Delivery Service (p. 138)	service / Canonical
HCM-SV-discharge-dossier-packaging-service	Discharge Dossier Packaging Service (p. 167)	service / Canonical

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HCM-SV-discharge-readiness-telemetry	Discharge Readiness Telemetry (p. 221)	service / Canonical
HCM-SV-dispense-event-ingress	Dispense Event Ingress (p. 141)	service / Canonical
HCM-SV-document-metadata-registry-query	Document Metadata Registry Query (p. 148)	service / Canonical
HCM-SV-ed-operational-status-query	ED Operational Status Query (p. 195)	service / Canonical
HCM-SV-ed-triage-stream-ingress	ED Triage Stream Ingress (p. 164)	service / Canonical
HCM-SV-emergency-fast-path-lhr-query	Emergency Fast-Path LHR Query (p. 164)	service / Canonical
HCM-SV-encounter-context-resolution	Encounter Context Resolution (p. 127)	service / Canonical
HCM-SV-encounter-lifecycle-event-notification	Encounter Lifecycle Event Notification (p. 128)	service / Canonical
HCM-SV-encounter-location-history-query	Encounter Location History Query (p. 129)	service / Canonical
HCM-SV-exchange-profile-specification-query	Exchange Profile Specification Query (p. 262)	service / Canonical
HCM-SV-federated-health-information-query	Federated Health Information Query (p. 230)	service / Canonical
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HCM-SV-healthcare-subject-context-resolution	Healthcare Subject Context Resolution (p. 110)	service / Canonical
HCM-SV-hie-submission-gateway-service	HIE Submission Gateway Service (p. 229)	service / Canonical
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HCM-SV-immunisation-history-query	Immunisation History Query (p. 176)	service / Canonical
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HCM-SV-location-directory-query	Location Directory Query (p. 92)	service / Canonical
HCM-SV-location-hierarchy-resolution	Location Hierarchy Resolution (p. 93)	service / Canonical
HCM-SV-longitudinal-clinical-record-query	Longitudinal Clinical Record Query (p. 225)	service / Canonical
HCM-SV-longitudinal-medication-history-query	Longitudinal Medication History Query (p. 173)	service / Canonical
HCM-SV-medication-administration-ingress	Medication Administration Ingress (p. 142)	service / Canonical
HCM-SV-medication-order-ingress	Medication Order Ingress (p. 141)	service / Canonical
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HCM-SV-operational-work-status-query	Operational Work Status Query (p. 212)	service / Canonical
HCM-SV-order-cancellation-service	Order Cancellation Service (p. 134)	service / Canonical
HCM-SV-order-dispatch-service	Order Dispatch Service (p. 132)	service / Canonical
HCM-SV-order-outcome-notification	Order Outcome Notification (p. 134)	service / Canonical
HCM-SV-order-requisition-ingress	Order Requisition Ingress (p. 132)	service / Canonical
HCM-SV-order-status-tracking-query	Order Status & Tracking Query (p. 133)	service / Canonical
HCM-SV-organisation-contact-lookup	Organisation Contact Lookup (p. 90)	service / Canonical
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HCM-SV-organisational-hierarchy-resolution	Organisational Hierarchy Resolution (p. 89)	service / Canonical
HCM-SV-outreach-batch-reconciliation-service	Outreach Batch Reconciliation Service (p. 182)	service / Canonical
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HCM-SV-person-identifier-correlation	Person Identifier Correlation (p. 106)	service / Canonical
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HCM-SV-practitioner-directory-query	Practitioner Directory Query (p. 82)	service / Canonical
HCM-SV-practitioner-endpoint-lookup	Practitioner Endpoint Lookup (p. 84)	service / Canonical
HCM-SV-practitioner-role-resolution	Practitioner Role Resolution (p. 83)	service / Canonical
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HCM-SV-preventive-screening-status-query	Preventive Screening Status Query (p. 176)	service / Canonical
HCM-SV-primary-care-summary-ingress	Primary Care Summary Ingress (p. 158)	service / Canonical
HCM-SV-procedural-documentation-ingress	Procedural Documentation Ingress (p. 144)	service / Canonical
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HCM-SV-referral-context-query	Referral Context Query (p. 121)	service / Canonical
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HCM-SV-referral-submission-service	Referral Submission Service (p. 121)	service / Canonical
HCM-SV-regional-health-system-capacity-query	Regional Health System Capacity Query (p. 203)	service / Canonical
HCM-SV-remote-telemetry-ingress	Remote Telemetry Ingress (p. 184)	service / Canonical
HCM-SV-representative-authority-verification	Representative Authority Verification (p. 113)	service / Canonical
HCM-SV-request-service-assurance	Request Service Assurance (p. 267)	service / Canonical
HCM-SV-request-service-assurance-status	Request Service Assurance Status (p. 267)	service / Canonical
HCM-SV-secure-clinical-message-dispatch	Secure Clinical Message Dispatch (p. 154)	service / Canonical
HCM-SV-security-audit-ingress	Security Audit Ingress (p. 240)	service / Canonical
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HCM-SV-semantic-conformance-verification-service	Semantic Conformance Verification Service (p. 262)	service / Canonical
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HCM-SV-service-availability-query	Service Availability Query (p. 97)	service / Canonical
HCM-SV-service-eligibility-evaluation	Service Eligibility Evaluation (p. 98)	service / Canonical
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HCM-SV-specimen-transit-tracking-service	Specimen Transit Tracking Service (p. 218)	service / Canonical
HCM-SV-staff-presence-telemetry-query	Staff Presence Telemetry Query (p. 206)	service / Canonical
HCM-SV-standards-communication-gateway-service	Standards Communication Gateway Service (p. 235)	service / Canonical
HCM-SV-synthetic-task-execution-service	Synthetic Task Execution Service (p. 252)	service / Canonical

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HCM-SV-temporal-context-resolution	Temporal Context Resolution (p. 256)	service / Canonical
HCM-SV-terminology-binding-specification-lookup	Terminology Binding Specification Lookup (p. 261)	service / Canonical
HCM-SV-terminology-concept-resolution-service	Terminology Concept Resolution Service (p. 242)	service / Canonical
HCM-SV-terminology-mapping-service	Terminology Mapping Service (p. 242)	service / Canonical
HCM-SV-theatre-case-progression-telemetry	Theatre Case Progression Telemetry (p. 193)	service / Canonical
HCM-SV-to-do-management-service	To Do Management Service (p. 251)	service / Canonical
HCM-SV-unified-calendar-projection-query	Unified Calendar Projection Query (p. 255)	service / Canonical
HCM-SV-virtual-encounter-context-resolution	Virtual Encounter Context Resolution (p. 184)	service / Canonical
HCM-SV-ward-operational-summary-query	Ward Operational Summary Query (p. 190)	service / Canonical
HCM-SV-ward-round-clinical-briefing-query	Ward Round Clinical Briefing Query (p. 166)	service / Canonical
HCM-SV-work-dispatch-service	Work Dispatch Service (p. 211)	service / Canonical
HCM-SV-work-order-coordination-service	Work Order Coordination Service (p. 250)	service / Canonical

Part I.

Governed RAD semantic detail

8. Canonical Business Capability Model (18 CT1 Domains)

RAD source: `docs/markdown/02-strategy/capabilities/business-capabilities.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Canonical Business Capability Model (18 CT1 Domains)

Overview & Purpose

Business Capabilities describe **what the health enterprise/business must be capable of doing**. They are organisational abilities independent of technology, products, departments, professions or automation. This catalogue is the canonical R1/R2 reconciliation of the adjudicated Business Capability decisions recorded in DN-BC-001 through DN-BC-037. The scratchpad retains working reasoning only; the definitions, explicit tier/parent assertions and boundaries here govern the reconciled model. Provisional and unresolved matters are not promoted as approved hierarchy.

Business Enabling Capabilities describe **reusable enabling abilities Harmonia provides or supports**. The two hierarchies are distinct:

RAD source listing (presentation only; no inferred ancestry).

> Business Capability: CT1 → CT2 → CT3
>
> enabled-by / supported-by
>
> Business Enabling Capability: CT1 → CT2 → CT3 → Feature

The relationship is enablement/support, never decomposition or implementation equivalence. **Business Capabilities do not decompose into Harmonia Features.** Features are ultimately framed by Business Enabling CT3s. This catalogue establishes no new BC↔BEC mapping and does not reframe existing Features. Previously approved B2.1 and assurance support relationships retain their bounded authority without being extended to the new BC descendants.

Technology-sounding names such as Health Information Exchange, Digital Communication Channel, Identity Assurance and Access Control Enforcement describe organisational abilities. Physical, procedural and digital mechanisms illustrate exercise/enablement, not software-component identities. An interpretation requiring a particular software product to explain a BC is suspect. Where BC and BEC concern the same subject, their names must distinguish organisational ability from system enablement where reuse would imply equivalence or ownership. The existing Exchange name collision is explicitly unresolved below.

R1/R2 Scope and Decomposition Rules

> Capability decomposition SHALL be sufficient to establish Harmonia's architectural context and derivation requirements. It SHALL NOT pursue exhaustive decomposition of healthcare-enterprise capabilities where additional detail does not materially affect Harmonia responsibility, information semantics, interoperability, security, workflow enablement, traceability or another Harmonia architectural concern.
>
> Scope may determine decomposition depth. Scope must not distort decomposition semantics.

The stopping test asks whether distinguishing another ability would materially change how Harmonia behaves, governs, represents, exchanges, secures, coordinates or traces something. Once a branch is decomposed, its materially distinct peers must be coherent from the business perspective, independently of Harmonia's implementation interest. This does not require identical depth, a Feature for every peer or a complete healthcare-enterprise taxonomy. An undecomposed CT1/CT2 does not authorise a skipped tier or an invented CT3.

The metamodel governs tier meaning. Explicit CT1/CT2/CT3 and parent statements below establish the stated BC hierarchy; display ordinals, indentation and regional membership alone do not. CT4 and deeper tiers are outside scope. No ordinal is an allocated local code or Canonical ID. BEC tiers/ancestry remain separately unresolved under K9. Absent decomposition does not deny real-world abilities or assign them to an existing node.

Capability Modelling Quality Rules

Rule	Requirement
CM-R01 Coverage	Preserve the eighteen broad enterprise domains and five regions; decomposition is bounded by R1/R2 semantic sufficiency rather than exhaustive enterprise detail.
CM-R02 Orthogonality	Preserve distinct responsibilities, outcomes and authority even where capabilities share evidence, context or implementation machinery.
CM-R03 Semantic Clarity	Use clear business meanings; examples do not redefine capabilities as technology.
CM-R04 No Junk Drawers	Keep cohesive purposes; do not use residual categories to hide materially distinct abilities.
CM-R05 Global Name Uniqueness	Capability names are globally unique across the architecture. The unresolved BC/BEC Exchange collision is reported; neither identity is merged or renamed here.

Capability decomposition and educational documentation

Decomposition follows architectural distinction, not explanatory detail. Definition, state change, boundary, illustrative activities, care contexts and exclusions explain capabilities without establishing more children. History-taking, diagnosis, prescribing, medication administration, surgery, therapy and counselling are examples unless an explicit adjudication establishes a Capability identity.

B2-R05 — Capability Is Independent of Care Delivery Context

Setting, delivery mode, temporal pattern and organisational arrangement do not by themselves establish separate Business Capabilities.

B2-R06 — Context Independence Test

If changing setting, mode or time context does not materially change the business ability, it remains the same Capability. Acute, Planned/Scheduled and Longitudinal Care are not Individual Care Delivery's CT2 decomposition. Inpatient, ambulatory, community, home, virtual, emergency and elective remain illustrative contexts.

Harmonia Relevance Taxonomy

Classification	Meaning
Reference	Enterprise context without material effect on Harmonia responsibilities or processing obligations.
Adjacent	Harmonia consumes, represents or supplies contextual information without material participation in execution or coordination.
Harmonia-Relevant	Harmonia materially enables, connects, routes, coordinates or supplies information used in performing the capability.
Harmonia-Core	Harmonia provides substantive direct platform responsibility for the capability within its assigned boundary.

An established classification has exactly one of these values. Unresolved is a status, not a fifth value. CT1 relevance describes material participation across its bounded scope; it does not classify every descendant identically or confer enterprise-domain authority. Descendant relevance remains unallocated unless explicitly established. Contextual resource/state consumption alone warrants neither Core nor automatic Relevant classification. AX-18 preserves originating and domain authority. A Harmonia-Core enabling responsibility may support a Harmonia-Relevant BC without Harmonia owning that business activity.

The 18 CT1 Business Capability Domains

Five natural regions group the eighteen CT1s. Regions are contextual views, not capability ancestors, organisational boundaries or application ownership. They are distinct from the five BEC contextual views. All eighteen named catalogue domains below are explicitly **CT1 roots**; each branch statement explicitly establishes its CT2 parent and each table its CT3 parent.

Catalogue ordinal	CT1 Business Capability	Region	Relevance
01	Individual Care Delivery	Care & Health Delivery	Harmonia-Relevant
02	Care Access & Coordination	Care & Health Delivery	Harmonia-Relevant
03	Health Rights, Advocacy & Participation	Care & Health Delivery	Adjacent
04	Diagnostic, Therapeutic & Clinical Support Services	Care & Health Delivery	Harmonia-Relevant
05	Health Products & Clinical Technology	Care & Health Delivery	Harmonia-Relevant
06	Clinical Quality, Safety & Improvement	Care & Health Delivery	Harmonia-Relevant
07	Community Health & Wellbeing	Care & Health Delivery	Harmonia-Relevant
08	Population Health & Health-System Planning	Care & Health Delivery	Harmonia-Relevant
09	Health Information & Knowledge Management	Health Information & Digital Health	Harmonia-Core
10	Standards, Semantics & Reference Governance	Health Information & Digital Health	Harmonia-Core
11	Connected Health Services	Health Information & Digital Health	Harmonia-Core
12	Security, Privacy & Digital Trust	Health Information & Digital Health	Harmonia-Core
13	Health Research & Innovation	Research & Innovation	Harmonia-Relevant
14	Health Service Direction & Stewardship	Health Service Management	Harmonia-Relevant

Catalogue ordinal	CT1 Business Capability	Region	Relevance
15	Workforce & Organisational Capability	Health Service Management	Harmonia-Relevant
16	Corporate Resources & Enterprise Services	Health Service Management	Harmonia-Relevant
17	Health Service Governance	Governance & Assurance	Unresolved
18	Health Service Assurance	Governance & Assurance	Harmonia-Relevant

Region 1: Care & Health Delivery

01. Individual Care Delivery

Tier and ancestry: CT1 root.

Definition: Assess, clinically determine, plan, intervene, monitor and review individual care independently of setting, mode, time pattern or organisational arrangement.

Classification: Harmonia-Relevant.

The six CT2s are Individual Health Assessment, Clinical Determination, Individual Care Planning, Clinical Intervention, Individual Health Monitoring and Individual Care Review. Their semantic state changes do not prescribe execution order.

01.01 Individual Health Assessment

Tier and ancestry: CT2; parent Individual Care Delivery (CT1).

Definition: The capability to establish an understanding of an individual's current health state through the collection, consideration and evaluation of relevant clinical, health and contextual information.

State Change: `Healthinformation→Assessedhealthstate/findings`

Boundary: Assessment establishes the assessed health state or findings. Existing records can contribute, while direct examination, observation, history-taking or measurement may establish information independently of Health Information Access.

Illustrative Activities: History taking, physical examination, consideration of available clinical records and measurement of current observations.

Example Care Contexts: An emergency presentation, a scheduled consultation or a home visit may exercise the same assessment ability.

Explicit Exclusions: Establishing clinical conclusions, selecting the care response and performing an intervention are not the assessment itself.

01.02 Clinical Determination

Tier and ancestry: CT2; parent Individual Care Delivery (CT1).

Definition: The capability to interpret assessed health information and establish clinically meaningful conclusions about an individual's health state, condition, risk or prognosis.

State Change: `Assessedfindings→Clinicallyinterpretedunderstanding`

Boundary: Clinical Determination interprets findings and establishes their clinical meaning, including the clinical significance of change identified by monitoring.

Illustrative Activities: Diagnosis, clinical risk interpretation and prognostic judgement.

Example Care Contexts: Interpretation during acute care or a longitudinal follow-up consultation remains the same business ability.

Explicit Exclusions: Clinical Determination does not include establishing care needs or selecting the response to the determination.

01.03 Individual Care Planning

Tier and ancestry: CT2; parent Individual Care Delivery (CT1).

Definition: The capability to establish the intended clinical response to an individual's assessed and clinically interpreted health state, including the objectives, interventions and monitoring required to progress their care.

State Change: `Clinicallyinterpretedunderstanding→Intendedcourseofcare`

Boundary: Planning establishes what is intended to happen. It does not imply that the planned care has occurred.

Illustrative Activities: Establishing care objectives, selecting intended therapy, prescribing as an expression of clinical intent and planning monitoring.

Example Care Contexts: An intended course of care may be established during an inpatient encounter, an ambulatory consultation or a virtual consultation.

Explicit Exclusions: Performing planned care is Clinical Intervention. Ordering, requesting, scheduling and allocating care do not themselves establish that an intervention has occurred. Operational service/resource availability information is not Health Information Access merely because it informs planning.

01.04 Clinical Intervention

Tier and ancestry: CT2; parent Individual Care Delivery (CT1).

Definition: The capability to perform clinical actions intended to alter, maintain or otherwise manage an individual's health state in accordance with an established care intent.

State Change: `Intendedcare→Careactionperformed`

Boundary: Clinical Intervention establishes that a care action was performed. An intervention need not produce a successful clinical outcome. Access to an intended intervention does not confer authority to perform it.

Illustrative Activities: Medication administration, surgery, therapy and counselling when performed as clinical actions under an established care intent.

Example Care Contexts: An intervention may be performed in inpatient, community or home care, or through a suitable virtual delivery mode.

Explicit Exclusions: Ordering, requesting, scheduling or allocating an intervention is not itself the Clinical Intervention; selecting the intended response belongs to planning.

01.05 Individual Health Monitoring

Tier and ancestry: CT2; parent Individual Care Delivery (CT1).

Definition: The capability to observe and evaluate an individual's health state over time in order to identify clinically meaningful change, stability or deviation from an expected state or trajectory.

State Change: `Observedhealthstatesovertime→Change/stability/deviationidentified`

Boundary: Monitoring establishes change, stability or deviation. Interpretation of the clinical significance of that change remains Clinical Determination.

Illustrative Activities: Comparing serial observations with a baseline or expected trajectory and identifying a trend or deviation.

Example Care Contexts: Serial inpatient observations and longitudinal home monitoring exercise the same ability across different time patterns and settings.

Explicit Exclusions: Clinical interpretation of an identified change and selection of the care response are not monitoring. Evaluating care effectiveness against its objectives belongs to Individual Care Review.

01.06 Individual Care Review

Tier and ancestry: CT2; parent Individual Care Delivery (CT1).

Definition: The capability to evaluate the progress and effectiveness of an individual's care against its intended objectives and establish the continuing appropriateness of the current care intent.

State Change: `Careprogress/outcomes→Effectivenessandcontinuingappropriatenessestablished`

Boundary: Care Review evaluates the care itself against its intent and objectives; it is not simply another name for reassessment.

Illustrative Activities: Reviewing response to delivered therapy against care objectives and evaluating whether the current care intent remains appropriate.

Example Care Contexts: Review during an admission, a scheduled follow-up or longitudinal care retains the same evaluation purpose.

Explicit Exclusions: Reassessment alone does not establish care effectiveness. Establishing a revised intended course of care belongs to Individual Care Planning.

Clinical Determination and Individual Care Planning have no adjudicated CT3 set in the supplied decisions. Clinical Intervention's Therapy & Counselling Intervention / Medication Intervention / Procedural Intervention candidate remains provisional under DN-BC-010; it is not promoted as a final taxonomy. Examples cannot close these gaps.

The previously approved B2.1 support trace is preserved without extending it to CT3s or establishing runtime dependencies.

CT2 — Individual Health Assessment

Tier and ancestry: CT2; parent Individual Care Delivery (CT1).

Definition: Establish an understanding of an individual's current health state.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Individual Health Assessment; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Health History Assessment	Establish relevant health knowledge from the individual's history and applicable historical/contextual information.
Clinical Examination	Establish health findings through clinical examination.
Health Status Measurement	Establish individual health observations or measurements.

Boundary: The decomposition distinguishes means of establishing knowledge. Functional, nutritional, cognitive, mental-health and similar labels are assessment domains, not automatically peer abilities. Risk interpretation belongs to Clinical Determination. Care setting, profession, specialty and information type do not define the decomposition.

CT2 — Individual Health Monitoring

Tier and ancestry: CT2; parent Individual Care Delivery (CT1).

Definition: Observe health over time to identify change, stability or deviation.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Individual Health Monitoring; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Health State Monitoring	Establish an aggregate, temporally situated view of the individual's health from applicable monitored data.
Care Response Monitoring	Scope that health-state view to a particular problem/condition and the treatment applied to it.

Boundary: Care Response Monitoring consumes health-state services provided through Health State Monitoring without independently recreating the aggregate view. This semantic consumption establishes no named Service, interface or runtime invocation. Measurement supplies observations; monitoring establishes a view; Clinical Determination interprets its clinical significance. Deterioration, improvement and stability are possible monitored states, not established peer CT3s.

CT2 — Individual Care Review

Tier and ancestry: CT2; parent Individual Care Delivery (CT1).

Definition: Evaluate care against intent and objectives.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Individual Care Review; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Care Appropriateness Review	Evaluate whether the current intended care remains suitable in the individual's current context.
Care Effectiveness Review	Evaluate whether delivered care is achieving or has achieved its intended goals and outcomes.

Boundary: An effectiveness finding may inform appropriateness without replacing it. Care may be appropriate yet ineffective, or effective yet no longer appropriate to continue. Monitoring the response is distinct from judging effectiveness; identifying a need to reconsider care does not establish the replacement plan. Replanning belongs to Individual Care Planning.

02. Care Access & Coordination

Tier and ancestry: CT1 root.

Definition: Managing patient intake, referral triage, appointment scheduling, multidisciplinary care team coordination, cross-facility patient transfers, and care continuity across organisational boundaries.

Classification: Harmonia-Relevant.

CT2 — Care Access Management

Tier and ancestry: CT2; parent Care Access & Coordination (CT1).

Definition: Establish and manage access to applicable care.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Care Access Management; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Care Intake Management	Establish and manage the intake context through which an individual seeks or enters care.

CT3 Business Capability	Definition
Care Referral Management	Manage a request for another participant/service to assess, manage or assume applicable care responsibility.
Care Eligibility Determination	Determine whether applicable eligibility conditions for care access are satisfied.

Boundary: Intake, referral and eligibility are distinct abilities. Referral submission or receipt does not establish acceptance, care delivery or responsibility actually assumed; eligibility does not itself allocate or deliver care.

CT2 — Care Scheduling & Capacity Management

Tier and ancestry: CT2; parent Care Access & Coordination (CT1).

Definition: Manage scheduled opportunities, care places, usable operational capacity and care-demand resourcing.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Care Scheduling & Capacity Management; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Care Appointment Management	Establish and manage a scheduled opportunity for care.
Care-Place Allocation	Allocate an applicable care place to a care requirement.
Care-Place Readiness Management	Establish and manage whether a particular care place is ready for allocation or use.
Care Resource Deployment	Apply legitimately available and appropriately authorised resources to a current or planned operational care requirement, particularly using roster/on-call and service-coverage context.
Operational Care Flow Coordination	Coordinate operational care flow and its progression within the care-delivery environment.
Operational Care Capacity Management	Understand and manage the service's usable capacity to accommodate care in its current operational context.

Boundary: Available staff, equipment or an unoccupied bed does not establish usable service capacity. Available for service coverage ≠ deployed to a care requirement ≠ assigned operational work. Care Resource Deployment consumes availability, role, qualification, engagement and privilege context without changing authoritative rosters, qualifications, employment or privilege records. It establishes no Harmonia authority over clinical work, enterprise assets or hospital-wide logistics. Operational capacity is distinct from strategic/tactical capacity planning and from collective service readiness (OI-BC-002).

CT2 — Care Coordination & Transition

Tier and ancestry: CT2; parent Care Access & Coordination (CT1).

Definition: Coordinate care participation, actual responsibility transition and discharge.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Care Coordination & Transition; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Care-Team Coordination	Coordinate participating care-team context and collaboration around care.
Care Responsibility Transition	Establish and coordinate who has actually assumed applicable care responsibility.
Care Discharge Coordination	Coordinate applicable discharge requirements and progression.

Boundary: Patient moved ≠ care responsibility transferred. Arrival, referral submission, message receipt, discharge readiness or physical departure does not itself establish acceptance

of responsibility. Information assembly for clinical handover does not confer clinical-handover management authority on Harmonia.

CT2 — Care Support Logistics Management

Tier and ancestry: CT2; parent Care Access & Coordination (CT1).

Definition: Coordinate bounded operational support needed for care access and fulfilment.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Care Support Logistics Management; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Patient Movement Coordination	Coordinate physical patient transit and applicable mobility constraints.
Clinical Material Logistics Coordination	Coordinate clinically relevant material movement, custody, availability or disposition needed for care or diagnostic fulfilment.
Care Support Work Coordination	Coordinate authorised operational support work: what needs doing, who performs it and how progression, completion or escalation is coordinated.

Boundary: These abilities do not establish a general hospital logistics, supply-chain, ERP, facilities or workforce-management architecture. Support work coordination does not determine clinical work; assignment/dispatch does not establish availability, qualification, privilege or clinical authority.

03. Health Rights, Advocacy & Participation

Tier and ancestry: CT1 root.

Definition: Protecting patient rights, facilitating informed consent, managing advance care directives, supporting patient advocacy, and enabling consumer participation in health decisions.

Classification: Adjacent.

No CT2/CT3 set is established by this reconciliation. Consent and record-access evidence can support rights without assigning advocacy or participation management to Harmonia.

04. Diagnostic, Therapeutic & Clinical Support Services

Tier and ancestry: CT1 root.

Definition: The specialised clinical services that support direct patient care, including pathology and laboratory testing, diagnostic imaging (radiology), pharmacy and medication dispensing, and allied health therapies.

Classification: Harmonia-Relevant.

The following diagnostic and pharmacy branches are established. Other therapeutic/support scope remains enterprise context; no further subordinate identity is manufactured.

CT2 — Diagnostic Support Provision

Tier and ancestry: CT2; parent Diagnostic, Therapeutic & Clinical Support Services (CT1).

Definition: Provide diagnostic services and manage their evidence, interpretation and governed reporting.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Diagnostic Support Provision; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Diagnostic Service Appointment Management	Manage the diagnostic-service-specific scheduled opportunity, including preparation, modality, urgency, duration and other service-specific constraints.
Diagnostic Service Delivery	Perform the diagnostic activity that obtains or produces diagnostic evidence.
Diagnostic Specimen Management	Maintain a specimen's identity, integrity, suitability, custody and applicable lifecycle context.
Diagnostic Data Management	Manage acquired or produced diagnostic evidence as information, including identity, association, integrity, organisation, availability and lifecycle.
Diagnostic Interpretation	Establish what diagnostic evidence demonstrates within the diagnostic service's professional scope.
Diagnostic Report Management	Establish and govern the authoritative expression of diagnostic findings, including applicable creation, review, amendment, verification, finalisation and authorisation/signing.
Diagnostic Report Distribution	Provide an established report to applicable authorised recipients or consuming contexts under applicable access, privacy, security and communication controls.

Boundary: Scheduling ≠ performance ≠ evidence management ≠ interpretation ≠ governed report ≠ distribution. Specimen and data responsibilities remain distinct. Diagnostic interpretation does not establish patient-level Clinical Determination from the total clinical context. Generic appointments are distinct from diagnostic-specific constraints. Diagnostic Order Management is not established here as a subordinate capability. Signing/finalisation are report behaviours, not additional CT3s. Services may exercise a subset, concurrently or iteratively; no mandatory diagnostic workflow is established.

CT2 — Medication & Pharmacy Services

Tier and ancestry: CT2; parent Diagnostic, Therapeutic & Clinical Support Services (CT1).

Definition: Provide professional medication/pharmacy services under applicable controls.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Medication & Pharmacy Services; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Prescription Medication Dispensing	Professionally prepare and provide medication according to an applicable authorised prescription/order and dispensing requirements.
Non-Prescription Medication Provision	Provide medication not requiring an applicable prescription, subject to professional, regulatory and safety controls.
Medication Advice	Provide professional medication-specific advice about appropriate use, precautions, interactions, administration, adherence or safe handling.
Medication Dose Administration Aid Provision	Prepare and provide organised medication doses supporting administration according to an already established regimen.

Boundary: Medication prescribed ≠ dispensed ≠ provided ≠ administered. Advice does not establish diagnosis, prescribing or authority to alter treatment. Dose aid provision does not establish the regimen; commercial packaging examples do not define the capability. These are pharmacy responsibilities, distinct from clinical intervention, and do not prescribe a medication sequence. Medication Advice's detailed scope remains subject to refinement. OI-BC-001 remains unresolved; compounding/preparation is neither established, rejected nor silently subsumed.

05. Health Products & Clinical Technology

Tier and ancestry: CT1 root.

Definition: Manage health products and clinical technology in the care environment, with distinct device inventory, functional/release, deployment, utilisation, integration and lifecycle responsibilities. Product procurement and supply context is retained without inferring further Health Product Management decomposition.

Classification: Harmonia-Relevant.

CT2 — Clinical Technology Management

Tier and ancestry: CT2; parent Health Products & Clinical Technology (CT1).

Definition: Manage clinical technology as a coherent business concern rather than only the device information Harmonia happens to use.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Clinical Technology Management; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Clinical Device Inventory Management	Establish what devices exist and their managed asset identity, specification, ownership/custody and inventory context.
Clinical Device Feature & Release Management	Manage functional/release state, including applicable software, firmware, configuration and enabled device characteristics.
Clinical Device Loan & Deployment Management	Manage device loan, issue, assignment, placement, return and deployment.
Clinical Device Utilisation Support	Enable and support otherwise appropriately authorised users to use the particular device safely and effectively.
Clinical Device Integration Provision	Establish and maintain the device's capability and configuration to participate in applicable information exchanges.
Clinical Device Data Integration	Acquire, exchange or convey device data through established integration capability.
Clinical Device Lifecycle Management	Govern introduction, continued service, review, replacement, withdrawal, decommissioning and retirement within the overall device estate.

Boundary: Inventory ≠ release state ≠ deployment ≠ user enablement ≠ use in care. Professional qualification/privilege does not establish device-specific enablement. Integration provisioned ≠ data integrated; neither confers clinical interpretation or downstream information authority. Device monitoring remains externally provided enablement, with provider unestablished. Monitoring, maintenance and lifecycle disposition have distinct meanings. The Maintenance membership discrepancy is preserved under Unresolved Business Capability matters; no eighth CT3 is inferred.

06. Clinical Quality, Safety & Improvement

Tier and ancestry: CT1 root.

Definition: Operational clinical quality, safety and improvement: monitoring clinical outcomes; identifying quality and safety concerns; adverse event and incident management, reporting and response; infection control; operational quality and safety monitoring; continuous quality improvement; and coordination of operational responses to identified concerns.

Classification: Harmonia-Relevant.

CT2 — Clinical Safety Management

Tier and ancestry: CT2; parent Clinical Quality, Safety & Improvement (CT1).

Definition: Manage the clinical-risk distinctions materially required for R1/R2.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Clinical Safety Management; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Clinical Risk Identification	Recognise and characterise a clinical safety risk from applicable evidence.
Clinical Risk Mitigation Management	Manage prospective/preventative measures reducing identified risk likelihood, exposure or consequence.
Clinical Risk Response Management	Manage active response, containment and remediation where risk has materialised, is emerging or requires active intervention.

Boundary: Monitoring supplies evidence but does not itself identify/characterise clinical risk. Monitoring providers remain unestablished. Mitigation/response may require action by other capabilities without acquiring those capabilities' ownership. Response is not a generic enterprise incident-management container. The three CT3s are deliberately non-exhaustive specialist context; identified, mitigated and actively responded-to states are distinct, not a mandatory sequence.

CT2 — Clinical Quality Management

Tier and ancestry: CT2; parent Clinical Quality, Safety & Improvement (CT1).

Definition: Manage the clinical-quality distinctions materially required for R1/R2.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Clinical Quality Management; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Clinical Quality Criteria Management	Establish and manage measures, indicators, expectations and criteria against which clinical quality is considered.
Clinical Quality Evidence Management	Manage the body of applicable evidence used for clinical-quality consideration without owning its originating activity.
Clinical Quality Review & Assessment Management	Review evidence against applicable criteria and establish a clinical-quality assessment, finding or conclusion.
Clinical Quality Reporting	Communicate established clinical-quality measures, assessments, findings and conclusions to authorised consumers.

Boundary: Criteria ≠ evidence ≠ assessment ≠ reporting ≠ subsequent improvement. Observation alone is not quality judgement and reporting does not originate the reported conclusion. Clinical Quality Performance Monitoring is not established as a separate CT3; the provider of monitoring evidence remains unestablished. Continuous Improvement is not a subordinate CT3 here; generic placement remains unestablished. Health Service Continuous Improvement has the separately established service-wide purpose under 14. The four CT3s do not exhaust specialist Clinical Quality.

07. Community Health & Wellbeing

Tier and ancestry: CT1 root.

Definition: Support community health and wellbeing, including the bounded preventative-health abilities required for R1/R2. Wider environmental health, outreach and social determinants remain enterprise context without inferred additional decomposition.

Classification: Harmonia-Relevant.

CT2 — Preventative Health

Tier and ancestry: CT2; parent Community Health & Wellbeing (CT1).

Definition: Address preventative-health needs through organised activity with sufficient R1/R2 distinctions.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Preventative Health; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Preventative Health Problem Identification	Identify and characterise a preventative-health problem warranting organised attention.
Preventative Health Programme Management	Establish and manage an organised response to a preventative-health need/objective without absorbing its delivery capabilities.
Preventative Health Education Services	Provide education intended to prevent adverse health outcomes and influence preventative knowledge or behaviour.
Preventative Health Programme Monitoring	Observe programme operation, reach, participation, coverage, progress and applicable performance.
Preventative Health Reporting	Report established preventative-health problems, programmes, cohorts, activity, performance and outcomes.
Preventative Health Cohort Management	Establish and manage the target population, inclusion/exclusion criteria, membership, stratification and applicable entry/exit context.

Boundary: Screening and immunisation are programme types, not automatically separate CT3s. Population at risk ≠ defined cohort ≠ individual clinical problem. Programme management does not own clinical intervention, appointments or other delivery abilities. Education is purpose-specific rather than generic communication. Programme monitoring is distinct from population-health assessment, individual monitoring and assurance/review conclusions. This is deliberate specialist under-decomposition; unmodelled responsibilities are not assigned to existing CT3s.

08. Population Health & Health-System Planning

Tier and ancestry: CT1 root.

Definition: Understand population health through assessment/surveillance and applicable event/notification management; assess health-service demand and plan required capacity and its distribution.

Classification: Harmonia-Relevant.

CT2 — Population Health Surveillance

Tier and ancestry: CT2; parent Population Health & Health-System Planning (CT1).

Definition: Understand population-health state and manage organised surveillance, significant events and applicable notification.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Population Health Surveillance; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Population Health Longitudinal Assessment	Establish population-health state, patterns and change over time from applicable evidence.
Population Health Surveillance Management	Establish and manage surveillance purposes, populations, concerns and regimes.
Population Health Event Identification	Identify and characterise significant population-health events/patterns from applicable evidence.
Notifiable Health Event Management	Manage formal notification applicability, required information, recipient, status, submission, correction/supplementation and applicable follow-up.

CT3 Business Capability	Definition
Population Health Reporting	Communicate established population-health assessments, surveillance, events, trends and findings.

Boundary: Population state ≠ individual state; observed change ≠ identified event ≠ notifiability ≠ obligation discharged. Notification management neither defines the obligation nor owns operational response, and is distinct from general reporting. Surveillance may provide evidence for preventative problems/cohorts, demand planning, clinical quality or safety without establishing those outcomes. Population assessment differs from preventative-programme performance. Individual observations may contribute population evidence without becoming population conclusions. Epidemiology, outbreak response and public-health scope are deliberately not exhaustively modelled.

CT2 — Health-System Planning

Tier and ancestry: CT2; parent Population Health & Health-System Planning (CT1).

Definition: Assess health-service demand and plan the capacity and footprint required to meet it.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Health-System Planning; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Health Service Demand Assessment	Assess current/expected health-service demand across a population, geography, service domain or health-system context.
Health Service Capacity Planning	Determine required strategic/tactical capacity to address assessed demand.
Health Service Distribution Planning	Determine the distribution of planned capacity across a geographical, organisational, site or service-network footprint.

Boundary: Population-health evidence does not predetermine a demand conclusion. Individual care demand differs from health-system demand; required capacity differs from established or operationally usable capacity. Planning owns neither underlying resources nor operational allocation/deployment. Planned footprint differs from managed locations and identifiable offerings. Detailed commissioning, funding, capital, infrastructure and workforce planning remain intentionally unmodelled.

Region 2: Health Information & Digital Health

09. Health Information & Knowledge Management

Tier and ancestry: CT1 root.

Definition: Manage governed health-information content, structures, lifecycle, catalogues, compartments, correlation and retrieval, alongside reusable clinical knowledge and its governed standing.

Classification: Harmonia-Core.

CT2 — Health Information Management

Tier and ancestry: CT2; parent Health Information & Knowledge Management (CT1).

Definition: Manage health information as governed, semantically meaningful content.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Health Information Management; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Health Information Data Management	Manage actual health-information content within applicable governed structures.
Health Information Structure Management	Manage structures, element/reference definitions, constraints and semantic bindings giving content unambiguous meaning.
Health Information Lifecycle Management	Govern state, amendment, correction, supersession, currency, retention and disposition while preserving provenance and historical meaning.
Health Information Catalogue and Index Management	Manage descriptive catalogues/indexes through which information existence and location can be established.
Health Information Compartmentalisation	Manage contextual association, partitioning and bounded information groupings.
Health Information Data Correlation	Establish meaningful relationships between independently managed information items/sets.
Health Information Data Retrieval	Obtain identified information under an applicable authorised request/context.

Boundary: Semantic authority ≠ semantic binding ≠ content; a persisted value is not self-describing health information. Structure/binding management does not govern the referenced terminology/value set. Catalogue existence/location ≠ retrieval ≠ communication ≠ presentation; discoverability does not authorise access. Compartment membership may inform but does not determine authorisation. Correlation establishes neither clinical interpretation nor entity identity. Current state and immutable historical operational truth remain distinct; managing information does not originate its facts. Structures are not equated with FHIR resources, physical schemas or storage mechanisms.

CT2 — Clinical Knowledge Management

Tier and ancestry: CT2; parent Health Information & Knowledge Management (CT1).

Definition: Identify, extract, formulate, associate, govern and share reusable clinical knowledge.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Clinical Knowledge Management; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Clinical Knowledge Context Identification	Identify sources/contexts potentially containing reusable clinical knowledge.
Clinical Knowledge Context Extraction	Extract relevant material while preserving its source/context relationship.
Clinical Knowledge Information Formulation	Formulate explicit reusable knowledge, including applicability, qualifications, rationale and evidence relationships.
Clinical Knowledge Concept Association	Associate knowledge with applicable governed clinical concepts without acquiring concept authority.
Clinical Knowledge Review & Approval Management	Manage review, endorsement, approval/rejection, conditions and applicable history for intended governed use.
Clinical Knowledge Publication & Sharing	Make approved knowledge available to applicable authorised consumers and use contexts.

Boundary: Source identified ≠ extracted material ≠ formulated knowledge ≠ reviewed/approved knowledge ≠ publication ≠ individual application. Reusable knowledge differs from source health information and individual clinical facts. Formulation is distinct from Health Information Structure Management and does not establish approval. Concept association does not redefine terminology or semantic authority. Publication establishes neither individual applicability nor a clinical conclusion/decision. Clinical knowledge and source provenance/authority remain distinguishable; no mandatory knowledge workflow is prescribed.

10. Standards, Semantics & Reference Governance

Tier and ancestry: CT1 root.

Definition: Govern organisational adoption, oversight and appropriate use of health-information models, standards, terminology, classifications and semantic mappings.

Classification: Harmonia-Core.

CT2 — Health Information Standards Governance

Tier and ancestry: CT2; parent Standards, Semantics & Reference Governance (CT1).

Definition: Govern organisational adoption and use of health-information models and applicable standards.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Health Information Standards Governance; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Information Model Discovery	Identify information models relevant to the organisation's health-information needs.
Information Model Adoption	Adopt applicable information models, including intrinsic assessment of adoption impacts.
Information Model Education	Support organisational understanding and appropriate use of adopted information models.
Information Model Compliance	Address organisational conformity to applicable information-model requirements.
Information Model Governance Oversight	Oversee and evaluate effectiveness, consistency and appropriate operation of information-model governance.
Standards Adoption	Adopt applicable standards within the organisation.
Standards Impact Identification	Identify standards impacts across clinical/work practice, permitted medications, procedures, policy, safety, obligations and reporting as applicable.

Boundary: Oversight does not perform its discovery, adoption, education or compliance peers. Model-adoption impact is intrinsic to adoption; standards impacts can extend beyond information-system implementation and warrant their distinct established identity. These are the peers explicitly named in DN-BC-025; no additional standards peer is inferred to fill out a conventional taxonomy. Conceptual progression does not establish a workflow.

CT2 — Clinical Semantics Governance

Tier and ancestry: CT2; parent Standards, Semantics & Reference Governance (CT1).

Definition: Govern clinical meaning and appropriate organisational use of semantic artefacts.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Clinical Semantics Governance; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Terminology Governance	Manage governed terminology/reference-set lifecycle and standing, including acceptance, release, update, supersession/retirement and authority/provider coordination.
Terminology Utilisation Oversight	Oversee actual terminology/reference-set use and identify inappropriate, inconsistent or drifting usage.
Terminology Localisation	Manage controlled local responses where adopted terminology does not meet legitimate organisational semantic requirements.
Classification Governance	Govern applicable classification definitions and their organisational standing.

CT3 Business Capability	Definition
Classification Utilisation Oversight	Oversee appropriate organisational application of classifications.
Semantic Mapping Governance	Govern mappings between applicable semantic artefacts without collapsing their source meanings or authorities.

Boundary: Compartmentalisation concerns business/context relationships; classification concerns sensitivity and consequences of exposure/mishandling; terminology concerns consistent meaning. They are orthogonal even where metadata/mechanisms are shared. Classification Localisation is not established and SHALL NOT be invented for symmetry. Governance differs from structure binding and knowledge-to-concept association.

11. Connected Health Services

Tier and ancestry: CT1 root.

Definition: Exchange health information appropriately and provide digitally mediated communication, notifications and collaborative interaction between health-service participants.

Classification: Harmonia-Core.

CT2 — Health Information Exchange

Tier and ancestry: CT2; parent Connected Health Services (CT1).

Definition: Exchange health information appropriately between participating parties, systems and services.

No CT3 decomposition is established here.

Boundary: This organisational ability is not a FHIR server, interface engine, protocol or interoperability product. The established rename from Health Information Connectivity does not allocate additional CT3s. Health Information Sharing Programme Management is a named working capability in DN-BC-027 whose precise tier/parent is not supplied there; it is retained separately as unresolved. The collision with the existing BEC name remains for explicit adjudication; no BEC identity is merged or renamed.

CT2 — Digital Communication Channel

Tier and ancestry: CT2; parent Connected Health Services (CT1).

Definition: Provide digitally mediated communication, notification and collaborative interaction between patients, practitioners and other health-service participants.

No CT3 decomposition is established here.

Boundary: Appointment notifications, pharmacy queries, results and collaboration illustrate participant communication. This is not the general UI/presentation capability: exercising another capability through a UI does not itself exercise this communication ability. General presentation enablement remains parked for BEC reconciliation. No CT3 set is established by DN-BC-027.

12. Security, Privacy & Digital Trust

Tier and ancestry: CT1 root.

Definition: Establish adequate identity confidence, coherent access controls, privacy/protection safeguards, governed activity attribution and accountability for security-relevant activity and information use.

Classification: Harmonia-Core.

CT2 — Digital Identity & Access Governance

Tier and ancestry: CT2; parent Security, Privacy & Digital Trust (CT1).

Definition: Govern sufficient identity confidence and coherent access controls across the organisation.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Digital Identity & Access Governance; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Identity Assurance	Establish sufficient confidence that a person is who they claim to be through appropriate evidence and processes.
Identity Modelling & Interoperability	Establish identity models and interoperability arrangements appropriate to organisational identity use.
Identity Qualification Control	Determine whether presented identity characteristics, provenance and assurance quality are sufficient for its intended context.
Identity Identification & Authentication	Identify an actor and authenticate the asserted identity.
Access Control Context Management	Determine what information, functions, activities or other concerns require control, under what circumstances and how requirements are modelled.
Access Control Assurance & Reporting	Establish organisational assurance/evidence of whether access-control arrangements are appropriately established and operating.
Access Control Collaboration	Coordinate arrangements across independently controlled domains/parties while retaining their control ownership.
Access Control Enforcement	Ensure physical, procedural and digital control mechanisms coherently impose intended controls.

Boundary: Who are you? ≠ confidence in the claimed identity ≠ identity quality sufficient here. Federation and Single Sign-On are examples, not equivalent concepts or universal identity-quality guarantees. Context management is not merely runtime attribute assembly; enforcement is not a software authorisation engine. Strong local control does not imply end-to-end control. Identity Assurance is an organisational ability; the possible future BEC name Identity Assurance Representation remains unestablished.

CT2 — Health Information Protection

Tier and ancestry: CT2; parent Security, Privacy & Digital Trust (CT1).

Definition: Govern privacy consequences and adequate protection of the information estate and its environment.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Health Information Protection; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Health Privacy Safeguard Management	Establish organisational safeguards and assurance processes concerning privacy consequences of information supply, exposure or use.
Health Information Safeguarding	Ensure information and its operational environment are protected and resilient against loss, compromise, inappropriate access or operational failure.
Health Information Access Monitoring	Evaluate actual organisational access/handling practice, including controls, accessibility, compartments, classification and physical/printed handling.
Health Information Compartment Assurance	Assure appropriate establishment and application of compartmentalisation without establishing access decisions.

CT3 Business Capability	Definition
Health Information Classification Assurance	Assure appropriate classification according to sensitivity and potential consequences of exposure/mishandling.

Boundary: Privacy consequences differ from estate/environment protection. Storage, backups, disaster recovery, physical controls and testing illustrate organisational safeguards rather than define software components. Access Monitoring evaluates practice using IT, audit, physical-access or other evidence; it is not merely observation of software access events. Evidence supply does not transfer business accountability. These subject-specific protection abilities are not separate macro-level Health Service Assurance trees.

CT2 — Digital Accountability & Trust

Tier and ancestry: CT2; parent Security, Privacy & Digital Trust (CT1).

Definition: Establish governed attribution and accountability for activity and information use.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Digital Accountability & Trust; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Governed Activity Attribution	Preserve who participated, what was done, the attributable person/role/unit/organisation capacity and applicable permission/authority/constraint context.
Security & Information-Use Accountability	Evaluate whether attributed security-relevant activity/information use was appropriate within policy, obligation and authority constraints, and establish accountability.

Boundary: Actor identified ≠ activity attributed ≠ activity permitted. Attribution without permission/constraint context cannot establish entitlement and is richer than simple audit logging or provenance. Accountability evaluates appropriateness rather than merely recording participation. Expected consumption by governance/assurance or other enablement establishes no structural BC↔BEC relationship.

Region 3: Research & Innovation

13. Health Research & Innovation

Tier and ancestry: CT1 root.

Definition: Provide existing health information for authorised research under applicable constraints and evaluate health innovation; deliberately under-decomposed for R1/R2.

Classification: Harmonia-Relevant.

CT2 — Research Information Support

Tier and ancestry: CT2; parent Health Research & Innovation (CT1).

Definition: Provide the existing health information needed for authorised research within the deliberately bounded R1/R2 responsibility.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Research Information Support; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Governed Research Information Provision	Provide existing health information for an authorised research purpose under applicable ethics, governance, privacy, consent, security and information-use constraints.

Boundary: This is deliberate under-decomposition, not a claim that information provision exhausts research support. Research Assessment and Governance, Research/Trials Oversight, clinical-trial management, genomic analysis and other specialist research abilities are intentionally unmodelled unless Harmonia requirements make them material. Their absence does not assign them to information provision.

CT2 — Health Innovation

Tier and ancestry: CT2; parent Health Research & Innovation (CT1).

Definition: Consider health-service and clinical innovation within the R1/R2 business context.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Health Innovation; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Health Innovation Evaluation	Evaluate proposed/emerging innovation for suitability, value, implications and potential adoption.

Boundary: Evaluation does not establish adoption or the clinical/operational authority to implement innovation. The single CT3 is deliberately sufficient, not an exhaustive innovation lifecycle.

Region 4: Health Service Management

14. Health Service Direction & Stewardship

Tier and ancestry: CT1 root.

Definition: Model needed health services and their viability requirements, support establishment/improvement and align services with the wider ecosystem of dependencies, resources, financial arrangements and organisational change.

Classification: Harmonia-Relevant.

CT2 — Health Service Modelling

Tier and ancestry: CT2; parent Health Service Direction & Stewardship (CT1).

Definition: Establish what service should exist and what it needs to become viable.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Health Service Modelling; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Health Service Need Identification	Establish an unmet health-service need requiring response.
Health Service Capability Identification	Identify the ordinary business abilities needed to satisfy the service need across clinical, workforce, facility, technology, information and support concerns.
Health Service Implementation Support	Support establishment of the framed service through applicable organisational preparation and programmes of work.
Health Service Continuous Improvement	Evaluate the established service and identify correction/improvement needs that may feed back into modelling and implementation support.

Boundary: Capability Identification uses ordinary business meaning; it is not Harmonia CT-tier allocation. Implementation support may require recruitment, training, facility, equipment or technology change without absorbing HR, facilities, ICT or procurement responsibility. Identification, fulfilment requirements, support and review describe semantic relationships, not a prescribed workflow.

CT2 — Health Service Alignment

Tier and ancestry: CT2; parent Health Service Direction & Stewardship (CT1).

Definition: Position the service coherently in its wider health-service ecosystem.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Health Service Alignment; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Health Service Co-Dependency Management	Identify and manage what the service needs from others and what others need from it.
Health Service Resource Identification	Identify required workforce, facilities, equipment, clinical technology, ICT, administration and support resources without owning them.
Health Service Financial Management	Establish the service's fit within funding, financial and charging arrangements without absorbing enterprise finance/accounting.
Health Service Implementation Planning	Establish and coordinate surrounding organisational/service changes needed to introduce or materially change the service.

Boundary: Alignment does not establish the authoritative service catalogue; it positions the service within the environment represented by that catalogue. Modelling determines what service is needed; alignment determines how it fits; Offering Management establishes the identifiable offering. Expected future Service Catalogue richness explains architectural relevance, but establishes neither a BEC identity/hierarchy nor a BC↔BEC mapping. Alignment does not resolve operational-readiness ancestry.

15. Workforce & Organisational Capability

Tier and ancestry: CT1 root.

Definition: Govern practitioner qualification, engagement/privilege and enterprise service coverage, together with organisation identity/structure, recognised locations and identifiable healthcare service offerings.

Classification: Harmonia-Relevant.

Workforce & Organisational Capability remains the established CT1. Health Workforce Management and Healthcare Organisation & Service Structure Management are its distinct CT2 frames; directories represent their facts without replacing business governance of those facts.

CT2 — Health Workforce Management

Tier and ancestry: CT2; parent Workforce & Organisational Capability (CT1).

Definition: Govern practitioners' professional standing, engagement/privilege and whole-enterprise service coverage.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Health Workforce Management; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Practitioner Qualification Management	Govern professional/clinical standing and qualifications, including applicable review, escalation, complaints, penalties/issues, suspension and revocation.
Practitioner Engagement & Privilege Management	Manage practitioners as service participants, their roles/ capacities, applicable privileges and qualification context above ordinary HR employment administration.
Workforce Service Coverage Management	Observe whether required enterprise work/service obligations can be fulfilled, identify coverage gaps/risks and adjust coverage in response.

Boundary: Qualified ≠ engaged ≠ privileged for a particular activity. Coverage is broader than rostering; it asks whether required work can be covered and how coverage should change. Care Resource Deployment applies available resources to a particular care requirement; Care Support Work Coordination progresses particular support work. Neither consumption of qualification/coverage information nor directory representation transfers the underlying workforce authority to Harmonia.

CT2 — Healthcare Organisation & Service Structure Management

Tier and ancestry: CT2; parent Workforce & Organisational Capability (CT1).

Definition: Manage organisational identity/structure, recognised locations and identifiable organisational service offerings.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Healthcare Organisation & Service Structure Management; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Healthcare Organisation Identity & Structure Management	Establish and manage identity and structural relationships of participating organisations and organisational units.
Healthcare Location Definition Management	Establish and manage recognised locations and their applicable business characteristics and relationships.
Healthcare Service Offering Management	Establish/manage what identifiable service is offered, by whom, where, when and under which availability conditions/constraints.

Boundary: Location definition is distinct from Facilities Management. Service exists ≠ offered here ≠ available at this time/context. Temporal/conditional availability is material: a walk-in offering may operate 06:00–22:00 while another pathway covers other hours. Offering availability does not establish collective operational readiness; OI-BC-002 remains unplaced. Organisation, location and practitioner associations do not substitute for material Healthcare Service context.

16. Corporate Resources & Enterprise Services

Tier and ancestry: CT1 root.

Definition: Provide the bounded clinical-activity charging information and enterprise support context required for R1/R2, without assigning enterprise finance, HR, facilities or ICT operating authority to Harmonia.

Classification: Harmonia-Relevant.

CT2 — Financial & Commercial Services

Tier and ancestry: CT2; parent Corporate Resources & Enterprise Services (CT1).

Definition: Provide the financial/commercial business context materially required for R1/R2.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Financial & Commercial Services; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Clinical Activity Charging Information Provision	Identify and provide information about chargeable or cost-relevant clinical/health-service activity where required for applicable funding, financial or charging purposes.

Boundary: The single CT3 is deliberately narrow. Referral forwarding, diagnostic requests, service types, quantities and utilisation context can supply evidence; not every mediated activity is chargeable. Evidence provision does not confer pricing, billing, claims, accounting or revenue-management responsibility. Comprehensive Medicare, insurance, activity-based funding and revenue-cycle architecture is not established.

CT2 — Corporate Support Services

Tier and ancestry: CT2; parent Corporate Resources & Enterprise Services (CT1).

Definition: Provide enterprise support whose resources, availability, state or constraints may materially affect service activity.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Corporate Support Services; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Back-Office Services	Provide administrative/support services on which health-service activity depends.
Facilities Management	Manage physical facilities/services and applicable resource, location, availability and facility-state context.
IT Infrastructure Management	Operate/manage underlying technology infrastructure and its availability.
Communications & Publicity	Manage organisational communication/publication arrangements relevant to information availability/distribution.
People Management	Manage people across the organisation, distinct from practitioner-specific Health Workforce Management.
Operational Oversight	Observe broader operational conditions affecting the organisation’s ability to work or deliver services.

Boundary: These are relevant enterprise abilities even where Harmonia only consumes their state/resource/constraint context; contextual consumption alone does not make them Harmonia-Core. They are not decomposed further for R1/R2 absent a material requirement. Consumption transfers neither resource ownership nor corporate operating authority and does not resolve OI-BC-002.

Region 5: Governance & Assurance

17. Health Service Governance

Tier and ancestry: CT1 root.

Definition: Establish and maintain the obligations, authorities, constraints, policies and governance framework under which health-service activity must operate, and define how adherence will be assessed, checked and reported.

Classification: Unresolved.

Health Service Governance establishes what the organisation must do and how adherence will be determined and governed. It does not determine operational means or acquire progression/management of governed work. Enabling derivation remains unestablished; consumption of its requirements alone does not settle its relevance classification.

CT2 — Governance Framework Management

Tier and ancestry: CT2; parent Health Service Governance (CT1).

Definition: Establish the authoritative WHAT and legitimate working parameters.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Governance Framework Management; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Policy / Obligation Governance	Identify, interpret, establish and maintain authoritative legislative, regulatory, policy, standards, guideline, contractual and other obligations, attributable and traceable to source authority.
Authority & Constraint Governance	Establish who may make, approve or govern organisational decisions/artefacts and the bounds within which authority may be exercised.

Boundary: Obligation = what must be satisfied; authority = who may decide/approve; constraint = within what bounds. Engagement with regulators, standards/compliance bodies or other authorities can establish legitimate parameters without transferring their authority.

CT2 — Compliance Governance

Tier and ancestry: CT2; parent Health Service Governance (CT1).

Definition: Define how adherence to the authoritative WHAT will be determined, checked and reported.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Compliance Governance; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Compliance Assessment Definition	Define the evidence, measures, criteria, controls and assessment arrangements sufficient to evaluate compliance with established obligations, authorities and constraints.
Governance Reporting & Checkpoint Definition	Establish where/how adherence is checked and what governance/compliance information must be reported.

Boundary: Obligation definition establishes what must be satisfied; assessment definition establishes the basis for determining satisfaction. This replaces the candidate Compliance Obligation Definition wording. Compliance Governance does not itself perform assurance or assume operational management.

18. Health Service Assurance

Tier and ancestry: CT1 root.

Definition: Establish and maintain justified confidence that health services can be, and are being, delivered in accordance with applicable expectations, obligations, constraints and required levels of performance.

Classification: Harmonia-Relevant.

Health Service Assurance establishes whether sufficient evidence exists to give justified confidence in the organisation's ability to deliver its health services appropriately. Its macro-level subject is that ability. Security, information, technology, process, clinical and other concerns may contribute criteria/evidence without becoming separate Business Assurance Capability trees. The independently governed evaluation already established in the canonical architecture is a required assurance approach where REQ-FND-005 applies; the reconciliation below preserves its requirements.

CT2 — Health Service Assurance Specification

Tier and ancestry: CT2; parent Health Service Assurance (CT1).

Definition: Specify how macro-level service-delivery assurance will be undertaken and what sufficiently supports confidence.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Health Service Assurance Specification; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Assurance Approach Definition	Define scope, evidence expectations, responsibilities, methods, applicable degree of independence and timing/frequency for service-delivery assurance.
Assurance Evaluation Criteria Governance	Establish and maintain criteria against which sufficient assurance can be determined.

Boundary: Criteria may draw on workforce, facilities, clinical practice, information, technology, security, privacy, governance, dependencies and operational state. These contributing concerns do not create separate Business Assurance trees. An assurance approach SHALL preserve every applicable higher-level independence requirement; it cannot designate self-attestation as sufficient where REQ-FND-005 requires independent governed assurance.

CT2 — Assurance Evaluation

Tier and ancestry: CT2; parent Health Service Assurance (CT1).

Definition: Determine what justified confidence the applicable evidence supports.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Assurance Evaluation; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Assurance Evidence Assessment	Evaluate available evidence against the established assurance approach and criteria.
Assurance Conclusion Determination	Establish the legitimately supported conclusion, including insufficiency, qualification and unresolved concerns where applicable.

Boundary: Evidence assessment ≠ conclusion determination ≠ operational response. Insufficient evidence establishes neither assurance nor non-assurance. Assurance manages its own evaluation, not its subject. These BC responsibilities do not remap existing Assurance Design, Assurance Criteria Management or Governed Assurance BEC ownership.

Assurance Independence Reconciliation

Independence is a required property of applicable assurance approaches/evaluations, rather than the exhaustive definition of the macro-level Business Capability. This does **not** make Harmonia's already-required independent governed assurance optional. REQ-FND-005 requires it for activities, information and outcomes **for which assurance is required**. Its applicability is bounded; it neither defines every enterprise assurance approach nor requires assurance of every activity.

The approved Strategy derivation and Business assurance responsibilities implement that obligation through independently governed progression and conclusion authority. Assurance Approach Definition may determine applicable methods, responsibilities and degree of independence but SHALL NOT weaken those requirements. The subject may contribute evidence; it shall not solely determine, suppress, manufacture or retrospectively alter its assurance outcome. Evidence dependency is distinct from control dependency. No external assessor or physical separation is implied.

The broader organisational capability concerns justified confidence in service delivery. Harmonia's assigned participation remains assurance of explicitly required facts/behaviour; it does not acquire clinical-adequacy assurance or management of the service. Thus the old independent-evaluation definition is preserved as the established governed-assurance obligation within the broader capability, rather than discarded or universalised to all enterprise evaluations. Operational uncertainty under AX-15/REQ-FND-004 remains distinct from evidence insufficiency under REQ-FND-005.

Clinical Services Delivery Assurance Boundary

Health Service Assurance SHALL NOT be represented as Clinical Services Delivery Assurance. The enterprise scope's reference to independent clinical audit/evaluation does not make Harmonia responsible for assuring clinical judgement, professional practice, clinical adequacy of care, or delivery of clinical services by healthcare practitioners or healthcare organisations.

Information concerning clinical service delivery MAY be the subject of governed assurance where an explicit applicable requirement establishes that assurance concern. This does not transfer responsibility for Clinical Services Delivery Assurance to Harmonia. Capability 06 — Clinical Quality, Safety & Improvement retains its established clinical quality/safety responsibilities.

> **Harmonia may assure facts and behaviour concerning clinical activity where explicitly required; it does not thereby assure the clinical adequacy of care.**

Meaning of Independent

Independent Assurance requires an independently governed assurance activity/workflow whose progression and assurance conclusion are not controlled by the activity or mechanism responsible for performing or managing the subject being assured.

Independence does not inherently require an external auditor, a different organisation, department, human actor, application, deployment, database or Kubernetes cluster, or physical infrastructure separation. This is a responsibility and authority boundary consistent with REQ-FND-005, not an Application or Technology Architecture decision about physical execution separation.

> **Evidence dependency does not compromise assurance independence; control dependency does.**

The subject being assured may supply evidence. It shall not thereby be solely responsible for determining, suppressing, manufacturing or retrospectively altering its assurance conclusion.

Assurance / Operational Management Boundary

Health Service Assurance does not thereby assume operational management of its subject, assignment, delegation, reassignment, operational escalation, operational remediation, workflow progression, Incident Management or Incident Response. Findings, exceptions and conclusions may inform or initiate appropriate operational activity; responsibility for that activity remains with its operational capability.

> **Assurance is the QA service, not the Job Foreman.**

An assurance activity may manage its own internal assurance workflow without thereby assuming management of the subject being assured.

Governance, Management, Monitoring and Assurance Boundaries

Concern	Purpose and responsibility	Boundary
Governance	Establishes what must be done or must be true; applicable obligations, constraints and authority; reporting required to demonstrate compliance; and checkpoints required to determine whether obligations and constraints are satisfied.	Establishes the normative operating envelope without determining operational means.
Management	Establishes and coordinates what must be done operationally and how it will be performed within constraints; operational-state monitoring, activity progression, progress reporting, assignment/ reassignment, escalation necessary for completion, response and remediation.	Owns progression of the managed activity. Being governed does not relocate activity management to Region 5.
Management Monitoring	Observation and assessment of the current state of an activity for the purpose of determining progress, completion, fulfilment or impediments, so that the containing management or workflow activity can determine what should happen next.	May cause or inform progression, pause, retry, escalation, reassignment, response or completion. Observation alone is not Independent Assurance.
Independent Assurance	Evaluation of a governed subject against applicable obligations, constraints, controls, performance expectations, quality criteria or compliance checkpoints, using sufficient trustworthy evidence to establish an assurance conclusion.	Independently governs its evaluation and conclusion; does not progress or manage its subject.

Management Monitoring evaluates activity state in order to progress or manage the activity. Independent Assurance evaluates a governed subject against governing criteria in order to establish an assurance conclusion.

Evidence Distinctions

Evidence production does not itself perform assurance. Security, privacy, clinical, operational and information-management capabilities may produce trustworthy evidence used by Health Service Assurance without becoming assurance capabilities.

RAD source listing (presentation only; no inferred ancestry).

Provenance ≠ Assurance

Audit information / AuditEvent ≠ Assurance

Operational monitoring ≠ Assurance

Workflow progression ≠ Assurance

Business Outcome ≠ Assurance Outcome

Workflow completion ≠ Assurance

Incident ≠ Assurance Finding

Incident Response ≠ Assurance

Risk ≠ Assurance Finding

Risk ≠ Assurance Outcome

Subject-Specific Governance

> **Subject-specific governance remains with the capability responsible for that subject where governance is intrinsic to managing the subject itself.**

Health Products & Clinical Technology (05), Standards, Semantics & Reference Governance (10), Security, Privacy & Digital Trust (12), and Workforce & Organisational Capability (15) retain their existing governance semantics and regional placement. Capability 12's compliance auditing and audit-record production do not, merely by that wording, establish independent assurance. Region 5 concerns governance and independent assurance as primary responsibilities.

Classification Reconciliation and Rationale

CT1	Previous → reconciled classification	Semantic rationale
05 Health Products & Clinical Technology	Reference → Harmonia-Relevant	Device integration provision/data integration and service-relevant device context materially affect information exchange and care enablement. Supply-chain and maintenance authority remain external.
07 Community Health & Wellbeing	Reference → Harmonia-Relevant	The established preventative programme/ cohort distinctions materially frame information, notification and care enablement. Wider public-health enterprise activity is not assigned to Harmonia.
08 Population Health & Health-System Planning	Adjacent → Harmonia-Relevant	Formal notification and governed population information materially support required health activity; planning remains externally governed and is not Core through context consumption.
13 Health Research & Innovation	Adjacent → Harmonia-Relevant	Governed provision of existing information for an authorised research purpose is material support, not passive context or ownership of trials/research. Innovation evaluation does not acquire a separate platform assignment.
14 Health Service Direction & Stewardship	Reference → Harmonia-Relevant	Service needs, viability, dependencies and alignment facts materially frame Harmonia's service information and derivation requirements. This does not assign executive/service-management authority or establish a future BEC.
15 Workforce & Organisational Capability	Harmonia-Relevant retained	Professional standing, privilege, coverage and organisational/service-offering facts materially constrain care enablement. Directory support does not make workforce/service governance Core.
16 Corporate Resources & Enterprise Services	Reference → Harmonia-Relevant	Required clinical activity/utilisation evidence is material financial/charging support. Corporate support context also matters, but contextual consumption alone does not classify every support CT3 Relevant or Core.
17 Health Service Governance	Unresolved retained	The normative and compliance-definition responsibilities are established, but material platform participation/ enablement is not yet sufficiently assigned to distinguish Adjacent from Relevant/Core.

CT1	Previous → reconciled classification	Semantic rationale
18 Health Service Assurance	Unresolved → Harmonia-Relevant	Already-approved independent governed assurance materially supports service confidence. Harmonia does not own macro-level clinical/service delivery assurance, so the whole BC is not classified Core.
01–04, 06, 09–12	Established assignments retained	Their material enabling/direct platform responsibilities remain established; organisational definitions do not turn clinical judgement, consent authority or enterprise decisions into software responsibilities. Core is bounded to assigned platform activity.

Unresolved Business Capability Matters

OI-BC-001 — Medication Compounding and Preparation

Unresolved. Whether compounding/specialised preparation warrants a distinct CT3 beneath Medication & Pharmacy Services is neither established nor rejected. Absence does not mean subsumption by Prescription Medication Dispensing, irrelevance, exclusion or representation by another Capability. Adjudication must preserve peer completeness and the R1/R2 materiality test.

OI-BC-002 — Health Service Operational Readiness Placement

The identified semantic concern asks: **Can this Health Service Offering actually be delivered from this operational context?** It establishes collective readiness from workforce, administration, spaces, equipment/devices, materials, utilities, ICT/communications and other dependencies without owning them. Ongoing site/service readiness and readiness for a session are presently exercises of the same concern; session-level exercise does not establish another Capability.

Its provisional name/identified identity and boundary are retained, but **CT2 parent, CT1 ancestry and canonical placement remain unresolved**. No complete CT1→CT2→CT3 path is asserted. Offering defined/temporally available ≠ collectively ready; individual resource ready ≠ service ready; care-place readiness ≠ service readiness; readiness oversight ≠ clinical execution or supporting-resource ownership. It SHALL NOT be placed beneath Health Service Alignment, Healthcare Service Offering Management, Care Scheduling & Capacity Management, Corporate Support Services, Health Product Management or another convenient parent without explicit adjudication.

Other Preserved Uncertainties and Canonical Conflicts

- Clinical Determination and Individual Care Planning have no established CT3 set in the supplied decisions. Clinical Intervention's three-child candidate remains provisional; DN-BC-010's taxonomy question is not closed by the queue.
- DN-BC-015 defines **Clinical Device Maintenance Management** as maintaining or restoring acceptable device condition through maintenance, testing, calibration, inspection, fault remediation and repair, distinct from monitoring and overall lifecycle disposition. Its Working Decision and closing guardrail nevertheless specify seven CT3s omitting Maintenance. Its semantics are preserved, but its inclusion/tier/ancestry requires adjudication; no eighth child is manufactured.
- **Health Information Sharing Programme Management** is named by DN-BC-027's spelling correction, but that note supplies no precise tier/parent assertion. Its name is retained without constructing ancestry or an additional definition.

- **Health Information Exchange** is the approved BC CT2 name and an existing BEC name. Their identities/meanings remain distinct. This conflicts with CM-R05 and the BC/BEC naming guardrail; neither a BEC rename nor an identity merger is authorised here. The naming point remains stopped pending adjudication.
- Generic Continuous Improvement placement, external device/safety/quality monitoring providers, the possible Identity Assurance Representation BEC name and general UI/ presentation enablement remain unestablished/parked. No standards or other peer is added merely for symmetry. Other undecomposed enterprise scope remains explicitly unmodelled.

Semantic Relationships and Downstream Boundary

Business capabilities may provide evidence, context, identified events or established information to other business capabilities while retaining their own outcomes and source authority. Unless explicitly required otherwise, such provision is contextual rather than a universal dependency. **Information consumed ≠ ownership transferred; context supplied ≠ target outcome established.** These relationships are not decomposition, workflow order, runtime invocation, Service identity or application dependency. Any RADS relationship must preserve the exact meaning; where no governed type fits, preserve it as unresolved rather than substitute a stronger convenient type.

Established BC Evidence and Context Relationships

The following relationships preserve the adjudicated business meaning. They do not require every source instance to produce an input for every target instance, or make one capability perform another. A conditional relationship remains conditional. No named Business Service or machine-readable relationship type is allocated by this table.

Source	Recipient	Established meaning
Population Health Surveillance Management	Population Health Longitudinal Assessment; Population Health Event Identification	Supplies the population, concern and purpose of managed surveillance.
Population Health Longitudinal Assessment	Population Health Event Identification	Supplies trends/change evidence that may support recognition of a significant event.
Population Health Event Identification	Notifiable Health Event Management	Supplies an identified event for consideration under applicable notification obligations.
Population Health Longitudinal Assessment; Population Health Surveillance Management; Population Health Event Identification; Notifiable Health Event Management	Population Health Reporting	Supply their established assessments, surveillance state, events and notification information for reporting.
Population Health Longitudinal Assessment; Population Health Event Identification	Preventative Health Problem Identification	Supply population-health evidence/ identified concerns without determining the preventative response.
Population Health Longitudinal Assessment	Preventative Health Cohort Management	Supplies population characteristics relevant to cohort definition, stratification or review.
Population Health Event Identification	Preventative Health Cohort Management	May supply event context for identifying the population requiring preventative attention.

Source	Recipient	Established meaning
Individual Care Delivery	Population Health Surveillance	Individual information may contribute population evidence without becoming a population conclusion.
Population Health Surveillance	Individual Care Delivery	Population findings may inform individual care without establishing an Individual Clinical Determination.
Population Health Surveillance	Clinical Quality Management; Clinical Safety Management	May supply evidence relevant to their distinct assessments or risk identification; their information may also contribute population evidence.
Population Health Longitudinal Assessment; Population Health Event Identification	Health Service Demand Assessment	Supply population state/change or identified-event evidence relevant to expected service demand.
Health Service Demand Assessment	Health Service Capacity Planning	Supplies assessed demand evidence relevant to required capacity.
Health Service Capacity Planning	Health Service Distribution Planning	Supplies planned-capacity requirements relevant to the service footprint.
Operational Care Capacity Management	Health Service Demand Assessment; Health Service Capacity Planning	May supply operational pressure/ utilisation and capacity experience as planning evidence.
Health Service Distribution Planning	Healthcare Service Offering Management	May supply planned-footprint context for identifiable offerings.
Standards, Semantics & Reference Governance	Health Information Structure Management	Supplies governed semantic artefacts for reference/binding while retaining their authority.
Health Information Structure Management	Health Information Data Management	Supplies the structure, references, constraints and bindings through which content is meaningful.
Health Information Data Management	Health Information Lifecycle Management; Health Information Catalogue and Index Management; Health Information Compartmentalisation; Health Information Data Correlation	Supplies the managed information subject/content for these distinct responsibilities.
Health Information Catalogue and Index Management	Health Information Data Retrieval	Supplies existence, discovery and location context.
Health Information Compartmentalisation	Health Information Data Retrieval	Supplies applicable compartment scope/ context without making an access decision.
Health Information Data Correlation	Health Information Data Retrieval	May supply relationships relevant to identifying related requested information.
Health Information Structure Management; Health Information Lifecycle Management	Health Information Data Retrieval	Supply interpretation and applicable current/historical/lifecycle context.
Clinical Knowledge Context Identification	Clinical Knowledge Context Extraction	Supplies the identified source/context for extraction.
Clinical Knowledge Context Extraction	Clinical Knowledge Information Formulation	Supplies relevant extracted material for formulation.
Clinical Knowledge Information Formulation	Clinical Knowledge Concept Association; Clinical Knowledge Review & Approval Management	Supplies formulated knowledge for association and governed review.
Standards, Semantics & Reference Governance	Clinical Knowledge Concept Association	Supplies governed concepts/artefacts without transferring semantic authority.

Source	Recipient	Established meaning
Clinical Knowledge Concept Association	Clinical Knowledge Review & Approval Management	Supplies semantic-association context relevant to review.
Clinical Knowledge Review & Approval Management	Clinical Knowledge Publication & Sharing	Supplies knowledge whose applicable approval standing is established.
Clinical Knowledge Publication & Sharing	Individual Care Delivery	Supplies reusable knowledge; the individual-care capability establishes its clinical application/conclusion.
Health Information Management	Clinical Knowledge Context Identification	May supply source information containing potential reusable knowledge.

Material Traceability Requirements

Later derivation SHALL preserve established CT3 identity and the distinction between the Business purpose it supports and its owning BEC CT3 Feature frame. It SHALL NOT attach a Harmonia Feature directly to a BC CT3. Where the following contexts are material, traceability must preserve them without prescribing an Information Concept, representation, interface or implementation:

Concern	Context/evidence to preserve
Clinical risk	Evidence potentially indicating risk, the characterised risk, mitigation, active response and resulting state remain distinct.
Clinical quality	Underlying criterion authority; evidence source and purpose; criteria/evidence supporting an assessment; assessment authority distinct from its report and subsequent improvement.
Preventative health	Evidence supporting problem identification; target cohort; membership reasoning, inclusions/exclusions and applicable entry/exit or stratification. Programme type does not create another BC.
Population health	Population/cohort/geographical and temporal scope; evidence supporting assessment/event identification; notification obligation, recipient/authority, status and supporting event evidence.
Health-system planning	Demand basis, assumptions and scope; capacity requirements and distribution context; planned capacity distinguishable from operational evidence.
Health information	Applicable structures/reference targets/bindings and semantic authority; lifecycle state/history; catalogue versus content; compartment versus access decision; correlations and source provenance; retrieved information context.
Clinical knowledge	Source/extraction provenance and supporting formulation material; concept authority; review/approval state, conditions and history; publication preserving standing and limits; individual use distinct from general knowledge authority.

Later traceability must preserve source provenance, applicable context, temporal standing and these distinct business outcomes. Criteria do not replace governing requirements; reported representations do not originate the facts/conclusions they present. Programme-specific implementation does not establish another BC. Implementation reuse does not collapse BC peers or reverse-derive their meaning.

The separately approved assurance support derivation and B2.1 trace remain intact, with no extension to the newly reconciled CT3s. This reconciliation establishes no BEC ancestry, no framing of the 137 Features, no new BC↔BEC mapping, no allocation of Canonical IDs/local codes and no Domain05 B2.2 work. The 69 unresolved enabling descriptions remain unresolved. Those are subsequent phases.

- Business Enabling Capabilities
- Capability decomposition and derivation model

- Historical five-region reconciliation
- Business Architecture reference candidate

9. Canonical Business Enabling Capability Model & Features

RAD source: `docs/markdown/02-strategy/capabilities/business-enabling-capabilities.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Canonical Business Enabling Capability Model & Features

Overview & Architecture

The Business Enabling Capability model defines **what systems and information infrastructure must enable or provide in support of the healthcare enterprise**.

Where Business Capabilities define the high-level healthcare landscape (what the enterprise does), Business Enabling Capabilities specify the functional abilities systems must deliver to make healthcare delivery, administration, and operations possible.

Architectural Principles Governing this Model

1. **Authentic Healthcare Operating Contexts**: Capabilities are grouped into five authentic healthcare contextual views. They reflect genuine healthcare operating environments (e.g., Ward Operations, Order Administration, Patient Clinical Record), **not** low-level integration engine plumbing or middleware mechanics.
2. **Independence from Commercial Product Boundaries**: Capabilities are defined independently of commercial software product boundaries (such as PAS, EMR, LIS, RIS, HRMS, or FMIS). A commercial product may implement multiple capabilities, drawing ”dotted lines” around parts of the model; the capability model remains orthogonal to vendor packaging.
3. **Atomic Feature Decomposition**: Within established Harmonia-Relevant and Harmonia-Core Capability contexts, system behaviour is decomposed into atomic **Features**—the smallest useful, testable, technology-neutral statements of system-enabled behaviour. Reference and Adjacent capabilities are intentionally not decomposed into features.
4. **Technology Neutrality**: No capability or feature is defined in terms of specific software products (HAPI FHIR, Infinispan, PostgreSQL, ActiveMQ Artemis, Camel, Netty, Vue, Spring) or runtime plumbing (threads, queues, ports, DDL).
5. **Architectural Scope Sufficiency**: The Business Enabling Feature catalogue provides the established R1.x/R2.x Feature baseline for the capabilities and contextual views presently decomposed. Completeness SHALL NOT be interpreted as requiring artificial Feature decomposition of capabilities for which no Feature-level decomposition has been architecturally established. Roadmap items (such as authoritative master-patient EMPI reconciliation) are explicitly demarcated as uncommitted Harmonia 3.x candidates.

Under approved G1 K9, the five views, catalogue numbering, indentation and decomposition presentation do not establish Architectural Element identity, CT1 / CT2 / CT3 assignment, ancestry or root status. The 137 established Features retain their FT type, names and identifiers. Affected complete Capability ancestry and structural Canonical IDs remain unresolved. Derivation may proceed from established ownership and responsibility without inventing intermediate elements.

The fixed Capability decomposition requires **CT1 → CT2 → CT3 → FT** for every fully reconciled Feature. Every Feature SHALL be framed by CT3 with CT2 and CT1 parents; tiers SHALL NOT

be skipped. A Feature cannot compensate for incomplete upstream Capability modelling. Existing Features without established CT3 frames remain valid evidence with Capability ancestry/decomposition requiring separate reconciliation; their current placement does not establish direct CT1/CT2 → FT permission. Neither the 69 unresolved enabling catalogue descriptions nor any other presentation entry becomes a CT3 Capability through this obligation. No Capability, tier assignment, Feature, local ID or complete Canonical ID is created or changed here.

The approved Health Service Assurance derivation additionally establishes **Assurance Design**, **Assurance Criteria Management** and **Governed Assurance** as Business Enabling Capabilities supporting BC-18. Their definitions and assurance principles are recorded below. Their contextual-view placement, relevance classification, Capability Tier, complete ancestry and structural Canonical IDs remain unresolved; they are not assigned through document position or lexical similarity. No additional Feature catalogue is derived.

The Five Contextual Views

The Business Enabling capabilities are structured across five contextual views:

RAD source listing (presentation only; no inferred ancestry).

1. ENTITY MANAGEMENT

   Governance and administration of healthcare entities throughout their lifecycle: Clients, Providers, Organisations, Locations, Services, Products, and Devices.

2. SERVICE ADMINISTRATION

   Administrative, logistical, and financial progression of care: Client Intake, Referrals, Encounters, Orders, Diagnostics, Care Plans, Clinical Messaging, and Billing Extraction.

3. SERVICE DELIVERY

   Systems enablement across clinical delivery settings: Primary Care, Acute Inpatient, Emergency, Surgical, Diagnostic, Medication, Allied Health, Community, and Remote/Virtual Care.

4. HEALTH SERVICE OPERATIONS

   Facility logistics, bed and care-place management, capacity, clinical resource coordination, work allocation and dispatch, patient transport, and discharge progression.

5. INTRINSIC / SHARED ENABLEMENT

   Reusable system capabilities: Longitudinal Clinical Record, Health Information Exchange, Information Access, Policy & Security Control, Collaboration, Workflow Coordination, and Presentation.

Architectural Scope Statement (Harmonia 1.x/2.x vs. 3.x Roadmap)

> The Business Enabling Feature catalogue provides the established R1.x/R2.x Feature baseline for the capabilities and contextual views presently decomposed. Completeness SHALL NOT be interpreted as requiring artificial Feature decomposition of capabilities for which no Feature-level decomposition has been architecturally established.

- **Established Coverage**: The model records established capability responsibilities and the 137-Feature baseline. BC-17 enablement and the assurance capabilities' unestablished contextual-view, tier, ancestry and Feature relationships remain explicit gaps; structural symmetry is not a completion requirement.

- **Sufficient Does Not Mean Frozen Forever**: Additional capabilities and features may be introduced where future Harmonia scope requires them.
- **Roadmap Demarcation — Patient Identity**:
  - **Harmonia 1.x/2.x Scope**: Includes cross-system identifier resolution, identifier correlation, identity alias association, identity provenance, identity federation across regional nodes, and governed administrative correction.
  - **Harmonia 3.x Roadmap Candidate**: Authoritative master-patient identity reconciliation, probabilistic matching algorithms, and Enterprise Master Person Index (EMPI) merge/unmerge engines remain an uncommitted Harmonia 3.x candidate. Harmonia 1.x/2.x relies on authoritative upstream PAS/MPI feeds rather than acting as the master clinical EMPI authority.

View 1: Entity Management

Entity Management encompasses systems enablement for identifying, governing, and managing core healthcare entities throughout their operational lifecycles.

RAD source listing (presentation only; no inferred ancestry).

1. Client Administration

1.1 Person Identity [Harmonia-Core]

1.2 Healthcare Subject [Harmonia-Core]

1.3 Client Relationship [Harmonia-Relevant]

1.4 Client Privacy [Harmonia-Core]

2. Provider Administration

2.1 Practitioner [Harmonia-Core]

2.2 Practitioner Role [Harmonia-Core]

2.3 Provider Relationship [Harmonia-Relevant]

2.4 Provider Engagement [Harmonia-Relevant]

3. Organisation Administration

3.1 Healthcare Organisation [Harmonia-Core]

3.2 Organisational Structure [Harmonia-Core]

3.3 Organisation Contact [Harmonia-Relevant]

4. Location Administration

4.1 Healthcare Location [Harmonia-Core]

4.2 Location Structure [Harmonia-Core]

4.3 Care-Place Definition [Harmonia-Core]

5. Health Service Administration

5.1 Healthcare Service Definition [Harmonia-Core]

5.2 Service Provision Relationship [Harmonia-Core]

5.3 Service Availability Definition [Harmonia-Core]

5.4 Service Eligibility Definition [Harmonia-Relevant]

6. Health Product Administration

6.1 Health Product Definition [Adjacent]

6.2 Product Formulary & Catalogue [Adjacent]

6.3 Product Batch & Serial Tracking [Reference]

7. Clinical Device Administration

7.1 Device Definition [Harmonia-Relevant]

7.2 Device Instance [Harmonia-Relevant]

7.3 Device Association [Harmonia-Relevant]

7.4 Device Connectivity [Harmonia-Relevant]

Detailed Capability & Feature Catalogue: Entity Management

1. Client Administration

Governs the representation, identification, and demographic context of individuals receiving care.

- **1.1 Person Identity** (`Harmonia-Core`)
  - *Definition*: Resolving, maintaining, and correlating identities and identifiers assigned to an individual across different authorities.
  - `FEAT-EM-01`: **Identifier Resolution**: Ingest and resolve external identifiers against registered issuing authorities and namespace systems.
  - `FEAT-EM-02`: **Cross-Authority Identifier Correlation**: Correlate disparate local identifiers (e.g., hospital MRN, clinic client ID, national IHI) belonging to the same individual.
  - `FEAT-EM-03`: **Identity Alias Association**: Maintain explicit associations between active identifiers and historical or demographic aliases without destructive overwriting.
  - `FEAT-EM-04`: **Governed Identity Correction**: Apply administrative identity updates, linked-record corrections, and split/merge notifications propagated from authoritative sources.
- **1.2 Healthcare Subject** (`Harmonia-Core`)
  - *Definition*: Managing the core healthcare subject context bound to clinical records and operational activities.
  - `FEAT-EM-05`: **Subject Context Binding**: Bind authenticated person identity to clinical encounters, orders, and observations as the enduring subject of care with verifiable association.
  - `FEAT-EM-06`: **Demographic Context Governance**: Manage essential demographic attributes (date of birth, sex, vital status) with full provenance and timestamping.
- **1.3 Client Relationship** (`Harmonia-Relevant`)
  - *Definition*: Representing relationships between the client and carers, guardians, or contacts.
  - `FEAT-EM-07`: **Next-of-Kin & Guardian Association**: Associate nominated emergency contacts, legal guardians, and care agents with the client record.
  - `FEAT-EM-08`: **Relationship Validity Tracking**: Maintain effective date intervals and authority scopes for nominated client relationships.
- **1.4 Client Privacy** (`Harmonia-Core`)
  - *Definition*: Enforcing client-level privacy preferences, consent directives, and confidentiality markers.
  - `FEAT-EM-09`: **Consent Directive Evaluation**: Evaluate client consent directives against requested information disclosure and exchange actions.
  - `FEAT-EM-10`: **Confidentiality Flag Enforcement**: Enforce restricted access flags (e.g., VIP, sensitive diagnosis, restricted record) across query and presentation interfaces.

2. Provider Administration

Governs healthcare practitioners, their professional qualifications, operational roles, and organisational engagements.

- **2.1 Practitioner** (`Harmonia-Core`)
  - *Definition*: Identifying and verifying individual healthcare professionals.

  - FEAT-EM-11: **National Practitioner Verification**: Ingest, validate, and verify national practitioner identifiers (e.g., HPI-I) and professional registration status.
  - FEAT-EM-12: **Practitioner Profile Governance**: Maintain core professional demographic details, qualifications, and active practice status.
- **2.2 Practitioner Role** (Harmonia-Core)
  - *Definition*: Modeling the formal role, specialty, and operational capacity of a practitioner at a given organisation or location.
  - FEAT-EM-13: **Role & Specialty Modeling**: Define and associate clinical specialties, practice roles, and service capacities for a practitioner.
  - FEAT-EM-14: **Electronic Endpoint Binding**: Associate validated communication endpoints (secure messaging addresses, direct routing queues) with specific practitioner roles.
- **2.3 Provider Relationship** (Harmonia-Relevant)
  - *Definition*: Managing organizational affiliations and multidisciplinary team relationships.
  - FEAT-EM-15: **Practitioner-Organisation Affiliation**: Maintain temporal affiliations between practitioners and healthcare organizations.
- **2.4 Provider Engagement** (Harmonia-Relevant)
  - *Definition*: Capturing terms of service and clinical privilege agreements.
  - FEAT-EM-16: **Clinical Privileging Record**: Track active clinical admitting and practice privileges within healthcare facilities.

3. Organisation Administration

Governs healthcare delivery organizations, facilities, and network structures.

- **3.1 Healthcare Organisation** (Harmonia-Core)
  - *Definition*: Authoritative identification and profiling of legal and operational healthcare entities.
  - FEAT-EM-17: **National Organisation Verification**: Validate national organization identifiers (e.g., HPI-O) and legal entity classifications.
  - FEAT-EM-18: **Organisation Profile Management**: Govern directory profiles, operational status, and published service descriptions.
- **3.2 Organisational Structure** (Harmonia-Core)
  - *Definition*: Managing parent-child hierarchies, departments, and corporate relationships.
  - FEAT-EM-19: **Hierarchical Structure Navigation**: Model, traverse, and resolve nested departmental, divisional, and regional healthcare networks.
- **3.3 Organisation Contact** (Harmonia-Relevant)
  - *Definition*: Maintaining verified administrative and clinical communication channels.
  - FEAT-EM-20: **Directory Contact Management**: Manage verified phone, email, and administrative contact channels for healthcare facilities.

4. Location Administration

Governs the physical spaces, facilities, and care environments where healthcare is delivered.

- **4.1 Healthcare Location** (Harmonia-Core)
  - *Definition*: Identifying and profiling geographic locations, physical campuses, and health facilities.
  - FEAT-EM-21: **Location Geospatial & Address Resolution**: Validate physical addresses, geospatial coordinates, and location operational status.
- **4.2 Location Structure** (Harmonia-Core)
  - *Definition*: Modeling the physical architecture of healthcare campuses.
  - FEAT-EM-22: **Physical Hierarchy Modeling**: Represent campus, building, wing, floor, and ward architectural relationships.

- **4.3 Care-Place Definition** (`Harmonia-Core`)
  - *Definition*: Modeling the atomic care delivery places (beds, bays, treatment chairs, surgical tables).
  - `FEAT-EM-23`: **Atomic Care-Place Modeling**: Define specific beds, care-bays, and examination rooms with their physical capabilities (e.g., telemetry-capable, negative-pressure).

5. Health Service Administration

Governs the healthcare services offered by providers and organizations.

- **5.1 Healthcare Service Definition** (`Harmonia-Core`)
  - *Definition*: Specifying clinical and administrative health services offered to the public or clinical referrers.
  - `FEAT-EM-24`: **Service Catalogue Publishing**: Publish structured healthcare service definitions, clinical specialties, and intake criteria to regional directories.
- **5.2 Service Provision Relationship** (`Harmonia-Core`)
  - *Definition*: Binding health services to specific organizations, locations, and practitioner roles.
  - `FEAT-EM-25`: **Service-Location-Provider Binding**: Correlate which organization provides a named health service at which physical location with which practitioner roles.
- **5.3 Service Availability Definition** (`Harmonia-Core`)
  - *Definition*: Modeling regular operating hours, exceptional closures, and emergency availability.
  - `FEAT-EM-26`: **Operating Schedule Specification**: Define temporal availability, clinic operating hours, and on-call service windows.
- **5.4 Service Eligibility Definition** (`Harmonia-Relevant`)
  - *Definition*: Capturing referral criteria, catchment boundaries, and clinical eligibility.
  - `FEAT-EM-27`: **Eligibility & Catchment Specification**: Define clinical referral thresholds and geographic catchment boundaries.

6. Health Product Administration

Governs medications, blood products, implants, and consumable health goods.

- **6.1 Health Product Definition** (`Adjacent`): Classification of pharmaceutical and consumable products.
- **6.2 Product Formulary & Catalogue** (`Adjacent`): Management of approved facility medication formularies.
- **6.3 Product Batch & Serial Tracking** (`Reference`): Supply chain batch and lot management. *(No features decomposed for Reference/Adjacent capabilities).*

7. Clinical Device Administration

Governs medical equipment, patient monitors, diagnostic instruments, and telehealth devices.

- **7.1 Device Definition** (`Harmonia-Relevant`)
  - *Definition*: Specifying medical equipment types, models, and regulatory classifications.
  - `FEAT-EM-28`: **Device Type Specification**: Catalog medical device models, regulatory approvals, and capabilities.
- **7.2 Device Instance** (`Harmonia-Relevant`)
  - *Definition*: Tracking physical device hardware instances and serial numbers.
  - `FEAT-EM-29`: **Device Serial Tracking**: Ingest and maintain unique device identifier (UDI) records and serial numbers.
- **7.3 Device Association** (`Harmonia-Relevant`)

  - *Definition*: Associating clinical devices with patients, beds, or rooms.
  - `FEAT-EM-30`: **Device-Patient/Location Assignment**: Track dynamic association between a telemetry monitor, patient, and physical bed.
- **7.4 Device Connectivity** (`Harmonia-Relevant`)
  - *Definition*: Registering communication protocols and network endpoints for medical devices.
  - `FEAT-EM-31`: **Device Endpoint Registration**: Register communication interfaces and security credentials for diagnostic telemetry feeds.

View 2: Service Administration

Service Administration encompasses systems enablement for the operational, administrative, logistical, and financial progression of care.

RAD source listing (presentation only; no inferred ancestry).

1. Client-Centred Administration [Harmonia-Relevant]
2. Referral Administration [Harmonia-Relevant]
3. Scheduling Administration [Harmonia-Relevant]
4. Episode & Encounter Administration [Harmonia-Core]
5. Order Administration [Harmonia-Core]
6. Diagnostic Administration [Harmonia-Core]
7. Medication Administration [Harmonia-Relevant]
8. Procedure Administration [Harmonia-Relevant]
9. Clinical Record Administration [Harmonia-Core]
10. Care Plan Administration [Harmonia-Relevant]
11. Clinical Communication Administration [Harmonia-Core]
12. Funding & Coverage Administration [Adjacent]
13. Clinical Charging & Claims Administration [Adjacent]

Critical Domain Distinctions: Service Administration

Distinction: Referral vs. Order

- **Referral Administration**: A **Referral** formally requests another care participant or service provider to assess, manage or assume some degree of clinical care responsibility. Referral intent, disposition, responsibility actually assumed and Transfer of Care are distinct. Referral may lead to shared or assumed responsibility or transfer of ongoing care, but does not itself establish acceptance, delivery, assumption or Transfer of Care, or universally require an Appointment, Encounter or Order.
- **Order Administration**: An **Order** requests a defined, bounded **fulfilment activity** (e.g., execute a diagnostic test, dispense a medication, perform a procedure) and strictly supports a **closed-loop progression** returning an outcome to the requester.

Distinction: Technical ACK vs. Business ACK

- **Technical Delivery Acknowledgement**: A technical acknowledgement establishes only the acceptance state defined by the applicable interaction contract. It SHALL NOT be interpreted as durable acceptance, committed business state, successful processing or successful business disposition unless that meaning is explicitly established by the governing

contract. HTTP 200 does not universally establish successful business disposition, and HTTP 202 does not universally establish durable queue acceptance; either may have stronger meaning under an explicit contract.
- **Business Acknowledgement**: A clinical/operational message generated by the receiving application or clinical staff confirming that the order or referral has been reviewed, accepted for execution, scheduled, or rejected. Technical Delivery Acknowledgement ≠ Business Acknowledgement

Acknowledgement of a handoff is not, by itself, evidence of eventual downstream business outcome. REQ-FND-001 continues to require the applicable durable acceptance boundary before Harmonia positively acknowledges ingress acceptance.

Business acknowledgement is contextual and may communicate acceptance, rejection or another explicit disposition; it does not establish a universal reviewed → accepted → scheduled progression or transfer originating authority.

Order Closed-Loop Progression Semantics

All order-related capabilities must support the end-to-end closed loop: Destination → Route → Deliver → Acknowledge → Progress → Complete → Outcome

Detailed Capability & Feature Catalogue: Service Administration

1. Client-Centred Administration (`Harmonia-Relevant`)

- **1.1 Client Intake & Demographic Update**
  - `FEAT-SA-01`: **Demographic Update Ingestion**: Ingest and validate patient admission and registration updates from external PAS systems.
  - `FEAT-SA-02`: **Demographic Change Distribution**: Fan out verified client demographic updates to downstream clinical subsystems.

2. Referral Administration (`Harmonia-Relevant`)

- **2.1 Referral Intake & Progression**
  - `FEAT-SA-03`: **Referral Document Ingestion**: Ingest structured clinical referral messages across integration boundaries.
  - `FEAT-SA-04`: **Referral Supporting Information**: Collate and associate supporting clinical documentation, diagnostic history, and patient context with referral requests.
  - `FEAT-SA-05`: **Referral Status & Outcome Tracking**: Track referral operational progression and communicate referral outcomes (acceptance, decline, or redirection) to referring providers.

3. Scheduling Administration (`Harmonia-Relevant`)

- **3.1 Appointment & Booking Administration**
  - `FEAT-SA-06`: **Appointment Notification Ingestion**: Ingest scheduled appointment events from departmental scheduling systems.
  - `FEAT-SA-07`: **Appointment Status Synchronization**: Propagate booking cancellations, reschedules, and attendances across care coordination systems.

4. Episode & Encounter Administration (`Harmonia-Core`)

- **4.1 Encounter Lifecycle Management**

- `FEAT-SA-08`: **Encounter Context Creation**: Create and maintain authoritative encounter tracking contexts for hospital admissions, outpatient visits, and community contacts.
- `FEAT-SA-09`: **Encounter State Progression**: Track encounter status transitions (`PLANNED` → `ARRIVED` → `IN_PROGRESS` → `ON_LEAVE` → `DISCHARGED`).
- `FEAT-SA-10`: **Encounter Bed/Location Association**: Dynamically maintain the association between an encounter, care team, and physical bed or ward location.

5. Order Administration (`Harmonia-Core`)

- **5.1 Closed-Loop Order Progression**
  - `FEAT-SA-11`: **Order Request Ingestion**: Ingest clinical diagnostic, medication, and procedure order requests with validated ordering clinician context.
  - `FEAT-SA-12`: **Order Destination Resolution & Routing**: Resolve fulfillment endpoints and reliably route orders to performing service systems.
  - `FEAT-SA-13`: **Order Closed-Loop Progression Tracking**: Track order progression across destination determination, routing, delivery, receipt acknowledgement (distinguishing technical delivery acknowledgement from business acknowledgement), execution progress, completion, and outcome association.
  - `FEAT-SA-14`: **Order Cancellation & Modification Coordination**: Coordinate order cancellation and amendment requests, ensuring synchronization with performing systems.

6. Diagnostic Administration (`Harmonia-Core`)

- **6.1 Diagnostic Workflow Management**
  - `FEAT-SA-15`: **Diagnostic Request Correlation**: Correlate diagnostic test orders with collected clinical specimens and patient encounters.
  - `FEAT-SA-16`: **Diagnostic Report Ingestion & Binding**: Ingest finalized laboratory and radiology reports, verifying content integrity and binding them to the parent order.
  - `FEAT-SA-17`: **Diagnostic Report Distribution**: Distribute finalized laboratory, pathology, and imaging reports to ordering clinicians and designated care teams.

This Feature retains its finalised-report responsibility. Broader Domain03 Diagnostic Administration behaviour may also handle preliminary, corrected and addendum/correction information without redefining this narrower Feature.

7. Medication Administration (`Harmonia-Relevant`)

- **7.1 Medication Order & Dispense Management**
  - `FEAT-SA-18`: **Medication Order Ingestion**: Ingest inpatient and discharge prescription orders.
  - `FEAT-SA-19`: **Dispense Event Tracking**: Ingest medication dispense verification records from pharmacy management systems.
  - `FEAT-SA-20`: **Medication Administration Record (MAR) Ingestion**: Ingest point-of-care medication administration events.

8. Procedure Administration (`Harmonia-Relevant`)

- **8.1 Procedure & Surgical Administration**
  - `FEAT-SA-21`: **Procedure Booking Ingestion**: Ingest scheduled surgical case bookings and procedural requests.
  - `FEAT-SA-22`: **Procedural Documentation Ingestion**: Ingest operative reports, anesthesia records, and post-procedure summaries.

9. Clinical Record Administration (`Harmonia-Core`)

- **9.1 Clinical Documentation Lifecycle**
  - `FEAT-SA-23`: **Clinical Document Ingestion**: Ingest structured and unstructured clinical documents (discharge summaries, consult notes, letters).
  - `FEAT-SA-24`: **Document Versioning & Supersession**: Manage document classification, author attribution, amendment attribution, versioning, and supersession or withdrawal without destructive data loss.
  - `FEAT-SA-25`: **Document Metadata Indexing**: Index clinical document metadata (author, specialty, encounter, date, confidentiality) for rapid discovery.

10. Care Plan Administration (`Harmonia-Relevant`)

- **10.1 Care Plan Coordination**
  - `FEAT-SA-26`: **Care Plan Ingestion & Distribution**: Ingest multidisciplinary care plans and syndicate updates to authorized care team members.

11. Clinical Communication Administration (`Harmonia-Core`)

- **11.1 Secure Clinical Communication**
  - `FEAT-SA-27`: **Clinical Communication Distribution**: Distribute secure clinical communications between verified healthcare providers and designated care team endpoints.
  - `FEAT-SA-28`: **Delivery Acknowledgement Tracking**: Correlate recipient delivery and read receipts with outbound clinical communications, distinguishing transport delivery from recipient acknowledgement. Receipt/read evidence does not by itself establish business acceptance, clinical review, incorporation, approval, finality or originating authority.
  - `FEAT-SA-29`: **Communication Audit Logging**: Maintain an enduring, non-PHI, tamper-evident log of all clinical communication transactions.

12. Funding & Coverage Administration (`Adjacent`)

- Ingestion of patient insurance coverage and eligibility flags. *(No features decomposed)*.

13. Clinical Charging & Claims Administration (`Adjacent`)

- Export of clinical activity summaries and charging records to billing systems. *(No features decomposed)*.

View 3: Service Delivery

Service Delivery encompasses systems enablement across genuine healthcare delivery settings and clinical care delivery contexts.

These established Business Enabling contextual entries are not the CT2 decomposition of 01 Individual Care Delivery. The Business Capability's six CT2 abilities remain independent of these contexts under B2-R05 and B2-R06.

RAD source listing (presentation only; no inferred ancestry).

1. Clinical Assessment [Adjacent]
2. Primary Care [Harmonia-Relevant]
3. Acute Care [Harmonia-Relevant]

4. Emergency Care [Harmonia-Relevant]
5. Inpatient Care [Harmonia-Relevant]
6. Ambulatory Care [Adjacent]
7. Surgical & Procedural Care [Adjacent]
8. Diagnostic Services [Harmonia-Relevant]
9. Therapeutic Services [Adjacent]
10. Medication Therapy [Harmonia-Relevant]
11. Allied Health [Adjacent]
12. Preventive Care [Harmonia-Relevant]
13. Rehabilitation [Adjacent]
14. Community Care [Harmonia-Relevant]
15. Outreach Care [Harmonia-Relevant]
16. Remote & Virtual Care [Harmonia-Relevant]

Architectural Principles: Service Delivery

1. **Heterogeneous Axes Preserved**: The 16 Service Delivery capabilities represent heterogeneous clinical settings (acute, emergency, primary), modalities (virtual, outreach), and specialties (diagnostic, therapeutic). They are deliberately preserved as authentic healthcare contexts rather than normalized into artificial abstractions.
2. **Zero Harmonia-Core in Service Delivery**: No Service Delivery capability is classified as `Harmonia-Core`. Harmonia provides integration, transport, state management, and notification; **Harmonia does not practice medicine or deliver clinical therapy**.
3. **Canonical Enabling Pattern for Relevant Features**: Provide Context → Receive Clinical State/Outcome → Correlate → Preserve Provenance → Make Available → Enable Next Activity

Detailed Capability & Feature Catalogue: Service Delivery

2. Primary Care (`Harmonia-Relevant`)

- **2.1 Primary Care Coordination**
  - `FEAT-SD-01`: **General Practice Shared Record Ingestion**: Ingest shared health summaries, encounter notes, and immunization records from primary care practice management systems.
  - `FEAT-SD-02`: **Primary Care Notification Dispatch**: Dispatch emergency department arrival, acute admission, and discharge notifications to the patient's registered general practitioner.

3. Acute Care (`Harmonia-Relevant`)

- **3.1 Acute Care Monitoring & Event Coordination**
  - `FEAT-SD-03`: **Acute Clinical State Event Capture**: Ingest clinical deterioration notifications and acute clinical status updates.
  - `FEAT-SD-04`: **Acute Care Clinical Context Provision**: Provide consolidated longitudinal history and active medication lists to acute care clinicians at point of triage.

4. Emergency Care (`Harmonia-Relevant`)

- **4.1 Emergency Care Progression**
  - `FEAT-SD-05`: **Emergency Department Arrival & Triage Ingestion**: Ingest emergency presentation, triage category, and chief complaint records.
  - `FEAT-SD-06`: **Emergency Care Cross-Facility Correlation**: Correlate incoming emergency presentations with recent regional admissions, prior discharge summaries, and known allergy profiles.

5. Inpatient Care (`Harmonia-Relevant`)

- **5.1 Multidisciplinary Inpatient Care Coordination**
  - `FEAT-SD-07`: **Inpatient Rounding Context Provision**: Aggregate diagnostic results, current medication administration records, and nursing notes for multidisciplinary inpatient rounds.
  - `FEAT-SD-08`: **Inpatient Clinical Progression Tracking**: Capture inpatient clinical milestone completions (e.g., medical stability achieved, allied health clearance).

8. Diagnostic Services (`Harmonia-Relevant`)

- **8.1 Diagnostic Service Integration**
  - `FEAT-SD-09`: **Laboratory & Imaging Result Distribution**: Reliably distribute structured diagnostic reports (pathology, radiology) to ordering clinicians and designated secondary recipients.
  - `FEAT-SD-10`: **Diagnostic History Consolidation**: Consolidate historical diagnostic trends (e.g., cumulative laboratory results) across multiple regional providers into an integrated view.

10. Medication Therapy (`Harmonia-Relevant`)

- **10.1 Medication Therapy Provision and Monitoring**
  - `FEAT-SD-11`: **Medication History Reconciliation Ingestion**: Ingest reconciled medication histories compiled during clinical transitions of care.
  - `FEAT-SD-12`: **Medication Administration Tracking**: Distribute medication therapy orders, dispensing updates, and administration records across clinical care boundaries.

12. Preventive Care (`Harmonia-Relevant`)

- **12.1 Screening and Immunisation Enablement**
  - `FEAT-SD-13`: **Immunisation Event Submission**: Ingest point-of-care immunisation administration records and syndicate to national immunization registers.
  - `FEAT-SD-14`: **Screening Recall Notification Distribution**: Distribute clinical screening reminders and recall notices to designated healthcare providers.

14. Community Care (`Harmonia-Relevant`)

- **14.1 Community Care Coordination**
  - `FEAT-SD-15`: **Community Encounter Ingestion**: Ingest home visit notes, community nursing observations, and social support assessments.
  - `FEAT-SD-16`: **Community Care Plan Synchronization**: Synchronize community care plan updates between hospital outreach teams and community health organizations.

15. Outreach Care (`Harmonia-Relevant`)

- **15.1 Outreach Care Continuity**
  - `FEAT-SD-17`: **Outreach Visit Context Provision**: Package and deliver portable clinical summaries and offline-capable care summaries for remote outreach clinics.
  - `FEAT-SD-18`: **Outreach Encounter Outcome Ingestion**: Ingest and reconcile clinical documentation gathered during rural and remote outreach visits.

16. Remote & Virtual Care (`Harmonia-Relevant`)

- **16.1 Remote Patient Monitoring**
  - `FEAT-SD-19`: **Remote Telemetry Ingestion**: Ingest home patient monitoring data, biometric telemetry, and patient-reported outcome measures (PROMs).
  - `FEAT-SD-20`: **Virtual Care Session Context Binding**: Associate synchronous virtual/telehealth consultations with the longitudinal health record and active care episode.

(Capabilities 1 Clinical Assessment, 6 Ambulatory Care, 7 Surgical & Procedural Care, 9 Therapeutic Services, 11 Allied Health, and 13 Rehabilitation remain Adjacent or Reference and are not decomposed into atomic features).

View 4: Health Service Operations

Health Service Operations encompasses systems enablement for the operational management, logistics, capacity, and resource coordination of healthcare facilities and services.

RAD source listing (presentation only; no inferred ancestry).

1. Clinic & Practice Operations [Harmonia-Relevant]
2. Ward Operations [Harmonia-Relevant]
3. Theatre Operations [Harmonia-Relevant]
4. Emergency Department Operations [Harmonia-Relevant]
5. Outpatient Operations [Harmonia-Relevant]
6. Bed & Care-Place Management [Harmonia-Core]
7. Clinical Resource Management [Harmonia-Relevant]
8. Service Capacity Management [Harmonia-Relevant]
9. Clinical Workforce Management [Adjacent]
10. Clinical Rostering [Adjacent]
11. Mobile Staff Management [Harmonia-Relevant]
12. On-Call Management [Harmonia-Relevant]
13. Work Allocation & Dispatch [Harmonia-Core]
14. Clinical Training & Education [Reference]
15. Clinical Competency Management [Reference]
16. Clinical Qualification Management [Harmonia-Relevant]
17. Clinical Supervision [Reference]
18. Patient Transport [Harmonia-Core]
19. Clinical Logistics Coordination [Harmonia-Relevant]
20. Discharge Management [Harmonia-Core]

Architectural Principles: Health Service Operations

1. **Facility Logistics, Not IT Machinery**: Health Service Operations reflects real-world health facility logistics and coordination (wards, beds, staff allocation, patient transfer); it must **never** be redefined or reinterpreted as protocol mediation, message queuing, deduplication, or integration software machinery.
2. **Healthcare Service Context Rule**:

   > **Where operational activity occurs in the context of a Healthcare Service, that service context must be explicitly associated with the activity and state rather than inferred solely from practitioner, organisation, or location.**

1. **General Operational Progression Pattern**: Event → Entity Context → Required Activity → Assignment → Dispatch → ACK → Progress → Outcome → Entity State Change → Next Activity

Detailed Capability & Feature Catalogue: Health Service Operations

1. Clinic & Practice Operations (`Harmonia-Relevant`)

- **1.1 Session & Queue Coordination**
  - `FEAT-HSO-01`: **Clinic Session Tracking**: Ingest and track clinic operating session start, delay, and completion states.
  - `FEAT-HSO-02`: **Operational Queue Progression**: Track client queue progression through reception, waiting area, consultation, and billing.

2. Ward Operations (`Harmonia-Relevant`)

- **2.1 Ward Logistics Coordination**
  - `FEAT-HSO-03`: **Ward Patient Flow Coordination**: Coordinate inpatient ward occupancy, care-place allocation, and patient bed status across care facilities.
  - `FEAT-HSO-04`: **Clinical Handover Context Assembly**: Collate current ward patient lists, active medical concerns, and pending tasks for nursing shift handovers.

3. Theatre Operations (`Harmonia-Relevant`)

- **3.1 Perioperative Logistics**
  - `FEAT-HSO-05`: **Operating Theatre Case Progression**: Track surgical case state transitions (`CALLED` → `IN_THEATRE` → `ANAESTHETISED` → `INCISION` → `CLOSED` → `RECOVERY`).
  - `FEAT-HSO-06`: **Perioperative Resource Notification**: Dispatch alerts to portering, sterilization, and recovery teams based on surgical case progression milestones.

4. Emergency Department Operations (`Harmonia-Relevant`)

- **4.1 ED Operational Coordination**
  - `FEAT-HSO-07`: **ED Bed & Bay Tracking**: Track occupancy and status of emergency cubicles, resuscitation bays, and short-stay beds.
  - `FEAT-HSO-08`: **ED Length of Stay Tracking**: Monitor operational wait times and length-of-stay milestones, generating operational escalation notifications.

5. Outpatient Operations (`Harmonia-Relevant`)

- **5.1 Outpatient Flow Enablement**
  - `FEAT-HSO-09`: **Outpatient Arrival Registration**: Ingest self-service check-in and clinic arrival events, notifying treating clinicians.
  - `FEAT-HSO-10`: **Post-Clinic Order Coordination**: Capture follow-up booking and diagnostic order requirements generated at clinic completion.

6. Bed & Care-Place Management (`Harmonia-Core`)

- **6.1 Bed State & Turnover Management**
  - `FEAT-HSO-11`: **Bed Availability Tracking**: Maintain operational status of all registered beds and care-places (`VACANT`, `OCCUPIED`, `BLOCKED`, `RESERVED`, `DIRTY`, `CLEANING`, `CLEAN`).
  - `FEAT-HSO-12`: **Bed State Progression**: Progress bed states in response to clinical and environmental events (e.g., patient discharge automatically transitions bed to `DIRTY`).
  - `FEAT-HSO-13`: **Bed Readiness Notification**: Notify bed allocation managers and clinical units when a cleaned bed achieves `READY` status.

7. Clinical Resource Management (`Harmonia-Relevant`)

- **7.1 Specialized Equipment Coordination**
  - `FEAT-HSO-14`: **Mobile Clinical Equipment Tracking**: Track physical location and allocation status of shared clinical equipment (infusion pumps, ventilators, mobile telemetry).

8. Service Capacity Management (`Harmonia-Relevant`)

- **8.1 Facility Demand & Capacity Telemetry**
  - `FEAT-HSO-15`: **Operational Capacity Metric Aggregation**: Aggregate operational capacity telemetry (occupancy rates, wait times, diversion status) across facilities.

11. Mobile Staff Management (`Harmonia-Relevant`)

- **11.1 Mobile Worker Dispatch & Safety**
  - `FEAT-HSO-16`: **Mobile Worker Task Dispatch**: Dispatch community care tasks and urgent visits to roving clinicians' mobile devices.
    - **Architectural scope qualification (2026-10-09 human adjudication)**: Dispatch here concerns integration with external/business mobile work-management mechanisms: exchanging externally established assignment, dispatch and operational progression/status information with applicable worker/context information. Harmonia does not thereby determine or manage clinical work, clinical handover, clinical allocation or EMR worklists. External "work/task" vocabulary does not redefine Harmonia synthetic Task. The Feature name/identifier are retained; the proposed name `MobileWorkTaskManagementIntegration` requires separate human confirmation. See the Business integration boundary.
  - `FEAT-HSO-17`: **Mobile Visit Status Tracking**: Ingest arrival, visit progression, and safety check-in milestones from roving staff.

12. On-Call Management (`Harmonia-Relevant`)

- **12.1 On-Call Directory Resolution**

- FEAT-HSO-18: **Active On-Call Provider Resolution**: Resolve currently active on-call specialists, registrars, and operational managers for a given specialty and hospital at any point in time.

13. Work Allocation & Dispatch (`Harmonia-Core`)

- **13.1 Operational Work Assignment & Supervision**
  - FEAT-HSO-19: **Work Item Instantiation**: Instantiate operational units of work from clinical triggers (e.g., bed cleaning required, specimen transport required).
  - FEAT-HSO-20: **Worker Matching & Allocation**: Match work items against available staff roles, credentials, and locations based on allocation policies.
  - FEAT-HSO-21: **Work Dispatch Delivery**: Deliver work orders to target workers, devices, or team queues with technical and operational acknowledgement capture.
  - FEAT-HSO-22: **Work Progress Oversight & Escalation**: Monitor work progression against time-to-completion thresholds, escalating stalled or unacknowledged tasks.

16. Clinical Qualification Management (`Harmonia-Relevant`)

- **16.1 Credential Verification Checkpoint**
  - FEAT-HSO-23: **Operational Credential Check**: Validate that a practitioner possesses active credentials and required competency certs prior to dispatching specialized work.

18. Patient Transport (`Harmonia-Core`)

- **18.1 Patient Transfer Coordination**
  - FEAT-HSO-24: **Transport Request Ingestion**: Ingest internal (portering) and external (ambulance, inter-hospital) patient transfer requests with clinical mobility requirements.
  - FEAT-HSO-25: **Transport Dispatch & Progress Tracking**: Dispatch transport orders, tracking departure, transit, and destination arrival checkpoints.

19. Clinical Logistics Coordination (`Harmonia-Relevant`)

- **19.1 Specimen & Supply Courier Tracking**
  - FEAT-HSO-26: **Pathology Specimen Transport Tracking**: Track pickup, custody chain, transport conditions, and laboratory delivery of clinical specimens.

20. Discharge Management (`Harmonia-Core`)

- **20.1 Discharge Progression & Coordination**
  - FEAT-HSO-27: **Discharge Milestone Tracking**: Track clinical criteria-led discharge milestones (medications dispensed, transportation arranged, home care confirmed).
  - FEAT-HSO-28: **Discharge Readiness & Coordination Oversight**: Track and aggregate multidisciplinary discharge milestones, home care confirmations, and transit readiness across care teams.
  - FEAT-HSO-29: **Discharge Notification Dispatch**: Dispatch departure notifications, updated bed states, and finalized discharge summaries to external care providers upon physical patient exit.

View 5: Intrinsic / Shared Enablement

Intrinsic / Shared Enablement encompasses the horizontal, platform-wide system capabilities required to support all healthcare operating contexts.

RAD source listing (presentation only; no inferred ancestry).

1. Patient Clinical Record [Harmonia-Core]
2. Health Information Exchange [Harmonia-Core]
3. Health Information Access [Harmonia-Core]
4. Health Information Communication [Harmonia-Core]
5. Health Information Control [Harmonia-Core]
6. Clinical Knowledge Services [Harmonia-Relevant]
7. Clinical Collaboration [Harmonia-Core]
8. General Collaboration [Harmonia-Relevant]
9. Workflow & Activity Coordination [Harmonia-Core]
10. Calendar Management [Harmonia-Core]
11. Presentation Services [Harmonia-Core]
12. Information Design Governance [Harmonia-Core]

Critical Domain Distinctions: Intrinsic / Shared Enablement

Health Information Exchange: Four Interaction Models

1. **Submission**: *"I send this to you."* (Point-to-point push of an authored artifact).
2. **Retrieval**: *"I ask for this."* (Synchronous or asynchronous pull of specific records).
3. **Distribution**: *"I deliberately send this to identified recipients."* (Addressed fan-out delivery to designated systems or clinical actors).
4. **Syndication**: *"I publish information/change and subscribers receive matching interests."* (Publish-subscribe topic/criteria distribution).

Workflow & Activity Coordination: Three Work Units

1. **Work Order**: A unit of work where a **human is doing something** physical or operational (e.g., clean bed 4B, transfer patient to X-ray, draw blood sample).
2. **To Do**: A unit of work where a **human is reviewing, updating, or deciding**, including clinical approvals, sign-offs, and triage reviews.
3. **Task**: A unit of **non-human executable work** (internally a synthetic task) executed by platform automation, daemons, or services. *Execution Semantics*: Workflow Management provides the coordination and state engine through which these three activity types progress.

Clinical Collaboration Context

Clinical Collaboration is **Harmonia-Core** because Harmonia provides **Longitudinal Health Record (LHR)-aware collaboration spaces**. Clinicians can converse within secure multidisciplinary team rooms and query patient context directly (e.g., *"What were Fred's last three HbA1c results?"*). *Important Boundary*: A clinical statement made within a collaboration chat does **not** automatically become an authoritative clinical Observation or medical record entry. It remains collaborative discourse unless formally authored into a clinical document.

Presentation Services Decoupling

Presentation Services consumes established identity, security, and clinical context. Presentation does **not** own:

- Authentication or credential storage.
- Identity lifecycle or EMPI reconciliation.
- Consent decisions or access-policy evaluation.
- Authoritative clinical state preservation.

Detailed Capability & Feature Catalogue: Intrinsic / Shared Enablement

1. Patient Clinical Record (`Harmonia-Core`)

- **1.1 Longitudinal Clinical Record Governance**
  - `FEAT-ISE-01`: **Longitudinal Health Record (LHR) Assembly**: Assemble disparate encounters, diagnostic reports, medications, and clinical documents into a unified, vendor-neutral longitudinal timeline.
  - `FEAT-ISE-02`: **Active Clinical Record Access**: Provide low-latency, active distributed access to consolidated patient clinical summaries.
  - `FEAT-ISE-03`: **Durable Clinical Record Preservation**: Durably maintain governed, vendor-neutral longitudinal clinical representation and preservation with verifiable integrity controls.

2. Health Information Exchange (`Harmonia-Core`)

- **2.1 Multi-Model Information Exchange**
  - `FEAT-ISE-04`: **Direct Submission Ingestion & Delivery**: Support push submissions of clinical documents and messages across boundaries.
  - `FEAT-ISE-05`: **Clinical Record Retrieval Mediation**: Mediate query-based retrieval requests across external and internal repositories.
  - `FEAT-ISE-06`: **Addressed Clinical Distribution**: Fan out clinical events and diagnostic reports to designated external and internal recipients with per-destination delivery tracking.
  - `FEAT-ISE-07`: **Event-Driven Syndication**: Publish clinical and directory change events to subscribed systems matching interest filters.

3. Health Information Access (`Harmonia-Core`)

- **3.1 Governed Query & Search**
  - `FEAT-ISE-08`: **Federated Clinical Query**: Execute bounded, indexed queries across distributed clinical resources and directory graphs.
  - `FEAT-ISE-09`: **Access-Controlled Information Filtering**: Filter search results dynamically according to requesting user authorizations and patient consent directives.

4. Health Information Communication (`Harmonia-Core`)

- **4.1 Boundary Protocol Mediation**
  - `FEAT-ISE-10`: **Standards-Based Boundary Exchange**: Provide standards-conformant boundary exchange for healthcare messaging and resource representations.

5. Health Information Control (`Harmonia-Core`)

- **5.1 Policy & Security Governance**
  - `FEAT-ISE-11`: **Health Information Access & Consent Control**: Evaluate patient consent preferences, practitioner authority boundaries, and access control policies for managed clinical records.
  - `FEAT-ISE-12`: **Security Context Propagation**: Propagate tamper-evident, attributable security context across asynchronous processing boundaries.
  - `FEAT-ISE-13`: **Non-PHI Compliance Auditing**: Generate tamper-evident, verifiable audit records of security-significant events without emitting unmasked PHI.

6. Clinical Knowledge Services (`Harmonia-Relevant`)

- **6.1 Terminology & Reference Mapping**
  - `FEAT-ISE-14`: **Clinical Concept Resolution**: Resolve local clinical codes against standard terminologies (SNOMED CT, LOINC, ICD-10).
  - `FEAT-ISE-15`: **Bidirectional Terminology Mapping**: Translate between legacy local code systems and canonical national healthcare profiles.

7. Clinical Collaboration (`Harmonia-Core`)

- **7.1 LHR-Aware Multidisciplinary Collaboration**
  - `FEAT-ISE-16`: **Care-Team Clinical Collaboration Coordination**: Coordinate secure multidisciplinary clinical communication and consultation exchange bound to patient encounters or clinical topics.
  - `FEAT-ISE-17`: **In-Conversation LHR Query Resolution**: Resolve clinical data queries within an authorized collaboration context, presenting verified clinical summaries directly to care team members.
  - `FEAT-ISE-18`: **Zero-PHI Collaboration Metadata Governance**: Ensure collaboration channel titles, topics, and aliases use opaque identifiers free of patient names, MRNs, or sensitive details.

8. General Collaboration (`Harmonia-Relevant`)

- **8.1 Operational Messaging**
  - `FEAT-ISE-19`: **Operational Team Chat Enablement**: Provide secure instant messaging channels for departmental, administrative, and facility support staff.

9. Workflow & Activity Coordination (`Harmonia-Core`)

- **9.1 Multi-Work-Unit Orchestration**
  - `FEAT-ISE-20`: **Work Order Progression**: Manage execution states and dispatch of human operational work orders.
  - `FEAT-ISE-21`: **To Do Decision & Approval Coordination**: Coordinate human clinical review items, sign-offs, and approval workflows.
  - `FEAT-ISE-22`: **Synthetic Task Orchestration**: Execute non-human, automated platform activity units and sequence orchestration.
  - `FEAT-ISE-23`: **Workflow Timeout & Escalation Oversight**: Supervise operational deadlines, executing automated timeout notifications and supervisory escalations.

10. Calendar Management (`Harmonia-Core`)

- **10.1 Operational & Clinical Calendar Sync**
  - `FEAT-ISE-24`: **Schedule Projection Integration**: Project clinician rosters, on-call schedules, and facility operating calendars into operational views.
  - `FEAT-ISE-25`: **Temporal Event Correlation**: Correlate incoming clinical events with scheduled appointments and operational calendar milestones.

11. Presentation Services (`Harmonia-Core`)

- **11.1 Decoupled Human Interaction & Dashboards**
  - `FEAT-ISE-26`: **Longitudinal Clinical & Operational Presentation**: Deliver contextual clinical timelines, directory administration views, and operational management dashboards.

12. Information Design Governance (`Harmonia-Core`)

- **12.1 Canonical Semantics & Schema Governance**
  - `FEAT-ISE-27`: **Information Standards & Semantic Governance**: Govern canonical clinical data definitions, terminology bindings, and exchange profiles across regional systems.
  - `FEAT-ISE-28`: **Semantic Conformance Verification**: Verify inbound and outbound representations against published semantic rules and conformance constraints.

Health Service Assurance: Approved Business Enabling Capabilities

REQ-FND-005 — Independent Assurance of Governed Activity motivates BC-18 — Health Service Assurance. BC-18 requires the three Business Enabling Capabilities defined here. They are collaboratively realised by the approved contributions of existing Enterprise Capabilities and EC-14 — Service Guardian, as recorded in the contribution matrix.

These are Strategy capabilities and semantic boundaries. Their presentation does not create a sixth contextual view, allocate them to an existing view, establish CT1 / CT2 / CT3 ancestry, or derive Business Functions, Services, Processes, Features or assurance Information Concepts.

Assurance Design

- **Definition**: The capability to determine and define how satisfaction of a governed requirement, constraint or expected behaviour will be assured, including its assurance disposition, applicable criteria, required evidence and evaluation expectations.
- **Fundamental Question**: **How will satisfaction of this requirement, constraint or expected behaviour be assured?**
- **Boundary**: Assurance Design is a peer concern to Behaviour Design and Failure/Recovery Design when implementing governed behaviour. This relationship establishes design concerns, not additional capability entries, application design or implementation specifications.

Assurance Criteria Management

- **Definition**: The capability to establish and manage reusable, governed and temporally identifiable assurance criteria through which subjects may be evaluated.
- **Scope**: Criteria may express governance rule boundaries, quality constraints, performance expectations, compliance criteria, temporal criteria, evidence requirements or completeness expectations. These are examples, not an exhaustive taxonomy.

- **Boundary**: Criteria management does not own or replace the authoritative governance requirement from which a criterion may derive. Not every assurance criterion is required to derive from formal Governance. Criteria may have their own lifecycle, version and temporal applicability.

  Assurance Criteria may operationalise a governing requirement for evaluation, but SHALL NOT silently alter, weaken, strengthen or replace the authoritative requirement from which they derive.

Governed Assurance

- **Definition**: The capability to independently evaluate a governed subject against applicable assurance criteria through the assembly and evaluation of sufficient trustworthy evidence, establishing explicit findings, exceptions and assurance conclusions.
- **Fundamental Question**: **What can we independently establish about this subject against these criteria from the available trustworthy evidence?**
- **Assurance Semantics**: Establishing the subject/context of assurance, applicable assurance criteria, evidentiary association, assessment, adjudication, findings, exceptions and conclusions.
- **Boundary**: Governed Assurance does not manage, remediate, assign, delegate, escalate operationally or complete the subject activity. It may manage its own assurance activity; findings may inform operational response without transferring responsibility for that response.

Assurance Disposition

Every implemented behaviour has an assurance aspect. Every governed obligation, functional requirement, non-functional requirement or other defined implementation requirement SHALL have an explicit assurance disposition identifying how its satisfaction is assured.

The approved disposition concepts are:

- **Generic Assurance sufficient**
- **Existing Specific Assurance applies**
- **Existing Assurance must be extended**
- **New Specific Assurance required**
- **Not Assurable**
- **Not Worth Assuring**

These are architectural semantics, not an information model or software enumeration. **Not Assurable** does not mean the requirement is not required. **Not Worth Assuring** does not mean the requirement is optional or unimportant. Both are explicit governed assurance decisions, not absence of assurance configuration. An explicit disposition does not require specific assurance of every individual activity, information item or outcome; REQ-FND-005 applies where assurance is required.

Generic Processing Assurance and System Operations

Harmonia-managed workflow behaviour SHALL be subject to applicable generic processing assurance. This concerns business-level processing behaviour above and beyond System Operations. Potential concerns include progression, completion, timeliness, expected processing pathway, failure behaviour, recovery behaviour, outcome, and aggregate or pattern behaviour. This is not a final exhaustive criteria catalogue.

System Operations asks whether the technical machinery is operating. Generic processing assurance asks whether governed business behaviour is operating within expected boundaries.

Operational Health ≠ Processing Assurance.

Failure and recovery are part of the expected behavioural space of the activity being executed. The activity/Praxis remains responsible for handling its own expected failure and recovery behaviour. Assurance independently establishes whether normal, failure and recovery behaviour remain within applicable expectations. This responsibility boundary does not allocate assurance execution to Praxis or another solution construct.

EC-12 Operational Assurance retains its established resilience, concurrency, exception/recovery and telemetry responsibilities and may contribute directly to generic operational/processing assurance. EC-14 Service Guardian supplies the distinguishing assurance-specific semantics; neither replaces System Operations or the other Enterprise Capability.

Assurance Evidence Assembly and Information Ownership

Assurance Evidence Assembly is the evidence-assembly concern within Governed Assurance; this statement does not establish a fourth Business Enabling Capability or a downstream Function, Service or Information Concept.

Underlying information used as assurance evidence remains owned and managed by the capability responsible for that information. Assurance owns the contextual association that particular information constitutes evidence for a particular assurance purpose. Being evidence is contextual, not an intrinsic universal property of source information.

Evidence may be associated through reference, linkage, aggregation, representation, mirroring or derivation. These possibilities do not prescribe technical storage or replication. An association may identify a particular version, state or temporal context of the underlying information.

> **Evidence inclusion may be temporally and version-specific. Association of information as assurance evidence SHALL be capable of distinguishing the particular information state relevant to the assurance activity from the information's current state.**

Temporally Appropriate Evidence and Criteria

Applicable assurance criteria may themselves be versioned and temporally applicable. A conclusion may depend both on the information state relevant to the evidence and on the criterion state applicable to the subject at the relevant time. Assurance evaluates temporally appropriate evidence against temporally appropriate criteria. No persistence, active-state, replication or retention mechanism is derived here.

Assessment, Adjudication and Explicit Insufficiency

Concern	Fundamental question	Semantic boundary
Assessment	What does this evidence demonstrate with respect to the applicable criterion?	May supply quantitative, qualitative, compound, temporal or confidence-bearing information.
Adjudication	What assurance finding or conclusion follows from that assessment?	Establishes the finding or conclusion from assessed detail; it is distinct from assessment and operational response.

Assurance SHALL NOT be reduced universally to PASS / FAIL / UNKNOWN. Insufficient evidence is not equivalent to either satisfaction or non-satisfaction. Under REQ-FND-005, where available evidence cannot establish the required assurance conclusion with sufficient confidence, that inability and evidence insufficiency SHALL remain explicit.

An assurance activity may execute correctly while lacking sufficient evidence to establish a conclusion about its subject. This is distinct from an indeterminate execution outcome or execution failure. REQ-FND-004 and AX-15 continue to govern uncertainty about operational state, effect or outcome, including the assurance activity's own execution where applicable.

Assurance Independence

Independent Assurance is performed through an independently governed activity whose progression and assurance conclusion are not controlled by the activity or mechanism responsible for performing or managing the subject being assured.

Independence does not require a different organisation, person, application, Kubernetes cluster or database, an external auditor, or physical separation. The subject activity may supply evidence but SHALL NOT solely determine, suppress, manufacture or retrospectively alter its own assurance conclusion.

> **Evidence dependency does not compromise assurance independence; control dependency does.**

Assurance may manage its own assurance activity without assuming management of the subject activity. No technical or organisational separation is prescribed by this boundary.

Clinical Services Delivery Assurance Boundary

Health Service Assurance SHALL NOT be represented as Clinical Services Delivery Assurance. Harmonia does not thereby assume responsibility for assuring clinical judgement, professional practice, clinical adequacy of care, or delivery of clinical services by healthcare practitioners or healthcare organisations.

Information concerning clinical service delivery MAY be the subject of governed assurance where an explicit applicable requirement establishes that concern. This does not transfer responsibility for Clinical Services Delivery Assurance to Harmonia. Capability 06 — Clinical Quality, Safety & Improvement retains its established clinical quality/safety responsibilities.

> **Harmonia may assure facts and behaviour concerning clinical activity where explicitly required; it does not thereby assure the clinical adequacy of care.**

Strategy Derivation Boundary

The approved derivation view stops at collaborative Enterprise Capability realisation. No strategic logical or application component is allocated these capabilities or EC-14. No Dokimasia, Ponos, Praxis, Pragma, Digital Twin, Mneme, Mnemosyne, Calliope, Iris, Pylai or other implementation relationship is established. Business Architecture, Information Architecture and solution derivation remain subsequent activities.

Downstream Progression

The Business Enabling Capability catalogue and its atomic Features establish what systems must enable for the healthcare enterprise. These features serve as the empirical basis for deriving the reusable, technology-neutral platform capabilities:

- Enterprise Capabilities (EC-01 .. EC-14): The established EC-01 through EC-13 and approved EC-14 Service Guardian; the Health Service Assurance contribution model is established without adding Feature or component allocations.
- Health Service Assurance Derivation: REQ-FND-005 → BC-18 → the three approved Business Enabling Capabilities → collaborative Enterprise Capability realisation.
- Capability Tier Progression Model: Detailed vertical derivation rules and representative composition examples.
- ICT Foundation Lenses: Technical enablement considerations guiding technology realization.

10. Business Architecture Metamodel & Modelling Rules

RAD source: `docs/markdown/03-business-architecture/metamodel/business-architecture-metamodel.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Business Architecture Metamodel & Modelling Rules

1. Overview & Metamodel Hierarchy

The Harmonia Business Architecture metamodel defines the structural and behavioural relationships governing how healthcare capabilities are operationalised into functions, exposed services, stateful processes, and information responsibilities.

RAD diagram: textual source notation.

```
graph TD
    subgraph OWNER["Owning Business / Business Enabling Capability or Feature responsibility context"]
        subgraph BC["Business Capability"]
            BC1["BC CT1"] -->|Decomposes| BC2["BC CT2"]
            BC2 -->|Decomposes| BC3["BC CT3"]
        end
        subgraph BEC["Business Enabling Capability"]
            CT1["BEC CT1"] -->|Capability decomposition| CT2["BEC CT2"]
            CT2 -->|Capability decomposition| CT3["BEC CT3"]
            CT3 -->|Frames every Harmonia Feature| FT["FT Feature<br/>(Capability Specialisation)"]
        end
        BC -.->|Enabled-by / supported-by where established| BEC
    end
    OWNER -->|Delivers| FUNC["Business Function(s)"]
    OWNER -->|Provides to clients| SERV["Business Service(s)"]
    SERV -->|May use directly| FUNC
    SERV -->|May use| PROC
    FUNC -->|May require| PROC
    PROC -->|Chains| FUNC
    OWNER -->|Encompasses where progression is material| PROC["Business Process(es)"]
    OWNER -->|Establishes| INFO["Business Information Responsibility"]
```

The diagram distinguishes two Capability hierarchies and preserves the permissible behavioural responsibility contexts. Business Capabilities describe organisational abilities; Business Enabling Capabilities describe reusable enabling abilities Harmonia provides or supports. Enablement/support is not decomposition or implementation equivalence, and the aggregate diagram establishes no individual BC↔BEC mapping. Business Capability decomposition does not terminate in Harmonia Features. Every fully reconciled Harmonia Feature requires complete **Business Enabling CT1 → CT2 → CT3** ancestry. Functions, Services and Processes may still anchor directly at their established owning Capability or Feature; they do not require an intervening Feature. Service, Function and Process form a graph rather than a mandatory hierarchy. A Function need not have either a Service or a Process.

1.1 Capability Decomposition and Terminology

The following terminology SHALL be used canonically:

Term	Meaning
Layer / Architectural Layer	Architectural layering within the TOGAF / ArchiMate architecture model.

Term	Meaning
Capability Tier	Hierarchical decomposition within a Capability Model.
Derivation / Derivation Progression	Progression between different architectural constructs or models. A Derivation Stage is a stage in that progression.
CT1	Capability Tier 1: establishes the broad Capability frame within Harmonia's architectural scope.
CT2	Capability Tier 2: decomposes the CT1 frame into constituent Capabilities.
CT3	Capability Tier 3: establishes the finest Capability frame required by Harmonia. Within the Business Enabling hierarchy, CT3 provides the context within which Harmonia Features are defined.
FT	Feature: expresses required system-enabled behaviour within its owning Business Enabling CT3 Capability context.

`Layer≠CapabilityTier≠DerivationStage`. `L1`, `L2` and `L3` are deprecated as canonical capability classifications.

Harmonia SHALL use the fixed three-tier Capability decomposition:

RAD source listing (presentation only; no inferred ancestry).

Business Capability CT1 → CT2 → CT3

enabled-by / supported-by

Business Enabling Capability CT1 → CT2 → CT3 → FT Feature

Every Harmonia Feature SHALL be immediately framed by a **Business Enabling CT3** Capability. Every established CT3 SHALL have a CT2 parent and every CT2 a CT1 parent within the same Capability category. Capability tiers SHALL NOT be skipped. Complete Business Enabling CT1 → CT2 → CT3 ancestry is required for a fully reconciled Feature; direct CT1 → FT and CT2 → FT framing SHALL NOT be used. Business Capability CT3s do not require or own Harmonia Features. An intentionally undecomposed BC branch does not indicate missing Feature framing.

> **A Feature cannot compensate for incomplete Capability decomposition. If Harmonia requires a Feature, the CT3 Capability that frames that Feature must first be identified. Failure to identify a meaningful CT3 indicates incomplete upstream Capability modelling, not permission to skip the tier.**

Complete decomposition is a modelling obligation, not authority to fabricate architectural elements. CT3 Capabilities SHALL NOT be manufactured merely to satisfy structural symmetry. Where an existing branch contains a Feature without an established CT3 frame or complete ancestry, the Feature, its established responsibility and local identifier SHALL be preserved, and its Capability ancestry/decomposition SHALL be recorded as requiring reconciliation under AX-17. It SHALL NOT be presented as fully reconciled or silently reattached to an invented owner. Established partial relationships remain valid evidence until separately adjudicated.

A Feature remains a finer-grained **Capability Specialisation** within Business Enabling responsibility and inherits Capability semantics. This retained classification does not make FT another Capability Tier, a Business Capability child or an arbitrary-depth frame. CT1 → CT2 and CT2 → CT3 denote same-category Capability decomposition; Business Enabling CT3 provides the mandatory Feature frame. BC↔BEC support relationships do not supply missing ancestry in either hierarchy.

CT3 is Harmonia's intentional Capability decomposition boundary. Harmonia's Capability Model is bounded to the level required to derive and govern Harmonia; it does not attempt to model the complete Enterprise Architecture of a hospital, health service provider, health department or other healthcare enterprise. Harmonia SHALL NOT define CT4, CT5 or deeper Capability tiers. Further detail is not authority to extend the Capability hierarchy.

Established higher-tier ancestry SHALL be represented in a lower-tier Capability's Canonical ID. Every Feature's complete structural Canonical ID SHALL include its CT1, CT2 and CT3 ancestry. An unidentified ancestor SHALL NOT permit omission of that tier; required but unresolved ancestry SHALL leave the Canonical ID unresolved, as specified in §8.

The five-stage Domain02 derivation progression SHALL NOT be reinterpreted as CT1 / CT2 / CT3 / FT. Enablement, derivation and contextual-view membership do not, by themselves, establish a Capability Tier assignment.

The approved fixed-decomposition decision record records the rationale and superseded permissive position. It changes the target metamodel, not the reconciliation status, tier allocation or identity of existing catalogue elements. K9's uncertainty and evidence rules remain controlling.

1.2 Hierarchy Naming & Uniqueness Rules

1. **Global Uniqueness**: Capability names across CT1, CT2 and CT3 are globally unique across the entire Harmonia architecture.
2. **Feature Scoping**: Feature names must be unique within their owning CT1 Capability hierarchy. An unresolved CT1 ancestor SHALL be recorded as unresolved rather than assigned speculatively.
3. **No Disconnected Elements**: Business Functions, Business Services, and Business Processes must **never** be modelled as free-standing global catalogues disconnected from an owning Capability or Feature context.

2. Capability & Feature Semantics

A Capability or Feature establishes:

- **Context & Scope**: Sets the boundary of healthcare operational relevance.
- **Architectural Responsibility**: Establishes unambiguous ownership of behaviour and state.
- **Business Functions**: Defines the internal behaviour delivered within the boundary.
- **Exposed Business Services**: Declares behaviour made available to external or cross-capability consumers.
- **Business Processes**: Encompasses progression lifecycles where activity state tracking is material.
- **Information Responsibility**: Owns the conceptual information created or governed by its functions.
- **Dependencies**: Explicitly records consumption of services provided by other capabilities.

3. Function and Service Semantics

Domain03 SHALL NOT establish a Feature identity that Strategy has not established. A Strategy Feature may be associated with Business behaviour only where that behaviour materially realises the semantic responsibility expressed by the Feature. Shared subject matter, terminology, information, Actor, Capability context or clinical purpose is insufficient. Where semantic realisation is not established, preserve the Feature, Function, Service and established owning Capability, and record **Feature association not established** without assigning a replacement Feature.

The following are **Harmonia architecture definitions**, reconciled by the 2026-10-13 B1 task decision. They apply across architectural views and need not conform to the exact semantics of ArchiMate or another modelling framework. Frameworks may represent them where useful but SHALL NOT redefine their Harmonia meaning. Domain03's owning Capability/Feature and exposure rules continue to qualify Business behaviour; they do not replace these definitions.

3.1 Business Function Rule

> A **Function** is a mechanism that moves the state of information, or the state of a capability concern as a whole, from one status to another. A Function may operate

over a small, medium or large concern. Its architectural meaning derives from what it does to or within the capability, not from what it exposes externally.

A Function answers **what change is effected?** Function accountability concerns achievement of the defined state transition. Function size does not determine whether something is a Function.

> **A Business Function represents behaviour delivered within the responsibility boundary of its owning Capability or Feature.**

A Business Function describes *what* the capability does internally to discharge its architectural mandate. A Business Function may operate upon:

- information owned by its Capability / Feature;
- information obtained through Services exposed by another Capability / Feature;
- information used under an explicit cross-capability dependency.

Therefore, **ownership of a Function does not imply ownership of every information item upon which that Function operates**.

3.2 Business Service Exposure Rule

> A **Service** is a mechanism through which a client interacts with a capability to request change, request information, or be informed of change. A Service represents the interface between the capability and its client and operates according to a Service Contract. A Service may use one or more Functions and/or Processes to fulfil that contract.

A Service answers **what can the client ask for, obtain, or be informed about?** Service accountability concerns fulfilment of the **Service Contract**. A Service may use Functions directly, Processes, or both; a Function need not be exposed through a Service.

> **When Function behaviour is exposed for consumption outside the owning Capability or Feature responsibility boundary, that behaviour is exposed through a Business Service.**

RAD source listing (presentation only; no inferred ancestry).

> Capability A (Owner) provides Service X used by Capability B (Client)
>
> fulfilled using
>
> Function(s) and/or Process(es)

3.3 Ownership and Non-Equivalence Invariants

1. **Ownership Preservation**: When Capability A exposes `FunctionX` as `BusinessServiceX`, and Capability B consumes `BusinessServiceX`, **Capability A retains sole architectural ownership of Function X and its underlying state**. Capability B is merely a consumer.
2. **No 1:1 Mechanical Generation**: A Business Service is **not** created mechanically for every Business Function. A Service is modelled *only* when an identifiable consumer exists outside the owning responsibility boundary.
3. **Consumption Scope**:
   - **Harmonia-Internal Consumption**: Services consumed by other Harmonia capabilities (e.g., Service Administration consuming Person Identifier Resolution from Entity Management).
   - **Enterprise/External Consumption**: Services consumed by external healthcare participants, client systems, or national registries (e.g., Referral Submission exposed to external GP practices).

3.4 Significance-Driven Relationships and Typed Dependencies

Role, Actor, Function, Service, Interaction, Collaboration and Process relationships SHALL be established where they materially explain responsibility, participation, exposure, collaboration or Business progression. Their absence alone does not establish missing architecture. Exhaustive Function-to-Role, Function-to-Service, Interaction-to-Service or Actor-to-Function matrices are not required.

A Service dependency requires consuming behaviour that needs behaviour actually exposed by that Service. Related information or subject matter is insufficient. Capability dependency, Service consumption, Interaction participation and Collaboration involvement SHALL retain their distinct types and meanings. Descriptive or abbreviated references do not create aliases. An unestablished element or relationship SHALL remain explicitly unresolved; no replacement dependency is inferred.

3.5 Clinical and Operational Privilege

Clinical Privilege concerns whether a practitioner is authorised/credentialed to undertake applicable clinical activity. **Operational Privilege** concerns whether an actor is authorised to undertake applicable operational activity. They are distinct responsibilities; an activity may require both.

Practitioner Role information may contextualise or contribute to a privilege determination, but Practitioner Role Resolution establishes neither Clinical Privilege nor Operational Privilege. These distinctions do not allocate new Roles, Services or policy mechanisms.

3.6 Business Responsibility and Reusable Composition

Business Architecture SHALL describe responsibility meaningful to the Business at its declared architectural layer. Shared downstream machinery does not establish a shared Business Function. In service-delivery operational contexts, **Capacity Management** concerns understanding and managing available, committed and utilised service capacity. Contexts may have materially different capacity semantics. A generalised operational-state capture mechanism is a downstream derivation concern and SHALL NOT replace those Business responsibilities. The HSO capacity responsibility boundary distinguishes context-specific capacity from resource state and higher-order consumers.

A domain-specific Business Enabling Feature MAY be realised through composition of established reusable capabilities and configured workflow/Praxis without a correspondingly specialised Business Function. The composition must sufficiently express the required responsibility; clinical purpose or setting alone does not establish another Harmonia information-management responsibility. Do not create a Feature-to-Function relationship where only a composition-level sufficiency decision is established.

3.7 Clinical Work Integration Boundary

Harmonia may integrate and coordinate around clinical work; it does not thereby manage clinical work. It may exchange work information, receive externally established requirements, route operational activity, observe progression, coordinate Harmonia-managed consequences and apply established qualification, privilege and policy constraints. It SHALL NOT thereby determine clinical work, manage clinical handover, acquire clinical allocation or decision authority, own clinical worklists or assume EMR workflow ownership. Information assembly for handover is distinct from performing or managing the handover.

Clinical facts retain their originating authority when consumed by patient-flow, capacity or discharge coordination. Clinical progression and operational patient progression SHALL remain distinct. A clinical/professional clearance may affect discharge readiness without the operational consumer acquiring authority to establish that clearance. Clinical Privilege and Operational Privilege retain §3.5's distinct meanings.

Harmonia terminology remains **Work Order = human doing**, **To Do = human review/ update/approval**, and **Task = synthetic task**. External work/task vocabulary does not redefine Harmonia Task. Coordination of a To Do supports an authorised review or approval; it does not confer clinical-work management authority.

4. Business Process Semantics

> A **Process** is an ordered chain of Functions used to realise a complex change. A Process provides a convenient mechanism for decomposing, structuring and managing change that would otherwise be difficult to understand or control as a single Function. A Function may itself require one or more Processes where the change for which that Function is responsible requires further decomposition.

A Process answers **how is a complex change decomposed and coordinated?** Process accountability concerns progression and coordination of its constituent Functions. Function and Process may be recursively composed:

RAD source listing (presentation only; no inferred ancestry).

> Function
>
> may require → Process
>
> Function
>
> Function
>
> Function
>
> may require → Process

A Business Process applies this meaning to progressing Business behaviour within an owning Capability or Feature responsibility boundary. There is no mandatory Service → Process → Function hierarchy.

4.1 Process Justification Rule

> **Model a Business Process where ordered Function decomposition is material to understanding, structuring or managing a complex Business change and its progression.**

Meaningful states, dispositions and outcomes may justify that decomposition. Not all Functions require a Process. Search, lookup, mapping, verification or presentation labels alone neither establish a Process nor exclude one where the bounded change requires decomposition. Primary Information traceability remains with the responsible Functions; Process-level aggregation does not replace it. Existing named Business Processes and responsibility boundaries are preserved; this rule creates no new catalogue entry.

4.2 Cross-Capability Process Guardrail

If a proposed Business Process appears to span multiple Capability responsibility boundaries:

1. Examine whether the process has been scoped incorrectly (conflating independent sequential activities with a single process); or
2. Examine whether an architectural responsibility is missing or misassigned.

Cross-capability processes must never be used to erase or blur Capability ownership boundaries. Cross-boundary coordination is achieved via event notifications and service invocations between bounded capabilities.

4.3 Behaviour and Process Consistency

A Business Process may elaborate progression for a defined activity within its owning Capability or Feature without reproducing the Behaviour stage list. It SHALL preserve established responsibility, ownership, authority boundaries and supported obligations. Scoped applicability, compound checkpoints and alternative dispositions SHALL remain explicit; additional Process detail SHALL NOT silently become a universal requirement or redefine owning responsibility.

Omission of a Process checkpoint from a Behaviour summary does not invalidate that checkpoint; omission of Behaviour responsibility from a Process does not remove that responsibility. Stage-count equality is not required. Greater Process detail does not override established responsibility.

4.4 Illustrative Progression and Material Outcome Uncertainty

Illustrative progression SHALL NOT imply an authoritative transition model. Only established states/transitions may be normative. Where alternatives, terminal behaviour or post-completion relationships remain unresolved, both the surrounding text and the affected diagram SHALL visibly identify the uncertainty.

Where Harmonia manages or governs an activity with a materially significant outcome and available evidence cannot establish that outcome, the uncertainty SHALL remain explicit. Absence of acknowledgement, response, observation or evidence does not by itself establish success or failure. This follows REQ-FND-004 and AX-15; it creates no universal INDETERMINATE Process state or downstream execution-state machinery.

5. Cross-Cutting Responsibility Rule

Harmonia establishes an essential architectural guardrail to maintain component cohesion and prevent boundary erosion:

> **Cross-cutting concern does not imply cross-cutting responsibility. A concern becomes cross-cutting because Capabilities depend upon Services or Functions delivered by other Capabilities whose responsibilities remain explicitly bounded.**

Concrete Guardrails:

- **Security & Control**: Security does not become partly owned by every capability that consumes security services. Authorisation evaluation and security context propagation are owned exclusively by *Health Information Control*.
- **Semantic Governance**: Terminology and canonical model definitions do not become owned by capabilities that use them. They are owned by *Information Design Governance* and *Clinical Knowledge Services*.
- **Workflow & Activity Coordination**: Coordinating tasks and to-dos does not grant ownership of the underlying clinical or operational activities. Coordination semantics are owned by *Workflow & Activity Coordination*.
- **Presentation**: Presentation capabilities do not acquire ownership of the clinical information they render.
- **Information Exchange**: Transporting or mediating messages across networks does not grant ownership of the clinical payloads transported. Exchange state is owned by *Health Information Exchange*, while clinical document ownership remains with *Clinical Record Administration*.

6. Business Information Responsibility

> **Business information ownership derives strictly from Capability responsibility and the Functions/Processes through which that responsibility is discharged.**

- **No Ownership Transfer**: Access, use, transformation, transport, search, presentation, caching, persistence, or coordination of information does not transfer architectural ownership.
- **Pure Business Semantics**: Business information concepts must retain authentic clinical/business definitions. They must **never** be prematurely translated into:
  - FHIR Resource structures (e.g., `Patient`, `Observation`, `Task`);
  - Database schemas, tables, or DDL;
  - JSON schemas, DTOs, or wire payloads;
  - Java classes or programming interfaces.

Domain04 formalises conceptual information meaning, semantic relationships and responsibility traceability. Realised representations are downstream: application logical/software representation in Domain05; exchange contracts, interoperability profiles, wire payloads and transformations in Domain06; physical platform/runtime/product/deployment realisation in Domain07. Persistence realisation may involve Domain05 and Domain07; a more precise allocation is not established. Representation does not define upstream business or conceptual meaning.

6.1 Management, Originating Authority and Historical Truth

Managing, preserving, governing access to, communicating, contextualising or maintaining the history of information SHALL NOT imply originating authority for the represented facts.

Within Harmonia's operational boundaries, Harmonia may maintain durable, immutable, append-only historical truth concerning what it received, knew, asserted, managed, decided, communicated or did. Subsequent correction, supersession or changed knowledge SHALL NOT retrospectively alter that historical record. Current authoritative state and historical operational truth remain distinct. This does not make every transient observation durable evidence or weaken the active/durable-state separation.

6.2 Business Terminology and Context

Interpret an element according to its declared architectural layer and Business meaning. Cache, index, routing, topic, session and similar terms do not prescribe a technology or product merely because those words also have technical uses. Preserve clear Business meanings; implementation structures or bindings require explicit downstream authority.

Where Healthcare Service context is material to an activity's meaning, authority, coordination, progression or accountability, that context SHALL remain identifiable through relevant Business behaviour and information responsibility. Organisation, Location, Practitioner or Role association does not imply that context. The obligation is contextual, not universal, and defines no representation, identifier structure, persistence mechanism, FHIR representation or implementation binding.

7. Qualified Architectural Reference Grammar

To unambiguously express participants, their acting capacity, and situational context without inventing transient metamodel entities, Harmonia uses the **Qualified Architectural Reference Grammar**:

RAD source listing (presentation only; no inferred ancestry).

```
<EntityType>#<Entity>-as-<Role>[#<ContextQualifier>]
```

7.1 Grammar Structure

- `<EntityType>`: A rendering of the canonical Business Actor category (`Person`, `Group`, `Organisation`, `OrganisationalUnit`, `Government/RegulatoryBody`, `System`, `Device`).
- `<Entity>`: The specific business instance identifier or name.
- `<Role>`: The canonical Business Role representing the stable business/functional capacity being fulfilled.
- `[#<ContextQualifier>]`: *(Optional)* The contextual function or situational qualifier in a specific interaction or collaboration.

Qualified references resolve to canonical architectural identities. For compact rendering, CamelCase SHOULD be used where possible: Information Supplier → `InformationSupplier`, Service Provider → `ServiceProvider`, Organisational Unit → `OrganisationalUnit`. This is syntactic rendering, not an alias or independent element. Preserve meaning and disambiguate where a compact expression could resolve to more than one identity; compact spelling alone SHALL NOT establish identity.

7.2 Canonical Examples

Qualified Expression	Actor Category	Entity Name	Fulfilled Role	Context Qualifier
`Organisation#ACTPathology-as-InformationSupplier`	`Organisation`	ACT Pathology	`InformationSupplier`	*(none)*
`Organisation#ACTHealth-as-ServiceProvider#ReferralSource`	`Organisation`	ACT Health	`ServiceProvider`	`ReferralSource`
`System#ACTPathologyLIS-as-InformationSupplier#DiagnosticResultSource`	`System`	ACT Pathology LIS	`InformationSupplier`	`DiagnosticResultSource`
`Person#Fred-as-Patient`	`Person`	Fred	`Patient`	*(none)*
`Person#DrSmith-as-Clinician#Reviewer`	`Person`	Dr Smith	`Clinician`	`Reviewer`
`OrganisationalUnit#FinanceDirectorate-as-InformationCustodian`	`OrganisationalUnit`	Finance Directorate	`InformationCustodian`	*(none)*
`GovernmentRegulatoryBody#AHPRA-as-Regulator`	`Government/RegulatoryBody`	AHPRA	`Regulator`	*(none)*

7.3 Governance Guardrails

1. The **Role** identifies stable architectural/business capacity.
2. The **Context Qualifier** identifies situational nuance; it does **not** create a new Role.
3. This notation is a **reference grammar**, not a new architectural metamodel class.

8. Architectural Element Identity and Canonical Identification

8.1 Common Identity Properties

Every governed Business Architecture element SHALL have the following identity properties:

Property	Cardinality	Meaning
Element Type	Exactly one	The element's architectural semantic type.
Canonical Name	Exactly one	The current authoritative architectural name.
Canonical ID	Exactly one current value	The Canonical Architectural Identifier expressing current architectural responsibility context.
Name Alias	Zero or more	A retained alternative or historical name for the same element.
ID Alias	Zero or more	A retained non-canonical identifier resolving directly to the same current element.

These properties SHALL preserve the semantic distinctions between Capability, Feature, Business Function, Business Service and Business Process. Existing canonical-name uniqueness and ownership rules remain applicable.

This convention establishes the identification requirement; it does not assert that the existing architecture corpus already conforms. Where required ancestry or identifier allocation is not established, the Canonical ID SHALL remain explicitly unresolved. A placeholder or speculative identifier SHALL NOT be treated as an allocated Canonical ID.

8.2 Typed Canonical Identifier Grammar

The general form is `<architectural-context>.<element-identifier>`. A root CT1 Capability has no preceding context. For Capability, Feature, Business Function, Business Service and Business Process elements, the structural grammar is:

RAD source listing (presentation only; no inferred ancestry).

canonical-id =

capability-path

| feature-path

| capability-path ”.” behaviour-token

| feature-path ”.” behaviour-token

capability-path = ct1-token [”.” ct2-token [”.” ct3-token]]

ct3-path = ct1-token ”.” ct2-token ”.” ct3-token

feature-path = ct3-path ”.” feature-token

ct1-token = ”CT1-” <approved-local-code>

ct2-token = ”CT2-” <approved-local-code>

ct3-token = ”CT3-” <approved-local-code>

feature-token = ”FEAT-” <approved-feature-code>

behaviour-token =

”FN-” <approved-local-code>

| ”SV-” <approved-local-code>

| ”PR-” <approved-local-code>

Codes SHALL be approved, nonempty tokens without the context separator `..` This grammar establishes no code allocation or numeric width. Existing established Feature identifiers SHALL retain their canonical spelling, including `FEAT-EM-01`; `FEAT-EM01` SHALL NOT be substituted or assumed equivalent.

Capability tokens SHALL explicitly carry their Capability Tier type. Untyped forms such as `CT01`, `CT02` and `CT03` SHALL NOT be used as canonical Capability tokens. The FN / SV / PR prefixes identify behaviour types and SHALL NOT constitute additional Capability Tiers.

The following are syntax examples only; they SHALL NOT assign identifiers or Capability Tiers to existing architectural elements:

RAD source listing (presentation only; no inferred ancestry).

CT1-01

CT1-01.CT2-02

CT1-01.CT2-02.CT3-03

CT1-01.CT2-02.CT3-03.FEAT-EM-01

CT1-01.FN-01

CT1-01.CT2-02.PR-01

CT1-01.CT2-02.CT3-03.SV-01

CT1-01.CT2-02.CT3-03.FEAT-EM-01.FN-01

Every ancestor prefix in a current Canonical ID SHALL identify the corresponding current Capability or Feature in the established responsibility ancestry. `CT1-01.CT3-02` SHALL NOT be generated merely because CT2 ancestry has not been identified. Unresolved ancestry SHALL remain unresolved rather than being omitted or manufactured.

The shorter `capability-path` forms identify CT1 or CT2 Capabilities and permit direct behavioural anchoring; they SHALL NOT frame a Feature token. A Feature, or behaviour anchored beneath it, requires `feature-path` with complete Business Enabling CT1 → CT2 → CT3 ancestry. A syntactically valid path under a Business Capability does not establish valid Harmonia Feature ownership. These are grammar rules only and allocate no identifier to an existing element.

The common identity properties apply to other governed Business Architecture element types, but this decision does not establish their token vocabularies or structural contexts. Those SHALL NOT be inferred from this grammar. The Qualified Architectural Reference Grammar in §7 remains distinct and unchanged.

8.3 Behavioural Anchoring and Contextual Containment

Business Functions, Business Services and Business Processes MAY be anchored directly at any Capability Tier or Feature establishing responsibility for that behaviour. CT2, CT3 or FT elements SHALL NOT be invented merely to provide behavioural anchoring.

Canonical identifier containment SHALL express architectural responsibility context. It SHALL NOT redefine semantic relationships or Element Type, and SHALL NOT be interpreted as implementation containment. A Function beneath CT2 remains a Function; a Service beneath CT1 remains a Service; a Process beneath a Feature remains a Process. Function delivery, Service exposure and justified Process progression retain the rules in §§2–4.

The complete Canonical ID SHALL preserve contextual architectural standing in flat documents, generated diagrams, models, traceability matrices, cross-document references and AI contexts without depending on Markdown nesting. A flat representation retaining that context is permitted; disconnected ownership is not.

8.4 Rename, Relocation and Alias Rules

A Canonical Name change alone SHALL NOT change architectural identity or require a Canonical ID change. A previous name MAY be retained as a Name Alias for historical or search purposes.

The approved rename `ClinicalCredentialManagement→ClinicalQualificationManagement` preserves the existing practitioner professional / clinical qualification and competency responsibility. `ClinicalCredentialManagement` MAY be retained as a Name Alias only for that same element. It SHALL remain distinct from Domain03 `CredentialManagement`, whose separately established

responsibility concerns operational badges and physical access. The rename SHALL NOT establish equivalence or aliasing between those elements.

Where an element changes architectural responsibility context, its Canonical ID SHALL change to express current structural truth. The previous Canonical ID SHALL be retained as an ID Alias. This rule also applies to descendant elements whose responsibility paths change. The following relocation is a syntax example only:

RAD source listing (presentation only; no inferred ancestry).

Previous Canonical ID: CT1-01.CT2-02.FN-01

Current Canonical ID: CT1-01.CT2-02.CT3-03.FN-01

ID Alias: CT1-01.CT2-02.FN-01

An ID Alias SHALL:

- refer to the same architectural element;
- be searchable and resolvable;
- be non-canonical;
- resolve directly to the current architectural element, without requiring an alias chain;
- never be used for a new authoritative architectural reference.

Canonical IDs and ID Aliases SHALL occupy one globally unique, non-reusable identifier namespace. Once an identifier has referred to an element, canonically or as a historical Canonical ID, it SHALL NOT subsequently identify another element. An ID Alias SHALL NOT collide with a current Canonical ID or an ID Alias assigned to another element. All retained aliases SHALL continue resolving directly after subsequent relocations. New authoritative references SHALL use the current Canonical ID.

Name Aliases are names, not members of the identifier namespace. Neither Name nor ID Aliases SHALL establish equivalence merely from similar wording or nested behaviour. The approved G1 K5 rules remain controlling: finer-grained Functions or Services SHALL NOT be promoted to Features, or assigned a broader Feature's identity, solely to obtain traceability symmetry.

8.5 Machine-Validation and Generation Intent

This convention is intended to support future automated architectural navigation, diagram generation, model generation, traceability matrices, cross-document references and AI-assisted architecture interpretation. It SHALL permit validation of at least:

- Canonical ID uniqueness;
- ID Alias uniqueness;
- Canonical ID / ID Alias collisions;
- ancestor existence and complete, correctly typed ancestry;
- valid element-type suffix or terminal token;
- valid Capability / Feature responsibility context;
- resolvable references;
- direct, unambiguous alias resolution;
- orphaned identifiers.

This decision implements no validators, schemas or classes. Generation intent does not require the exhaustive capability matrices excluded by Domain02's modelling position.

8.6 Approved K9 Derivation and Uncertainty Rules

Approved G1 K9 found no affected legacy owner assignment sufficiently validated to authorise mechanical conversion to CT1 / CT2 / CT3. Established identities and responsibilities remain valid; affected Capability Tier, complete ancestry and root status remain unresolved.

Unresolved Capability Tier or ancestry SHALL NOT prevent downstream derivation from an otherwise established owning Capability or Feature and its relevant responsibility. Downstream

traceability SHALL reference that element without manufacturing structural ancestry. Document structure, catalogue grouping, numbering, indentation and decomposition presentation SHALL NOT by themselves establish Architectural Element identity.

`ClientAdministration→PersonIdentity→IdentifierResolution[FT/FEAT-EM-01]` is an established partial responsibility chain and SHALL be preserved. Its full structural Canonical ID remains unresolved until actual Capability Tier ancestry and identifier allocation are architecturally established. The former incorrect L3 example SHALL NOT supply that ancestry.

The fixed model in §1.1 does not retrospectively validate legacy L1 / L2 / L3 assignments or convert a known owner into CT3. Existing Features with incomplete ancestry remain valid architectural evidence requiring Capability decomposition reconciliation; downstream derivation from their established responsibility does not declare them structurally reconciled. Missing CT3 frames SHALL be separately established before subsequent Feature definition, without using catalogue presentation to manufacture them.

These rules preserve AX-01/AX-04 business meaning, AX-14 distinctions, AX-15 uncertainty and AX-17 architectural authority and explicit uncertainty. They allocate no structural IDs, manufacture no ancestry and do not authorise Package2 Information Family derivation.

The subsequent R1/R2 Business Capability reconciliation explicitly establishes its stated BC tiers/parents independently of catalogue indentation. It does not resolve any affected historical BEC ancestry, convert any of the 69 enabling descriptions into Capabilities, frame the 137 Features or allocate Canonical IDs. K9 continues to govern all unestablished dimensions.

9. R1.x/R2.x Semantic Sufficiency Boundary

The human architectural adjudication of 2026-10-09 establishes semantic sufficiency rather than graph density. Domain03 must express the Business behaviour required by Strategy without mirroring Strategy's structure. Complete identifier allocation is not a prerequisite for semantic completeness; deterministic corrections preserve established identities, while unresolved ancestry, tiers and code allocations remain explicit.

Semantic completeness requires sufficient Business expression to derive Strategy's required behaviour. It requires neither one Function per Feature, one Service per Function, exhaustive Role/Function or Feature/Function mappings, identical decomposition nor structural symmetry. A composition/workflow realisation under §3.6 may suffice without a specialised Function; unestablished individual relationships remain unestablished. The residual SD-03, SD-08 and SD-14 decisions are recorded in the Service Delivery catalogue, while HSO-09 and HSO-16 are reconciled within their operational responsibilities.

The current approved assurance authority boundary and the three assurance Process purposes/progression descriptions are sufficient for this baseline. Detailed approval allocation and lifecycle transitions remain unestablished and may be revisited if later architecture demonstrates a genuine Business requirement. Evidentiary use alone establishes no Service, Interaction or dependency. The approved assurance catalogue remains two Roles, five Functions, three Processes, three Services and three Interactions; financial governance exclusion and non-recursion remain intact.

The approved G1 capability-scoped corrections and unresolved Feature/dependency relationships remain valid unless a separately authorised semantic decision establishes more. No exhaustive Value Stream derivation, participant/exposure matrix, Twin Business Actor or workflow, or organisational risk-management responsibility is required for structural completeness.

11. Harmonia Capability Decomposition & Derivation Progression

RAD source: `docs/markdown/02-strategy/capability-maps/capability-tier-model.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Harmonia Capability Decomposition & Derivation Progression

Overview

This model distinguishes hierarchical Capability decomposition from the derivation progression, composition relationships, and derivation rules connecting high-level healthcare enterprise capabilities with reusable platform capabilities and downstream architectural responsibility boundaries.

Canonical terminology SHALL distinguish:

- **Layer / Architectural Layer**: architectural layering within the TOGAF / ArchiMate architecture model.
- **Capability Tier**: hierarchical decomposition within a Capability Model.
- **Derivation / Derivation Progression**: progression between different architectural constructs or models; a Derivation Stage is a stage in that progression.

`Layer≠CapabilityTier≠DerivationStage`.

Business Capability decomposition SHALL use `CT1→CT2→CT3`; Business Enabling Capability decomposition SHALL use `CT1→CT2→CT3→FT`. BC↔BEC enabled-by/support is a separate relationship, not decomposition or implementation equivalence. In these fixed models: `CT1` means Capability Tier 1, `CT2` Capability Tier 2, `CT3` Capability Tier 3 and `FT` Feature. `L1`, `L2` and `L3` remain deprecated as canonical capability classifications.

Tier / type	Architectural role
CT1	Establishes the broad Capability frame within Harmonia's architectural scope.
CT2	Decomposes the CT1 frame into constituent Capabilities.
CT3	Establishes the finest Capability frame required by Harmonia and within the Business Enabling hierarchy, the Capability context within which Harmonia Features are defined.
FT	Expresses required system-enabled behaviour within its owning Business Enabling CT3 Capability context.

Every Harmonia Feature SHALL be immediately framed by Business Enabling CT3; Business Capabilities do not decompose into Harmonia Features; every CT3 SHALL have a CT2 parent; every CT2 SHALL have a CT1 parent. Capability tiers SHALL NOT be skipped. Complete CT1 → CT2 → CT3 ancestry is required for a fully reconciled Feature. CT1 → CT2 and CT2 → CT3 are Capability decomposition relationships.

> **A Feature cannot compensate for incomplete Capability decomposition. If Harmonia requires a Feature, the CT3 Capability that frames that Feature must first be identified. Failure to identify a meaningful CT3 indicates incomplete upstream Capability modelling, not permission to skip the tier.**

This modelling obligation SHALL NOT authorise manufactured CT3 Capabilities for structural symmetry. Existing Features without established CT3 frames SHALL be preserved with their

established responsibilities and local IDs; their Capability ancestry/decomposition requires reconciliation. Missing ancestry SHALL remain explicitly unresolved, rather than being omitted, invented or treated as direct CT1/CT2 Feature framing in the target model.

CT3 is the intentional Capability decomposition boundary. Harmonia models the level required to derive and govern Harmonia, rather than the complete Enterprise Architecture of a hospital, health service provider, health department or other healthcare enterprise. CT4, CT5 and deeper Capability tiers are outside that scope and SHALL NOT be defined. The approved decision record records the rationale and supersession; the Domain03 metamodel governs the fixed hierarchy.

The Domain03 Business Architecture metamodel governs typed Canonical IDs, identity, aliases and behavioural anchoring. The five-stage derivation progression below SHALL NOT be reinterpreted as CT1 / CT2 / CT3 / FT or used to assign Capability Tiers or identifier codes to existing elements. Approved G1 K9 establishes that affected legacy owner assignments are insufficiently evidenced; they SHALL NOT be mechanically translated to CT1 / CT2 / CT3. Capability Tier, complete ancestry, root status and structural Canonical IDs remain unresolved where not established.

Unresolved Capability Tier or ancestry SHALL NOT prevent downstream derivation where the owning Capability or Feature and its relevant responsibility are established. Traceability SHALL reference that element without manufacturing ancestry, as required by AX-17. Document structure, grouping, numbering, indentation and decomposition presentation SHALL NOT by themselves establish Architectural Element identity.

The model ensures that:

1. Enterprise healthcare practice is clearly demarcated from software enablement.
2. System-enabled capabilities reflect authentic clinical, administrative, and operational contexts rather than integration software mechanics.
3. System behaviours are decomposed into atomic, testable features.
4. Reusable capabilities (EC-01 .. EC-14) are derived systematically without creating centralized bottleneck services.
5. Technical realisations are guided by ICT Foundation lenses without allowing physical software products to define architectural capabilities.

1. The Five-Stage Derivation Progression

The framework progresses through five derivation stages relating different architectural constructs and models. Their stage numbers SHALL NOT denote Capability Tiers or Architectural Layers:

RAD source listing (presentation only; no inferred ancestry).

STAGE 1: BUSINESS CAPABILITY

What the enterprise must be capable of doing

(18 catalogued capabilities across 5 natural healthcare regions)

requires enablement by

▼

STAGE 2: BUSINESS ENABLING CAPABILITY

What systems must enable or provide in support of the business

(Structured across 5 authentic healthcare contextual views)

specifies atomic behaviour as

▼

STAGE 3: FEATURE (FT)

Smallest useful statement of required system-enabled behaviour

(Independently understandable, testable, technology-neutral)

derived into reusable

▼

STAGE 4: ENTERPRISE CAPABILITY (EC-01 .. EC-14)

Reusable ICT functionality recurring across multiple domains

(Decoupled from concurrency, message brokers, and storage DDL)

viewed through

▼

STAGE 5: ICT FOUNDATION CAPABILITY LENSES (18 Lenses)

Cross-cutting technical enablement considerations

(Evaluates technical realization via technology-substitution test)

realised by

▼

ARCHITECTURE REALISATION

Domains 03–07: Business, Information, Application, Integration, Tech

2. Derivation Stage Definitions

Stage 1: Business Capability

- **Definition**: What the healthcare enterprise must be capable of doing to deliver healthcare services, govern operations, and ensure patient safety.
- **Independence**: Exists independently of IT systems, software products, commercial organizational hierarchies, or automation.
- **Structure**: 18 CT1 Business Capability roots grouped into five natural regions, with the explicit CT2/CT3 assertions and deliberate stopping points in the canonical BC catalogue. Their tiers are established by that reconciliation, not derivation-stage position; Canonical IDs remain unallocated:
  - *Care & Health Delivery* (01–08)
  - *Health Information & Digital Health* (09–12)
  - *Research & Innovation* (13)
  - *Health Service Management* (14–16)
  - *Governance & Assurance* (17–18)
- **Harmonia Principle**: Harmonia provides core technical enablement for Health Information and Connected Health capabilities (09–12) and materially enables delivery and workforce capabilities (01, 02, 04, 06, 15) without asserting platform ownership over healthcare business practice.

Natural regions are contextual groupings, not capability tiers, organisational structures, application boundaries or ownership hierarchies, and are distinct from Business Enabling contextual views. Health Service Governance (17) retains unresolved relevance classification/enabling derivation; Health Service Assurance (18) is Harmonia-Relevant under the reconciled BC catalogue, preserving its approved independent governed-assurance participation. The approved Health Service Assurance derivation establishes only REQ-FND-005 → BC-18 → the three named Business Enabling Capabilities → collaborative Enterprise Capability realisation. Stage diagrams and existing examples do not establish additional relationships.

Stage 2: Business Enabling Capability

- **Definition**: What software systems and information infrastructure must enable or provide to support the enterprise healthcare capabilities.
- **Structuring Axis**: Organized into five contextual views reflecting genuine healthcare operating environments:

1. *Entity Management*: Governance of practitioners, organizations, locations, services, products, and devices.
2. *Service Administration*: Administrative progression of encounters, referrals, orders, scheduling, and billing.
3. *Service Delivery*: Systems enablement across clinical delivery settings (primary, acute, emergency, diagnostic, virtual).
4. *Health Service Operations*: Healthcare facility logistics, bed management, work allocation, dispatch, and discharge coordination.
5. *Intrinsic / Shared Enablement*: Longitudinal clinical records, health information exchange, clinical collaboration, and workflow coordination.

- **Independence**: Independent of commercial software product boundaries (PAS, EMR, LIS, RIS) and internal middleware engines.

The approved Assurance Design, Assurance Criteria Management and Governed Assurance additionally support BC-18. Their contextual-view placement, relevance classification, Capability Tier, complete ancestry, root status and structural Canonical IDs remain unresolved. Their direct Strategy derivation does not create intermediate Features, a sixth view or inferred hierarchy.

Stage 3: Feature (FT)

- **Definition**: The **smallest useful statement of required system-enabled behaviour** within its owning CT3 Business Enabling Capability context. Existing owners whose tier/ancestry is unresolved remain established responsibility evidence requiring reconciliation. FT is a Feature classification; derivation stage 3 SHALL NOT classify it as CT3.
- **Criteria**:
  - *Independently understandable*: Conveys a self-contained operational meaning.
  - *Testable in principle*: Allows clear verification that the behavior occurred or was enabled.
  - *Technology-independent*: Contains no product names, protocols, or storage mechanisms.
  - *Component-independent*: Does not prescribe which software module executes it.
- **Scope Demarcation**: Formally authored for Harmonia-Relevant and Harmonia-Core capabilities in the established catalogue. Reference and Adjacent capabilities are intentionally not decomposed into features. This scope statement allocates no Capability Tier to existing elements and does not waive the fixed ancestry required for a fully reconciled Feature.

Stage 4: Enterprise Capability (EC-01 .. EC-14)

- **Definition**: Reusable architectural functionality derived from recurring system-enabled features across multiple healthcare domains.
- **Derivation Logic**: Derived by analyzing features and asking: *"How is this function delivered?"* Functions that recur vertically across multiple distinct healthcare domains are abstracted into reusable Enterprise Capabilities.
- **Catalogue**: The established EC-01 Managed Entity & Relationship through EC-13 Semantic Governance & Conformance, plus approved EC-14 Service Guardian.
- **Collaborative Nature**: Enterprise Capabilities represent bounded reusable responsibilities. Their realisation may require participation, consumption or enforcement across multiple architectural elements, but this does not imply shared semantic ownership or prohibit establishment of a responsibility centre. **Distributed participation does not imply distributed responsibility. Cross-cutting concern does not imply cross-cutting responsibility.**

EC-14 supplies the assurance-specific semantics needed to bind existing reusable capabilities into governed assurance. The approved first-pass contribution matrix establishes collaborative realisation without extending EC-12's semantics or allocating EC-14 to a component. The generic progression through Features describes the existing derivation convention; it does not manufacture Feature relationships for this explicitly approved assurance derivation.

Stage 5: ICT Foundation Capability Lenses

- **Definition**: Technical enablement lenses that provide cross-cutting engineering criteria, architectural considerations, and technology-substitution testing for realizing Enterprise Capabilities.
- **Role**: They are technical lenses, NOT peers of the Business Enabling capabilities.
- **Test**: Governed by the *Technology-Substitution Test*: *"If the named product, protocol, programming language, or deployment mechanism were replaced tomorrow, would the capability still exist?"*

3. The Capability Composition Formula

Harmonia capabilities are composed according to the following foundational relationship:

Harmonia Capability = domain-specific enabling behaviour + reusable Enterprise Capabilities

Where:

- **Domain-Specific Enabling Behaviour**: The specific clinical or operational logic, context rules, and progression semantics unique to a healthcare domain (e.g., healthcare identifier system rules, diagnostic order workflows, bed turnover readiness).
- **Reusable Enterprise Capabilities**: The horizontal, platform-wide capabilities (EC-01 through EC-14) that provide state management, context propagation, policy evaluation, provenance capture, interoperable transport and the approved assurance-specific semantics.

Informing Strategic Logical Component Boundaries

In Harmonia Strategy:

- **Capabilities describe *what* the platform achieves**.
- **Strategic Logical Components describe *how responsibility is organized***.

When specific combinations of Enterprise Capabilities and domain-specific behaviours recur coherently across multiple features, they inform candidate **Strategic Logical Component responsibility boundaries**:

RAD source listing (presentation only; no inferred ancestry).

RECURRING COMPOSITION PATTERN

Domain-specific enabling behaviour

+ Reusable Enterprise Capabilities

informs clustering for

▼

STRATEGIC LOGICAL RESPONSIBILITY MODEL

- Mneme (Managed Information & Active State)
- Mnemosyne (Durable Preservation & Recovery)
- Ponos (Managed Operational Activity Execution)
- Pylai (Standards-Conformant Ingress/Egress Membrane)
- Calliope (Semantic Authority & Conformance)
- Iris (Contextual Human Interaction & Presentation)
- Digital Twin (Entity Coordination Archetype)

Architectural Principle: This composition informs component boundaries; it does **not** imply a rigid, mathematical one-to-one derivation. Components represent cohesive responsibility centres, not one-to-one wrappings of individual capabilities.

The displayed component-clustering examples remain the established EC-01 through EC-13 derivations. This convention does not allocate EC-14 or the three assurance Business Enabling Capabilities to a strategic logical component or establish a relationship to an existing implementation construct.

4. Representative Derivation Examples

To demonstrate how Enterprise Capabilities recur across distinct Business Enabling features without constructing an exhaustive matrix, three representative derivation examples are detailed below.

Example 1: Person Identifier Resolution

In healthcare integration, identifying an individual across disparate clinics, hospitals, and diagnostic centres requires resolving identifiers (MRNs, national healthcare identifiers, local practice IDs) issued by multiple independent authorities.

- **Domain-Specific Enabling Behaviour**:
  - Healthcare identifier system semantics (e.g., verifying issuing authority jurisdiction, assigning authority namespaces).
  - Scope of identifier validity (temporary vs. permanent, national vs. regional).
  - Client privacy and consent flags associated with identifier disclosure.
- **Reusable Enterprise Capability Contributions**:
  - **EC-01 Managed Entity & Relationship**: Modeling the Person entity and linking multiple scoped identifiers and alias relationships.
  - **EC-05 Search & Discovery**: Locating existing entity records matching supplied identifier sets and resolving candidate matches.
  - **EC-03 Managed State & Lifecycle**: Governing the lifecycle status (active, superseded, suspended) of identifier records.
  - **EC-07 Provenance & Traceability**: Recording the authoritative origin, verification timestamp, and issuing system attribution for each identifier.
  - **EC-06 Policy & Control**: Enforcing default-deny access policies governing which requesting systems may view or resolve specific identifier authorities.
  - **EC-12 Operational Assurance**: Ensuring deterministic matching behavior and transactional consistency during concurrent resolution requests.

Example 2: Order Closed Loop

A clinical diagnostic or procedure order requires governed progression from initial request to final diagnostic report.

- **Domain Progression Semantics**: Destination → Route → Deliver → Acknowledge → Progress → Complete → Outcome
- **Critical Healthcare Distinction**: Technical Delivery Acknowledgement ≠ Business Acknowledgement A technical acknowledgement establishes only the acceptance state defined by the applicable interaction contract. It SHALL NOT be interpreted as durable acceptance, committed business state, successful processing or successful business disposition unless that meaning is explicitly established by the governing contract. HTTP 200/202 and MLLP acknowledgements are interpreted under their applicable contracts. A handoff acknowledgement alone does not establish the eventual downstream business outcome or the receiving laboratory's clinical responsibility. REQ-FND-001 retains its explicit Harmonia durable-ingress requirement.

- **Reusable Enterprise Capability Contributions**:
  - **EC-02 Context Management**: Establishing and propagating the clinical encounter, ordering provider, and subject-of-care context across all order lifecycle events.
  - **EC-05 Search & Discovery**: Resolving the target fulfillment service endpoint and receiving facility capabilities.
  - **EC-01 Managed Entity & Relationship**: Associating the Order entity with the Subject, Ordering Practitioner, Performing Organization, and Target Service.
  - **EC-08 Interoperability & Exchange**: Standards-compliant boundary protocol adaptation, message transformation, delivery dispatch, and transport acknowledgement capture.
  - **EC-03 Managed State & Lifecycle**: Tracking the formal Order state progression (`PLACED` → `RECEIVED` → `ACCEPTED` → `IN_PROGRESS` → `COMPLETED`).
  - **EC-10 Activity & Execution**: Instantiating and executing operational units of work, tracking timeouts, managing escalations, and supervising fulfillment deadlines.
  - **EC-07 Provenance & Traceability**: Capturing complete chain-of-custody, transformation history, and delivery timestamps for clinical auditability.
  - **EC-12 Operational Assurance**: Guaranteeing dual-write safety, duplicate suppression, and exception recovery across asynchronous boundary handoffs.

Example 3: Work Allocation & Dispatch

Operational healthcare activities (e.g., patient transport, ward bed preparation, discharge coordination, emergency triage assessment) require assigning and dispatching work to qualified clinical or operational staff.

- **Domain Progression Semantics**: Event → Entity Context → Required Activity → Assignment → Dispatch → ACK → Progress → Outcome
- **Healthcare Service Context Rule**: Where operational activity occurs in the context of a Healthcare Service, that service context must be explicitly associated with the activity/state rather than inferred solely from practitioner, organization, or location.
- **Cross-Domain Recurrence**: This operational progression pattern recurs across:
  - *Discharge Progress*: Coordinating multidisciplinary discharge milestones and readiness oversight.
  - *Patient Transport*: Allocating transfer tasks to portering staff based on mobility requirements.
  - *Mobile Work*: Dispatching community health visits to roving practitioners.
  - *ED Operational Coordination*: Progressing triage queue assessments.
  - *Bed Readiness*: Dispatching environmental cleaning staff upon patient discharge.
- **Reusable Enterprise Capability Contributions**:
  - **EC-02 Context Management**: Binding the patient, physical bed/room, care team, and healthcare service context to the operational work item.
  - **EC-03 Managed State & Lifecycle**: Managing the lifecycle of the operational activity (`REQUESTED` → `ASSIGNED` → `ACCEPTED` → `IN_PROGRESS` → `COMPLETED`).
  - **EC-05 Search & Discovery**: Discovering available, rostered, and credentialed practitioners or operational personnel matching task criteria.
  - **EC-01 Managed Entity & Relationship**: Linking the assigned worker, patient, location, and equipment entities.
  - **EC-10 Activity & Execution**: Governing the assignment logic, dispatch mechanism, human notification, timeout monitoring, and escalation triggers.
  - **EC-08 Interoperability & Exchange**: Delivering work orders to mobile devices or external task management systems where external dispatch is required.
  - **EC-12 Operational Assurance**: Preventing duplicate assignment and ensuring resilient recovery if a dispatched worker does not acknowledge within defined thresholds.
  - **EC-07 Provenance & Traceability**: Recording assignment timestamps, worker acceptance, and completion evidence.

5. Architectural Modelling Position: Omission of Exhaustive N × M Matrices

Harmonia explicitly rejects the generation of exhaustive N × M traceability matrices between Business Capabilities and Business Enabling Capabilities, or between Features and Enterprise Capabilities.

Formal Rationale

> **The Business Enabling Capability model was derived with explicit consideration of the Business Capability model and is intended to provide the system-enabled capabilities necessary to support that business landscape. A formal many-to-many mapping between Business Capabilities and Business Enabling Capabilities has not been produced, as such traceability is not required for the current Harmonia architecture and strategy objectives. Such mapping may be developed subsequently where required for benefits realisation, investment analysis, business-case development or other value-traceability purposes.**

This statement records the existing derivation. BC-17 enabling derivation and relationships beyond the approved BC-18 derivation remain unresolved under AX-17. The Health Service Assurance view adds only the approved three Business Enabling Capabilities and bounded fourteen-row EC contribution matrix. That explicit matrix is not an exhaustive Business Capability-to-Business Enabling or Feature-to-Enterprise mapping, does not imply other coverage, and does not change EC-12's established operational semantics.

Utilitarian Justification

1. **Avoidance of False Precision**: In a complex regional HIE, a single Business Capability (e.g., *Care Access & Coordination*) draws upon dozens of enabling capabilities across Entity Management, Service Administration, and Shared Enablement. An exhaustive matrix suggests mechanical one-to-one connections where real systems operate through dynamic, contextual composition.
2. **Maintenance Overhead vs. Value**: Maintaining a static matrix of hundreds of capabilities across thousands of cells creates substantial documentation drift without providing actionable guidance to software engineers or architects.
3. **Clarity of Representative Derivations**: As shown in Section 4, representative derivation patterns provide clear, verifiable architectural rationale for how enterprise needs compose reusable capabilities, without the clutter of exhaustive combinatorial tables.

Topic Navigation

- Domain 02 Strategy Overview
- Capability Framework Index
- Business Capabilities
- Business Enabling Capabilities
- Enterprise Capabilities
- Health Service Assurance Strategy Derivation
- ICT Foundation Lenses

12. Health Service Assurance — Approved Strategy Derivation

RAD source: `docs/markdown/02-strategy/capability-maps/health-service-assurance-derivation.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Source qualification: The opening 2026-10-14 reconciliation qualifies the older unresolved BC-18 tier/relevance statements later in this source. Later approved Domain03 assurance behaviour is reproduced separately; earlier Strategy stop boundaries retain their own scope.

Health Service Assurance — Approved Strategy Derivation

> **BC reconciliation — 2026-10-14:** The current BC catalogue establishes BC-18 as a CT1, its stated BC CT2/CT3 decomposition and Harmonia-Relevant classification. Its macro-level confidence purpose preserves REQ-FND-005's applicable independent governed-assurance requirement. This qualifies the earlier unresolved BC-tier/relevance statements below; all existing BEC/EC support assertions remain unchanged and are not redistributed among the new BC descendants. BEC tiers, ancestry and identity allocations remain unresolved.

Purpose and Architectural Authority

This view records the approved Health Service Assurance derivation in Domain02 Strategy. It connects the existing Motivation requirement and Business Capability to three approved Business Enabling Capabilities and their collaborative Enterprise Capability realisation. Canonical definitions remain in the linked capability catalogues.

AX-17 — Architectural Authority and Explicit Uncertainty governs truthful traceability: only the relationships explicitly approved here are established. Missing relationships remain unresolved. The requested Strategy reconciliation is approved architectural input; it does not complete later architecture domains.

The materially relevant axioms also include:

- AX-14 — Semantic Distinctions Are Preserved: assurance is distinct from monitoring, evidence production, operational management, clinical adequacy, assessment and adjudication.
- AX-06 — Information Authority Is Explicit, AX-07 — Security Is Intrinsic to Managed Operations and AX-08 — Evidence Records Meaning, Not Machinery: trustworthy evidence and governed assurance preserve their existing authority, security and meaningful-evidence constraints without transferring source-information ownership.
- AX-09 — Transient Operational State Is Ephemeral by Default: contextual evidence association does not automatically make all observations or operational diagnostics durable evidence.
- AX-10 — Distribution, Load and Failure Are Normal Operating Conditions and AX-15 — Uncertainty Is Preserved Until Resolved: expected failure/recovery remains the subject activity's responsibility, and operational uncertainty remains distinct from insufficient assurance evidence.
- AX-05 — Active State and Authoritative Durable State Are Distinct: temporal/version-specific evidence does not alter the approved state-responsibility separation or derive a persistence/active-state implementation.

Motivation to Strategy Traceability

RAD diagram: textual source notation.

```
graph TD
    REQ["REQ-FND-005<br/>Independent Assurance of Governed Activity"] --> BC["BC-18<br/>Health Service
    ↪  Assurance"]
    BC --> AD["Business Enabling Capability<br/>Assurance Design"]
    BC --> ACM["Business Enabling Capability<br/>Assurance Criteria Management"]
    BC --> GA["Business Enabling Capability<br/>Governed Assurance"]
    AD --> COL["Collaborative Enterprise Capability realisation<br/>Approved contribution matrix"]
    ACM --> COL
    GA --> COL
    COL --> EXIST["Applicable existing EC-01 through EC-13<br/>Direct or Supporting as explicitly approved"]
    COL --> SG["EC-14<br/>Service Guardian"]
```

The final branches identify contributions to collaborative realisation, not ownership, execution order or component allocation. The matrix below distinguishes the contribution to each Business Enabling Capability; the aggregate diagram does not assert every existing EC contributes to each capability.

Established relationship	Authoritative Strategy reference	Boundary
REQ-FND-005 → BC-18	Approved requirement; Health Service Assurance	Independent conclusion authority and explicit evidence insufficiency; no clinical-adequacy responsibility transfer.
BC-18 → Assurance Design	Assurance Design	Defines how satisfaction is assured, not application design.
BC-18 → Assurance Criteria Management	Assurance Criteria Management	Manages criteria without replacing governing requirements.
BC-18 → Governed Assurance	Governed Assurance	Independently evaluates and concludes without managing the subject.
Three Business Enabling Capabilities → collaborative EC realisation	Contribution matrix; Enterprise catalogue	Direct does not mean sole realisation; no extra relationships implied.
EC-14 → assurance-specific contribution	Service Guardian	Reusable assurance semantics, distinct from EC-12; component allocation remains unresolved.

BC-18 refers to the existing Business Capability catalogue entry **18 Health Service Assurance**. This reference does not assign a new Capability Tier or structural Canonical ID. The three Business Enabling capability names identify the approved responsibilities without inventing CT1 / CT2 / CT3 ancestry or new Feature identifiers.

Collaborative Enterprise Capability Contribution Matrix

This is the approved **first-pass contribution model**. Existing canonical EC names are preserved. **Direct** identifies a direct contribution, not sole realisation or ownership. **Supporting** identifies a supporting contribution. **No Material Role** records the approved absence of a material contribution in this model; it is not an invitation to invent a relationship.

Enterprise Capability	Assurance Design	Assurance Criteria Management	Governed Assurance
EC-01 Managed Entity & Relationship	Supporting	No Material Role	Supporting
EC-02 Context Management	Supporting	Supporting	Direct
EC-03 Managed State & Lifecycle	Supporting	Direct	Direct
EC-04 Information Management	Direct	Direct	Direct

Enterprise Capability	Assurance Design	Assurance Criteria Management	Governed Assurance
EC-05 Search & Discovery	Supporting	Supporting	Supporting
EC-06 Policy & Control	Direct	Direct	Supporting
EC-07 Provenance & Traceability	Supporting	Supporting	Direct
EC-08 Interoperability & Exchange	No Material Role	Supporting	Supporting
EC-09 Event & Subscription	Supporting	No Material Role	Direct
EC-10 Activity & Execution	Supporting	No Material Role	Direct
EC-11 Interaction & Experience	Supporting	Supporting	Supporting
EC-12 Operational Assurance	Supporting	Supporting	Direct, limited to generic operational/ processing assurance contribution
EC-13 Semantic Governance & Conformance	Supporting	Direct	Supporting
EC-14 Service Guardian	Direct	Direct	Direct

The three Business Enabling Capabilities are collaboratively realised. EC-14 contributes the assurance-specific semantics not truthfully represented by EC-01 through EC-13. Their contributions do not transfer source-information ownership, governing-requirement authority or subject-activity management to assurance. In particular, EC-10's direct contribution does not make Governed Assurance responsible for managing the subject activity, and it does not establish a relationship to an execution component.

This bounded, approved matrix is not an exhaustive Business Capability-to-Business Enabling or Feature-to-Enterprise mapping. It is consistent with the existing representative-derivation convention and does not manufacture missing traceability.

Preserved Semantic Boundaries

- **EC-12 / EC-14**: EC-12's established operational responsibilities remain intact. Its direct Governed Assurance contribution is limited to generic operational/processing assurance. EC-14 supplies assurance disposition, criteria applicability, evidentiary association, assessment, adjudication and independent findings/conclusions; EC-12 is not expanded to absorb them.
- **Clinical assurance**: Health Service Assurance is distinct from Clinical Services Delivery Assurance. Explicitly required assurance of facts or behaviour concerning clinical activity does not make Harmonia responsible for clinical judgement, professional practice or clinical adequacy of care. Capability 06 retains its established responsibility.
- **Evidence and time**: Source-information ownership and contextual evidence association remain distinct. Evidence and criteria may each be temporal/version-specific without deriving storage, replication or persistence mechanisms.
- **Assurance and operations**: Generic processing assurance evaluates business behaviour, including expected failure/recovery; the subject activity manages its own behaviour. Operational Health ≠ Processing Assurance.
- **Assessment and conclusion**: Assessment, adjudication and explicit insufficiency preserve quantitative, qualitative, compound, temporal and confidence-bearing meaning. Insufficient evidence establishes neither satisfaction nor non-satisfaction.
- **Independence**: Assurance independence concerns control of progression and conclusion, not physical separation. Evidence dependency does not compromise assurance independence; control dependency does.

Unresolved Relationships and Downstream Boundary

This approved derivation establishes only REQ-FND-005 → BC-18 → the three named Business Enabling Capabilities → the first-pass collaborative EC contribution model, including EC-14. The five-region/eighteen Business Capability model, BC-17 Health Service Governance, capabilities 06 and 14, the existing thirteen EC semantics and approved AX-05 reconciliation remain intact.

The following remain deliberately unresolved or unestablished:

- Relevance classification for BC-17/BC-18; BC-17 enabling derivation; relationships beyond the approved BC-18 derivation.
- Contextual-view placement, relevance classification, Capability Tier, complete ancestry, root status and structural Canonical IDs for the three assurance Business Enabling Capabilities; additional Feature decomposition.
- Courses of Action, Strategic Resources, Value Stream/stage relationships and additional capability contributions not explicitly approved here.
- Downstream Business Functions, Services, Processes, actors, roles, assurance Information Concepts, outcome taxonomies, criterion catalogues and applicability/confidence models beyond the approved Strategy semantics.
- Strategic logical component, application component, execution, persistence, protocol, deployment and implementation allocation for EC-14 and the three assurance Business Enabling Capabilities.

No relationship to Dokimasia, Ponos, Praxis, Pragma, Digital Twin, Mneme, Mnemosyne, Calliope, Iris, Pylai or another implementation construct is derived. Existing Dokimasia material is not reconciled or modified. Domain03 Business Architecture and Domain04 Information Architecture are unchanged. Existing or future suggestions cannot establish an allocation through lexical similarity, familiar implementation or the conceptual behavioural sequence of Service Guardian.

Navigation

- Domain02 Strategy
- Capability Framework
- Business Capability Catalogue
- Business Enabling Capability Catalogue and Assurance Principles
- Enterprise Capability Catalogue
- Capability Derivation Progression
- Capability Maps Index

13. B2.1 — Individual Care Delivery ↔ Health Information Access

RAD source: `docs/markdown/05-information-systems-architecture/traceability/b2-1-individual-care-delivery-health-information-access.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

B2.1 — Individual Care Delivery ↔ Health Information Access

B2.1 — Individual Care Delivery ↔ Health Information Access: PASS

1. Authority and bounded result

This record reconciles the explicitly agreed Domain05 Step B B2.1 decisions, 2026-10-13. The Domain02 Business Capability catalogue defines Individual Care Delivery and its six L2 capabilities. The established Health Information Access responsibility defines the enabling anchor. The Step B method governs relationship semantics and Health Information Access scope.

PASS applies only to the L1 relationship and six L2 relationships below. It is not a runtime verification, a declaration of exhaustive capability coverage or an application/component allocation. This record establishes no additional capability decomposition, Feature, Service, Function or Information mapping. **B2.2 is not commenced.** No other Business Capability or Business Enabling Capability is mapped by this reconciliation.

2. L1 relationship and rationale

`IndividualCareDeliveryenabled-byHealthInformationAccess`

Individual Care Delivery requires governed access to relevant longitudinal and current health information to support clinical assessment, diagnosis, treatment decisions, interventions and monitoring. Health Information Access enables access to that information but does not determine its clinical significance or perform the clinical decision-making of Individual Care Delivery.

3. L2 relationships and rationales

These are material capability-enablement relationships. They do not transfer clinical accountability or require each runtime exercise to retrieve information through Health Information Access. The L2 labels identify the agreed Domain02 decomposition; no additional structural Canonical ID scheme is established.

01.01 Individual Health Assessment

`IndividualHealthAssessmentenabled-byHealthInformationAccess`

Individual Health Assessment requires governed access to relevant existing and current health information so that available clinical, health and contextual information can be considered when establishing an understanding of the individual's health state. Health Information Access enables retrieval of that information but does not perform the assessment or establish the assessed findings.

Health Information Access does not own every mechanism by which assessment information is acquired. Direct examination, observation, history-taking or measurement may establish assessment information independently.

01.02 Clinical Determination

`ClinicalDeterminationenabled-byHealthInformationAccess`

Clinical Determination requires governed access to relevant current and historical health information where that information is necessary to interpret assessed findings and establish clinically meaningful conclusions about the individual's health state, condition, risk or prognosis. Health Information Access enables retrieval of that information but does not interpret its clinical significance or establish the clinical determination.

01.03 Individual Care Planning

`IndividualCarePlanningenabled-byHealthInformationAccess`

Individual Care Planning requires governed access to relevant current and historical health and care information where that information is necessary to establish an appropriate intended clinical response, including care objectives, interventions and monitoring. Health Information Access enables retrieval of that information but does not establish the care intent, select the clinical response or perform the planning.

Information concerning operational availability of services, resources, staff, facilities or appointments is not Health Information Access merely because it influences care planning.

01.04 Clinical Intervention

`ClinicalInterventionenabled-byHealthInformationAccess`

Clinical Intervention requires governed access to relevant health and care information where that information is necessary to safely and appropriately perform an intended clinical action, including information concerning the individual's current health state, established care intent and clinically relevant history. Health Information Access enables retrieval of that information but does not authorise, select or perform the clinical intervention.

Access to information describing an intended intervention does not confer authority to perform that intervention.

01.05 Individual Health Monitoring

`IndividualHealthMonitoringenabled-byHealthInformationAccess`

Individual Health Monitoring requires governed access to relevant current and historical health information where that information is necessary to establish change, stability or deviation in an individual's health state over time. Health Information Access enables access to prior, baseline and current health information required for comparison, but does not perform the monitoring or interpret the clinical significance of an identified change.

01.06 Individual Care Review

`IndividualCareReviewenabled-byHealthInformationAccess`

Individual Care Review requires governed access to relevant health and care information in order to evaluate delivered care and observed health progression against the intended care objectives. Health Information Access enables access to the care intent, relevant care history and health outcomes required for that evaluation, but does not perform the review or establish the continuing appropriateness of the care intent.

4. Preserved boundaries

The Domain02 decomposition and context rules apply: the six established children are now explicitly reconciled as BC CT2s. The current catalogue establishes the stated assessment, monitoring and review CT3s; the other branches retain their recorded uncertainty/stopping points. Under the fixed Capability model, subsequently required Harmonia Features must first have meaningful **Business Enabling CT3** frames established. BC decomposition does not terminate in Features and this B2.1 support trace establishes no ancestry or new CT3 enablement edge. None may be invented for structural completeness. Retained L1/L2 display labels alone do not allocate CT tiers. Examples are not subordinate Capabilities, and care setting, delivery mode, temporal pattern and organisational arrangement do not themselves define different business abilities. Acute, Planned/Scheduled and Longitudinal Care are not the ICD L2 decomposition.

B2-R07 distinguishes individual clinical / health / care information from operational / workforce / financial / corporate / service-resource information. No exhaustive edge-case taxonomy or allocation to other enabling capabilities is introduced.

`enabled-by` does not mean that Health Information Access interprets, judges, plans, authorises or performs clinical care. It expresses material enablement, not a mandatory runtime dependency. B2.1 PASS does not change retained candidate deferrals, declare B1 PASS or authorise subsequent Step B mappings.

14. Principal Business Processes

RAD source: `docs/markdown/03-business-architecture/processes/business-processes.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Principal Business Processes

1. Process Demarcation & Justification Rules

The named owning Capabilities retain their established responsibilities. Affected Capability Tier, complete ancestry, root status and full structural Canonical IDs remain unresolved under approved G1 K9. The explicit `ClientAdministration→PersonIdentity` relationship remains a partial chain, not a tier allocation.

Processes may elaborate scoped progression without reproducing Behaviour stage lists. Established ownership, responsibility, authority boundaries and supported obligations remain controlling. Omitted Process checkpoints are not invalidated by a Behaviour summary; omitted Behaviour responsibilities are not removed by a Process. Stage-count equality is not required, and extra detail does not establish universal applicability.

Illustrations are not complete normative transition models. Their scoped applicability and visible uncertainty annotations retain independent Business meaning; detailed execution-state machinery remains downstream. For a materially significant managed outcome that available evidence cannot establish, preserve uncertainty rather than infer success or failure from an absent acknowledgement, response or observation. No universal additional Process state is prescribed.

Where Healthcare Service context is material to the activity's meaning, authority, coordination, progression or accountability, retain that context in the relevant activity/information responsibility. Location, Organisation, Practitioner or Role cannot substitute for it; no representation or implementation binding follows.

In the Harmonia Business Architecture, a **Business Process** applies the canonical Process definition: ordered Function decomposition of complex change within the owning Capability or Feature responsibility. Existing activity-instance states, dispositions and outcomes retain their established meaning; no new transition model is inferred.

1.1 Process Inclusion Rule

> **Model a Business Process where ordered Function decomposition is material to understanding, structuring or managing a complex Business change and its progression.**

A Function may require further Processes recursively. Search, query, resolution, translation, mapping, verification, lookup and presentation do not automatically require a Process, but their labels do not exclude decomposition where justified. Service fulfilment may use Functions directly, Processes, or both. Function-level Information relationships remain primary; this reconciliation adds no named Process to the catalogue.

1.2 Canonical Business Process Catalogue

The sixteen pre-existing Business Processes remain in the current R1.x/R2.x baseline with the authorised identity/discharge reconciliations and explicit lifecycle qualifications. Three approved Health Service Assurance Processes extend this catalogue to **nineteen Business Processes**; their existing authority/progression boundaries are sufficient for this baseline while detailed lifecycle states and transitions remain unestablished.

RAD source listing (presentation only; no inferred ancestry).

Principal Business Processes

Entity & Administrative Processes

1. Governed Person Identity Correction
2. Practitioner Verification

Clinical Lifecycle & Service Administration Processes

3. Referral Progression
4. Encounter Lifecycle
5. Closed-Loop Order Progression
6. Clinical Document Lifecycle

Healthcare Facility & Logistics Operations Processes

7. Theatre Case Progression
8. Clinic / Operational Queue Progression
9. Bed Turnover
10. Operational Work Progression
11. Patient Transport
12. Specimen Transport
13. Discharge Progression

Horizontal Workflow Coordination Processes

14. Work Order Progression (Human Doing)
15. To Do Progression (Human Reviewing / Deciding)
16. Synthetic Task Progression (Automated System Work)

Health Service Assurance Processes

17. Assurance Process Design
18. Assurance Process Execution
19. Assurance Process Reporting / Communication

2. Entity & Administrative Processes

2.1 Governed Person Identity Correction Process

- **Owning Capability**: `PersonIdentity` (under `ClientAdministration`)
- **Process Purpose**: Processes externally authoritative identity corrections and merge/unlink outcomes, applies their consequences to Harmonia-managed identity information and identifier associations, and communicates resulting changes with an audit history.
- **Authority boundary**: Person correction/merge decisions originate externally. Internal triage, evidence verification and approval concern applicability and governed processing of that authoritative outcome, not person matching, golden-record determination, master-person selection or originating merge adjudication.

- **State Progression Lifecycle**: Correction Requested → HIM Triage → Evidence Verified → Correction Approved → Merge/Unlink Executed → Change Broadcasted → Closed
- **Key State Dispositions**:
  - `CorrectionRequested`: Externally authoritative correction or merge/unlink outcome received for governed application.
  - `HIMTriage`: Health Information Manager reviews the received decision/outcome and its applicability.
  - `EvidenceVerified`: Authoritative source decision and supporting provenance/documentation verified.
  - `CorrectionApproved`: Governed application of the externally authoritative outcome approved; this does not originate a person-merge decision.
  - `Merge/UnlinkExecuted`: External merge/unlink consequences applied to Harmonia's managed identifier associations and audit history.
  - `ChangeBroadcasted`: Downstream clinical systems notified of merged or rectified identity.

2.2 Practitioner Verification Process

- **Owning Capability**: `ProviderAdministration`
- **Process Purpose**: Governs the lifecycle of validating, certifying, and periodically re-verifying a healthcare practitioner's professional credentials, registration status, and clinical scope of practice.
- **State Progression Lifecycle**: Verification Initiated → Regulatory Registry Queried → Credentials Validated → Privileges Endorsed → Active Verified → Expired / Suspended
- **Key State Dispositions**:
  - `VerificationInitiated`: New practitioner registration or periodic re-credentialing triggered.
  - `RegulatoryRegistryQueried`: Real-time query executed against national registry (e.g., AHPRA).
  - `CredentialsValidated`: Primary qualifications, specialist endorsements, and sanctions checked.
  - `PrivilegesEndorsed`: Health service medical administration endorses admitting/procedural scope.
  - `ActiveVerified`: Practitioner verified for clinical ordering and electronic communication.
  - `Expired/Suspended`: Verification lapses or regulatory sanction triggers privilege suspension.

3. Clinical Lifecycle & Service Administration Processes

3.3 Referral Progression Process

- **Owning Capability**: `ReferralAdministration`
- **Process Purpose**: Governs the end-to-end operational progression of an incoming clinical referral from receipt through specialist clinical triage, booking, and final service acceptance.
- **Scope qualification**: This illustrates a specialist booking/attendance pathway; Referral does not universally require acceptance, scheduling, attendance or discharge, or establish delivery, responsibility actually assumed or Transfer of Care. Decline, rejection and redirection remain valid dispositions without invented transition paths. `Accepted/Waitlisted` is a local compound checkpoint, not universal equivalence between acceptance and waitlisting.
- **State Progression Lifecycle**: Submitted → Intake Validated → Clinically Triaged → Accepted / Waitlisted → Scheduled → Consultation Attended → Discharged / Rejected (Illustrative specialist pathway; dispositions vary)
- **Key State Dispositions**:
  - `Submitted`: Electronic referral received from GP or external facility.
  - `IntakeValidated`: Administrative validation of patient details, mandatory fields, and tests.
  - `ClinicallyTriaged`: Senior clinician assigns urgency category (e.g., Cat 1 urgent within 30 days).

- `Accepted/Waitlisted`: Referral accepted and placed onto the specialty waitlist.
- `Scheduled`: Outpatient appointment allocated and patient notified.
- `ConsultationAttended`: Patient seen; initial specialist assessment completed.
- `Discharged/Rejected`: Referral completed and discharged back to primary care, or formally rejected with clinical rationale.

3.4 Encounter Lifecycle Process

- **Owning Capability**: `Episode&EncounterAdministration`
- **Process Purpose**: Governs the clinical and administrative lifecycle of an acute, emergency, inpatient, or outpatient encounter between a patient and healthcare services.
- **State Progression Lifecycle**: Planned / Booked → Arrived → Triaged / Ingested → Active In-Progress → Discharged → Completed / Encoded (Scoped illustration; ON_LEAVE relationship unresolved)
- **Key State Dispositions**:
  - `Planned/Booked`: Elective admission or clinic appointment scheduled.
  - `Arrived`: Patient presents at facility or emergency desk.
  - `Triaged/Ingested`: Clinical triage category assigned; encounter record activated.
  - `ActiveIn-Progress`: Inpatient care, bedside monitoring, and clinical orders actively underway.
  - `Discharged`: Patient clinically discharged; care-place vacated.
  - `Completed/Encoded`: Clinical documentation finalized, ICD/DRG coding completed, and encounter legally closed.
- **Scope qualification**: These completion criteria retain their locally described applicability; they do not establish universal signing, legal closure or originating authority over clinical information. Finalisation, signing and legal qualification remain distinct. The relationship with Strategy `ON_LEAVE`, including applicability and transitions, remains unresolved.

3.5 Closed-Loop Order Progression Process

- **Owning Capability**: `OrderAdministration`
- **Process Purpose**: Governs the rigorous, closed-loop tracking of diagnostic pathology, radiology, and procedural orders from requisition to result correlation, preventing dropped or unfulfilled investigations.
- **Scope qualification**: These examples describe a diagnostic/procedural progression and do not exclude medication from the broader Order Administration responsibility. Specimen and result checkpoints are scoped. The signed requisition/report requirements remain within this illustrated scope; signing is not universally equivalent to finalisation or authority. Result consumption/binding does not define originating validity, authority, authorship or legal status.
- **State Progression Lifecycle**: Requisition Placed → Order Dispatched → Specimen Collected / Scheduled → In-Execution → Preliminary Result Bound → Final Result Bound → Closed / Verified (Scoped diagnostic/procedural illustration)
- **Key State Dispositions**:
  - `RequisitionPlaced`: Electronic order created and signed by requesting clinician.
  - `OrderDispatched`: Order routed and accepted by performing diagnostic service. This local compound checkpoint retains routing and recipient acceptance as distinct facts; dispatch alone establishes neither receipt nor acceptance. Behaviour `Received` is not equated with this checkpoint without evidence.
  - `SpecimenCollected/Scheduled`: Bio-specimen collected or imaging appointment booked.
  - `In-Execution`: Laboratory analysis or diagnostic scanning underway.
  - `PreliminaryResultBound`: Critical or interim observations published and linked to order.
  - `FinalResultBound`: Authoritative diagnostic report signed and bound to order.
  - `Closed/Verified`: Requesting clinician acknowledges result; order closed. This Order-closure checkpoint does not establish diagnostic-content verification, approval or clinical incorporation.

3.6 Clinical Document Lifecycle Process

- **Owning Capability**: `ClinicalRecordAdministration`
- **Process Purpose**: Governs the versioning, clinical sign-off, addenda, superseding, and legal status of clinical documents (discharge summaries, specialist letters, advance care directives).
- **State Progression Lifecycle**: Draft → Preliminary → Final Signed → Amended / Addended → Superseded → Entered-in-Error (Scoped illustration; no universal document lifecycle)
- **Key State Dispositions**:
  - `Draft`: Incomplete clinical document saved during consultation.
  - `Preliminary`: Document authored pending senior registrar or consultant countersignature.
  - `FinalSigned`: Authoritative clinical document legally signed and published.
  - `Amended/Addended`: Governed supplementary clinical addendum appended to signed document.
  - `Superseded`: Entire document replaced by a newer revision; prior version retained for audit.
  - `Entered-in-Error`: Document formally retracted due to wrong patient or invalid content; content struck through with retraction notice.
- **Scope qualification**: `Final≠automaticallyFinalSigned`. The described signing/countersigning and legal qualifications remain local requirements, not a universal document lifecycle. Authorship, attestation, approval, authentication, verification, signature, finalisation, authority and legal qualification remain distinct. Amendment, supersession and entered-in-error are not mandatory stages for every document, and consumer processing does not establish originating authority or legal status.

4. Healthcare Facility & Logistics Operations Processes

4.7 Theatre Case Progression Process

- **Owning Capability**: `TheatreOperations`
- **Process Purpose**: Governs the perioperative operational milestones of a surgical procedure within the operating theatre suite.
- **State Progression Lifecycle**: Case Called → In Anaesthetic Bay → Anaesthesia Commenced → Patient in Theatre → Knife-to-Skin → Procedure Finished → In PACU Recovery → Ward Handover Complete
- **Key State Dispositions**:
  - `CaseCalled`: Theatre team requests ward to send patient for surgery.
  - `InAnaestheticBay`: Patient arrives in theatre holding and identity/consent verified.
  - `AnaesthesiaCommenced`: Anaesthetic induction underway.
  - `PatientinTheatre`: Patient transferred onto operating table.
  - `Knife-to-Skin`: Surgical incision initiated (operative start).
  - `ProcedureFinished`: Wound closure completed and dressings applied.
  - `InPACURecovery`: Patient monitored in post-anaesthesia care unit.
  - `WardHandoverComplete`: Post-operative clinical handover to inpatient ward completed.

4.8 Clinic / Operational Queue Progression Process

- **Owning Capability**: `Clinic&PracticeOperations`
- **Process Purpose**: Governs the movement and state tracking of outpatients through an ambulatory specialty clinic session.
- **State Progression Lifecycle**: Scheduled → Patient Arrived → Checked-In → Roomed / Pre-Consult → Consultation In-Progress → Consultation Concluded → Departed
- **Key State Dispositions**:
  - `Scheduled`: Appointment booked on session list.
  - `PatientArrived`: Patient enters waiting area.
  - `Checked-In`: Reception verifies demographics and marks patient ready.

- `Roomed/Pre-Consult`: Nursing staff record baseline vitals and room patient.
- `ConsultationIn-Progress`: Clinician actively conducting consultation.
- `ConsultationConcluded`: Clinical discussion finished; follow-up orders placed.
- `Departed`: Patient checks out and departs clinic facility.

4.9 Bed Turnover Process

- **Owning Capability**: `Bed&Care-PlaceManagement`
- **Process Purpose**: Governs the rapid physical turnover, cleaning, sanitisation, and re-allocation of inpatient and emergency care-places.
- **State Progression Lifecycle**: Bed Vacated → Cleaning Dispatched → Sanitisation In-Progress → Terminal Cleaning Verified → Bed Ready / Available → Bed Allocated / Occupied
- **Key State Dispositions**:
  - `BedVacated`: Prior patient discharged or transferred.
  - `CleaningDispatched`: Cleaning job allocated to environmental services team.
  - `SanitisationIn-Progress`: Ward cleaning or isolation decontamination underway.
  - `TerminalCleaningVerified`: Ward coordinator or supervisor verifies care-place readiness.
  - `BedReady/Available`: Bed marked available for incoming patient allocation.
  - `BedAllocated/Occupied`: Incoming patient allocated and physically placed in bed.

4.10 Operational Work Progression Process

- **Owning Capability**: `WorkAllocation&Dispatch`
- **Process Purpose**: Governs the dispatch, assignment, execution, and completion of facility operational jobs (e.g., equipment moves, waste removal, linen supply).
- **State Progression Lifecycle**: Work Requested → Queued in Dispatch → Assigned to Worker → Job Accepted → Work In-Progress → Work Completed / Aborted
- **Key State Dispositions**:
  - `WorkRequested`: Facility support task submitted by clinical unit.
  - `QueuedinDispatch`: Job prioritized in operational queue.
  - `AssignedtoWorker`: Job allocated to specific staff member or team.
  - `JobAccepted`: Worker acknowledges dispatch on mobile device.
  - `WorkIn-Progress`: Task actively being executed.
  - `WorkCompleted/Aborted`: Job marked finished or formally cancelled with reason.

4.11 Patient Transport Process

- **Owning Capability**: `PatientTransport`
- **Process Purpose**: Governs the physical portering and transport of patients between hospital departments, wards, diagnostic suites, and external facilities.
- **State Progression Lifecycle**: Transport Requested → Porter Dispatched → Patient Collected → In-Transit → Delivered at Destination → Clinical Handover Complete
- **Key State Dispositions**:
  - `TransportRequested`: Clinical unit books wheelchair, bed, or ambulance transport.
  - `PorterDispatched`: Wardsperson assigned and en route to pickup location.
  - `PatientCollected`: Patient identity verified and transfer initiated.
  - `In-Transit`: Patient actively moving through facility corridors or ambulance transit.
  - `DeliveredatDestination`: Patient arrives at target diagnostic suite or ward.
  - `ClinicalHandoverComplete`: Safe custody and clinical briefing transferred to receiving staff.

4.12 Specimen Transport Process

- **Owning Capability**: `ClinicalLogisticsCoordination`

- **Process Purpose**: Governs the physical courier chain-of-custody, transport, and delivery of pathology bio-specimens, blood products, and surgical biopsies.
- **State Progression Lifecycle**: Specimen Packaged → Courier Dispatched → Chain-of-Custody Signed → In-Transit (Monitored) → Lab Ingress Received → Specimen Accepted
- **Key State Dispositions**:
  - `SpecimenPackaged`: Clinical unit packages bio-sample with barcode manifest.
  - `CourierDispatched`: Courier route assigned for pickup.
  - `Chain-of-CustodySigned`: Courier scans barcode and accepts legal custody.
  - `In-Transit(Monitored)`: Specimen in transit under temperature/time constraints.
  - `LabIngressReceived`: Central pathology specimen reception logs physical receipt.
  - `SpecimenAccepted`: Specimen integrity verified and passed to analytical track.

4.13 Discharge Progression Process

- **Owning Capability**: `DischargeManagement`
- **Process Purpose**: Governs the multidisciplinary coordination of patient discharge planning, pharmacy reconciliation, transport, and community handover.
- **State Progression Lifecycle**: Discharge Planning Initiated → Clinical Readiness Confirmed → Medications Reconciled → Transport & Services Booked → Discharge Summary Signed → Physically Departed (Illustrative preparation; publication, authorisation and exit distinct)
- **Key State Dispositions**:
  - `DischargePlanningInitiated`: Estimated Date of Discharge (EDD) set on admission.
  - `ClinicalReadinessConfirmed`: Attending medical team declares patient fit for discharge.
  - `MedicationsReconciled`: Hospital pharmacy reconciles and dispenses discharge medications.
  - `Transport&ServicesBooked`: Community nursing, home equipment, and patient transport booked.
  - `DischargeSummarySigned`: Applicable signed summary is available/published and may be communicated before departure where the Business behaviour requires it; signing, finalisation, publication and discharge authorisation remain distinct.
  - `PhysicallyDeparted`: Confirmed physical exit triggers departure notifications, updated bed-state communication and finalised-summary dispatch to external care providers under `FEAT-HSO-29`; care-place turnover follows its own responsibility.
- **Timing reconciliation**: Preparation, information availability/publication, applicable discharge authorisation and physical exit are distinct. Earlier preparation/communication is permitted; it does not realise a Feature triggered by confirmed exit. This illustrated local pathway establishes neither universal publication timing nor a universal discharge state machine, and adds no inferred publication event or authorisation state.
- **Preserved uncertainty**: Summary signing and finalisation are not universal equivalents. Detailed applicability, discharge-authority workflows and timing beyond the established preparation/confirmed-exit distinction remain unestablished.

5. Horizontal Workflow Coordination Processes

5.14 Work Order Progression Process

- **Owning Capability**: `Workflow&ActivityCoordination`
- **Activity Archetype**: **Human Doing** — physical or operational task executed by healthcare staff.
- **State Progression Lifecycle**: Created → Ready → Dispatched → Accepted → In-Progress → Completed / Failed / Cancelled
- **Key State Dispositions**:
  - `Created`: Work order instantiated with scope, target, and required skills.
  - `Ready`: Preconditions satisfied; available for assignment.
  - `Dispatched`: Sent to target individual or worker group queue.
  - `Accepted`: Worker claims task.

- `In-Progress`: Execution underway.
- `Completed`: Task finished with recorded completion telemetry.

5.15 To Do Progression Process

- **Owning Capability**: `Workflow&ActivityCoordination`
- **Activity Archetype**: **Human Reviewing / Updating / Deciding** — clinical review, document countersignature, or administrative authorization task.
- **State Progression Lifecycle**:

RAD diagram: textual source notation.

```
graph LR
    I["Issued"] --> B["In-Inbox"]
    B --> R["Opened / Under-Review"]
    R --> A["Actioned / Decided"]
    D["Dismissed / Delegated<br/>Disposition/position unresolved"]
    NOTE["Illustrative progression, not a normative transition model.<br/>Relationship of dismissal/delegation
    ↪  to action/decision is unestablished."]
    classDef note fill:#fff8dc,stroke:#8a6d3b,stroke-dasharray:5 5;
    class NOTE,D note;
```

- **Key State Dispositions**:
  - `Issued`: To Do generated (e.g., review abnormal potassium result).
  - `In-Inbox`: Displayed in practitioner's actionable task list.
  - `Opened/Under-Review`: Clinician inspecting relevant clinical context.
  - `Actioned/Decided`: Decision executed (e.g., signed off, order placed, acknowledged).
  - `Dismissed/Delegated`: Task reassigned to registrar or dismissed with comment.
- **Preserved uncertainty**: Whether dismissal/delegation is an alternative disposition, a subsequent action or a scoped variant remains unresolved; its disconnected diagram node establishes no transition or position in the illustrative progression.

5.16 Synthetic Task Progression Process

- **Owning Capability**: `Workflow&ActivityCoordination`
- **Activity Archetype**: **Non-Human Executable Work** — automated system workflows, batch syndications, or policy evaluation tasks.
- **State Progression Lifecycle**:

RAD diagram: textual source notation.

```
graph LR
    D["Draft / Scheduled"] --> E["Executing"]
    E --> W["Awaiting-Dependency"]
    W --> C["Completed"]
    F["Progression Stalled / Failed<br/>Outcome/position unresolved"]
    NOTE["Illustrative progression, not a normative transition model.<br/>No Completed-to-Stalled/Failed
    ↪  transition is established."]
    classDef note fill:#fff8dc,stroke:#8a6d3b,stroke-dasharray:5 5;
    class NOTE,F note;
```

- **Key State Dispositions**:
  - `Draft/Scheduled`: Automated task instantiated and scheduled for processing.
  - `Executing`: System actively processing task.
  - `Awaiting-Dependency`: Paused awaiting asynchronous external reply or event.
  - `Completed`: Automated execution succeeded and output artifact produced.
  - `ProgressionStalled/Failed`: Transient fault triggers scheduled recovery attempt; permanent failure records failure evidence.
- **Preserved uncertainty**: Whether stalled/failed is an alternative outcome, reopening/post-completion behaviour or a presentation defect remains unresolved. Its disconnected diagram node does not make success and failure equivalent or establish post-completion transitions.

6. Health Service Assurance Processes

The approved human review decisions on 2026-10-08 establish exactly the following three assurance Processes, building on the assurance Strategy and the two established assurance Roles and five Functions. **Element Type: Business Process (PR)** applies to each. Capability ownership below follows the established responsibility; **Feature association is not established** and **Canonical IDs remain unresolved** because Capability Tier, complete ancestry and local identifier allocation remain unestablished. Catalogue numbering is navigation, not identifier allocation.

6.1 Assurance Process Design

- **Canonical Name**: `AssuranceProcessDesign`.
- **Owning Capability**: Assurance Design.
- **Process Purpose**: Establishes and maintains how satisfaction of a governed requirement, constraint or expected behaviour is to be assured.
- **Principal Performing Role**: Service Assurance Modeller.
- **Participating Functions**: Design Service Assurance and Manage Assurance Criteria. Manage Assurance Criteria retains ownership within Assurance Criteria Management; its participation does not subsume that Capability's criteria lifecycle or transfer its information responsibility to Assurance Design.
- **Conceptual Output**: Assurance Definition, retaining its conceptual Business Architecture output/boundary treatment.
- **Boundary**: Governance Authority establishes what is authoritative. Design does not grant Service Assurance Modeller approval authority. Detailed Assurance Definition lifecycle, approval/publication workflows, criteria lifecycle and information modelling remain unresolved. Participation of the two Functions does not establish a cross-capability lifecycle or a new exchange/Service between their owners.

6.2 Assurance Process Execution

- **Canonical Name**: `AssuranceProcessExecution`.
- **Owning Capability**: Governed Assurance.
- **Process Purpose**: Performs defined assurance for an identifiable governed subject in accordance with an applicable Assurance Definition and establishes an Assurance Finding / Conclusion from applicable trustworthy evidence.
- **Principal Performing Role**: Service Guardian.
- **Participating Functions**: Establish Assurance Context, Assess Assurance Evidence and Adjudicate Assurance Assessment, all retaining Governed Assurance ownership.
- **Conceptual Basis and Output**: Applicable Assurance Definition plus identifiable governed subject; Assurance Finding / Conclusion established through adjudication where sufficient trustworthy evidence permits. Correct execution with insufficient evidence does not imply satisfaction or non-satisfaction or force a conclusion.
- **Boundary**: Function completion does not define Process lifecycle states. No Context Established, Evidence Assessed, Adjudicated or Concluded states are inferred. Detailed lifecycle states, transitions and initiation mechanics remain unresolved. Requesting this Process does not grant control of evidence assessment, adjudication or the resulting finding/conclusion.

RAD diagram: textual source notation.

```
graph TD
    DEF["Applicable Assurance Definition<br/>Conceptual basis"] -->|Assurance basis| EXEC["PR: Assurance Process
    ↪  Execution<br/>Owner: Governed Assurance"]
    SUBJECT["Identifiable Governed Subject"] -->|Subject of assurance| EXEC
    EXEC -->|Participating Function| CONTEXT["FN: Establish Assurance Context"]
    EXEC -->|Participating Function| ASSESS["FN: Assess Assurance Evidence"]
    EXEC -->|Participating Function| ADJ["FN: Adjudicate Assurance Assessment"]
    EXEC -->|Establishes through adjudication| FINDING["Assurance Finding / Conclusion"]
```

6.3 Assurance Process Reporting / Communication

- **Canonical Name**: `AssuranceProcessReporting/Communication`.
- **Owning Capability**: Governed Assurance.
- **Process Purpose**: Communicates applicable established Assurance Findings and Conclusions to legitimate recipients according to their responsibilities, authority and information requirements, without altering the established assurance outcome or assuming responsibility for the recipient's subsequent action.
- **Starting Basis**: An already established Assurance Finding / Conclusion. This Process does not perform assessment or adjudication, alter or reinterpret the established outcome.
- **Communication Participation**: Service Guardian participates through Service Assurance Outcome Communication, associated with Communicate Service Assurance Outcome. No additional Function is introduced to populate this Process.
- **Boundary**: Reporting / Communication is not limited to documentary report production; an appropriate Business Interaction may communicate the outcome without a report document. It assumes no operational response, remediation, escalation, policy/compliance response, process improvement, clinical management, clinical judgement or other recipient action. Detailed communication progression and completion semantics remain unresolved.

6.4 Process-Family Relationship and Preserved Boundaries

RAD diagram: textual source notation.

```
graph TD
    DESIGN["PR: Assurance Process Design<br/>Owner: Assurance Design"] -->|Conceptual output| DEF["Assurance
    ↪  Definition"]
    DEF -->|Reusable assurance basis| EXEC["PR: Assurance Process Execution<br/>Owner: Governed Assurance"]
    EXEC -->|Established through adjudication| FINDING["Assurance Finding / Conclusion"]
    FINDING -->|Already established outcome| COMM["PR: Assurance Process Reporting / Communication<br/>Owner:
    ↪  Governed Assurance"]
```

Both diagrams express semantic Business Architecture relationships, not lifecycle states, mandatory runtime topology, application orchestration, deployment sequence, component structure, message flow or a complete implementation workflow. An applicable Assurance Definition may be reused by multiple Execution instances; each assurance activity does not require a newly executed Design instance. Using the modelling output establishes a responsibility/information dependency, not a Modeller-to-Guardian Interaction or Collaboration.

> **Governed Assurance does not recursively assure its own execution. Assurance activity execution integrity is provided by the established activity execution framework.**

Managed progression of assurance activity does not make that execution another Service Assurance subject. No recursive assurance Process, Function, Service or conclusion is established; the activity execution framework reference allocates no solution component. System Steward's status observation is management/stewardship of progression, not assurance of assurance. See the non-recursion boundary.

The sixteen pre-existing Processes retain their owners, definitions and state models. Clinical review, sign-off, countersignature and clinical decisions remain with applicable clinical processes/authorities; To Do completion is not an independent assurance conclusion. Generic Work Order, To Do and Synthetic Task progression do not supply assurance lifecycle states or an assurance execution allocation.

Service Guardian may manage its own assurance progression without managing its subject. Neither assurance Role acquires the subject's progression, assignment, delegation, failure/recovery, operational escalation, remediation or improvement responsibility. Execution failure/indeterminate outcome remain distinct from correctly executed assurance with insufficient evidence. Remaining decisions include lifecycle states/transitions, initiation mechanics, detailed information semantics and response mechanisms; the three Process names and purposes are resolved.

15. Health Service Assurance — Bounded Business Architecture Derivation

RAD source: `docs/markdown/03-business-architecture/behaviours/health-service-assurance.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Health Service Assurance — Bounded Business Architecture Derivation

1. Authority and Derivation Scope

This view carries the approved Domain02 Health Service Assurance derivation into Domain03. Its upstream responsibility chain is REQ-FND-005 — Independent Assurance of Governed Activity → BC-18 Health Service Assurance → **Assurance Design**, **Assurance Criteria Management** and **Governed Assurance**, with the approved collaborative Enterprise Capability contributions including **EC-14 Service Guardian**.

AX-14 requires the responsibility distinctions below to remain explicit. AX-17 and the Domain03 metamodel require established responsibility to be distinguished from unestablished naming, decomposition, exposure, progression and authority relationships. AX-06, AX-07, AX-08 and AX-09 continue to govern information authority, governed access, meaningful evidence and contextual evidence selection. AX-15 governs operational uncertainty without making it equivalent to insufficient assurance evidence.

The approved human review decisions on 2026-10-08 extend the previous bounded derivation with **Service Assurance Modeller** and **Service Guardian** Business Roles and exactly five Business Functions: **Design Service Assurance**, **Manage Assurance Criteria**, **Establish Assurance Context**, **Assess Assurance Evidence** and **Adjudicate Assurance Assessment**. Their capability ownership and Role performance are documented in §3. The assurance Role previously named Guardian is now Service Guardian; legal/personal-welfare Guardian remains unchanged. These naming and Function-decomposition decisions are resolved; further decomposition is not approved.

The next approved human review decisions on 2026-10-08 establish exactly three Business Processes, three Business Services and three Business Interactions, with the approved consumer/participant relationships. They also establish the Financial Governance deliberate exclusion and assurance non-recursion boundary. The five approved Functions are preserved without reinterpretation or further decomposition.

The three assurance capabilities remain established responsibility contexts with unresolved contextual-view placement, Capability Tier, complete ancestry, Feature decomposition and structural Canonical IDs. This document is a capability-scoped view, not a sixth contextual view or an identifier ancestor. Specific Actors and Collaborations remain unallocated; detailed Process states/transitions, information models and implementation remain unresolved.

EC-14 Service Guardian is an Enterprise Capability. Service Guardian is also the intentional canonical name of a distinct Business Role. Shared terminology does not equate their architectural types or make either a software component. The approved human review decisions supply the Business Architecture relationships; they are not inferred from the EC contribution model or its explanatory sequence. Previous completion reports supply execution context only and are not architectural authority.

2. Governance, Management, Guardianship and Clinical Authority

RAD source listing (presentation only; no inferred ancestry).

Governance defines

↓

Management performs

↓

Guardianship assures

This expresses semantic responsibilities, not organisational structure, a compulsory execution sequence or technical topology.

Responsibility	Business meaning	Preserved boundary
Governance	Defines applicable requirements, obligations, authority, constraints, controls, reporting requirements and checkpoints.	Establishes the normative envelope without taking over the operational means or progression of the governed activity. Subject-specific governance remains intrinsic to its established subject capability.
Operational management	Performs or manages the subject activity within its constraints, including monitoring for progression, assignment, delegation, operational escalation, response, remediation and expected failure/recovery.	Monitoring for operational completion or deciding what happens next does not establish independent assurance.
Service Assurance modelling	Service Assurance Modeller determines and models how satisfaction of governed requirements, constraints and expected behaviours is to be assured within Assurance Design and Assurance Criteria Management.	Modelling does not inherently approve the governing requirement, Assurance Definition or Assurance Criteria, perform independent assurance, or manage the subject.
Guardianship / governed assurance	Service Guardian independently evaluates governed activity, information, state or outcomes against applicable criteria and establishes findings and conclusions from sufficient trustworthy evidence.	Service Guardian may manage its own assurance activity; it does not thereby perform or manage the subject, direct its operational response or confer clinical authority.
Clinical review / clinical assurance	Exercises the applicable clinical process and authority's responsibility for clinical evaluation and professional judgement.	Neither assurance Role establishes clinical adequacy, clinical correctness or professional clinical judgement. Clinical quality, safety and improvement retain their existing responsibilities.

Governance Authority establishes what is authoritative. Service Assurance Modeller models how satisfaction will be assured. Service Guardian independently establishes what the applicable evidence demonstrates. Governance Authority references the existing mandated authority Roles, including Policy Authority for policy and Regulator for statutory authority, with existing steward mandates preserved. No new Governance Authority Role or extension of those mandates is established. Modelling does not grant approval authority for the governing requirement, Assurance Definition or Assurance Criteria; exact approval allocation/workflows remain unresolved.

RAD diagram: textual source notation.

```
graph TD
    GOV["Governance Authority<br/>Existing mandated authority Roles"] -->|Establishes authoritative basis|
    ↪  MOD["Business Role<br/>Service Assurance Modeller"]
```

```
MOD -->|Performs| DESIGN["Function: Design Service Assurance<br/>Owner: Assurance Design"]
MOD -->|Performs| CRITERIA["Function: Manage Assurance Criteria<br/>Owner: Assurance Criteria Management"]
MOD -->|Models| DEF["Assurance Definition<br/>Conceptual modelling output"]
DEF -->|Modelling boundary for assurance| GUARD["Business Role<br/>Service Guardian"]
GUARD -->|Performs| CONTEXT["Function: Establish Assurance Context<br/>Owner: Governed Assurance"]
GUARD -->|Performs| ASSESS["Function: Assess Assurance Evidence<br/>Owner: Governed Assurance"]
GUARD -->|Performs| ADJ["Function: Adjudicate Assurance Assessment<br/>Owner: Governed Assurance"]
ADJ -->|Establishes| FINDING["Assurance Finding / Conclusion"]
```

This is a responsibility/semantic model, not mandatory execution topology, organisational or application structure, deployment topology or a complete Business Process. The output labels do not introduce Information Objects or a formal information model. The diagram distinguishes performance through a Role from Function ownership by a Capability and does not define process ordering or an assurance initiation/approval exchange.

A real-world Actor may fulfil different Roles in different contexts where governance permits. That possibility does not relax REQ-FND-005: assurance progression and conclusion must remain independently governed rather than controlled by the performer or manager of the subject. The subject may contribute evidence but must not solely determine, suppress, manufacture or retrospectively alter its assurance conclusion. Evidence dependency and control dependency remain distinct.

Neither Service Assurance Modeller nor Service Guardian inherently possesses clinical authority or replaces professional clinical judgement, clinical peer review or clinical governance. Information and behaviour concerning clinical activity may be subject to governed assurance where an explicit applicable requirement establishes that concern. This does not constitute **Clinical Services Delivery Assurance** or transfer clinical-care responsibility to Guardianship or Harmonia. The approved clinical boundary, Service Delivery care-enablement principle, and existing clinical review responsibilities remain controlling. No generic clinical-assurance authority Role is derived.

Neither assurance Role manages the subject merely because it designs or performs assurance. Service Guardian may manage progression of its own assurance activity without assuming progression of its subject. Findings may cause another responsible party to initiate management, escalation, remediation or other operational action; no such response responsibility is allocated to Service Guardian.

3. Direct Capability-Scoped Responsibility Consequences

The following five Functions are approved Business Architecture. Each is behaviour within its established owning Capability, performed through the stated Business Role. **Element Type: Business Function (FN)** applies to each. **Canonical IDs remain unresolved** because Capability Tier, complete ancestry and local identifier allocation are unestablished. **Feature association is not established** for any of these Functions; no Feature or structural ID is invented to anchor them. Function ownership does not transfer source-information ownership to an assurance Role or Capability.

3.1 Assurance Design — Service Assurance Modeller

- **Owning Capability**: Assurance Design.
- **Performing Business Role**: Service Assurance Modeller.

Design Service Assurance

- **Canonical Name**: `DesignServiceAssurance`.
- **Definition**: Defines how satisfaction of a governed requirement, constraint or expected behaviour is to be independently established, including the applicable assurance disposition, assurance criteria, required evidence and evaluation expectations.

- **Boundary**: Concerns assurance design. It does not establish the underlying governance requirement, grant authority to it, perform the resulting assurance, manage the subject activity or prescribe application or implementation mechanisms. Modelling the resulting Assurance Definition does not grant approval authority. The Strategy peer relationship to behaviour design and failure/recovery design creates no additional Capability or Function.

3.2 Assurance Criteria Management — Service Assurance Modeller

- **Owning Capability**: Assurance Criteria Management.
- **Performing Business Role**: Service Assurance Modeller.

Manage Assurance Criteria

- **Canonical Name**: `ManageAssuranceCriteria`.
- **Definition**: Establishes and manages reusable, governed and temporally identifiable assurance criteria through which governed subjects may be evaluated.
- **Scope**: Criteria may express governance rule boundaries, quality constraints, performance expectations, compliance criteria, temporal criteria, evidence requirements or completeness expectations. These examples are not an exhaustive taxonomy. Criteria may have lifecycle, version and temporal applicability.
- **Boundary**: Managing a criterion does not transfer authority for the governing requirement from which it may derive. Criteria may operationalise a requirement without silently altering, weakening, strengthening or replacing it; not every criterion must derive from formal Governance. Modelling/managing criteria does not inherently confer approval authority. Detailed criterion lifecycle states and approval workflows remain unresolved.

3.3 Governed Assurance — Service Guardian

- **Owning Capability**: Governed Assurance.
- **Performing Business Role**: Service Guardian.

Establish Assurance Context

- **Canonical Name**: `EstablishAssuranceContext`.
- **Definition**: Establishes the governed subject, applicable assurance purpose and criteria, relevant temporal context, and contextual association of sufficient trustworthy source information as evidence required for an assurance activity.
- **Scope**: Includes the evidence-assembly responsibility previously described as Assurance Evidence Assembly; no separate evidence-assembly Function is established. Sufficiency is required for conclusion, not assumed merely because context is established; unavailable or insufficient evidence remains explicit.
- **Information Boundary**: Source information remains owned by its responsible source Capability. Its use as evidence does not transfer ownership or originating authority. Governed Assurance owns the contextual association that particular information constitutes evidence for a particular assurance purpose. Evidence association and applicable criteria may each be temporal and version-specific; association distinguishes the information state relevant to assurance from its current state.
- **Realisation Boundary**: Uses generic EC-02 context-management capability together with EC-14 assurance-specific semantics as documented in §4.1. Establishment of Assurance Context prescribes no copying, persistence, aggregation, caching or other technical mechanism.

Assess Assurance Evidence

- **Canonical Name**: `AssessAssuranceEvidence`.
- **Definition**: Evaluates the available trustworthy evidence against the applicable assurance criteria and establishes what that evidence demonstrates for each relevant criterion or assessment concern.
- **Boundary**: Assessment remains distinct from adjudication. It may identify satisfaction, exception, insufficiency or other criterion-level results where applicable; no universal result taxonomy is established. Quantitative, qualitative, compound, temporal and confidence-bearing assessment remain possible. **Insufficient evidence is not equivalent to either satisfaction or non-satisfaction.**

Adjudicate Assurance Assessment

- **Canonical Name**: `AdjudicateAssuranceAssessment`.
- **Definition**: Determines the assurance finding or conclusion that follows from the assessed evidence and applicable assurance criteria.
- **Boundary**: Establishment of the finding/conclusion is part of adjudication. It remains distinct from evidence assessment, operational response, remediation, escalation, workflow progression and clinical judgement. No separate conclusion-establishment or reporting Function is introduced merely because a conclusion is established or may be communicated.

3.4 Preserved Assurance Obligations

The approved assurance dispositions remain explicit governed decisions. **Not Assurable** does not remove the requirement; **Not Worth Assuring** does not make it optional. Disposition does not require specific assurance of every individual activity, information item or outcome.

The approved generic processing assurance obligation constrains Harmonia-managed workflow behaviour. Evaluating business progression, completion, timeliness, pathways, failure/recovery and outcomes for assurance is distinct from monitoring to progress that work. The existing operational Processes and Functions retain their owners; neither successful completion nor an operationally healthy environment establishes an assurance conclusion. No final criterion catalogue or applicability model is supplied by these examples.

Assessment asks what the evidence demonstrates against applicable criteria; adjudication establishes what finding or conclusion follows. The approved decisions now establish these as distinct Functions performed through Service Guardian; they do not require separate Roles or process stages. Assurance is not universally reducible to PASS / FAIL / UNKNOWN. An assurance activity may execute correctly while lacking enough evidence to conclude. That insufficiency is distinct from its own execution failure or indeterminate execution outcome and must not imply satisfaction or non-satisfaction.

4. Information and Dependency Consequences

The Business Information Responsibility material records the directly supported demarcations: Assurance Design defines how satisfaction will be assured; Assurance Criteria Management manages criteria; Governed Assurance owns contextual evidentiary association and its findings/conclusions. Underlying source information remains owned and managed by its established capability. Evaluating it does not acquire its originating authority.

Evidence association must distinguish the relevant information state from its current state where temporal/version-specific inclusion matters. Evaluation must use temporally appropriate evidence and criteria. Evidence collection, provenance, audit information and operational completion are not themselves assurance conclusions. No assurance Information Object, outcome taxonomy, representation, custody, retention rule or persistence structure is defined here.

An evidence need does not identify a Business Service/provider or authorise a new consumption edge. The dependency assessment preserves that distinction, including the prohibition on

subject control of assurance progression or conclusion. The approved Enterprise Capability contribution matrix remains a Strategy contribution model, not a Business Service consumption matrix.

4.1 Context Management Contribution

EC-02 Context Management supplies generic establishment, validation, propagation and preservation of meaningful context. EC-14 Service Guardian supplies assurance-specific subject/purpose, criterion applicability and contextual evidentiary-association semantics. Their contributions together support **Establish Assurance Context**, owned by Governed Assurance and performed through the Service Guardian Business Role.

RAD diagram: textual source notation.

```
graph TD
    CONTEXT["Enterprise Capability<br/>EC-02 Context Management"] -->|Generic context semantics| FN["Business
    ↪  Function<br/>Establish Assurance Context<br/>Owner: Governed Assurance"]
    ASSURANCE["Enterprise Capability<br/>EC-14 Service Guardian"] -->|Assurance-specific semantics| FN
```

This documents the approved semantic contribution relationship. In the Domain02 collaborative contribution matrix, EC-02 and EC-14 each contribute directly to Governed Assurance. Neither is a sole realiser or Function owner; the other approved capability contributions remain intact. This view is not an exhaustive Function-to-EC allocation matrix, a new Business Service consumption edge or an application/runtime allocation. How context establishment is realised belongs to later architecture; no loader/unloader mechanism or implementation pattern is derived.

4.2 Assurance Definition — Conceptual Modelling Output

Assurance Definition describes the conceptual output/boundary between Service Assurance modelling and governed assurance: how satisfaction is to be assured, including assurance disposition, applicable criteria, required evidence and evaluation expectations. This output description follows directly from **Design Service Assurance** within **Assurance Design**, performed through **Service Assurance Modeller**; governed assurance uses that modelling boundary without transferring governance authority or establishing an exposed Service or approval exchange.

The existing Domain03 rules permit documenting this business output and its responsibility context. They do not establish its formal information structure, identity, lifecycle, representation or approval model. The term therefore remains a conceptual responsibility/output description, not a newly allocated formal Business Information element, Information Object or Data Object. Formal information modelling is deferred. Modelling an Assurance Definition does not make it authoritative or grant its modeller approval authority; applicable governance authority must establish that standing.

5. Approved Governed Assurance Business Services

Exactly three assurance Services are established below. **Element Type: Business Service (SV)** applies to each; each is owned/exposed by **Governed Assurance** with Service Guardian participating through its associated Business Interaction. Identifiable consumers/recipients outside that responsibility boundary justify the exposure under the metamodel. **Feature association is not established** and **Canonical IDs remain unresolved**; no Capability Tier, ancestry or local code is invented.

The Services expose approved request, activity-progression observation and established-outcome communication behaviour. They do not mechanically correspond one-to-one with the five Functions, nor introduce new request, status or reporting Functions. Detailed internal Service-to-Function mappings and contracts beyond the approved behaviour remain unestablished. Status observation concerns assurance's own permitted activity progression, not assessment of the subject or a new status Process.

5.1 Request Service Assurance

- **Canonical Name**: `RequestServiceAssurance`.
- **Definition**: Enables an authorised consumer to request that Service Assurance be performed for an applicable governed subject in accordance with an established Assurance Definition.
- **Approved Consumers**: `CareCoordinator`, `ServiceCoordinator`, `Regulator`, `PolicyAuthority`, `SystemSteward`.
- **Associated Interaction**: Service Assurance Request, with Service Guardian and one authorised requesting Role.
- **Process Relationship**: Requests Assurance Process Execution under an applicable established Assurance Definition. It does not require a new Assurance Process Design instance for each request.
- **Independence Boundary**: The requesting Role identifies the assurance need within its responsibility and authority. Requesting does not grant authority to define or alter the Assurance Definition, alter Assurance Criteria, direct evidence assessment, determine adjudication or control the resulting Assurance Finding / Conclusion. REQ-FND-005 control independence remains applicable, including where a requester performs or manages the subject. Detailed payloads, contracts, initiation mechanics and states remain unresolved.

5.2 Request Service Assurance Status

- **Canonical Name**: `RequestServiceAssuranceStatus`.
- **Definition**: Enables the System Steward to obtain the current business progression state of an identifiable Service Assurance activity for stewardship purposes.
- **Only Approved Consumer**: `SystemSteward`.
- **Associated Interaction**: Service Assurance Status, between System Steward and Service Guardian.
- **Status Boundary**: Concerns **activity progression state**. It does not communicate provisional findings, incomplete assessment results as conclusions, predicted outcomes or unadjudicated conclusions. Other requesting Roles do not gain status access merely because they requested assurance. No detailed activity-status taxonomy is established.

Service Assurance Status answers ”where is the assurance activity in its progression?” It does not answer ”what has the assurance established?” The latter requires an adjudicated Assurance Finding / Conclusion and subsequent outcome communication; neither status nor execution completion establishes that conclusion.

Management Monitoring evaluates activity state in order to progress or manage activity. Assurance evaluates activity against governing criteria in order to establish an assurance conclusion. System Steward’s use of this Service is stewardship / management observation of assurance progression, not itself assurance or recursive assurance. The non-recursion boundary in §7.2 applies.

5.3 Communicate Service Assurance Outcome

- **Canonical Name**: `CommunicateServiceAssuranceOutcome`.
- **Definition**: Communicates applicable established Assurance Findings and Conclusions to authorised recipients according to their responsibilities, authority and information requirements.
- **Approved Potential Recipients**: `CareCoordinator`, `ServiceCoordinator`, `Regulator`, `PolicyAuthority`, `SystemSteward`.
- **Associated Interaction**: Service Assurance Outcome Communication, with Service Guardian and one or more authorised recipient Roles.
- **Process Relationship**: Exposes the communication behaviour of Assurance Process Reporting / Communication, beginning from an already established finding/conclusion. It does not perform assessment or adjudication or alter/reinterpret the established outcome. A documentary report is not required.

- **Recipient Boundary**: No mandatory broadcast is established. A particular outcome may be communicated to one, some or all legitimate recipients according to applicability, authority, responsibility and information requirements. Requester, status consumer and outcome recipient need not be the same Role. Communication does not transfer responsibility for recipients' interpretation within their authority, operational/clinical management, remediation, escalation, policy/compliance response, process improvement or other subsequent action to Service Guardian.

5.4 Approved Consumer and Recipient Concerns

The existing Role definitions and mandates remain controlling. These are approved consumer/recipient concerns, not exhaustive Role responsibilities or new Roles. Each use requires applicable authority; Role eligibility alone does not grant unrestricted information access.

Business Role	Request Service Assurance	Request Service Assurance Status	Potential recipient of Communicate Service Assurance Outcome	Principal assurance concern
Care Coordinator	Approved within authority	Not approved	Approved where applicable/ authorised	Clinical management
Service Coordinator	Approved within authority	Not approved	Approved where applicable/ authorised	Operational management and potentially task / people process improvement
Regulator	Approved within authority	Not approved	Approved where applicable/ authorised	Service governance / compliance
Policy Authority	Approved within authority	Not approved	Approved where applicable/ authorised	Service governance / compliance and policy effectiveness / improvement
System Steward	Approved within authority	Only approved consumer	Approved where applicable/ authorised	ICT process improvement and stewardship of assurance activity progression

Care Coordinator's request or receipt may inform Clinical Management; clinical interpretation, judgement and action remain with the applicable clinical authority. Service Coordinator retains operational management/improvement responsibilities; System Steward's status access does not make Service Guardian responsible for task progression, assignment, delegation, operational escalation, remediation or process improvement. Regulator and Policy Authority retain their established governance/compliance mandates. None of these consumer relationships transfers source-information ownership, governance authority or control of assurance assessment/adjudication.

6. Additional Business Elements Not Yet Established

The two assurance Roles, five Functions, three Processes, three Services, three Interactions and approved Role relationships are established. The following further concerns remain explicitly **unresolved and non-authoritative**; their inclusion does not add them to a catalogue. Financial Governance's scope exclusion and assurance non-recursion are settled boundaries, not gaps.

Candidate concern assessed	What approved architecture establishes	Architectural decision required before adding an element
Further Function decomposition / Feature association	Exactly the five approved Functions in §3, including evidence assembly within context establishment and conclusion establishment within adjudication.	Additional Functions and Feature associations require a further decision. No separate applicability/criteria-resolution, evidence-assembly, conclusion-establishment, reporting, remediation or escalation Function is approved.
Further Business Services / contracts	Exactly the three Services in §5 with their approved consumers/ recipients, owner and behaviour.	Additional criteria/evidence Services, exact evidence/criteria providers, detailed Service contracts and internal Service-to-Function mappings are unestablished. No extra Service is inferred from reportability, reuse or evidence need.
Detailed Process progression	Assurance Process Design, Execution and Reporting / Communication have established purposes, participating Functions where approved, conceptual boundaries and ownership.	Detailed lifecycle states/transitions, initiation mechanics, completion/ disposition semantics, criteria lifecycle and approval/publication workflows remain unresolved. Function completion, EC-14's sequence and existing coordination lifecycles do not supply assurance states.
Further Business Interactions	Service Assurance Request, Status and Outcome Communication with approved Role participants and associated Services.	Evidence acquisition/exchange Interactions remain unresolved; source-owned information may be available through existing governed information access. No Evidence Contribution or Modeller-to-Guardian Interaction is created from evidence association or use of an Assurance Definition. Further exchange commitments require separate approval.
Assurance Role collaborations	Roles participate in the three approved Interactions; capability realisation is collaborative.	Establish an enduring, structured Actor/ Role association with a defined purpose and permitted control relationships. Interaction does not mechanically imply Collaboration; no existing membership or new assurance collective is established.
Further responsibility relationships / dependencies	Approved request/status/ outcome Role relationships, governance mandates and independence; source ownership, operational management and clinical authority remain separate.	Detailed Actor eligibility/mandates beyond approved Roles, coverage/applicability, initiation mechanics, exact approval allocation, evidence/criteria providers, findings-to-operational-response mechanisms and exact authorised external financial-governance recipients remain unresolved. Financial governance responsibility itself is deliberately excluded.
Further Business Information responsibilities	Responsibility demarcations and contextual/temporal evidence obligations in §4; conceptual Assurance Definition; progression status distinct from established findings/conclusions and their communication.	Detailed request/status/outcome information semantics, formal Assurance Definition modelling, further information concepts/asset identity, custody/lifecycle, criterion catalogues, outcome taxonomies, confidence models and retention remain unresolved. No formal information element or source-information reassignment is inferred.

6.1 Previous Questions Resolved by Approved Human Review

Previous question	Approved resolution
Legal Guardian / assurance Guardian naming collision	**Resolved:** legal/personal-welfare Guardian remains intact; governed assurance uses the unambiguous **Service Guardian** Business Role. EC-14 retains the intentional shared name as a distinct Enterprise Capability.
Who models how satisfaction will be assured?	**Resolved: Service Assurance Modeller** performs Design Service Assurance within Assurance Design and Manage Assurance Criteria within Assurance Criteria Management, without inherent governance/approval authority.
Function names and bounded decomposition	**Resolved for this stage:** exactly five Functions are established in §3. Evidence assembly is included in Establish Assurance Context; finding/conclusion establishment is included in Adjudicate Assurance Assessment. No separate reporting Function is established.
Generic context management / assurance-specific context relationship	**Resolved to the approved extent:** EC-02 and EC-14 contribute together to Establish Assurance Context; sole realisation, Service exposure and implementation allocation are not implied.
Assurance Process names and purposes	**Resolved:** exactly Assurance Process Design, Assurance Process Execution and Assurance Process Reporting / Communication; detailed states/transitions remain unresolved.
Assurance Business Services and consumers	**Resolved:** Request Service Assurance for the five approved Roles; Request Service Assurance Status exclusively for System Steward; Communicate Service Assurance Outcome for applicable authorised recipients from the five Roles.
Assurance Business Interactions and participants	**Resolved:** Service Assurance Request, Service Assurance Status and Service Assurance Outcome Communication, with Service Guardian and the approved requesting/status/recipient Roles.
Assurance status versus outcome	**Resolved:** status concerns business progression only and exposes no unadjudicated finding/conclusion. Established-outcome communication remains separate and is not mandatory broadcast or responsibility for recipient action.
Financial Governance scope	**Deliberately excluded:** no distinct Harmonia financial-governance Role, Function, Capability, Service, Process or Collaboration. Financial relevance may justify authorised external receipt without responsibility transfer; exact external recipients remain unresolved.
Assurance recursion	**Resolved boundary:** Governed Assurance does not recursively assure its own execution; execution integrity belongs to the established activity execution framework without solution allocation.

Capability Tier, complete ancestry, contextual-view placement, Feature placement and structural Canonical IDs remain unresolved. Assurance Definition's formal information modelling remains deferred; documenting its conceptual output does not resolve those decisions. These bounded approvals do not complete Business Architecture or freeze/refreeze Domain03.

7. Deliberate Scope Boundaries

7.1 Financial Governance Exclusion

> **Financial Governance Exclusion** — Service Assurance findings may contain information that has financial relevance or may be consumed by external financial-governance activities. Harmonia does not establish a distinct Service Governance (Financials) Role, Function, Process or responsibility because financial governance is outside the presently established Harmonia business scope. Where financially relevant assurance outcomes are produced, they may be communicated to an appropriately authorised external recipient without Harmonia assuming financial-governance responsibility.

This is a deliberate scope exclusion, not an unresolved architectural gap. No Financial Governance Role, Function, Capability, Service, Process or Collaboration is created for symmetry. The allowance for authorised external receipt establishes no additional consumer Role or allocation in the approved Service/Interaction catalogues; exact external recipients and their mandates remain unresolved. Existing business responsibilities are preserved.

7.2 Assurance Non-Recursion Boundary

> **Governed Assurance does not recursively assure its own execution. Assurance activity execution integrity is provided by the established activity execution framework.**

This approved boundary distinguishes assurance execution integrity from independent evaluation of a governed subject. Managed execution/progression of an assurance activity does not make that execution another Service Assurance subject. No Assurance of Assurance, Guardian-of-Guardian, recursive Process, Function, Service or conclusion is introduced. System Steward's progression observation through Service Assurance Status is stewardship / management monitoring, not recursive assurance. The activity execution framework reference is solely a Business Architecture boundary and establishes no application/runtime responsibility allocation or implementation pattern.

8. Bounded Outcome and Navigation

The two assurance Roles and exactly five Functions, three Processes, three Services and three Interactions are established with approved Role relationships and capability, authority, clinical, operational and information boundaries. Further Business elements and detail remain unresolved as recorded above; Financial Governance is deliberately excluded and assurance non-recursion is established. Existing metamodel abbreviations **CT1** / **CT2** / **CT3** / **FT** and **FN** / **SV** / **PR**, established Feature identifiers and the Qualified Architectural Reference Grammar remain unchanged. No unresolved tier, ancestry, Feature placement or structural identifier is filled to accommodate assurance.

This derivation stops in Business Architecture. Information Architecture and all application, runtime, execution, representation, persistence and deployment allocation remain downstream. The completion report is execution evidence and confers no architectural authority.

- Domain03 orientation
- Capability-scoped behaviour index
- Service Assurance Modeller Role
- Service Guardian Role and boundaries
- Collaboration derivation boundary
- Approved assurance Services and consumer concerns
- Approved assurance Interactions
- Approved assurance Processes
- Financial Governance scope exclusion
- Assurance non-recursion boundary

16. Entity Management — Capability-Scoped Behaviour

RAD source: `docs/markdown/03-business-architecture/behaviours/01-entity-management.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Entity Management — Capability-Scoped Behaviour

1. Contextual Scope & Architectural Intent

The named owning Capabilities and established Features retain their responsibilities. Affected Capability Tier, complete ancestry, root status and structural Canonical IDs remain unresolved under approved G1 K9; document grouping and numbering do not establish architectural identity or hierarchy.

Entity Management establishes the canonical master data governance, identity correlation, and structural definitions for all core business entities participating in healthcare delivery: Clients/Patients, Healthcare Practitioners, Healthcare Organisations, Locations, Health Services, and Clinical Devices.

2. Client Administration

Client Administration governs the identity, demographic context, relationship associations, and privacy directives of persons receiving care.

2.1 Person Identity

- **Owning Capability**: `PersonIdentity` (under `ClientAdministration`)
- **Key Architectural Semantics**:
  - **Identifier Resolution**: Asks for authoritative detail and verification regarding a specific person identifier within a single domain namespace.
  - **Identifier Correlation**: Ingests and maintains cross-authority associations between identifier namespaces where the governing relationship is established by authoritative information.
  - **Identity Alias**: Contextual, legal, or alternative identity expressions (e.g., preferred names, maiden names, emergency aliases) by which a person is known. It is conceptually distinct from cross-domain correlation.
  - **R1.x/R2.x authority boundary**: Harmonia ingests and maintains authoritative identifiers, aliases and identity relationships, processes externally authoritative corrections/merge outcomes and propagates managed-state consequences. It does not originate probabilistic matching, golden-record determination, authoritative master-person selection or person-merge decisions. REQ-FND-003 and the Strategy scope statement govern this boundary.
- **Functions & Exposed Services**:
  - **Feature: Identifier Resolution**:
    - *Function*: `ResolvePersonIdentifier` — Validates and retrieves the authoritative demographic record bound to a specific identifier.
    - *Exposed Service*: `PersonIdentifierResolution` — Exposed to Service Administration and Service Delivery to disambiguate client identities.

- **Feature: Cross-Authority Identifier Correlation**:
  - *Function*: `CorrelatePersonIdentifiers` — Correlates local identifiers (e.g., Hospital MRN, National IHI, Pathology ID) using relationships established by authoritative information, retaining their source and namespace.
  - *Exposed Service*: `PersonIdentifierCorrelation` — Exposed across the HIE to establish patient cross-reference mappings.
- **Feature: Identity Alias Association**:
  - *Function*: `MaintainPersonIdentityAliases` — Records and qualifies alias names, titles, and emergency aliases for a governed person identity.
  - *Exposed Service*: *(Internal Function — accessed via Person Identifier Resolution)*.
- **Feature: Governed Identity Correction**:
  - *Function*: `ApplyGovernedPersonIdentityCorrection` — Processes externally authoritative corrections and merge/unlink outcomes, updates governed identity associations and propagates resulting managed-state consequences with audit history. Harmonia does not adjudicate the originating person-merge decision.
  - *Exposed Service*: `PersonIdentityCorrection` — Exposed to authorized HIM/Medical Records administrators.
  - *Governed Process*: **Governed Person Identity Correction Process**.
- **Information Responsibility**: Person Identity Record, Identifier Namespace Bindings, Cross-Authority Correlation Graph, Identity Aliases, Identity Merge/Split Audit Log.

2.2 Healthcare Subject

- **Owning Capability**: `HealthcareSubject`.
- **Owner derivation**: Strategy Healthcare Subject owns Subject Context Binding and Demographic Context Governance; the Functions below discharge those same responsibilities. “Healthcare Subject Context” describes information/behaviour context, not another Capability or alias.
- **Functions & Exposed Services**:
  - **Feature: Subject Context Binding**:
    - *Function*: `EstablishHealthcareSubjectContext` — Binds demographic, cultural, and communication requirements to a patient record.
    - *Exposed Service*: `HealthcareSubjectContextResolution` — Provides comprehensive demographic, language, and indigenous status context to clinical applications.
  - **Feature: Demographic Context Governance**:
    - *Function*: `GovernSubjectDemographicContext` — Applies validation rules to address changes, date of birth verification, and death notifications.
    - *Exposed Service*: `DemographicChangeDistribution` — Emits governed subject demographic updates to subscribed clinical systems.
- **Information Responsibility**: Healthcare Subject Profile, Demographic History, Communication Preferences.

2.3 Client Relationship

- **Owning Capability**: `ClientRelationship`.
- **Owner derivation**: Strategy Client Relationship owns Next-of-Kin & Guardian Association and Relationship Validity Tracking. Support-network wording describes those responsibilities without establishing a separate Capability, alias or specialisation.
- **Functions & Exposed Services**:
  - **Feature: Next-of-Kin & Guardian Association**:
    - *Function*: `MaintainClientRelationships` — Records links between patients and carers, next of kin, nominated representatives, and legal guardians.
    - *Exposed Service*: `ClientRelationshipQuery` — Discloses verified support network members and contact details to clinical teams.

  - **Feature: Relationship Validity Tracking**:
    - *Function*: `GovernClientRelationshipValidity` — Verifies power-of-attorney documents, guardianship orders, and proxy validity periods.
    - *Exposed Service*: `RepresentativeAuthorityVerification` — Confirms legal decision-making authority for substitute consent.
- **Information Responsibility**: Client Relationship Record, Representative Legal Mandate, Carer Contact Directory.

2.4 Client Privacy

- **Owning Capability**: `ClientPrivacy`.
- **Owner derivation**: Strategy Client Privacy owns Consent Directive Evaluation and Confidentiality Flag Enforcement. Consent-directive wording describes that responsibility without creating a separate Capability.
- **Functions & Exposed Services**:
  - **Feature: Consent Directive Evaluation**:
    - *Function*: `EvaluateClientConsent` — Evaluates patient opt-in, opt-out, and general information sharing preferences against proposed exchanges.
    - *Exposed Service*: `ConsentDecisionService` — Provides real-time consent evaluation decisions to Health Information Control.
  - **Feature: Confidentiality Flag Enforcement**:
    - *Function*: `EvaluateClientConfidentialityRestrictions` — Evaluates sensitive record flags (e.g., VIP masking, restricted department exclusions).
    - *Exposed Service*: `ConfidentialityRestrictionEvaluation` — Enforces record-level masking directives across the HIE.
- **Information Responsibility**: Client Consent Directive, Record Masking Rule, Disclosure Restriction Register.

3. Provider Administration

Provider Administration governs individual healthcare practitioners, their professional qualifications, clinical roles, electronic communication endpoints, and organizational affiliations.

- **Owning Capability**: `ProviderAdministration`
- **Key Architectural Semantics**:
  - **Practitioner ≠ Service Provider**: A *Practitioner* is an individual professional. A *Service Provider* is an organisation or facility offering health services. A Practitioner may act in the *Service Provider* role in private practice.
  - **Role and privilege**: Practitioner Role Resolution supplies role/capacity information; it does not establish Clinical Privilege or Operational Privilege. Clinical Privilege Verification retains its clinical scope. An activity may require both distinct privilege determinations; neither follows from Role information alone.
- **Functions & Exposed Services**:
  - **Feature: National Practitioner Verification**:
    - *Function*: `VerifyPractitionerRegistration` — Validates national registration, specialty endorsements, and disciplinary sanctions against regulatory registries (e.g., AHPRA).
    - *Exposed Service*: `PractitionerVerificationService` — Discloses verified registration and status to credentialing and clinical systems.
    - *Governed Process*: **Practitioner Verification Process**.
  - **Feature: Practitioner Profile Governance**:
    - *Function*: `GovernPractitionerProfile` — Maintains official names, professional identifiers (e.g., HPI-I, Prescriber Number), and clinical qualifications.
    - *Exposed Service*: `PractitionerDirectoryQuery` — Provides provider directory lookups across the healthcare network.

- **Feature: Role & Specialty Modeling**:
  - *Function*: `MaintainPractitionerRoles` — Assigns and maintains clinical roles (e.g., Specialist Physician, Registrar, General Practitioner) fulfilled by a practitioner.
  - *Exposed Service*: `PractitionerRoleResolution` — Resolves the specific clinical capacities of a practitioner.
- **Feature: Electronic Endpoint Binding**:
  - *Function*: `BindPractitionerElectronicEndpoints` — Associates secure messaging addresses, direct communication endpoints, and notification preferences to practitioner roles.
  - *Exposed Service*: `PractitionerEndpointLookup` — Provides secure message routing destinations for clinical communication.
- **Feature: Practitioner-Organisation Affiliation**:
  - *Function*: `MaintainPractitionerAffiliations` — Tracks employment, admitting rights, and consulting appointments at specific healthcare facilities.
  - *Exposed Service*: `PractitionerAffiliationResolution` — Discloses organizational ties and practice locations.
- **Feature: Clinical Privileging Record**:
  - *Function*: `MaintainClinicalPrivileges` — Records approved clinical scopes of practice, procedural authorizations, and prescribing privileges.
  - *Exposed Service*: `ClinicalPrivilegeVerification` — Verifies procedural and prescribing authority for clinical ordering systems.
- **Information Responsibility**: Practitioner Master Record, Professional Registration Status, Practitioner Role Bindings, Electronic Communication Endpoints, Scope of Practice Privileges.

4. Organisation Administration

Organisation Administration maintains the authoritative registry of healthcare organisations, networks, directorates, and administrative subdivisions.

- **Owning Capability**: `OrganisationAdministration`
- **Functions & Exposed Services**:
  - **Feature: National Organisation Verification**:
    - *Function*: `VerifyHealthcareOrganisation` — Validates enterprise legal identity, national facility identifiers (e.g., HPI-O), and accreditation status.
    - *Exposed Service*: `OrganisationVerificationService` — Provides verified organisation credentials to external and internal partners.
  - **Feature: Organisation Profile Management**:
    - *Function*: `GovernOrganisationProfile` — Maintains official trading names, enterprise registration numbers, and governing bodies.
    - *Exposed Service*: `OrganisationDirectoryQuery` — Discloses healthcare organisation profiles across the region.
  - **Feature: Hierarchical Structure Navigation**:
    - *Function*: `MaintainOrganisationalStructure` — Models parent-subsidiary relationships, clinical directorates, departments, and operational divisions.
    - *Exposed Service*: `OrganisationalHierarchyResolution` — Resolves department nesting and reporting hierarchies.
  - **Feature: Directory Contact Management**:
    - *Function*: `MaintainOrganisationContacts` — Manages administrative, clinical, and emergency contact details for facilities.
    - *Exposed Service*: `OrganisationContactLookup` — Provides official operational contact details.
- **Information Responsibility**: Healthcare Organisation Registry, National Facility Identifier Bindings, Department Hierarchy Graph, Organisation Contact Directory.

5. Location Administration

Location Administration defines the physical, functional, and geospatial structure of healthcare environments, including sites, buildings, floors, wards, rooms, bays, and bed care-places.

- **Owning Capability**: `LocationAdministration`
- **Key Architectural Semantics**:
  - **Definition vs. Operational State**: *Location Administration* defines what a physical location or care-place **IS** (its static attributes, type, capacity, hierarchy). *Health Service Operations* owns what is **operationally happening** to that care-place (occupancy, cleaning status, isolation locks, turnover).
- **Functions & Exposed Services**:
  - **Feature: Location Geospatial & Address Resolution**:
    - *Function*: `MaintainHealthcareLocation` — Records physical sites, campuses, outpatient clinics, and community health centres.
    - *Exposed Service*: `LocationDirectoryQuery` — Exposes location profiles and geospatial addresses.
  - **Feature: Physical Hierarchy Modeling**:
    - *Function*: `MaintainPhysicalLocationHierarchy` — Models the nested structural containment tree: `Campus` → `Building` → `Floor` → `Ward/Suite` → `Room/Bay` → `BedCare-Place`.
    - *Exposed Service*: `LocationHierarchyResolution` — Discloses physical containment and navigation paths for clinical facilities.
  - **Feature: Atomic Care-Place Modeling**:
    - *Function*: `MaintainCare-PlaceDefinition` — Defines static bed bays, treatment chairs, surgical tables, and emergency resuscitation cubicles, including equipment capabilities and oxygen/telemetry infrastructure.
    - *Exposed Service*: `Care-PlaceSpecificationLookup` — Discloses care-place physical capabilities to Bed & Care-Place Management.
- **Information Responsibility**: Location Master Registry, Physical Facility Hierarchy Tree, Care-Place Structural Definitions.

6. Health Service Administration

Health Service Administration governs the catalogue of clinical services offered across the healthcare network and resolves the critical multi-dimensional mapping between Services, Providers, Locations, and Practitioners.

- **Owning Capability**: `HealthServiceAdministration`
- **Key Architectural Semantics**:
  - **Service / Location / Provider Map**: This capability models the multi-way relationship: Healthcare Service ↔ Service Provider ↔ Healthcare Location ↔ Practitioner Role
  - **Ownership Invariant**: The mapping capability owns the **relationships and association rules**. It does **not** acquire ownership of the Service, Provider, Location, or Practitioner entities being mapped.
  - **Consumer Resolution Scenarios**: Enables clinical and administrative consumers to unambiguously resolve:
    1. *Who provides Service X?*
    2. *Where does Provider Y provide Service X?*
    3. *What Services are delivered at Location Z?*
    4. *Where can Service X be obtained?*
    5. *Which Practitioner Roles participate in Service X?*
- **Functions & Exposed Services**:
  - **Feature: Service Catalogue Publishing**:
    - *Function*: `MaintainHealthcareServiceDefinition` — Defines clinical service offerings (e.g., Cardiology Outpatient Clinic, MRI Imaging, Acute Stroke Care), specialty classifications, and clinical scope.

    - *Exposed Service*: `HealthcareServiceDirectoryQuery` — Exposes published clinical service offerings.
  - **Feature: Service-Location-Provider Binding**:
    - *Function*: `MaintainService/Location/ProviderMap` — Binds health service definitions to delivering organisations, physical clinic locations, and participating practitioner roles.
    - *Exposed Service*: `ServiceProvisionResolution` — Exposes multi-dimensional resolution queries answering who, where, and when healthcare services are delivered.
  - **Feature: Operating Schedule Specification**:
    - *Function*: `MaintainServiceAvailability` — Governs regular operating hours, emergency intake availability, and seasonal clinic schedules.
    - *Exposed Service*: `ServiceAvailabilityQuery` — Discloses operational intake hours and open status.
  - **Feature: Eligibility & Catchment Specification**:
    - *Function*: `MaintainServiceEligibilityRules` — Configures catchment criteria, age thresholds, referral requirements, and clinical indications for service access.
    - *Exposed Service*: `ServiceEligibilityEvaluation` — Evaluates referral eligibility for intake coordinators.
- **Information Responsibility**: Healthcare Service Catalogue, Service / Location / Provider Mapping Matrix, Service Availability Schedules, Service Eligibility Rule Sets.

7. Clinical Device Administration

Clinical Device Administration governs the master definition, instance identification, and electronic communication endpoints of medical equipment and point-of-care appliances.

- **Owning Capability**: `ClinicalDeviceAdministration`
- **Functions & Exposed Services**:
  - **Feature: Device Type Specification**:
    - *Function*: `MaintainDeviceDefinition` — Models medical device models, manufacturers, regulatory classifications (e.g., TGA/FDA classes), and calibration specifications.
    - *Exposed Service*: `DeviceDefinitionQuery` — Discloses approved medical device models and technical parameters.
  - **Feature: Device Serial Tracking**:
    - *Function*: `MaintainDeviceInstanceIdentity` — Tracks physical serial numbers, Unique Device Identifiers (UDI), asset tags, and firmware versions.
    - *Exposed Service*: `DeviceInstanceResolution` — Resolves physical device instances from scanned barcodes or UDIs.
  - **Feature: Device-Patient/Location Assignment**:
    - *Function*: `MaintainDeviceAssociation` — Records active associations between clinical devices, patient encounters, and specific care-places (e.g., Bed 4 Vital Signs Monitor).
    - *Exposed Service*: `DeviceAssociationQuery` — Discloses active device-to-patient and device-to-location bindings.
  - **Feature: Device Endpoint Registration**:
    - *Function*: `MaintainDeviceCommunicationEndpoints` — Manages network telemetry addresses, gateway ports, and protocol bindings for medical devices.
    - *Exposed Service*: `DeviceEndpointResolution` — Exposes network addresses for telemetry routing.
- **Information Responsibility**: Medical Device Type Registry, Physical Device Instance Directory, Device Association Ledger, Device Communication Endpoint Map.

17. Service Administration — Capability-Scoped Behaviour

RAD source: `docs/markdown/03-business-architecture/behaviours/02-service-administration.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Service Administration — Capability-Scoped Behaviour

1. Contextual Scope & Architectural Intent

The named owning Capabilities and established Features retain their responsibilities. Affected Capability Tier, complete ancestry, root status and structural Canonical IDs remain unresolved under approved G1 K9; document grouping and numbering do not establish architectural identity or hierarchy.

Service Administration governs the clinical administrative lifecycles, coordination workflows, and documentation exchanges that surround patient care episodes. It coordinates referrals, appointments, encounters, orders, diagnostic reports, medication events, care plans, and clinical documents across care boundaries.

2. Client-Centred Administration

- **Owning Capability**: `Client-CentredAdministration`
- **Key Architectural Semantics**:
  - **Administrative Coordination**: Handles operational demographic intake and administrative updates across departmental boundaries.
  - **Ownership Invariant**: Does **not** own authoritative person identity or identifier correlation (which is owned by *Client Administration / Person Identity*).
- **Functions & Exposed Services**:
  - **Feature: Demographic Update Ingestion**:
    - *Function*: `ReceiveClientDemographicChange` — Ingests administrative demographic updates from PAS/registration feeds.
    - *Exposed Service*: `DemographicIntakeService` — Exposed to departmental registration systems.
  - **Feature: Demographic Change Distribution**:
    - *Function*: `DistributeGovernedDemographicChange` — Propagates verified demographic modifications to subscribed clinical subsystems.
    - *Exposed Service*: `DemographicUpdateNotification` — Emits change notifications across the HIE.
- **Information Responsibility**: Administrative Intake Ledger, Subscribed Distribution Registry.

3. Referral Administration

- **Owning Capability**: `ReferralAdministration`
- **Key Architectural Semantics**:

  - **Referral vs. Order**: A **Referral** formally requests another care participant or service provider to assess, manage or assume some degree of clinical care responsibility. Intent, disposition, responsibility actually assumed and Transfer of Care are distinct; Referral itself establishes none of acceptance, delivery, assumption or Transfer of Care, and has no universal Appointment, Encounter or Order consequence. An **Order** concerns *bounded fulfilment of a specific diagnostic or therapeutic task*.
- **Functions & Exposed Services**:
  - **Feature: Referral Document Ingestion**:
    - *Function*: `ReceiveReferral` — Validates and ingests structured electronic referrals, clinical indications, and priority categories.
    - *Exposed Service*: `ReferralSubmissionService` — Exposed to external GP practices and referring health facilities.
  - **Feature: Referral Supporting Information**:
    - *Function*: `AssembleReferralContext` — Gathers relevant clinical history, medications, and diagnostic investigations into the referral package.
    - *Exposed Service*: `ReferralContextQuery` — Provides comprehensive referral dossiers to intake triage clinicians.
  - **Feature: Referral Status & Outcome Tracking**:
    - *Function*: `ManageReferralProgression` — Tracks referral disposition in an **illustrative specialist pathway; dispositions vary**: *Submitted* → *Triaged* → *Accepted/Waitlisted* → *Scheduled* → *Discharged*.
    - *Exposed Service*: `ReferralStatus&OutcomeService` — Exposes progression milestones to referrers and patients.
    - *Governed Process*: **Referral Progression Process**.
    - *Scope qualification*: The displayed specialist pathway is illustrative, not mandatory for every Referral; acceptance, waitlisting, decline and redirection retain their contextual meanings.
- **Information Responsibility**: Referral Master Record, Triage Decision Ledger, Referral Disposition Log.

4. Scheduling Administration

- **Owning Capability**: `SchedulingAdministration`
- **Key Architectural Semantics**:
  - **Coordination vs. Authoritative Booking**: Harmonia synchronises and communicates appointment milestones; it does **not** replace the authoritative booking/scheduling engines of host PAS or departmental scheduling platforms.
- **Functions & Exposed Services**:
  - **Feature: Appointment Notification Ingestion**:
    - *Function*: `ReceiveAppointmentNotification` — Ingests appointment creation, rescheduling, cancellation, and check-in events from host PAS engines.
    - *Exposed Service*: `AppointmentNotificationIngress` — Ingests booking events from departmental schedulers.
  - **Feature: Appointment Status Synchronization**:
    - *Function*: `SynchroniseAppointmentStatus` — Projects synchronised appointment timelines to clinical dashboards and care coordinators.
    - *Exposed Service*: `AppointmentScheduleQuery` — Exposes consolidated patient appointment schedules across facilities.
- **Information Responsibility**: Appointment Event History, Schedule Synchronisation State.

5. Episode & Encounter Administration

- **Owning Capability**: `Episode&EncounterAdministration`

- **Key Architectural Semantics**:
  - **Encounter Context**: Tracks the clinical and administrative interaction between a patient and healthcare provider during an emergency, inpatient, or outpatient episode.
- **Functions & Exposed Services**:
  - **Feature: Encounter Context Creation**:
    - *Function*: `EstablishEncounterContext` — Binds patient, attending practitioner, clinical class (e.g., Inpatient, Emergency, Ambulatory), and admitting diagnosis to a unique encounter identifier.
    - *Exposed Service*: `EncounterContextResolution` — Discloses active encounter details to clinical applications.
  - **Feature: Encounter State Progression**:
    - *Function*: `ProgressEncounterState` — Coordinates encounter progression (**scoped illustration; ON_LEAVE relationship unresolved**): *Planned → Arrived → In-Progress → Discharged → Completed*.
    - *Exposed Service*: `EncounterLifecycleEventNotification` — Emits encounter transition events across the HIE.
    - *Governed Process*: **Encounter Lifecycle Process**.
    - *Scope qualification*: Process completion qualifications remain local; their correspondence with Strategy `ON_LEAVE` is unresolved. A shorter summary neither removes established states nor creates universal signing or legal-closure requirements.
  - **Feature: Encounter Bed/Location Association**:
    - *Function*: `MaintainEncounterCare-PlaceAssociation` — Binds the encounter to physical wards, rooms, and beds over time.
    - *Exposed Service*: `EncounterLocationHistoryQuery` — Discloses patient location tracking history.
- **Information Responsibility**: Encounter Master Record, Encounter State Transition Log, Encounter Care-Place Movement Ledger.

6. Order Administration

- **Owning Capability**: `OrderAdministration`
- **Key Architectural Semantics**:
  - **Closed-Loop Progression**: Tracks clinical orders through closed-loop progression. Diagnostic, pathology, imaging and procedural examples illustrate scoped behaviour and do not exclude medication orders from Order Administration responsibility.
  - **Order ≠ Referral**: Closed-loop execution of requested investigations without transferring longitudinal care responsibility.
- **Functions & Exposed Services**:
  - **Feature: Order Request Ingestion**:
    - *Function*: `ReceiveOrderRequest` — Ingests structured orders containing clinical indication, requesting clinician, and test specifications.
    - *Exposed Service*: `OrderRequisitionIngress` — Ingests electronic orders from clinical ordering systems.
  - **Feature: Order Destination Resolution & Routing**:
    - *Function*: `ResolveOrderDestination` — Evaluates order routing rules to determine target laboratory, imaging centre, or procedural unit.
    - *Exposed Service*: `OrderDispatchService` — Transmits orders to performing diagnostic systems.
  - **Feature: Order Closed-Loop Progression Tracking**:
    - *Function*: `ManageOrderProgression` — Tracks order state (**scoped diagnostic/procedural illustration**): *Placed → Received → Specimen Collected → In-Progress → Preliminary Result → Final Result → Closed*.
    - *Exposed Service*: `OrderStatus&TrackingQuery` — Provides real-time order tracking to ordering clinicians.

    - *Governed Process*: **Closed-Loop Order Progression Process**.
    - *Scope qualification*: Specimen, investigation and result checkpoints describe the illustrated diagnostic/procedural progression. Dispatch, receipt and acceptance remain distinct; Behaviour `Received` is not equated to a Process checkpoint without evidence. Cancellation/modification responsibility remains valid even where absent from that sequence.
  - **Feature: Order Cancellation & Modification Coordination**:
    - *Function*: `CoordinateOrderModification/Cancellation` — Handles requests to amend clinical details or cancel unexecuted orders.
    - *Exposed Service*: `OrderCancellationService` — Dispatches cancel directives to performing systems.
  - **Capability-scoped behaviour: Order Result Association** (Feature identity not established):
    - *Function*: `AssociateOrderOutcome` — Correlates returning diagnostic reports and observations back to the originating order requisition.
    - *Exposed Service*: `OrderOutcomeNotification` — Emits completion notices to ordering practitioners.
- **Information Responsibility**: Clinical Order Master Record, Closed-Loop Tracking Ledger, Order-Result Correlation Matrix.

7. Diagnostic Administration

- **Owning Capability**: `DiagnosticAdministration`
- **Key Architectural Semantics**:
  - **Result Management & Distribution**: Manages the correlation, notification, and delivery of pathology, radiology, and point-of-care results.
  - **Ownership Guardrail**: Does **not** own the clinical diagnostic interpretation or specimen analysis (which is owned by the performing diagnostic service provider/LIS).
- **Functions & Exposed Services**:
  - **Feature: Diagnostic Request Correlation**:
    - *Function*: `CorrelateDiagnosticRequest` — Matches incoming lab/radiology results to active patient encounters and order requisitions.
    - *Exposed Service*: `DiagnosticCorrelationResolution` — Discloses correlated order/result linkages.
  - **Feature: Diagnostic Report Ingestion & Binding**:
    - *Function*: `BindDiagnosticReport` — Associates structured observations, reference ranges, and abnormal flags with patient health records.
    - *Exposed Service*: `DiagnosticReportIngress` — Ingests results from external LIS/RIS systems.
  - **Feature: Diagnostic Report Distribution**:
    - *Function*: `DistributeDiagnosticReport` — Delivers preliminary, final, and corrected reports to ordering and copied practitioners.
    - *Feature scope qualification*: Strategy `FEAT-SA-17–DiagnosticReportDistribution` retains its finalised-report responsibility. Preliminary, corrected and addendum/correction handling belongs to the broader Diagnostic Administration responsibility; it does not redefine that narrower Feature or confer diagnostic interpretation authority.
    - *Exposed Service*: `DiagnosticResultDeliveryService` — Emits secure result notifications.
- **Information Responsibility**: Diagnostic Request-Report Linkage Registry, Result Distribution Ledger.

8. Medication Administration

- **Owning Capability**: `MedicationAdministration`

- **Key Architectural Semantics**:
  - **Event Tracking**: Aggregates medication orders, pharmacy dispense records, and administration-event information into a coherent timeline; bedside events are one supported context.
  - **Non-Practice Guardrail**: Harmonia tracks and communicates medication events; it does **not** prescribe drugs, dispense pharmaceuticals, or perform clinical administration.
- **Functions & Exposed Services**:
  - **Feature: Medication Order Ingestion**:
    - *Function*: `ReceiveMedicationOrder` — Ingests inpatient and outpatient drug prescriptions.
    - *Exposed Service*: `MedicationOrderIngress` — Ingests prescription events.
  - **Feature: Dispense Event Tracking**:
    - *Function*: `TrackDispenseEvent` — Ingests community and hospital pharmacy dispensing records.
    - *Exposed Service*: `DispenseEventIngress` — Ingests pharmacy supply notifications.
  - **Feature: Medication Administration Record (MAR) Ingestion**:
    - *Function*: `ReceiveMedicationAdministrationEvent` — Records medication administration-event information, including nurse-administered doses, e-MAR documentation and infusion telemetry as non-exhaustive examples; scope is not limited to nursing, bedside or inpatient settings.
    - *Exposed Service*: `MedicationAdministrationIngress` — Ingests administration records.
- **Information Responsibility**: Medication Event Timeline, Prescription-Dispense-Administration Correlation Matrix.

9. Procedure Administration

- **Owning Capability**: `ProcedureAdministration`
- **Scope qualification**: Procedure information extends beyond surgery; Theatre retains its distinct surgical operational responsibility.
- **Functions & Exposed Services**:
  - **Feature: Procedure Booking Ingestion**:
    - *Function*: `ReceiveProcedureBooking/Request` — Ingests procedural and surgical requisitions.
    - *Exposed Service*: `ProcedureBookingIngress` — Receives surgical/procedural requests.
  - **Feature: Procedural Documentation Ingestion**:
    - *Function*: `ReceiveProceduralDocumentation` — Ingests operation reports, anaesthetic records, and post-procedure notes.
    - *Exposed Service*: `ProceduralDocumentationIngress` — Ingests procedural documentation, including surgical notes, into patient record streams.
- **Information Responsibility**: Procedure Request Register, Procedural Documentation Index.

10. Clinical Record Administration

- **Owning Capability**: `ClinicalRecordAdministration`
- **Key Architectural Semantics**:
  - **Document Governance**: Governs the ingestion, metadata indexing, versioning, addenda, and legal lifecycle of clinical documents (discharge summaries, specialist letters, advance care directives).
- **Functions & Exposed Services**:
  - **Feature: Clinical Document Ingestion**:
    - *Function*: `ReceiveClinicalDocument` — Receives clinical documents from information suppliers.

    - *Exposed Service*: `ClinicalDocumentIngress` — Exposes document submission endpoints across the HIE.
  - **Feature: Document Versioning & Supersession**:
    - *Function*: `GovernClinicalDocumentLifecycle` — Manages document states (**scoped illustration, not a universal transition model**): *Draft* → *Preliminary* → *Final* → *Amended* → *Superseded* → *Entered-in-Error*.
    - *Exposed Service*: `ClinicalDocumentLifecycleService` — Allows authoring clinicians to publish addenda or corrections.
    - *Governed Process*: **Clinical Document Lifecycle Process**.
    - *Scope qualification*: `Final≠automaticallyFinalSigned`. Authorship, attestation, approval, authentication, verification, signature, finalisation, authority and legal qualification remain distinct. Process signing/countersigning requirements retain their local scope, and amendment/supersession/error states are not mandatory for every document. Ingestion and consumer processing do not define originating validity, authority, authorship or legal status.
  - **Feature: Document Metadata Indexing**:
    - *Function*: `IndexClinicalDocumentMetadata` — Indexes author, specialty, encounter, date, and document type (LOINC/SNOMED).
    - *Exposed Service*: `DocumentMetadataRegistryQuery` — Discloses document registry metadata to clinical viewers.
- **Information Responsibility**: Clinical Document Registry, Document Version History, Document Lifecycle State Register.

11. Care Plan Administration

- **Owning Capability**: `CarePlanAdministration`
- **Functions & Exposed Services**:
  - **Feature: Care Plan Ingestion & Distribution**:
    - *Function*: `ReceiveCarePlan` — Ingests multidisciplinary care plans, clinical goals, and patient action steps.
    - *Exposed Service*: `CarePlanIngress` — Ingests shared care plans.
  - **Capability-scoped behaviour: Care Plan Context Association** (Feature identity not established):
    - *Function*: `AssociateCarePlanContext` — Links care plans to active diagnoses, health problems, and care team members.
    - *Exposed Service*: `CarePlanContextQuery` — Discloses active care plans to treating clinicians.
  - **Capability-scoped behaviour: Care Plan Update Distribution** (Feature identity not established):
    - *Function*: `DistributeCarePlanChange` — Broadcasts care plan updates to all enrolled care team participants.
    - *Exposed Service*: `CarePlanChangeNotification` — Emits care plan revision notices.
- **Information Responsibility**: Shared Care Plan Registry, Clinical Goal Ledger, Care Plan Revision History.

12. Clinical Communication Administration

- **Owning Capability**: `ClinicalCommunicationAdministration`
- **Key Architectural Semantics**:
  - **Technical vs. Business Acknowledgement**:
    - **Technical ACK**: May establish durable receiving-boundary acceptance into an integration queue; it does not establish business acceptance.

    - **Business Delivery ACK**: Records contextual recipient-reported facts or dispositions, which may include receipt, parsing, acceptance, rejection, filing or another explicit disposition. These are not a universal progression; business-acceptance and filing criteria remain unresolved.
- **Functions & Exposed Services**:
  - **Feature: Clinical Communication Distribution**:
    - *Function*: `DistributeClinicalCommunication` — Routes secure messages, referral letters, and critical alerts to practitioner endpoints.
    - *Exposed Service*: `SecureClinicalMessageDispatch` — Exposes message dispatch to clinical authors.
  - **Feature: Delivery Acknowledgement Tracking**:
    - *Function*: `TrackBusinessDeliveryAcknowledgement` — Tracks end-to-end delivery state and records business-level delivery/rejection acknowledgements.
    - *Receipt qualification*: Delivery/read evidence does not establish clinical review, agreement, approval or incorporation. Clinical Review itself does not automatically establish incorporation; no universal Clinical Incorporation concept is established.
    - *Exposed Service*: `CommunicationDeliveryStatusQuery` — Discloses delivery confirmation to message senders.
  - **Feature: Communication Audit Logging**:
    - *Function*: `RecordCommunicationTransaction` — Maintains an immutable audit trail of clinical message dispatch and receipt.
    - *Exposed Service*: `CommunicationAuditQuery` — Discloses transmission history for medico-legal verification.
- **Information Responsibility**: Clinical Message Dispatch Log, Business Delivery Acknowledgement Ledger, Medico-Legal Transmission Audit Register.
- **Authority boundary**: Acknowledgement, receipt, persistence, filing, indexing, assembly and review do not transfer originating authority or establish finality. Communication audit evidence concerns transmission; it does not establish the clinical content's authority or legal qualification.

18. Service Delivery — Capability-Scoped Behaviour

RAD source: `docs/markdown/03-business-architecture/behaviours/03-service-delivery.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Service Delivery — Capability-Scoped Behaviour

1. Contextual Scope & Architectural Intent

The named owning Capabilities and established Features retain their responsibilities. Affected Capability Tier, complete ancestry, root status and structural Canonical IDs remain unresolved under approved G1 K9; document grouping and numbering do not establish architectural identity or hierarchy.

Service Delivery represents the clinical operating contexts in which direct patient care is delivered across primary, acute, emergency, inpatient, diagnostic, and virtual settings.

The Fundamental Care Enablement Principle

> **Harmonia enables, informs, and coordinates healthcare delivery across care boundaries; it does not practice medicine, prescribe pharmaceuticals, make autonomous clinical decisions, or replace clinician judgment.**

Service Delivery capabilities within Harmonia define **integration enablement and clinical context correlation behaviour**. They reuse and consume core capabilities (*Patient Clinical Record, Health Information Exchange, Health Information Control, Episode & Encounter Administration, Clinical Record Administration*) rather than duplicating their responsibilities.

Residual Feature Sufficiency

The 2026-10-09 human adjudications apply the Business composition and clinical-work boundaries. General clinical-information capture, processing, preservation and distribution are expressed by Patient Clinical Record and Health Information Exchange, with applicable Service Administration responsibility. A clinical setting does not establish specialised capture or clinical-work ownership.

Strategy Feature	Adjudicated Business expression and boundary
`FEAT-SD-03–AcuteClinicalStateEventCapture`	Acute deterioration/status information is managed through general clinical-information responsibilities. No Acute-specific capture Function is required. The Strategy Feature's acute-specific framing is over-specialised if interpreted as a separate Harmonia responsibility; its existing name/definition are retained for bounded Strategy review, not silently redefined. Alert routing below remains distinct from capture.

Strategy Feature	Adjudicated Business expression and boundary
`FEAT-SD-08–InpatientClinicalProgressionTracking`	Relevant milestone information uses the same general responsibilities. Operational consequences may inform patient flow, capacity or discharge coordination; those consumers do not establish the originating clinical fact. An allied-health clearance may affect discharge readiness without becoming an operationally authored clearance. No Inpatient-specific clinical-tracking Function or EMR workflow is required. Dossier assembly below remains distinct.
`FEAT-SD-14–ScreeningRecallNotificationDistribution`	Recall is a domain-specific composition of clinical-information management, Calendar Management, Workflow & Activity Coordination and information communication/distribution, with applicable entity coordination. Configured workflow/Praxis may realise it without a screening-specific recall Function. Eligibility projection alone remains distinct from distribution. Exact workflow and provider contracts are downstream, not established by this sufficiency decision.

These are composition-level sufficiency decisions, not replacement Feature associations to the Functions below. The same reusable responsibilities can support follow-up, vaccination, diagnostic or review reminders without a new Business Function for each clinical purpose. Population/cohort identification may precede individual activity. Once activity concerns a specific managed patient, the entity-specific Digital Twin coordination rule applies; it establishes no Domain03 Twin Actor, Process or execution mechanism.

2. Clinical Care Enablement Contexts

RAD source listing (presentation only; no inferred ancestry).

Service Delivery Contexts

Primary Care

Acute Care

Emergency Care

Inpatient Care

Diagnostic Services

Medication Therapy

Preventive Care

Community Care

Outreach Care

Remote & Virtual Care

2.1 Primary Care

- **Owning Capability**: `PrimaryCare`
- **Clinical Operating Context**: General practice clinics, community medical centres, and family medicine practices providing longitudinal whole-person care.
- **Enabling Functions & Exposed Services**:
  - **Feature: General Practice Shared Record Ingestion**:
    - *Function*: `CorrelatePrimaryCareConsultationContext` — Binds GP consultation notes, vital signs, and encounter episodes to the regional Longitudinal Health Record (LHR).
    - *Exposed Service*: `PrimaryCareSummaryIngress` — Ingests GP event summaries into the HIE.
  - **Capability-scoped Functions / Services**:

  - *Feature association not established*: Strategy `FEAT-SD-02–PrimaryCareNotificationDispatch` remains valid, but semantic evidence does not establish that the following Function or Service belongs to, realises, specialises or is equivalent to it. No replacement Feature is assigned.
  - *Function*: `CoordinateMulti-AgencyChronicCarePlan` — Enables GPs to track multi-provider disease management milestones (e.g., Diabetes or Cardiovascular Care Plans).
  - *Exposed Service*: `ChronicCareTimelineQuery` — Exposes aggregated multidisciplinary care timelines.
- **Typed architectural dependencies**:
  - **Service consumption**: `PersonIdentifierResolution`; `LongitudinalClinicalRecordQuery` from Patient Clinical Record, already established in the consumption matrix.
  - **Capability dependency**: `CarePlanAdministration`.
  - **Unresolved reference**: "Referral Submission" — an exact Service identity/consuming relationship is not established here; the abbreviated wording is not an alias.

2.2 Acute Care

- **Owning Capability**: `AcuteCare`
- **Clinical Operating Context**: Hospital specialty wards, surgical suites, and intensive care units managing severe, urgent, or post-operative conditions.
- **Enabling Functions & Exposed Services**:
  - **Feature: Acute Care Clinical Context Provision**:
    - *Function*: `AssembleAcuteClinicalBaseline` — Rapidly aggregates previous discharge summaries, active medications, allergies, and diagnostic alerts upon acute hospital presentation.
    - *Exposed Service*: `AcuteAdmissionDossierQuery` — Delivers a high-priority clinical briefing to admitting teams.
  - **Capability-scoped Functions / Services**:
    - *Feature association not established*: `FEAT-SD-03–AcuteClinicalStateEventCapture` is adjudicated sufficient through general clinical-information responsibilities above. Routing a critical-result alert remains distinct from capture; no specialised capture Function or replacement association is assigned.
    - *Function*: `RouteAcuteCriticalResultAlert` — Immediately directs urgent lab/radiology alerts (e.g., critical troponin, intracranial bleed) to active acute attending clinicians.
    - *Exposed Service*: `AcuteClinicalAlertDispatch` — Dispatches urgent interruptive alerts to on-duty ward teams.
- **Typed architectural dependencies**:
  - **Service consumption**: `EncounterContextResolution`.
  - **Capability dependency**: `HealthInformationControl`; break-glass access describes the required governed context, not an independently allocated Service.
  - **Unresolved reference**: "Diagnostic Result Delivery" — intended exact Service/relationship remains unestablished; no alias or replacement edge is inferred.

2.3 Emergency Care

- **Owning Capability**: `EmergencyCare`
- **Clinical Operating Context**: Emergency Departments (ED), trauma centres, and urgent care clinics operating under time-critical, high-acuity constraints.
- **Enabling Functions & Exposed Services**:
  - **Feature: Emergency Department Arrival & Triage Ingestion**:
    - *Function*: `BindEDTriageAssessment` — Captures ATS (Australasian Triage Scale) category, presenting complaint, and ambulance paramedic handover data.
    - *Exposed Service*: `EDTriageStreamIngress` — Ingests emergency triage presentations.

    - **Feature: Emergency Care Cross-Facility Correlation**:
        - *Function*: `ProvideEmergencyLHRSummary` — Delivers rapid, override-capable access to Advance Care Directives, allergy records, and recent hospital admissions.
        - *Exposed Service*: `EmergencyFast-PathLHRQuery` — Provides low-latency emergency record summaries.
- **Typed architectural dependencies**:
    - **Service consumption**: `PersonIdentifierResolution`. "Fast-Match" does not establish a separate Service or probabilistic/master-person matching behaviour.
    - **Capability dependency**: `ClientPrivacy` for its established consent/privacy responsibility; "Break-Glass Override" does not allocate override authority or another element.
    - **Unresolved relationship to an established Feature**: `FEAT-SA-09—EncounterStateProgression`. A precise Service-consumption relationship is not established by naming that Feature.

2.4 Inpatient Care

- **Owning Capability**: `InpatientCare`
- **Clinical Operating Context**: Inpatient hospital wards, rehabilitation units, and sub-acute facilities managing multi-day hospital stays.
- **Enabling Functions & Exposed Services**:
    - **Feature: Inpatient Rounding Context Provision**:
        - *Function*: `ConsolidateInpatientClinicalRoundFeed` — Aggregates daily vital signs, overnight pathology, allied health notes, and medication administration records.
        - *Exposed Service*: `WardRoundClinicalBriefingQuery` — Discloses consolidated 24-hour clinical briefing views.
    - **Capability-scoped Functions / Services**:
        - *Feature association not established*: `FEAT-SD-08—InpatientClinicalProgressionTracking` is adjudicated sufficient through general clinical-information responsibilities above. Discharge-dossier assembly/publication remains distinct; no separate clinical-tracking Function or replacement association is assigned.
        - *Function*: `AssembleInpatientDischargeDossier` — Synthesises inpatient diagnoses, procedural summaries, reconciled discharge medications, and follow-up instructions into a canonical discharge summary package.
        - *Exposed Service*: `DischargeDossierPackagingService` — Emits canonical discharge summaries to GPs and community providers.
- **Typed architectural dependencies**:
    - **Capability dependency**: `DischargeManagement`.
    - **Unresolved references / relationships**: "Encounter Care-Place Movement," "Clinical Document Lifecycle," "Medication Administration Event Ingestion." Their intended element types and exact consumption/participation relationships are not established by these descriptions; no Function, Process or Service equivalence is inferred.

2.5 Diagnostic Services

- **Owning Capability**: `DiagnosticServices`
- **Clinical Operating Context**: Clinical pathology laboratories, diagnostic imaging networks (radiology/MRI/CT), nuclear medicine, and clinical neurophysiology departments.
- **Enabling Functions & Exposed Services**:
    - **Capability-scoped Functions / Services**:
        - *Feature association not established*: Strategy `FEAT-SD-09—Laboratory&ImagingResultDistribution` remains valid, but semantic evidence does not establish that the following Function or Service belongs to, realises, specialises or is equivalent to it. No replacement Feature is assigned.

    - *Function*: `ReceiveDiagnosticRequisitionFeed` — Normalises order requisitions from across the health network into modality worklists.
    - *Exposed Service*: `ModalityWorklistDistribution` — Supplies order worklists to diagnostic laboratory/imaging systems.
  - **Capability-scoped Functions / Services**:
    - *Feature association not established*: Strategy `FEAT-SD-10–DiagnosticHistoryConsolidation` remains valid, but semantic evidence does not establish that the following Function or Service belongs to, realises, specialises or is equivalent to it. No replacement Feature is assigned.
    - *Function*: `PublishGovernedDiagnosticReport` — Normalises and distributes structured diagnostic observations, reference ranges, and diagnostic image study links into the LHR.
    - *Exposed Service*: `DiagnosticPublicationService` — Publishes verified diagnostic reports across the enterprise.
- **Capability dependencies**: `OrderAdministration` for closed-loop order context; `InformationDesignGovernance` for governed terminology bindings. These contextual purposes establish neither an additional Feature nor a precise Service-consumption edge.

2.6 Medication Therapy

- **Owning Capability**: `MedicationTherapy`
- **Clinical Operating Context**: Hospital pharmacies, clinical pharmacotherapy review panels, community dispensing networks, and outpatient infusion suites.
- **Enabling Functions & Exposed Services**:
  - **Feature: Medication History Reconciliation Ingestion**:
    - *Function*: `ProjectLongitudinalMedicationHistory` — Aggregates community dispensing, hospital e-MAR administrations, and specialist prescriptions into a unified chronological medication profile.
    - *Exposed Service*: `LongitudinalMedicationHistoryQuery` — Discloses full medication timelines for medication reconciliation.
  - **Capability-scoped Functions / Services**:
    - *Feature association not established*: Strategy `FEAT-SD-12–MedicationAdministrationTracking` remains valid, but semantic evidence does not establish that the following Function or Service belongs to, realises, specialises or is equivalent to it. No replacement Feature is assigned.
    - *Function*: `CorrelateAdverseReaction&AllergyProfile` — Binds confirmed substance allergies, adverse drug reactions, and severity grades to the patient identity.
    - *Exposed Service*: `Allergy&AdverseReactionQuery` — Exposes active allergy profiles to clinical decision support systems.
- **Capability dependencies**: `MedicationAdministration`, `PatientClinicalRecord`, `HealthInformationControl`; no specific Service is allocated by this contextual list.

2.7 Preventive Care

- **Owning Capability**: `PreventiveCare`
- **Clinical Operating Context**: Public health screening programmes (breast/bowel/cervical), national immunisation registries, and health promotion initiatives.
- **Enabling Functions & Exposed Services**:
  - **Feature: Immunisation Event Submission**:
    - *Function*: `SyndicateImmunisationRecord` — Records vaccine encounters and syndicates records to national immunisation registers (e.g., AIR).
    - *Exposed Service*: `ImmunisationHistoryQuery` — Discloses complete vaccination history.
  - **Capability-scoped Functions / Services**:

  - *Feature association not established*: `FEAT-SD-14–ScreeningRecallNotificationDistribution` is adjudicated sufficient through reusable composition/workflow above. Projecting/querying eligibility dates remains distinct from distribution; no dedicated recall Function or replacement association is assigned.
  - *Function*: `ProjectScreeningEligibilityTimeline` — Tracks age-based and risk-based screening schedules across primary care.
  - *Exposed Service*: `PreventiveScreeningStatusQuery` — Discloses screening due dates to primary care providers.
- **Typed architectural dependencies**:
  - **Capability dependency**: `HealthInformationExchange` for syndication.
  - **Unresolved reference**: "Healthcare Subject Demographic Context" — exact element/relationship remains unestablished; descriptive context does not create a Capability alias or Service.

2.8 Community Care

- **Owning Capability**: `CommunityCare`
- **Clinical Operating Context**: District nursing, home-based palliative care, community mental health teams, and allied health community outreach.
- **Enabling Functions & Exposed Services**:
  - **Feature: Community Encounter Ingestion**:
    - *Function*: `SynchronizeCommunityVisitRecord` — Ingests clinical notes, wound assessments, and vital signs recorded in community and home settings.
    - *Exposed Service*: `CommunityCareEncounterIngress` — Ingests community care documentation.
  - **Capability-scoped Functions / Services**:
    - *Feature association not established*: Strategy `FEAT-SD-16–CommunityCarePlanSynchronization` remains valid, but semantic evidence does not establish that the following Function or Service belongs to, realises, specialises or is equivalent to it. No replacement Feature is assigned.
    - *Function*: `BindCommunitySupportNetworkContext` — Links community nursing teams with local non-clinical social care and home care packages.
    - *Exposed Service*: `CommunityCareTeamQuery` — Exposes community provider contact rosters.
- **Typed architectural dependencies**:
  - **Capability dependencies**: `ClientRelationship`, `Episode&EncounterAdministration`.
  - **Collaboration reference with unresolved involvement**: `Care-TeamCollaboration` is an established Collaboration; precise participation/involvement is not allocated by the former "Consumes" wording.

2.9 Outreach Care

- **Owning Capability**: `OutreachCare`
- **Clinical Operating Context**: Mobile specialist clinics, rural and remote fly-in fly-out (FIFO) clinical teams, and school health screening programmes.
- **Enabling Functions & Exposed Services**:
  - **Feature: Outreach Visit Context Provision**:
    - *Function*: `ProvideOutreachVisitContext` — Packages and delivers portable clinical summaries and offline-capable care summaries for remote outreach clinics.
    - *Derivation*: `FEAT-SD-17` in Strategy's Outreach Care Continuity establishes this preparation/provision responsibility. Outcome reconciliation below does not supply pre-visit context. This Function remains owned by Outreach Care, uses governed source information without acquiring its originating authority and establishes no copying, storage or offline implementation mechanism.

    - *Exposure boundary*: Remote outreach clinics are the established recipients. No separate named Service, Process, Role allocation or structural ID is introduced merely to mirror the Function.
  - **Feature: Outreach Encounter Outcome Ingestion**:
    - *Function*: `ReconcileOutreachClinicalBatch` — Reconciles and merges clinical documentation recorded during disconnected remote clinic operations.
    - *Exposed Service*: `OutreachBatchReconciliationService` — Reconciles batch clinical encounters upon network reconnection.
- **Typed architectural dependencies**:
  - **Capability dependencies**: `HealthInformationExchange`, `ClinicalRecordAdministration`.
  - **Established Service with unresolved consumption applicability**: `PersonIdentityCorrection` retains Person Identity ownership; how Outreach Care requires that Service is not established here. It cannot authorise originating person-merge decisions.

2.10 Remote & Virtual Care

- **Owning Capability**: `Remote&VirtualCare`
- **Clinical Operating Context**: Telehealth consultations, virtual hospital-in-the-home (vHIT), remote patient monitoring (RPM), and asynchronous specialist e-consults.
- **Enabling Functions & Exposed Services**:
  - **Feature: Virtual Care Session Context Binding**:
    - *Function*: `CorrelateVirtualCareConsultationContext` — Binds video consultation metadata, remote practitioner identity, and shared digital whiteboard artifacts to the encounter.
    - *Exposed Service*: `VirtualEncounterContextResolution` — Binds virtual session identifiers to patient encounters.
  - **Feature: Remote Telemetry Ingestion**:
    - *Function*: `IngestRemotePatientTelemetry` — Ingests continuous or periodic physiological measurements (e.g., blood pressure, pulse oximetry, continuous glucose) from home monitoring devices.
    - *Exposed Service*: `RemoteTelemetryIngress` — Ingests remote patient data streams into the LHR.
- **Capability dependencies**: `ClinicalDeviceAdministration`, `PatientClinicalRecord`, `ClinicalCommunicationAdministration`; no specific Service is allocated by this contextual list.

19. Health Service Operations — Capability-Scoped Behaviour

RAD source: `docs/markdown/03-business-architecture/behaviours/04-health-service-operations.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Health Service Operations — Capability-Scoped Behaviour

1. Contextual Scope & Architectural Intent

The named owning Capabilities and established Features retain their responsibilities. Affected Capability Tier, complete ancestry, root status and structural Canonical IDs remain unresolved under approved G1 K9; document grouping and numbering do not establish architectural identity or hierarchy.

Health Service Operations governs the operational logistics, physical resource management, facility capacity, staff dispatch, and operational movement required to support healthcare delivery across clinical facilities.

Operational Logistics vs. Software Plumbing

> **In Domain 03, "Operations" refers strictly to healthcare facility logistics (e.g., bed turnover, patient portering, theatre case scheduling, clinic queues, dispatch). It does not mean IT middleware monitoring, application deployment, or database administration.**

Where Healthcare Service context materially affects operational meaning, authority, coordination, progression or accountability, keep it identifiable with the activity and relevant information responsibility. Location, Organisation, Practitioner or Role does not imply that service context. This contextual obligation establishes no information representation, identifier or implementation binding.

The bounded coverage additions below derive from the established Strategy HSO Features. They reuse existing compound behaviour where its meaning supports the relationship, or add the minimum directly required behaviour. Additional Functions do not mechanically create Services, Processes, Roles or structural IDs.

Capacity Management Responsibility Boundary

The 2026-10-09 human adjudication establishes **Capacity Management** as Business responsibility for understanding and managing available, committed and utilised capacity in six service-delivery operational contexts. Clinic/practice, ward, theatre, ED, outpatient and mobile service capacity retain their distinct Business meanings. This does not establish a common capacity model, information schema or lifecycle. Generalised service/location/patient/operational-state capture belongs to downstream derivation and is not a Domain03 Function.

Context-specific responsibility establishes/manages service capacity. Resource state contributes to it: an available bed alone does not mean a ward can accept another patient; staffing, demand, acuity and operational constraints may also matter. Service Capacity Management

consumes/coordinates context-specific capacity; Work Allocation & Dispatch and Discharge Management may consume capacity/availability or capacity/progression information without becoming its owners. These are Business responsibility relationships, not newly allocated Services or consumption contracts.

HSO context	Context-specific capacity responsibility
Clinic & Practice Operations	Yes — session/queue behaviour contributes; overall clinic/ practice capacity is expressed separately in §2.1.
Ward Operations	Yes — existing operational coordination is reframed as ward capacity in §2.2.
Theatre Operations	Yes — case progression contributes; theatre capacity is expressed separately in §2.3.
Emergency Department Operations	Yes — existing departmental capacity behaviour is clarified in §2.4.
Outpatient Operations	Yes — existing session utilisation is reframed as outpatient capacity in §2.5.
Bed & Care-Place Management	No — establishes resource state contributing to ward capacity.
Clinical Resource Management	No — establishes equipment state/availability; resource readiness is distinct from service capacity.
Service Capacity Management	No — consumes/coordinates context-specific capacity.
Mobile Staff Management	Yes — presence and visit progression contribute; mobile service capacity is expressed separately in §2.9.
On-Call Management	No — resolves coverage rather than establishing context-specific capacity.
Work Allocation & Dispatch	No — may consume capacity/availability for operational work.
Credential Management	No — verifies operational access.
Patient Transport	No — coordinates transport requests/progression.
Clinical Logistics Coordination	No — coordinates logistics/custody.
Discharge Management	No — may consume capacity/progression for discharge coordination.
Clinical Qualification Management	No — verifies qualifications/competency for applicable dispatch.

Clinical work integration does not confer clinical-work management, clinical handover, clinical task determination, clinical allocation authority or EMR worklist ownership. Capacity and progression consumers retain the originating authority of clinical facts.

2. Facility Operational Units & Queue Coordination

RAD source listing (presentation only; no inferred ancestry).

Health Service Operations Contexts

1. Clinic & Practice Operations
2. Ward Operations
3. Theatre Operations
4. Emergency Department Operations
5. Outpatient Operations
6. Bed & Care-Place Management
7. Clinical Resource Management
8. Service Capacity Management
9. Mobile Staff Management
10. On-Call Management
11. Work Allocation & Dispatch
12. Credential Management

13. Patient Transport
14. Clinical Logistics Coordination
15. Discharge Management
16. Clinical Qualification Management

2.1 Clinic & Practice Operations

- **Owning Capability**: `Clinic&PracticeOperations`
- **Functions & Exposed Services**:
  - **Capability-scoped capacity behaviour**:
    - *Function*: `ManageClinic&PracticeCapacity` — Understands and manages available, committed and utilised clinic/practice capacity in relation to practitioner availability, sessions, rooms, appointments and patient demand.
    - *Responsibility boundary*: Operating-session tracking and individual queue progression contribute to this responsibility; neither alone expresses overall service capacity. Capacity coordination does not own source bookings, clinical decisions or practitioner rostering.
  - **Feature: Clinic Session Tracking**:
    - *Function*: `TrackClinicOperatingSession` — Ingests and tracks clinic operating-session start, delay and completion, distinct from an individual patient's queue progression.
    - *Derivation*: `FEAT-HSO-01` directly establishes this session-level behaviour; Manage Clinic Queue Progression and its Process concern individual patients and do not supply it.
    - *Capacity contribution*: Session start/delay/completion remains distinct observable Business behaviour contributing to clinic/practice capacity; it is not merged into capacity merely for structural neatness.
  - **Feature: Operational Queue Progression**:
    - *Function*: `ManageClinicQueueProgression` — Tracks patient arrival, rooming, consultation in-progress, and check-out states.
    - *Exposed Service*: `ClinicQueueTelemetryService` — Discloses real-time clinic waiting times and queue states.
    - *Governed Process*: **Clinic / Operational Queue Progression Process**.
- **Information Responsibility**: Clinic & Practice Capacity, Clinic Session Progression, Clinic Queue State, Patient Waiting Time Metrics.

2.2 Ward Operations

- **Owning Capability**: `WardOperations`
- **Functions & Exposed Services**:
  - **Capability-scoped capacity behaviour**:
    - *Function*: `ManageWardCapacity` — Coordinates available, committed and utilised ward capacity and its patient-flow implications, retaining acuity distribution, nurse-to-patient ratios and isolation cohorting alongside bed/care-place state, patient demand and operational constraints.
    - *Name Alias*: `CoordinateWardOperationalState` — the same established ward operational responsibility, clarified as Capacity Management; no new Function identity or identifier is allocated.
    - *Exposed Service*: `WardOperationalSummaryQuery` — Discloses ward-level operational telemetry.
    - *Existing Feature contribution*: Ward Patient Flow Coordination concerns ward occupancy, care-place allocation and patient bed status. Those patient-flow responsibilities remain within ward operational coordination; they do not define the whole capacity responsibility.

    - *Capacity boundary*: Bed & Care-Place Management contributes resource state. Ward capacity is assessed in its service context; an available bed alone does not establish capacity to accept a patient. Source staffing and clinical assertions retain their authority.
  - **Feature: Clinical Handover Context Assembly**:
    - *Function*: `AssembleWardHandoverContext` — Collates current ward patient lists, active medical concerns and pending tasks for nursing shift handover.
    - *Derivation*: `FEAT-HSO-04` directly requires this assembly. Ward acuity/cohorting telemetry alone does not provide the stated handover content. Source clinical information retains its originating authority; assembly does not perform clinical review, manage clinical handover, determine pending clinical work or acquire clinical judgement.
- **Information Responsibility**: Ward Capacity, Ward Shift Status Ledger, Acuity Balance Register, Ward Handover Context.

2.3 Theatre Operations

- **Owning Capability**: `TheatreOperations`
- **Functions & Exposed Services**:
  - **Capability-scoped capacity behaviour**:
    - *Function*: `ManageTheatreCapacity` — Understands and manages available, committed and utilised theatre capacity in relation to theatre, team and equipment availability, scheduled cases and case progression.
    - *Responsibility boundary*: Case milestones and targeted resource notifications contribute; they do not by themselves establish available or committed theatre service capacity. Clinical case determination and originating team/equipment information remain separately governed.
  - **Feature: Operating Theatre Case Progression**:
    - *Function*: `ManageTheatreCaseProgression` — Coordinates surgical case milestones: *Case Called → Patient in Anaesthetic Bay → Anaesthesia Commenced → Knife to Skin → Procedure Finished → In PACU/Recovery → Ward Handover Complete*.
    - *Exposed Service*: `TheatreCaseProgressionTelemetry` — Emits real-time surgical milestone updates.
    - *Additional Feature responsibility*: `FEAT-HSO-06–PerioperativeResourceNotification` is realised by dispatching progression-triggered alerts to portering, sterilisation and recovery teams within this same case-progression behaviour/exposure. Milestone observation alone does not establish notification; no new Function or Service is created.
    - *Governed Process*: **Theatre Case Progression Process**.
- **Information Responsibility**: Theatre Capacity, Theatre Slate Register, Perioperative Milestone Log.

2.4 Emergency Department Operations

- **Owning Capability**: `EmergencyDepartmentOperations`
- **Functions & Exposed Services**:
  - **Capability-scoped capacity behaviour**:
    - *Function*: `CoordinateEDDepartmentalCapacity` — Understands and manages available, committed and utilised ED capacity in relation to care-place availability, demand, patient presence/progression, acuity and operational constraints; retains waiting room occupancy, resuscitation bay availability and time-to-triage metrics.
    - *Exposed Service*: `EDOperationalStatusQuery` — Discloses facility ED occupancy and surge level.
    - *Existing Feature scope*: ED Length of Stay Tracking retains the existing wait/progression metrics context; capacity responsibility is broader than that Feature. This clarification does not assert a new whole-Feature realisation relationship.

    - *Additional Feature relationship*: `FEAT-HSO-07–EDBed&BayTracking` is materially realised by this capacity-tracking behaviour for occupancy/status of emergency cubicles, resuscitation bays and short-stay beds. Those care-place states are explicit within the existing Function and its operational status exposure.
    - *Capacity boundary*: Existing departmental capacity responsibility is clarified, not duplicated. Care-place state contributes to service capacity; capacity coordination neither performs clinical triage nor establishes clinical acuity assertions.
- **Information Responsibility**: ED Capacity, ED Operational Dashboard Ledger, Resuscitation Bay Occupancy Matrix.

2.5 Outpatient Operations

- **Owning Capability**: `OutpatientOperations`
- **Functions & Exposed Services**:
  - **Capability-scoped Functions / Services**:
    - *Function*: `ManageOutpatientCapacity` — Understands and manages available, committed and utilised outpatient capacity through clinic/session, practitioner and room availability, appointment demand, patient arrival/waiting and operational progression. Retains specialty room allocation and clinician arrival tracking.
    - *Name Alias*: `CoordinateOutpatientSessionUtilisation` — the existing responsibility reframed as outpatient Capacity Management; no new Function identity or identifier is allocated.
    - *Feature responsibility*: `FEAT-HSO-09–OutpatientArrivalRegistration` is realised within this capacity behaviour by ingesting patient check-in/arrival events and notifying treating clinicians. Patient arrival contributes demand/presence; it does not itself establish capacity. This bounded responsibility does not equate the whole capacity Function with the Feature or create a separate Arrival Registration Function.
    - *Exposed Service*: `AmbulatorySessionStatusQuery` — Exposes clinic room occupancy.
    - *Exposure qualification*: Query behaviour and Outpatient Operations ownership remain established; a meaningful outside consumer/mandate is not identified by the current description. That exposure relationship remains unestablished rather than populated with an inferred Actor, Role or Capability.
    - *Capacity boundary*: Operational coordination does not acquire authoritative booking/rostering or clinical-work management. Room-occupancy query remains narrower than the capacity responsibility; no enlarged exposure or new named Service is inferred.
  - **Feature: Post-Clinic Order Coordination**:
    - *Function*: `CapturePost-ClinicFollow-upRequirements` — Captures follow-up booking and diagnostic-order requirements generated at clinic completion.
    - *Derivation*: `FEAT-HSO-10` directly establishes this capture responsibility. The Clinic queue Process's consultation-concluded checkpoint describes orders placed, not Outpatient Operations' capture of follow-up requirements. Capture does not authorise clinical orders, own authoritative bookings or establish a new Service dependency.
- **Information Responsibility**: Outpatient Capacity, Outpatient Arrival/Waiting Demand, Outpatient Session Schedule, Room Allocation Register, Post-Clinic Follow-up Requirements.

2.6 Bed & Care-Place Management

- **Owning Capability**: `Bed&Care-PlaceManagement`
- **Key Architectural Semantics**:
  - **Operational State vs. Definition**: While *Location Administration* defines the physical care-place, *Bed & Care-Place Management* owns the real-time operational status (*Available*, *Occupied*, *Reserved*, *Blocked*, *Dirty*, *Cleaning In-Progress*, *Maintenance Lock*).
  - **Resource State vs. Capacity**: This state contributes to Ward Operations' capacity responsibility. Bed availability does not establish ward service capacity or authority to accept another patient.

- **Functions & Exposed Services**:
  - **Feature: Bed Availability Tracking**:
    - *Function*: `MaintainOperationalBedState` — Tracks real-time bed status and environmental locks.
    - *Exposed Service*: `BedAvailabilityQuery` — Discloses real-time bed availability across hospital directorates.
  - **Feature: Bed State Progression**:
    - *Function*: `ManageBedTurnoverProgression` — Coordinates cleaning, disinfection, and preparation: *Vacated* → *Cleaning Requested* → *Cleaning In-Progress* → *Inspected/Ready* → *Occupied*.
    - *Exposed Service*: `BedTurnoverLifecycleService` — Coordinates turnover requests with environmental services.
    - *Additional Feature responsibility*: `FEAT-HSO-13–BedReadinessNotification` requires the existing turnover behaviour/exposure to notify bed allocation managers and clinical units when a cleaned bed is ready. Readiness notification remains distinct from availability query and from physical placement; no new lifecycle state or Service is introduced.
    - *Governed Process*: **Bed Turnover Process**.
- **Information Responsibility**: Operational Bed Status Registry, Care-Place Lock Register, Bed Turnover Milestone Log.

2.7 Clinical Resource Management

- **Owning Capability**: `ClinicalResourceManagement`
- **Functions & Exposed Services**:
  - **Feature: Mobile Clinical Equipment Tracking**:
    - *Function*: `TrackMobileClinicalAssetOperationalState` — Manages the real-time location and operational readiness of mobile ventilators, infusion pumps, and telemetry transmitters.
    - *Exposed Service*: `ClinicalResourceAvailabilityQuery` — Exposes mobile equipment availability.
- **Information Responsibility**: Mobile Clinical Asset Operational Ledger, Maintenance Readiness Matrix.

2.8 Service Capacity Management

- **Owning Capability**: `ServiceCapacityManagement`
- **Functions & Exposed Services**:
  - **Feature: Operational Capacity Metric Aggregation**:
    - *Function*: `AggregateServiceCapacityTelemetry` — Consumes and coordinates context-specific service-capacity information for broader situational awareness, alongside bed occupancy, ICU surge capacity and ventilator availability across regional hospitals.
    - *Exposed Service*: `RegionalHealthSystemCapacityQuery` — Provides executive operational situational awareness.
    - *Consumer boundary*: Aggregation does not establish the underlying context-specific capacity. Resource occupancy/readiness and service capacity remain distinguishable; exact additional provider contracts are unestablished.
- **Information Responsibility**: Regional System Capacity Snapshot, Surge Level State.

2.9 Mobile Staff Management

- **Owning Capability**: `MobileStaffManagement`

- **Functions & Exposed Services**:
  - **Capability-scoped capacity behaviour**:
    - *Function*: `ManageMobileServiceCapacity` — Understands and manages available, committed and utilised mobile-service capacity in relation to workforce availability, geography, time, workload and applicable service demand.
    - *Responsibility boundary*: Staff presence and externally established assignments/visit progression contribute to capacity; they do not alone express it. Capacity coordination does not determine clinical work, own the source workforce roster or allocate underlying clinical duties.
  - **Capability-scoped Functions / Services**:
    - *Feature association not established*: `FEAT-HSO-16–MobileWorkerTaskDispatch` retains its Strategy identity with the integration boundary below. Presence/zone tracking remains distinct from mobile work-management integration; no replacement association to presence is assigned.
    - *Function*: `TrackOperationalStaffPresence` — Tracks active on-duty presence and physical zone assignment for porters, phlebotomists, and roving nurses.
    - *Exposed Service*: `StaffPresenceTelemetryQuery` — Discloses on-duty staff availability by hospital zone.
  - **Feature: Mobile Visit Status Tracking**:
    - *Function*: `ReceiveMobileVisitProgression` — Ingests arrival, visit-progression and safety check-in milestones from roving staff.
    - *Derivation*: `FEAT-HSO-17` directly requires visit-specific milestone ingestion. It remains distinct from presence, capacity assessment and work-management integration. Received progression can inform mobile capacity without determining clinical work or clinical completion.
  - **Feature: Mobile Worker Task Dispatch** (integration scope qualified in Strategy):
    - *Function*: `IntegrateMobileWorkManagement` — Exchanges externally established work/assignment, dispatch and operational progression/status information, with applicable worker/context information, between mobile work-management mechanisms and Harmonia operational coordination.
    - *Derivation and boundary*: The human adjudication of `FEAT-HSO-16` establishes integration with mobile work/task management. This responsibility is absent from presence observation and visit-specific milestone ingestion. It does not determine, allocate or manage the underlying clinical work, clinical handover or EMR worklists. Work Allocation & Dispatch retains non-clinical operational Work Order responsibility.
    - *Terminology*: External “work/task” vocabulary does not make human work a Harmonia Task. Work Order means human doing; To Do means human review/update/approval; Task means synthetic task. The preferred Strategy name `MobileWorkTaskManagementIntegration` remains a proposed naming correction pending human confirmation, not a new Feature or alias.
- **Information Responsibility**: Mobile Service Capacity, Staff Operational Presence Register, Zone Allocation Matrix, Mobile Visit Progression, Exchanged Mobile Work-Management Information. Originating work/assignment authority remains with its external/business work-management source.

2.10 On-Call Management

- **Owning Capability**: `On-CallManagement`
- **Functions & Exposed Services**:
  - **Feature: Active On-Call Provider Resolution**:
    - *Function*: `ResolveActiveOn-CallCoverage` — Resolves primary, secondary, and tertiary on-call clinical specialists for emergency escalation.
    - *Exposed Service*: `On-CallSpecialistLookup` — Discloses active on-call contacts for switchboard and emergency teams.
- **Information Responsibility**: On-Call Roster Projection, Escalation Contact Matrix.

2.11 Work Allocation & Dispatch

- **Owning Capability**: `WorkAllocation&Dispatch`
- **Key Architectural Semantics**:
  - **Ownership Guardrail**: Owns the dispatching, queuing, assignment, and operational progression of non-clinical work orders (e.g., patient transfers, cleaning, specimen pickups). It does **not** acquire ownership of Practitioners, Locations, Services, or Credentials.
  - **Capacity Consumer**: May use context-specific capacity/availability in operational allocation; it neither establishes that capacity nor acquires clinical-work allocation authority through mobile integration.
- **Functions & Exposed Services**:
  - **Feature: Work Item Instantiation**:
    - *Function*: `DispatchOperationalWorkOrder` — Allocates work requests to available wardspersons or support teams using applicable allocation policies, staff roles, credentials, location and skill, and delivers allocated work to the target worker, device or team queue.
    - *Exposed Service*: `WorkDispatchService` — Delivers allocated work and captures distinct technical and operational acknowledgements; acknowledgement alone does not establish completion.
    - *Additional Feature relationships*: `FEAT-HSO-20–WorkerMatching&Allocation` is realised by the existing allocation behaviour; `FEAT-HSO-21–WorkDispatchDelivery` by its dispatch exposure and acknowledgement capture. Applicable credential/privilege determinations remain separately governed; Role information is not itself privilege.
  - **Feature: Work Progress Oversight & Escalation**:
    - *Function*: `ManageOperationalWorkProgression` — Tracks execution states: *Queued → Assigned → Accepted → In-Progress → Completed / Aborted*.
    - *Exposed Service*: `OperationalWorkStatusQuery` — Discloses task progression to requesting wards.
    - *Governed Process*: **Operational Work Progression Process**.
- **Information Responsibility**: Operational Work Dispatch Ledger, Job Progression Milestone Ledger.

2.12 Credential Management

- **Owning Capability**: `CredentialManagement`
- **Functions & Exposed Services**:
  - **Capability-scoped behaviour: Operational Credential Check** (Feature association not established):
    - *Distinction*: Operational badge/access `CredentialManagement` is distinct from Strategy `ClinicalQualificationManagement`. Lexical similarity does not bind this behaviour to qualification/competency Feature `FEAT-HSO-23`.
    - *Function*: `VerifyOperationalAccessCredential` — Verifies physical access badges, specialty ward entry authorizations, and restricted area permissions.
    - *Exposed Service*: `OperationalCredentialCheckpointService` — Evaluates physical/operational access rights.
- **Information Responsibility**: Operational Access Authorization Ledger, Badge Credential Binding Register.

2.13 Patient Transport

- **Owning Capability**: `PatientTransport`
- **Functions & Exposed Services**:
  - **Feature: Transport Dispatch & Progress Tracking**:

    - *Function*: `ManagePatientTransportProgression` — Coordinates intra-facility and inter-facility patient movements: *Transport Requested → Porter Dispatched → Patient Collected → In-Transit → Delivered at Destination → Handover Completed*.
    - *Exposed Service*: `PatientTransportTrackingService` — Provides real-time transit telemetry.
    - *Governed Process*: **Patient Transport Process**.
    - *Additional Feature responsibility*: `FEAT-HSO-24–TransportRequestIngestion` is directly supported by the request stage of this same Function/Process. It ingests internal portering and external ambulance/inter-hospital requests with clinical mobility requirements; request ingestion is distinct from dispatch, transit and arrival.
- **Information Responsibility**: Patient Transport Request Master, Transit Milestone Log.

2.14 Clinical Logistics Coordination

- **Owning Capability**: `ClinicalLogisticsCoordination`
- **Functions & Exposed Services**:
  - **Feature: Pathology Specimen Transport Tracking**:
    - *Function*: `ManageSpecimenTransportProgression` — Tracks the physical transit of pathology bio-specimens, blood products, and surgical trays: *Collected → Courier Picked Up → In-Transit → Laboratory Ingress Received*.
    - *Exposed Service*: `SpecimenTransitTrackingService` — Exposes chain-of-custody tracking.
    - *Governed Process*: **Specimen Transport Process**.
- **Information Responsibility**: Specimen Courier Manifest, Chain-of-Custody Tracking Register.

2.15 Discharge Management

- **Owning Capability**: `DischargeManagement`
- **Capacity/Progression Consumer**: May use applicable service capacity and patient progression in discharge coordination. It does not establish the consumed capacity or originating clinical clearances; clinical progression, operational readiness, discharge authorisation and physical departure remain distinct.
- **Functions & Exposed Services**:
  - **Feature: Discharge Readiness & Coordination Oversight**:
    - *Function*: `ManageDischargeCoordinationProgression` — Synchronises multi-agency discharge readiness (**illustrative preparation; publication, authorisation and confirmed exit are distinct**): *Discharge Planning Initiated → Medications Reconciled → Transport Booked → Discharge Summary Finalised → Physically Departed*.
    - *Additional Feature relationship*: `FEAT-HSO-27–DischargeMilestoneTracking` is realised by this existing Function/Process's clinical readiness, medications, transport/services and home-care confirmation milestones.
    - *Departure-triggered responsibility*: `FEAT-HSO-29–DischargeNotificationDispatch` requires this behaviour to dispatch departure notifications, updated bed states and finalised discharge summaries to external care providers upon confirmed physical exit. Preparation/publication of applicable information may occur earlier; it does not satisfy this departure-triggered responsibility.
    - *Scope qualification*: Preparation, summary finalisation/signing, availability/publication, applicable discharge authorisation and confirmed physical departure remain distinct. The displayed preparation pathway is illustrative, not a universal state machine. Discharge readiness or available information does not establish discharge authorisation or departure.
    - *Exposed Service*: `DischargeReadinessTelemetry` — Discloses discharge barrier checklists and planned departure times.
    - *Governed Process*: **Discharge Progression Process**.

- **Information Responsibility**: Discharge Readiness Checklist Ledger, Estimated Date of Discharge (EDD) Register.

2.16 Clinical Qualification Management

- **Owning Capability**: `ClinicalQualificationManagement`.
- **Functions**:
  - **Feature: Operational Credential Check**:
    - *Function*: `VerifyClinicalQualificationforWorkDispatch` — Validates that a practitioner possesses active clinical credentials and required competency certificates before dispatch of specialised work.
    - *Derivation*: `FEAT-HS0-23` belongs to Strategy Clinical Qualification Management. Provider Administration establishes professional qualification/registration and privilege information, but the dispatch-specific qualification/competency checkpoint was not expressed by that information management alone.
    - *Boundary*: This checkpoint does not originate qualifications, confer clinical privileges, manage clinical competency training or establish a provider Service dependency. Clinical Privilege and Operational Privilege remain distinct, and an activity may require both. Physical badge/access Credential Management in §2.12 remains independently owned and is not associated with this Feature.
- **Information Responsibility**: Qualification/Competency Checkpoint Outcome and Applicable Work Context; source qualification and privilege information retains its established authority and ownership.

The section number is navigation only. No Capability Tier, ancestry, structural ID, new Role, Service or Process is allocated by this directly derived Function.

20. Intrinsic / Shared Enablement — Capability-Scoped Behaviour

RAD source: `docs/markdown/03-business-architecture/behaviours/05-intrinsic-enablement.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Intrinsic / Shared Enablement — Capability-Scoped Behaviour

1. Contextual Scope & Architectural Intent

The named owning Capabilities and established Features retain their responsibilities. Affected Capability Tier, complete ancestry, root status and structural Canonical IDs remain unresolved under approved G1 K9; document grouping and numbering do not establish architectural identity or hierarchy.

Intrinsic and Shared Enablement encompasses the horizontal platform capabilities that provide foundational security, longitudinal clinical record assembly, information exchange, workflow orchestration, collaborative spaces, and semantic governance across all healthcare contexts.

The Cross-Cutting Boundary Guardrail

> **These capabilities are bounded architectural responsibilities whose services are widely consumed across the HIE. Delivering a shared service (such as security enforcement, message exchange, workflow coordination, or UI presentation) does not grant ownership of the clinical payloads, business activities, or decisions being processed.**

2. Intrinsic Enablement Capabilities

RAD source listing (presentation only; no inferred ancestry).

Intrinsic / Shared Enablement Capabilities

1. Patient Clinical Record
2. Health Information Exchange (HIE)
3. Health Information Access
4. Health Information Communication
5. Health Information Control
6. Clinical Knowledge Services
7. Clinical Collaboration
8. General Collaboration
9. Workflow & Activity Coordination
10. Calendar Management
11. Presentation Services
12. Information Design Governance

2.1 Patient Clinical Record

- **Owning Capability**: `PatientClinicalRecord`
- **Architectural Mandate**: Governs the canonical, longitudinal, cross-institutional health record for every patient across the healthcare region.
- **Functions & Exposed Services**:
  - **Feature: Longitudinal Health Record (LHR) Assembly**:
    - *Function*: `AssembleLongitudinalClinicalRecord` — Synthesises clinical documents, encounters, diagnostic reports, and medication histories across disparate source systems into a unified chronological clinical view.
    - *Exposed Service*: `LongitudinalClinicalRecordQuery` — Discloses the comprehensive clinical timeline to authorised clinicians.
  - **Feature: Active Clinical Record Access**:
    - *Function*: `MaintainActiveClinicalRecord` — Maintains active problem lists, current medication regimens, allergies, and open clinical alerts for active care coordination.
    - *Exposed Service*: `ActiveProblem&AllergySummaryQuery` — Exposes immediate active clinical summaries.
  - **Feature: Durable Clinical Record Preservation**:
    - *Function*: `PreserveDurableClinicalRecord` — Preserves the governed longitudinal representation and durable, immutable, append-only history of what Harmonia received, knew, asserted or managed, with provenance and versioning. Corrections/supersession establish changed knowledge without retrospectively altering that history or acquiring originating clinical authority.
    - *Exposed Service*: `HistoricalClinicalRecordAccess` — Provides governed historical record retrieval for authorised clinical and legal review; preservation does not itself establish legal qualification or originating authority.
- **Information Responsibility**: Governed Longitudinal Health Record (LHR), Canonical Clinical Timeline, Active Problem List, Immutable Historical Record Store.

2.2 Health Information Exchange (HIE)

- **Owning Capability**: `HealthInformationExchange`
- **Architectural Mandate**: Manages the ingestion, transformation, routing, addressed distribution, and publish-subscribe syndication of health information across organizational and jurisdictional boundaries.
- **Ownership Invariant**: Health Information Exchange owns **exchange transaction state, delivery receipts, and routing policies**. It does **not** acquire ownership of the business information transported.
- **Functions & Exposed Services**:
  - **Feature: Direct Submission Ingestion & Delivery**:
    - *Function*: `ReceiveInformationSubmission` — Validates and ingests standards-compliant clinical messages from external health providers.
    - *Exposed Service*: `HIESubmissionGatewayService` — Ingress endpoint for regional data contributions.
  - **Feature: Clinical Record Retrieval Mediation**:
    - *Function*: `MediateInformationRetrieval` — Federates queries across participating hospital and diagnostic node repositories.
    - *Exposed Service*: `FederatedHealthInformationQuery` — Resolves distributed record queries.
  - **Feature: Addressed Clinical Distribution**:
    - *Function*: `DistributeAddressedInformation` — Directs point-to-point clinical documents to specific recipient practitioner endpoints.
    - *Exposed Service*: `AddressedDocumentDeliveryService` — Securely delivers documents to designated recipient mailboxes.
  - **Feature: Event-Driven Syndication**:

    - *Function*: `SyndicateInformationChange` — Broadcasts event notifications and record updates to subscribed regional care network systems.
    - *Exposed Service*: `ClinicalEventSyndicationStream` — Emits publish-subscribe clinical event feeds.
- **Information Responsibility**: Exchange Transaction Ledger, Route Resolution Map, Subscription & Syndication Matrix.

2.3 Health Information Access

- **Owning Capability**: `HealthInformationAccess`
- **Architectural Mandate**: Provides high-performance search, retrieval, and access-qualified filtering of clinical information across the longitudinal record.
- **Ownership Invariant**: Consumes access control and consent determinations from *Health Information Control*. Search behaviour does **not** own security policy.
- **Functions & Exposed Services**:
  - **Feature: Federated Clinical Query**:
    - *Function*: `SearchHealthInformation` — Executes structured semantic searches across diagnostic reports, clinical notes, and medication histories.
    - *Exposed Service*: `ClinicalInformationSearchService` — Discloses search capabilities to clinical user interfaces.
  - **Capability-scoped Functions / Services**:
    - *Feature association not established*: `FEAT-ISE-09–Access-ControlledInformationFiltering` requires access-controlled disclosure/filtering; retrieval of discrete information alone does not materially realise that responsibility. No replacement Feature is assigned.
    - *Function*: `RetrieveHealthInformation` — Fetches discrete clinical observations and original attachments.
    - *Exposed Service*: `ClinicalResourceRetrievalService` — Delivers structured clinical information to requesting applications.
  - **Capability-scoped behaviour: Access-Qualified Result Filtering** (Feature identity not established):
    - *Function*: `ApplyAccess-QualifiedResultFiltering` — Redacts or masks search results and clinical content based on the caller's verified security context and patient consent directives.
    - *Exposed Service*: *(Embedded within Search and Retrieval Services)*.
- **Information Responsibility**: Query Execution Context, Filtered Result Projection State.

2.4 Health Information Communication

- **Owning Capability**: `HealthInformationCommunication`
- **Architectural Mandate**: Encapsulates standards-based health information communication to mediate technical transmission between participating parties.
- **Ownership Invariant**: Does **not** own every business interaction communicated through it; provides protocol adaptation and secure transport mediation.
- **Functions & Exposed Services**:
  - **Feature: Standards-Based Boundary Exchange**:
    - *Function*: `MediateStandards-BasedHealthInformationCommunication` — Handles standards-based communication mediation, transaction coordination, and technical error handling.
    - *Exposed Service*: `StandardsCommunicationGatewayService` — Protocol interface for external client integrations.
- **Information Responsibility**: Transport Protocol Binding Map, Network Session State.

2.5 Health Information Control

- **Owning Capability**: `HealthInformationControl`
- **Architectural Mandate**: Enforces enterprise default-deny authorization, ABAC/RBAC policy evaluation, consent restriction enforcement, immutable security context propagation, and non-repudiation audit logging.
- **Functions & Exposed Services**:
  - **Feature: Health Information Access & Consent Control**:
    - *Function*: `EvaluateInformationAccessAuthority` — Evaluates user role, organisation, patient relationship, purpose of use, and break-glass overrides against default-deny policies.
    - *Exposed Service*: `PolicyEvaluation&AuthorisationService` — Evaluates access requests across the platform.
  - **Capability-scoped Functions / Services**:
    - *Feature association not established*: `FEAT-ISE-12–SecurityContextPropagation` propagates attributable security context; evaluating consent constraints does not materially realise that responsibility. No replacement Feature is assigned.
    - *Function*: `EvaluateConsentConstraint` — Cross-references access requests against client consent directives and sensitive health category restrictions.
    - *Exposed Service*: `ConsentEnforcementService` — Returns masking and redaction instructions.
  - **Capability-scoped behaviour: Security Context Binding** (Feature identity not established):
    - *Function*: `PropagateSecurityContext` — Generates and binds canonical, immutable security contexts to all intra-platform transactions.
    - *Exposed Service*: `SecurityContextValidationService` — Verifies transaction security credentials.
    - *Exposure qualification*: Validation is the declared Service behaviour. Its juxtaposition with Propagate Security Context does not establish a one-to-one Function/Service relationship; the precise internal mapping remains unestablished.
  - **Feature: Non-PHI Compliance Auditing**:
    - *Function*: `RecordSecurity-SignificantActivity` — Records tamper-evident audit evidence (who, what, when, why, patient MRN, policy outcome) without logging unmasked PHI.
    - *Exposed Service*: `SecurityAuditIngress` — Ingests audit events across all capabilities.
- **Information Responsibility**: Authorisation Policy Ruleset, Security Context Tokens, Tamper-Evident Security Audit Trail.

2.6 Clinical Knowledge Services

- **Owning Capability**: `ClinicalKnowledgeServices`
- **Architectural Mandate**: Resolves canonical medical ontologies (SNOMED-CT, LOINC, ICD-10, AMT) and executes cross-terminology concept mappings.
- **Functions & Exposed Services**:
  - **Feature: Clinical Concept Resolution**:
    - *Function*: `ResolveClinicalConcept` — Validates concept identifiers and retrieves preferred clinical rubrics and hierarchy paths.
    - *Exposed Service*: `TerminologyConceptResolutionService` — Exposes concept validation lookups.
  - **Feature: Bidirectional Terminology Mapping**:
    - *Function*: `MapClinicalConcept` — Translates local lab codes and legacy terms to canonical SNOMED/LOINC codes using governed translation maps.
    - *Exposed Service*: `TerminologyMappingService` — Translates codes across terminology systems.

- **Information Responsibility**: Canonical Clinical Ontologies, Semantic Value Sets, Cross-Terminology Mapping Tables.

2.7 Clinical Collaboration

- **Owning Capability**: `ClinicalCollaboration`
- **Architectural Mandate**: Governs real-time multidisciplinary clinical collaboration channels, patient care-team virtual workspaces, and secure clinical discussions.
- **Key Architectural Semantics**:
  - **Discourse vs. Record Guardrail**: Real-time collaborative discussion and messaging in team channels do **not** automatically constitute authoritative clinical health records or observations. Authoritative entries require explicit formal document submission.
- **Functions & Exposed Services**:
  - **Feature: Care-Team Clinical Collaboration Coordination**:
    - *Function*: `CoordinateCare-TeamCollaboration` — Manages care-team communication spaces, participant memberships, and clinical handovers.
    - *Exposed Service*: `ClinicalCollaborationSpaceService` — Manages team collaboration spaces and discussions.
  - **Feature: In-Conversation LHR Query Resolution**:
    - *Function*: `ResolveLHRQuerywithinCollaboration` — Contextually projects active patient summaries directly into collaborative clinical discussion channels.
    - *Exposed Service*: `CollaborationClinicalSummaryResolution` — Embeds clinical summaries in collaboration feeds.
    - *Established ownership*: `ClinicalCollaboration→In-ConversationLHRQueryResolution[FEAT-ISE-17]→ResolveLHRQuerywithinCollaboration→CollaborationClinicalSummaryResolution`.
    - *Unresolved dependency*: The precise source Service/provider, direct versus mediated retrieval, formal consuming Capability/Function for the exposed Service and intended referent of the withdrawn dependency row remain unestablished. Information ownership, Function ownership, Service ownership, consumption, information use and presentation are distinct; no substitute dependency is implied.
  - **Feature: Zero-PHI Collaboration Metadata Governance**:
    - *Function*: `GovernCollaborationMetadata` — Enforces privacy policies, participant access gates, and zero-PHI space metadata rules.
    - *Exposed Service*: `CollaborationSpaceGovernanceService` — Governs space lifecycles and membership.
- **Information Responsibility**: Collaboration Space Metadata, Care-Team Channel Ledger, Discussion Thread Index.

2.8 General Collaboration

- **Owning Capability**: `GeneralCollaboration`
- **Functions & Exposed Services**:
  - **Feature: Operational Team Chat Enablement**:
    - *Function*: `CoordinateOperationalCollaboration` — Provides communication spaces for facility logistics, disaster management teams, and administrative committees.
    - *Exposed Service*: `OperationalCollaborationService` — Provides non-clinical operational messaging channels.
- **Information Responsibility**: Operational Channel Index, Non-Clinical Messaging Log.

2.9 Workflow & Activity Coordination

- **Owning Capability**: `Workflow&ActivityCoordination`

- **Architectural Mandate**: Coordinates execution state, task dispatch, timers, deadlines, and escalations across three distinct activity archetypes:
  1. **Work Order**: Assigned physical/human operational tasks (*human doing* — e.g., transport a patient, clean a bay).
  2. **To Do**: Assigned clinical review or administrative decision tasks (*human reviewing/updating/deciding* — e.g., review abnormal lab result, countersign discharge summary).
  3. **Synthetic Task**: Automated system-executable activities (*non-human executable work* — e.g., generate summary, syndicate batch, evaluate rules).
- **Ownership Invariant**: Workflow & Activity Coordination owns **generic activity coordination semantics and timers**. It does **not** acquire ownership of the business meaning or clinical outcome of the activity.
- **Clinical-work boundary**: Coordination supports externally established work requirements and authorised human review/update/approval. It does not determine clinical tasks, manage clinical handover or own EMR clinical worklists. The clinical-work integration rule preserves clinical authority and the three work-unit meanings; Task remains synthetic work.
- **Outcome uncertainty**: For materially significant managed activity outcomes, missing acknowledgement, response or observation does not establish success or failure. Preserve uncertainty pending authoritative observation/reconciliation. This is a Business obligation, not a universal new lifecycle state; assurance evidence insufficiency remains distinct from execution uncertainty.
- **Functions & Exposed Services**:
  - **Feature: Work Order Progression**:
    - *Function*: `CoordinateWorkOrder` — Coordinates operational task lifecycles and dispatches.
    - *Exposed Service*: `WorkOrderCoordinationService` — Manages operational work progression.
    - *Governed Process*: **Work Order Progression Process**.
  - **Feature: To Do Decision & Approval Coordination**:
    - *Function*: `CoordinateToDo` — Coordinates human review/update/approval items, reminders and sign-off queues within Harmonia's operational responsibility; clinical requirements and decisions remain with accountable clinical authority.
    - *Exposed Service*: `ToDoManagementService` — Provides To Do inbox and sign-off coordination without acquiring underlying clinical-work management.
    - *Governed Process*: **To Do Progression Process**.
  - **Feature: Synthetic Task Orchestration**:
    - *Function*: `CoordinateSyntheticTask` — Orchestrates automated system workflows and operational progression.
    - *Exposed Service*: `SyntheticTaskExecutionService` — Coordinates automated tasks.
    - *Governed Process*: **Synthetic Task Progression Process**.
  - **Feature: Workflow Timeout & Escalation Oversight**:
    - *Function*: `SuperviseActivityTimeout/Escalation` — Monitors SLAs and triggers escalations when clinical or operational deadlines elapse.
    - *Exposed Service*: `ActivityEscalationTelemetry` — Discloses SLA breaches.
- **Information Responsibility**: Generic Activity Instance Registry, Task State Progression Graph, SLA Deadline & Timer Ledger.

2.10 Calendar Management

- **Owning Capability**: `CalendarManagement`
- **Architectural Mandate**: Projects unified chronological timelines across appointments, on-call rosters, and scheduled operational events.
- **Ownership Invariant**: Does **not** acquire authoritative ownership of rosters or appointment bookings (which remain owned by source scheduling and workforce systems).
- **Functions & Exposed Services**:
  - **Feature: Schedule Projection Integration**:

    - *Function*: `ProjectOperationalSchedule` — Aggregates and overlays multidisciplinary schedules onto a unified temporal view.
    - *Exposed Service*: `UnifiedCalendarProjectionQuery` — Discloses consolidated temporal schedules.
  - **Feature: Temporal Event Correlation**:
    - *Function*: `CorrelateActivitywithTemporalContext` — Associates clinical events with shift boundaries and appointment slots.
    - *Exposed Service*: `TemporalContextResolution` — Resolves operational shifts.
- **Information Responsibility**: Projected Unified Timeline, Temporal Correlation Index.

2.11 Presentation Services

- **Owning Capability**: `PresentationServices`
- **Architectural Mandate**: Delivers user-facing portals, clinical dashboards, administrative consoles, and mobile views.
- **Ownership Invariant**: Presentation owns **rendering, user interaction, and layout concerns**. It does **not** own the underlying clinical or operational information being rendered.
- **Functions & Exposed Services**:
  - **Feature: Longitudinal Clinical & Operational Presentation**:
    - *Function*: `PresentContextualHealthandOperationalInformation` — Composes clinical summaries, work queues, and patient records into responsive role-based user interfaces.
    - *Exposed Service*: `ContextualPresentationService` — Delivers contextual health and operational information for presentation.
- **Information Responsibility**: UI View Configuration, User Preference Profile, Presentation Layout Specification.

2.12 Information Design Governance

- **Owning Capability**: `InformationDesignGovernance`
- **Architectural Mandate**: Governs the enterprise canonical information model, data definitions, constraint schemas, terminology bindings, and exchange profiles.
- **Functions & Exposed Services**:
  - **Feature: Information Standards & Semantic Governance**:
    - *Function*: `GovernInformationSemantics` — Authors and validates canonical data concepts and entity relationships.
    - *Exposed Service*: `SemanticModelDefinitionService` — Discloses canonical model definitions.
  - **Capability-scoped behaviour: Information Definition & Schema Governance** (Feature identity not established):
    - *Function*: `GovernInformationDefinition` — Maintains governed data element dictionaries, data types, and structural rules.
    - *Exposed Service*: `InformationDictionaryQuery` — Provides data dictionary specifications.
  - **Capability-scoped behaviour: Terminology Binding Governance** (Feature identity not established):
    - *Function*: `GovernTerminologyBinding` — Binds canonical data elements to authoritative terminology value sets.
    - *Exposed Service*: `TerminologyBindingSpecificationLookup` — Discloses value set bindings.
  - **Capability-scoped behaviour: Exchange Profile Governance** (Feature identity not established):
    - *Function*: `GovernExchangeProfile` — Defines standards-based interchange profiles and message constraints.

    - *Exposed Service*: `ExchangeProfileSpecificationQuery` — Discloses interoperability schemas.
  - **Feature: Semantic Conformance Verification**:
    - *Function*: `VerifySemanticConformance` — Validates candidate messages against canonical governance schemas.
    - *Exposed Service*: `SemanticConformanceVerificationService` — Validates representations for compliance.
- **Information Responsibility**: Enterprise Canonical Semantic Model, Governed Data Dictionary, Terminology Binding Matrix, Exchange Profile Catalogue, Semantic Conformance Ruleset.

21. Canonical Enterprise Capability Model (EC-01 .. EC-14)

RAD source: `docs/markdown/02-strategy/capabilities/enterprise-capabilities.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Canonical Enterprise Capability Model (EC-01 .. EC-14)

Overview & Purpose

The Enterprise Capability Tier represents **reusable ICT architectural functionality** that recurs across multiple distinct healthcare business enabling capabilities and features.

Enterprise Capabilities answer the fundamental architectural question:

> **”How is this function delivered across our systems?”**

Where Business Enabling capabilities are defined within authentic clinical, operational, and administrative healthcare contexts (e.g., Ward Census, Medication Administration, Closed-Loop Orders), Enterprise Capabilities represent the **horizontal, domain-neutral capabilities** through which those healthcare features are delivered.

RAD source listing (presentation only; no inferred ancestry).

> BUSINESS ENABLING FEATURES (Hundreds of domain-specific behaviours)
>
> Example: Patient Bed Turnover, Pathology Order, Practitioner Lookup
>
> derived across features by asking:
>
> ”How is this function delivered?”
>
> ▼
>
> ENTERPRISE CAPABILITY TIER (EC-01 .. EC-14)
>
> 14 Reusable, Technology-Neutral Architectural Capabilities
>
> EC-01 .. EC-13 inform clustering of
>
> ▼
>
> STRATEGIC LOGICAL RESPONSIBILITY MODEL
>
> Mneme, Mnemosyne, Ponos, Pylai, Calliope, Iris, Digital Twin

The component-clustering progression describes established EC-01 through EC-13 derivations only. EC-14 is added by the approved Health Service Assurance Strategy derivation; no strategic logical or application component is allocated EC-14 by this diagram or catalogue.

Derivation Methodology & Architectural Principles

The established EC-01 through EC-13 were derived by systematically analyzing the system-enabled features across all five Business Enabling views. EC-14 Service Guardian is the approved addition derived from REQ-FND-005, BC-18 and the three assurance Business Enabling Capabilities. Their first-pass contribution model records the explicitly approved relationships without inventing intermediate Features or component allocations. The derivation adheres to five strict architectural principles:

1. Vertical Recurrence Test

A functional requirement qualifies as an Enterprise Capability only if it recurs vertically across multiple, diverse healthcare domains.

- If a function appears only within laboratory orders, it remains domain-specific Business Enabling behaviour.
- If a function recurs across clinical document versioning, directory update proposals, and bed state turnover, it reflects an underlying Enterprise Capability (**EC-03 Managed State & Lifecycle**).

2. Behavioural Properties vs. Top-Level Capabilities

Not every recurring function or algorithmic mechanism warrants promotion to a standalone top-level Enterprise Capability:

- **Correlation** is an intrinsic behavioural property appearing across multiple capabilities (e.g., entity correlation in EC-01, event correlation in EC-09, context correlation in EC-02), **not** a standalone `CorrelationService`.
- **Acknowledgement** is a reusable interaction pattern within exchange semantics (EC-08) and operational activity tracking (EC-10), **not** an independent capability.
- **Assignment and Dispatch** are core execution functions within **EC-10 Activity & Execution**.
- **Indexing and Caching** are technical enabling mechanisms within **EC-04 Information Management** and **EC-05 Search & Discovery**, not top-level architectural capabilities.
- **Retry and Circuit Breaking** are resilience properties within **EC-12 Operational Assurance**.

3. Reusable Capability ≠ Centralised Service

> **An Enterprise Capability describes reusable architectural functionality; it does NOT imply a monolithic, centralized runtime service.**

Enterprise Capabilities represent bounded reusable responsibilities. Their realisation may require participation, consumption or enforcement across multiple architectural elements, but this does not imply shared semantic ownership or prohibit establishment of a responsibility centre.

Distributed participation does not imply distributed responsibility. Cross-cutting concern does not imply cross-cutting responsibility.

Examples of participation in established reusable responsibilities include:

- **EC-02 Context Management**: Context is established at ingress (Pylai), maintained during active access (Mneme), propagated across execution units (Ponos), and preserved in audit (Kleio) where required. These participation points do not require a monolithic Context Service or establish shared semantic ownership.
- **EC-06 Policy & Control**: Policy evaluation guards ingress boundaries, persistence interfaces and presentation gateways while retaining bounded policy responsibility.
- **EC-07 Provenance & Traceability**: Required attribution and meaningful evidence are preserved at significant boundaries and transformations; routine technical activity does not automatically require durable evidence.
- **EC-12 Operational Assurance**: Resilience, duplicate suppression and concurrency integrity may require enforcement across messaging, storage and execution elements without distributing semantic ownership.

Responsibility centres follow established architectural responsibility. No capability is allocated merely to complete a diagram, and distributed participation does not prohibit a responsibility centre.

4. Zero Technology Leakage

Enterprise Capabilities are technology-neutral:

- They are free of concurrency primitives (threads, worker thread pools, execution daemons).
- They are free of message broker constructs (topics, queues, partitions, JMS headers).
- They are free of storage technologies (PostgreSQL, Infinispan, JPA, DDL, table locks).
- They are free of specific software packages (HAPI FHIR, Apache Camel, Netty, Spring Boot, Vue 3).

5. Capabilities vs. Software Components

An Enterprise Capability describes *what* functionality is provided. Subsystems (such as Pylai, Ponos, Mneme, Mnemosyne, Calliope, and Iris) represent *strategic logical component responsibility centres* defined in the existing Strategic Logical Component Responsibility Model. Software components are consumers, aggregators, or providers of capabilities; **a software component name is never a capability definition**.

The 14 Canonical Enterprise Capabilities

RAD source listing (presentation only; no inferred ancestry).

EC-01 Managed Entity & Relationship

EC-02 Context Management

EC-03 Managed State & Lifecycle

EC-04 Information Management

EC-05 Search & Discovery

EC-06 Policy & Control

EC-07 Provenance & Traceability

EC-08 Interoperability & Exchange

EC-09 Event & Subscription

EC-10 Activity & Execution

EC-11 Interaction & Experience

EC-12 Operational Assurance

EC-13 Semantic Governance & Conformance

EC-14 Service Guardian

EC-01: Managed Entity & Relationship

- **Architectural Scope**: Identify, classify, relate, and maintain the structural integrity of managed healthcare and administrative entities throughout the enterprise landscape.
- **Functional Responsibilities**:
  - Uniquely identifying core domain entities (Persons, Practitioners, Roles, Organisations, Locations, Services, Devices).
  - Resolving and maintaining multi-authority identifier systems and aliases without destructive overwriting.
  - Representing and traversing multi-dimensional relationships (affiliations, organizational hierarchies, physical location trees, care team memberships).
  - Maintaining referential integrity across interconnected entity graphs.
- **Anti-Responsibilities**: Does not own physical database relational tables or foreign key constraints (Domain 04/07).

EC-02: Context Management

- **Architectural Scope**: Establish, validate, propagate, and preserve the execution, clinical, security, and administrative context within which information and activities have meaning.
- **Functional Responsibilities**:
  - Binding the subject of care, encounter, ordering provider, and healthcare service context to in-flight messages and transactions.
  - Propagating tamper-evident, attributable security context across asynchronous, multi-hop operational boundaries.
  - Ensuring context preservation during cross-protocol transformations across disparate healthcare exchange formats and protocols.
  - Detecting and rejecting requests where mandatory operational or clinical context is missing or inconsistent.
- **Collaborative Nature**: Bounded reusable context responsibility may be realised through participation across gateways, activity execution and information-access boundaries. Participation does not transfer semantic ownership.

EC-03: Managed State & Lifecycle

- **Architectural Scope**: Govern the current and historical state, effective temporal periods, state transition validations, and lifecycle progression of managed integration artifacts and entities.
- **Functional Responsibilities**:
  - Defining and enforcing valid state transition state machines (e.g., `REQUESTED` → `VALIDATED` → `IN_PROGRESS` → `COMPLETED`).
  - Maintaining temporal validity intervals (effective start and end dates) for relationships, credentials, and states.
  - Supporting non-destructive state progression, preserving historical state versions for longitudinal review.
  - Rejecting illegal or out-of-order state transitions while accommodating idempotent replay.
- **Anti-Responsibilities**: Does not define thread-level mutexes or database transaction isolation levels.

EC-04: Information Management

- **Architectural Scope**: Validate, represent, persist, retrieve, version, and manage governed clinical, administrative, and operational information assets.
- **Functional Responsibilities**:
  - Validating inbound information structures against canonical information constraints.
  - Providing authoritative durable persistence for historical health records and administrative registries.
  - Managing active distributed working memory representations for low-latency retrieval.
  - Governing document versioning, supersession, amendments, and retractions without data loss.
- **Architectural Principle**: Enforces the foundational separation between *active distributed state access* and *authoritative durable state preservation* (Axiom AX-05).

EC-05: Search & Discovery

- **Architectural Scope**: Index, search, resolve, filter, and discover information assets, entities, directory endpoints, and healthcare services across the enterprise.
- **Functional Responsibilities**:
  - Providing multi-criteria indexed search across complex entity graphs (e.g., find active cardiologists at Location X accepting new referrals).

  - Resolving service and practitioner communication endpoints for addressed delivery.
  - Supporting bounded, paginated clinical queries across longitudinal patient records.
  - Applying dynamic security and consent filters to search results prior to egress.
- **Anti-Responsibilities**: Does not dictate specific search engine indexes, inverted index implementations, or SQL query syntaxes.

EC-06: Policy & Control

- **Architectural Scope**: Evaluate and enforce security, consent, privacy, and regulatory policies including authority, access control, and information handling rules.
- **Functional Responsibilities**:
  - Executing impartial, default-deny policy evaluations for all integration transactions.
  - Enforcing Role-Based and Attribute-Based Access Control (RBAC/ABAC) policies.
  - Evaluating patient consent directives and privacy restrictions at boundary and query checkpoints.
  - Enforcing statutory information disclosure boundaries (e.g., mental health, VIP records).
- **Collaborative Nature**: Evaluated across boundary gateways, internal bus dispatch, storage interfaces, and presentation layers.

EC-07: Provenance & Traceability

- **Architectural Scope**: Preserve required origin attribution, meaningful transformation history, significant execution checkpoints, tamper-evident audit evidence and end-to-end traceability. Harmonia SHALL preserve provenance, audit and operational evidence where required by governing obligations, architectural significance, accountability, reconstruction, security or assurance needs. Routine technical activity SHALL NOT automatically require durable evidentiary recording.
- **Functional Responsibilities**:
  - Recording attributable, verifiable origin metadata for information sources, authors, and timestamps.
  - Tracking information-significant transformation history and required mapping checkpoints as information crosses system seams.
  - Capturing non-PHI operational and security audit trails for regulatory compliance (HIPAA, GDPR).
  - Supporting end-to-end transaction tracing across asynchronous processing pipelines.
- **Collaborative Nature**: Participating elements preserve required provenance and audit context according to policy. Telemetry, operational facts, provenance, audit and assurance evidence remain distinct. Being evidence is contextual, not an intrinsic property of every recorded fact; preservation does not establish an assurance conclusion.

EC-08: Interoperability & Exchange

- **Architectural Scope**: Resolve destinations, transform schemas, route, transport, deliver, acknowledge, and coordinate standards-based information exchange across enterprise boundaries.
- **Functional Responsibilities**:
  - Providing multi-protocol boundary adaptation for healthcare standards (HL7 v2, FHIR REST, DICOM, secure messaging).
  - Executing bidirectional structural and semantic transformations between legacy formats and canonical representations.
  - Ensuring reliable addressed delivery and fan-out distribution to external clinical endpoints.
  - Processing and distinguishing technical delivery acknowledgements from business acknowledgements.

- **Boundary Rule**: Egress terminates Harmonia management of the emitted representation (Axiom AX-13).

EC-09: Event & Subscription

- **Architectural Scope**: Publish, ingest, correlate, filter, and syndicate events and subscription-driven changes across the enterprise landscape.
- **Functional Responsibilities**:
  - Ingesting operational and clinical event triggers from external systems and internal state changes.
  - Managing topic- and criteria-based event subscriptions for internal and external consumers.
  - Evaluating dynamic subscription filters to match event payloads with interested subscriber criteria.
  - Delivering asynchronous event notifications reliably to registered subscribers.
- **Decoupling Note**: Defined strictly in terms of event publishing, criteria matching, and notification delivery; entirely independent of broker topics, partitions, or queues.

EC-10: Activity & Execution

- **Architectural Scope**: Define, instantiate, match, assign, dispatch, supervise, and progress coordinated operational and clinical activity units.
- **Functional Responsibilities**:
  - Instantiating operational work items (Work Orders, To Dos, synthetic platform Tasks) in response to triggers.
  - Matching and assigning work units to eligible human or automated performers and roles based on policy rules.
  - Dispatching work units to their assigned performers with acknowledgement according to the applicable interaction contract.
  - Supervising execution progress, tracking timeouts, managing escalations, and handling activity interruptions.
- **Decoupling Note**: Free of worker threads, thread pools, execution daemons, or specific workflow runtime engines.

EC-11: Interaction & Experience

- **Architectural Scope**: Provide contextual interaction between human actors, Harmonia, and enterprise presentation services, including visualization, alerting, and collaboration integration.
- **Functional Responsibilities**:
  - Mediating presentation interactions via dedicated backend-for-frontend gateways decoupled from internal databases.
  - Rendering clinical timelines, provider directory management interfaces, and operational console dashboards.
  - Providing contextual notifications and alerts to human users across web and mobile channels.
  - Projecting longitudinal record context into secure multidisciplinary collaboration spaces.
- **Boundary Note**: Does not absorb the healthcare semantics of Clinical Collaboration or medical documentation. Consumes established identity and security context; does not own authentication, consent, or clinical truth.

EC-12: Operational Assurance

- **Architectural Scope**: Ensure concurrency integrity, system resilience, duplicate detection, exception handling, automated recovery, and operational telemetry across platform operations.
- **Functional Responsibilities**:
  - Enforcing sliding-window duplicate message detection and idempotent suppression.
  - Guaranteeing ingress dual-write safety (durable acceptance before emitting positive application ACK).
  - Managing structured failure recovery, exception escalation, and dead-letter isolation.
  - Emitting platform telemetry, operational health metrics, and queue processing metrics.
- **Collaborative Nature**: Realized collaboratively across boundary gateways, messaging infrastructure, execution workers, and storage layers.

EC-12 / EC-14 Boundary

EC-12 retains the established scope and responsibilities above. It may contribute directly to generic operational/processing assurance, limited to that contribution in the approved assurance matrix. It does not own the general semantics of assurance disposition, assurance-specific criteria applicability, evidentiary association, assessment against assurance criteria, adjudication, or independent assurance findings and conclusions. Those distinguishing responsibilities are introduced through EC-14 Service Guardian. Operational health and business-level processing assurance remain distinct; EC-12 is not expanded to absorb EC-14.

EC-13: Semantic Governance & Conformance

- **Architectural Scope**: Govern canonical models, schemas, terminologies, value sets, mapping tables, constraint rules, and semantic conformance validation across the enterprise.
- **Functional Responsibilities**:
  - Defining and publishing canonical healthcare data models and profile structures.
  - Governing clinical terminology mappings, value sets, and concept lookups (SNOMED CT, LOINC).
  - Validating inbound and outbound message representations against published semantic rules.
  - Governing semantic versioning, deprecation, and backward compatibility across schema revisions.
- **Component Relationship**: Informs the Calliope strategic logical component, but Calliope is the responsibility centre, not the capability definition.

EC-14: Service Guardian

- **Architectural Scope**: Service Guardian provides the reusable capability to independently establish whether governed service behaviour, state or outcomes remain within defined assurance expectations by associating applicable criteria and trustworthy evidence with a governed subject, assessing that evidence, adjudicating the assessment, and establishing reportable assurance findings and conclusions.
- **Functional Responsibilities**:
  - Supplying assurance-specific semantics for assurance disposition and assurance criteria applicability.
  - Associating a governed subject/context with applicable criteria and sufficient trustworthy evidence for a particular assurance purpose.
  - Assessing evidence against applicable criteria and adjudicating assessed detail to establish reportable findings, exceptions and conclusions.
  - Preserving assurance independence, temporal/version-specific evidence and criteria applicability, and explicit inability to establish a conclusion where evidence is insufficient.

- **Collaborative Nature**: Supplies the assurance-specific semantics required to bind existing reusable capabilities into governed assurance. It contributes directly to Assurance Design, Assurance Criteria Management and Governed Assurance; direct contribution is not sole realisation. The approved matrix preserves the contributions of EC-01 through EC-13.
- **Anti-Responsibilities**: Does not own the underlying governed subject, authoritative governance requirements, or source information used as evidence. Does not perform, manage, remediate, assign, delegate or operationally escalate the subject activity. Does not replace System Operations or EC-12 Operational Assurance, and does not provide Clinical Services Delivery Assurance.
- **Allocation Boundary**: No strategic logical component, application component, runtime service or implementation construct is allocated EC-14. The catalogue defines a capability, not a software component or deployment boundary.

Conceptual Behavioural Sequence

RAD source listing (presentation only; no inferred ancestry).

Define Good Behaviour

↓

Gather Evidence

↓

Assess Evidence against Good Behaviour

↓

Adjudicate Assessed Detail

↓

Report

This sequence explains the capability; it is not an implementation workflow specification or a derived Business Process. “Define Good Behaviour” identifies assurance expectations and criteria; it does not transfer ownership of governing requirements to Service Guardian. Source information ownership remains with its responsible capability; assurance owns its contextual evidentiary association.

The Business Enabling assurance principles govern disposition, generic processing assurance, evidence/criteria temporality, assessment/adjudication, independence, failure/recovery and explicit evidence insufficiency. These semantics are not universally reducible to PASS / FAIL / UNKNOWN and do not imply an assurance information model or persistence mechanism.

Summary Matrix: Enterprise Capabilities

ID	Enterprise Capability Name	Architectural Focus & Essence	Key Recurring Functions
EC-01	Managed Entity & Relationship	Entity identification, graph relationships, referential integrity	Identity resolution, relationship navigation, alias linking
EC-02	Context Management	Establishing, validating, and propagating operational context	Context binding, multi-hop propagation, security tokens
EC-03	Managed State & Lifecycle	State machines, temporal validity, lifecycle progression	State validation, effective periods, non-destructive updates

ID	Enterprise Capability Name	Architectural Focus & Essence	Key Recurring Functions
EC-04	Information Management	Validation, durable persistence, active distributed state	Schema validation, durable preservation and recovery, active access
EC-05	Search & Discovery	Indexing, multi-criteria search, endpoint resolution	Indexed query, criteria filtering, endpoint discovery
EC-06	Policy & Control	Impartial default-deny security, access control, consent	RBAC/ABAC evaluation, consent enforcement, VIP gating
EC-07	Provenance & Traceability	Origin attribution, audit evidence, transformation history	Tamper-evident audit logging, source attribution, chain of custody
EC-08	Interoperability & Exchange	Multi-protocol boundary exchange, transport, delivery	Protocol mediation, transformation, routing, delivery ACK
EC-09	Event & Subscription	Event publishing, criteria filtering, notification dispatch	Subscription management, criteria matching, syndication
EC-10	Activity & Execution	Work unit instantiation, assignment, dispatch, supervision	Matching, dispatching, progress tracking, escalation
EC-11	Interaction & Experience	Decoupled presentation, user interaction, dashboards	BEFE mediation, clinical view rendering, alert display
EC-12	Operational Assurance	Resilience, concurrency integrity, duplicate suppression	Dual-write safety, deduplication, recovery, telemetry
EC-13	Semantic Governance & Conformance	Canonical schemas, terminologies, conformance rules	Schema publishing, concept mapping, constraint checking
EC-14	Service Guardian	Independent governed assurance of service behaviour, state and outcomes	Assurance disposition, criteria applicability, evidentiary association, assessment, adjudication, findings and conclusions

Multi-Capability Composition Dynamics

Enterprise Capabilities do not operate in isolation; they compose harmoniously to deliver complex healthcare features. Below are three representative multi-capability compositions demonstrating this derivation logic:

Composition 1: Person Identifier Resolution

- **Healthcare Need**: Resolving multi-authority identifiers (MRN, national IHI, clinic client IDs) for an incoming patient transaction.
- **Composed Enterprise Capabilities**:
  - `EC-01ManagedEntity&Relationship`: Resolves entity models and alias relationships.
  - `EC-05Search&Discovery`: Searches existing identifier registers and resolves matches.
  - `EC-03ManagedState&Lifecycle`: Validates identifier active/superseded status.
  - `EC-07Provenance&Traceability`: Records origin authority, timestamp, and audit trail.
  - `EC-06Policy&Control`: Evaluates requesting system access permissions.
  - `EC-12OperationalAssurance`: Guarantees deterministic matching under concurrent load.

Composition 2: Closed-Loop Order Progression

- **Healthcare Need**: Ingesting a diagnostic order, routing to a laboratory, and tracking execution through to report outcome.
- **Composed Enterprise Capabilities**:
  - `EC-02ContextManagement`: Binds clinical encounter, patient, and ordering provider context.
  - `EC-05Search&Discovery`: Resolves target laboratory endpoint and service capability.
  - `EC-01ManagedEntity&Relationship`: Links order entity to patient, provider, and service.
  - `EC-08Interoperability&Exchange`: Manages boundary protocol delivery and captures technical ACK.
  - `EC-03ManagedState&Lifecycle`: Enforces order state progression (`PLACED` → `ACCEPTED` → `IN_PROGRESS` → `COMPLETED`).
  - `EC-10Activity&Execution`: Instantiates activity units, monitors timeouts, and supervises fulfillment.
  - `EC-07Provenance&Traceability`: Captures end-to-end chain of custody and delivery timestamps.
  - `EC-12OperationalAssurance`: Enforces dual-write safety and duplicate suppression.

Composition 3: Work Allocation & Dispatch

- **Healthcare Need**: Coordinating operational facility tasks (e.g., patient transport, ward bed cleaning).
- **Composed Enterprise Capabilities**:
  - `EC-02ContextManagement`: Binds patient, physical room/bed, and service context.
  - `EC-03ManagedState&Lifecycle`: Progresses work order state (`CREATED` → `DISPATCHED` → `IN_PROGRESS` → `DONE`).
  - `EC-05Search&Discovery`: Discovers available and credentialed staff matching task criteria.
  - `EC-01ManagedEntity&Relationship`: Links assigned worker, patient, location, and equipment.
  - `EC-10Activity&Execution`: Executes allocation logic, worker dispatch, and escalation oversight.
  - `EC-08Interoperability&Exchange`: Delivers work orders to mobile devices or external systems.
  - `EC-12OperationalAssurance`: Prevents duplicate assignment and ensures resilient retry.
  - `EC-07Provenance&Traceability`: Captures worker acceptance and task completion evidence.

Downstream Progression

Enterprise Capabilities represent what reusable functions the platform delivers. The existing Strategic Logical Component Responsibility Model describes the established responsibility compositions, and the Strategic Resources catalogue records resource adjudications. Downstream progression includes:

- **Domains 05–07 (Application, Integration, Technology)**: Implements these capabilities through concrete software packages, integration protocols, and runtime technologies.
- ICT Foundation Lenses: Cross-cutting technical enablement considerations guiding technology realization.
- Capability Tier Progression Model: Detailed vertical derivation rules.

The established component derivations and examples for EC-01 through EC-13 remain unchanged. EC-14 and the three assurance Business Enabling Capabilities stop at the approved Strategy derivation boundary: no allocation to Dokimasia, Ponos, Praxis, Pragma, Digital Twin, Mneme, Mnemosyne, Calliope, Iris, Pylai or any other logical/application construct is established. No downstream Business, Information or solution architecture is derived here.

22. Domain 03 to Domain 04 Traceability Framework

RAD source: `docs/markdown/04-information-architecture/traceability/domain03-traceability.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Domain 03 to Domain 04 Traceability Framework

1. The Four-Stage Derivation Framework

The Information Architecture (Domain 04) is derived systematically from the business semantics established in the Business Architecture (Domain 03).

Information Concepts, Relationships and Assemblies trace through established business responsibility using the following four-stage derivation framework. It does not allocate Capability Tiers or require invented hierarchy, Feature associations or dependencies:

RAD source listing (presentation only; no inferred ancestry).

1. Business Capability / Feature (Domain 02 / Domain 03)

↓ delivers

2. Business Function / Process (Domain 03)

↓ establishes

3. Business Information Responsibility (Domain 03 Matrix)

↓ realised as

4. Domain 04 Information Concepts, Relationships & Assemblies

RAD diagram: textual source notation.

```
graph TD
    CAP["1. Owning Business Capability / Feature<br/>(Domain 02 / Domain 03)"]
    FUNC["2. Business Function / Stateful Process<br/>(Domain 03 Behaviour & Processes)"]
    RESP["3. Business Information Responsibility<br/>(Domain 03 Ownership Matrix)"]
    D04["4. Domain 04 Information Concepts, Relationships & Assemblies<br/>(Entities, Activities, Events,
    ↪  Assertions, Definitions, Assemblies)"]

    CAP -->|"delivers"| FUNC
    FUNC -->|"establishes & governs"| RESP
    RESP -->|"formally realised as"| D04
```

1.1 Traceability Principles

1. **Derivation, Not Redefinition**: Domain 04 does not invent new business capabilities or alter the ownership boundaries established in Domain 03.
2. **Purity of Information Responsibilities**: The Business Information Responsibility matrix (`docs/markdown/03-business-architecture/information-responsibility/information-responsibility.md`) is the authoritative source for architectural information responsibilities. Architectural information responsibility is a capability property and is distinct from the governance role of `InformationSteward`.
3. **Bounded Responsibility Derivation**: Information Concepts for which Harmonia claims architectural responsibility SHALL trace to an owning Domain 03 Information Responsibility. Referenced, consumed, externally authoritative or contextual Information Concepts may be

represented where required to discharge a traced Harmonia responsibility, but SHALL NOT thereby acquire Harmonia ownership or authority.

4. **Truthful Traceability**: Under AX-17, traceability represents established relationships and explicitly preserves unestablished ones. Unresolved Capability Tier, complete ancestry or full Canonical ID does not prevent derivation from an established owning element and responsibility. Candidates remain non-authoritative until accepted.

2. Representative Derivation Examples

The following representative examples demonstrate how Domain 03 capabilities, functions, and information responsibilities realise concrete Domain 04 Information Concepts, Relationships, and Assemblies:

2.1 Representative Example 1: Health Service Administration (Mandatory Reference)

RAD source listing (presentation only; no inferred ancestry).

Health Service Administration (Capability / Feature: Service-Location-Provider Binding)

↓ delivers

Maintain Service / Location / Provider Map (Function)

↓ establishes

Service / Location / Provider Mapping Matrix (Information Responsibility)

↓ realised as

OfferedHealthcareService (Definition)

DeliverableHealthcareService (Contextual Binding)

Service-Provider-Location Relationships & Qualified Containment

↓ references participating entities; does not own their meaning

Service Provider / Healthcare Organisation (Entity; responsibility retained by its established owner)

Healthcare Location (Entity; Location Administration responsibility retained)

RAD diagram: textual source notation.

```
graph TD
    subgraph D03 ["Domain 03: Business Architecture"]
        HSA["Capability: Health Service Administration"]
        FUNC_MAP["Function: Maintain Service / Location / Provider Map"]
        RESP_MAP["Information Responsibility: Service / Location / Provider Mapping Matrix"]
        HSA --> FUNC_MAP --> RESP_MAP
    end

    subgraph D04 ["Domain 04: Information Architecture"]
        OFFERED["OfferedHealthcareService (Definition)"]
        DELIV["DeliverableHealthcareService (Contextual Binding)"]
        ORG["Healthcare Organisation (Entity)"]
        LOC["Healthcare Location (Entity)"]
        REL_MAP["Information Relationships:<br/>• Organisation.delivers(DeliverableHealthcareService)<br/>•
        ↪  DeliverableHealthcareService.locatedAt(HealthcareLocation)<br/>•
        ↪  OfferedHealthcareService.contains(OfferedHealthcareService)"]
    end

    RESP_MAP -.->|"realised as"| OFFERED
    RESP_MAP -.->|"realised as"| DELIV
    RESP_MAP -.->|"references mapped entity; responsibility retained"| ORG
    RESP_MAP -.->|"references mapped entity; responsibility retained"| LOC
    RESP_MAP -.->|"realised as"| REL_MAP
```

Under the Business ownership matrix, Health Service Administration owns the mapping associations; it does not own the mapped Provider, Location or Practitioner entities. The example references their participation without transferring their established information responsibility or originating authority. This clarification adds no Business relationship or replacement service-binding model.

2.2 Representative Example 2: Person Identity Administration

RAD source listing (presentation only; no inferred ancestry).

Person Identity (Capability / Feature: Cross-Authority Identifier Correlation)

↓ delivers

Correlate Person Identifiers (Function)

↓ establishes

Person Identity Record, Identifier Namespace Bindings, Cross-Authority Correlation Graph (Information Responsibility)

↓ realised as

Person (Entity)

Identifier Namespace Binding (Assertion)

Cross-Authority Correlation Link (Relationship with Qualification & Authority)

2.3 Representative Example 3: Order Administration

RAD source listing (presentation only; no inferred ancestry).

Order Administration (Capability)

↓ established Features

FEAT-SA-11 — Order Request Ingestion

FEAT-SA-12 — Order Destination Resolution & Routing

FEAT-SA-13 — Order Closed-Loop Progression Tracking

FEAT-SA-14 — Order Cancellation & Modification Coordination

↓ established Functions / exposed Services

Receive Order Request / Order Requisition Ingress

Resolve Order Destination / Order Dispatch Service

Manage Order Progression / Order Status & Tracking Query

Associate Order Outcome / Order Outcome Notification (Capability-scoped; Feature association not established)

↓ governed Process

Closed-Loop Order Progression Process

↓ establishes

Clinical Order & Closed-Loop Matrix (Information Responsibility)

Supporting: Clinical Order Master Record, Closed-Loop Tracking Ledger, Order-Result Correlation Matrix

↓ realised as

Order (Associated Direction Mechanism)

Order Routing Directive (Assertion / Instruction)

Order-Result Correlation Link (Relationship with Provenance & Status)

This retained Foundation example supplies canonical terminology, not Package2 derivation. A ledger/history label does not establish comprehensive retention; correlation does not establish conflict adjudication. Each Function/Service remains in its evidenced owning context without inferring a one-to-one Feature association.

2.4 Bounded Task / Work Responsibility Basis

The Task / Work model traces to Workflow & Activity Coordination's established Functions (`CoordinateWorkOrder`, `CoordinateToDo`, `CoordinateSyntheticTask`, `SuperviseActivityTimeout/Escalation`) and generic Activity Instance Registry, Task State Progression Graph and SLA Deadline & Timer Ledger information responsibilities. Work Order, To Do and Synthetic Task retain their distinct Business meanings and progression authority.

The authorised Task information concepts distinguish archetype, particular work, undertaking and governed ActionableTask resolution without assuming ownership of business meaning or clinical outcome. This responsibility basis allocates no unestablished definition-authoring/outcome-authority owner or specialised archetype/undertaking/outcome subclasses. ActionableTask governs its orthogonal execution-concurrency, outcome-participation and output-rule controls; that information policy does not transfer originating domain authority. ReportedTask and remaining detailed TaskOutcome questions remain unresolved.

3. Scope Boundary for Detailed Model Population

This foundation establishes the canonical derivation mechanism and validates it through representative references.

> **Scope Note**: Full, detailed derivation across the 137 established Strategy Features and their evidenced owning contexts and the complete Domain 03 Business Information Responsibility matrix remains reserved for subsequent, dedicated Domain 04 modelling packages.

23. Cross-Capability Dependencies & Service Exposure Matrix

RAD source: `docs/markdown/03-business-architecture/dependencies/cross-capability-dependencies.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Cross-Capability Dependencies & Service Exposure Matrix

1. Architectural Principles of Cross-Capability Dependencies

In the Harmonia Business Architecture, capabilities do not operate as isolated silos. Healthcare workflows require coordinated collaboration across bounded capabilities.

1.1 Service Exposure and Ownership Boundary Pattern

When Capability A delivers behaviour needed by Capability B, Capability A exposes that behaviour through a formal **Business Service**. Capability B consumes the Service without acquiring ownership of the underlying Function or information:

RAD diagram: textual source notation.

```
graph LR
    subgraph CapA["Capability A (Owner)"]
        FA["Function X"] -->|Exposed as| SA["Business Service X"]
    end
    subgraph CapB["Capability B (Consumer)"]
        CB["Business Behaviour"] -->|Consumes| SA
    end
```

1.2 High-Level Cross-Capability Dependency Topology

The diagram below illustrates the major inter-capability dependency relationships across the five healthcare contextual views:

These nodes are contextual views, not typed Capabilities or identifier ancestors. The arrows summarise established responsibility needs; they do not allocate particular Services, participating Interactions or Collaborations.

RAD diagram: textual source notation.

```
graph TD
    EM["Entity Management<br/>(Identity, Providers, Orgs, Locations, Services)"]
    SA["Service Administration<br/>(Encounters, Referrals, Orders, Documents)"]
    SD["Service Delivery<br/>(Primary, Acute, Emergency, Inpatient, Virtual)"]
    HSO["Health Service Operations<br/>(Wards, Beds, Dispatch, Logistics, Discharge)"]
    ISE["Intrinsic / Shared Enablement<br/>(LHR, HIE, Access, Control, Workflow, Collab)"]

    SA -->|Resolves entities & services| EM
    SD -->|Associates encounters & orders| SA
    SD -->|Retrieves longitudinal record & exchange| ISE
    HSO -->|Coordinates care-places & staff| EM
    HSO -->|Orchestrates operational workflows| ISE
    HSO -->|Synchronises discharge & admissions| SA
    SA -->|Enforces security context & audit| ISE
```

2. Cross-Capability Service Consumption Matrix

The matrix below documents the primary inter-capability service dependencies across Harmonia:

Consuming Capability Context	Exposed Business Service Consumed	Exposing / Owning Capability	Business Purpose
Referral Administration	`PersonIdentifierResolution`	**Person Identity** *(Client Administration)*	Validates source-attributed identifiers against their authoritative issuing information; does not determine a master person.
Referral Administration	`ServiceProvisionResolution`	**Health Service Administration**	Resolves target clinical specialty clinics, receiving providers, and referral catchment rules.
Episode & Encounter Administration	`Care-PlaceSpecificationLookup`	**Location Administration**	Resolves physical ward, room, and bed structural attributes for encounter bed placement.
Episode & Encounter Administration	`BedAvailabilityQuery`	**Bed & Care-Place Management**	Checks real-time operational bed readiness before confirming patient bed moves.
Order Administration	`ServiceProvisionResolution`	**Health Service Administration**	Determines performing diagnostic laboratory or imaging centre routing destinations.
Diagnostic Administration	`TerminologyMappingService`	**Clinical Knowledge Services**	Normalises local laboratory and radiology test codes to canonical SNOMED-CT/LOINC concepts.
Clinical Record Administration	`HealthcareSubjectContextResolution`	**Healthcare Subject**	Validates patient demographic context and indigenous/interpreter requirements for document headers.
Clinical Record Administration	`SemanticConformanceVerificationService`	**Information Design Governance**	Verifies clinical document structural and semantic compliance against governed interchange profiles.
Primary Care	`LongitudinalClinicalRecordQuery`	**Patient Clinical Record**	Retrieves regional patient history, past discharge summaries, and medication timelines for GP consultations.
Acute Care	`ConsentEnforcementService`	**Health Information Control**	Evaluates patient consent directives and break-glass emergency access permissions during acute admissions.

Consuming Capability Context	Exposed Business Service Consumed	Exposing / Owning Capability	Business Purpose
Emergency Care	`PersonIdentifierCorrelation`	**Person Identity** *(Client Administration)*	Resolves established, source-authoritative identity associations for emergency presentations; does not originate probabilistic matching or person-merge decisions.
Inpatient Care	`DischargeReadinessTelemetry`	**Discharge Management**	Coordinates multidisciplinary ward discharge checklists and pharmacy reconciliation.
Bed & Care-Place Management	`Care-PlaceSpecificationLookup`	**Location Administration**	Resolves physical care-place capabilities (e.g., negative pressure, telemetry wiring) for isolation placement.
Work Allocation & Dispatch	`StaffPresenceTelemetryQuery`	**Mobile Staff Management**	Allocates portering and cleaning work orders to active on-duty staff in the nearest physical zone.
Patient Transport	`EncounterLocationHistoryQuery`	**Episode & Encounter Administration**	Validates current patient ward location and target diagnostic department before dispatching porters.
Clinical Logistics Coordination	`SecureClinicalMessageDispatch`	**Clinical Communication Administration**	Sends automated electronic specimen tracking notifications and urgent critical result dispatch alerts.
Workflow & Activity Coordination	`PolicyEvaluation&AuthorisationService`	**Health Information Control**	Evaluates actor task-claiming permissions before allowing work order acceptance or document countersignature.
Presentation Services	`ClinicalInformationSearchService`	**Health Information Access**	Executes search queries across longitudinal clinical records for display in clinical portals.

3. Dependency Governance Rules

The former Clinical Collaboration consumption of `CollaborationClinicalSummaryResolution` from Patient Clinical Record is withdrawn as unsupported under approved G1 K12. Clinical Collaboration retains ownership of that exposed Service through `FEAT-ISE-17` and `ResolveLHRQuerywithinCollaboration`. Its precise source Service/provider, direct or mediated retrieval, formal Service consumer and the incorrect row's intended referent remain unresolved. No replacement dependency is established.

Patient Clinical Record retains longitudinal assembly, active record maintenance, durable preservation and canonical integrated synthesis; Clinical Collaboration retains spaces, membership, discourse, in-conversation resolution and contextual clinical-summary projection;

Health Information Access retains search, retrieval, access-qualified filtering, query execution context and result projection state. Information ownership, Function ownership, Service ownership, consumption, use and presentation remain distinct.

The Order Administration → Practitioner Role Resolution row is withdrawn: the Service resolves Role/capacity information, not Clinical Privilege or Operational Privilege. The Diagnostic Administration → Order Requisition Ingress row is withdrawn: that Service ingests requisitions, not the asserted correlation of incoming reports. Both Services retain their owners and valid exposed behaviour. Required privilege/correlation behaviour remains valid, but precise consuming Service relationships remain unestablished; no replacement edge is inferred.

Clinical Privilege concerns a practitioner's authority/credentials for applicable clinical activity; Operational Privilege concerns an actor's authority for applicable operational activity. Either or both may be required. Practitioner Role information may contribute context but establishes neither.

The former Discharge Management → Clinical Document Lifecycle Service "publication upon departure" assertion is withdrawn. That Service exposes authoring addenda/corrections; the alleged discharge-publication consumption is not established. Discharge Management retains a capability dependency on Clinical Record Administration for applicable discharge information, with the precise Service/provider contract unresolved. Preparation, information availability/publication, discharge authorisation and physical departure remain distinct. Applicable information may be prepared/communicated before departure; `FEAT-HS0-29` dispatch preserves confirmed physical exit as its trigger. No universal publication sequence, additional publication event or replacement Service edge is inferred.

Typed references distinguish Capability dependency, Service consumption, Interaction participation and Collaboration involvement. Abbreviated/descriptive wording is not an alias. Where identity or relationship is not established, retain the reference as unresolved rather than convert it into an edge.

1. **Unidirectional Service Coupling**: Dependency arrows point strictly from the consuming capability to the exposed Business Service. The exposing capability remains completely agnostic of which downstream capabilities consume its services.
2. **Encapsulated Implementation**: Consuming capabilities depend exclusively upon the abstract service contract and its business semantics, never upon internal algorithms or persistence structures.
3. **Fail-Closed Security Context**: All cross-capability service invocations must carry a valid, immutable `SecurityContext` evaluated by *Health Information Control*.

4. Assurance Dependency Derivation Boundary

The approved Strategy contribution matrix establishes contributions to Assurance Design, Assurance Criteria Management and Governed Assurance. It is not a Business Service consumption matrix: neither Direct nor Supporting identifies an exposed Service, its owning Function or an identifiable Business Service consumer. No assurance row or topology edge is added to §2 or §1.2 from those contributions.

Governed Assurance needs applicable criteria and trustworthy evidence. Source-information ownership and subject management remain with their established capabilities. The subject may supply evidence, but that dependency must not give it control of assurance progression or conclusion: **evidence dependency does not compromise assurance independence; control dependency does**. Service Guardian does not acquire assignment, delegation, escalation or remediation responsibility through a finding. Service Assurance Modeller also does not manage the subject merely by designing assurance concerning it.

The approved EC-02 / EC-14 contribution relationship supports **Establish Assurance Context** through generic context-management capability and assurance-specific semantics together. It does not identify an exposed Business Service, a Service consumer, sole realisation, a technical mechanism or an implementation allocation. The five approved Functions establish internal capability-owned behaviour; they do not mechanically create exposed Services or consumption rows.

The approved human review separately establishes the following three Governed Assurance Services and their Role-level consumption/communication relationships. These are approved relationships across the owning responsibility boundary, not inferred from EC contributions or Role-name similarity. No consuming Capability is allocated solely from a Role's name; these assurance relationships do not determine §2's other Capability rows or §1.2's contextual-view topology.

Approved Business Service	Exposing / Owning Capability	Approved consumer / potential recipient Roles	Associated Business Interaction
Request Service Assurance	**Governed Assurance**	Care Coordinator, Service Coordinator, Regulator, Policy Authority, System Steward — authorised requesters	Service Assurance Request
Request Service Assurance Status	**Governed Assurance**	**System Steward only** — stewardship of activity progression	Service Assurance Status
Communicate Service Assurance Outcome	**Governed Assurance**	Care Coordinator, Service Coordinator, Regulator, Policy Authority, System Steward — applicable authorised recipients	Service Assurance Outcome Communication

Requesting does not grant control of assurance assessment/adjudication or authority to alter the applicable Assurance Definition/Criteria. Status concerns assurance activity progression and does not expose unadjudicated findings/conclusions or provide general requester tracking. Outcome communication is not mandatory broadcast; requester, status consumer and recipient need not coincide. Recipients retain their own clinical, operational, governance/compliance and improvement responsibilities; communication transfers none of their subsequent action to Service Guardian.

Assurance Definition use is a responsibility/information dependency between modelling and execution; no Modeller-to-Guardian Interaction or Service is inferred. Source information may be available through existing governed access; evidence association does not create an Evidence Contribution Interaction. Exact evidence/criteria providers, additional exposed behaviour/Services, detailed contracts, consuming Capability allocation, evidence exchange and findings-to-operational-response mechanisms remain unresolved.

Financial Governance is deliberately excluded, while exact authorised external recipients of financially relevant outcomes remain unresolved. Governed Assurance does not recursively assure its own execution; System Steward's status observation is stewardship / management monitoring. Neither boundary establishes an implementation allocation. Existing Service ownership, consumption relationships, security rules and recorded uncertainties remain intact. Role participation alone does not grant information-access authority or bypass governed access.

24. Domain04 Package 2 — G1 Architectural Review Record

RAD source: `docs/markdown/04-information-architecture/reviews/package2-g1-review.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

11. K9 — Capability Metamodel Typing

Status: RESOLVED — APPROVED: CAPABILITY IDENTITIES AND RESPONSIBILITIES PRESERVED; CAPABILITY TIER / ANCESTRY UNRESOLVED

Disposition: Close K9 because the architectural investigation establishes what the current architecture does and does not determine. Preserve established architectural identities, responsibilities, behavioural ownership contexts and catalogued Feature classifications. Retain unresolved Capability Tiers, ancestry, root status and structural Canonical IDs explicitly. Completion of the full Business Enabling Capability hierarchy is not a prerequisite for Domain04 Package 2 derivation. Source reconciliation remains a later, separately authorised activity.

The approved identification prerequisite in §10, the Capability decomposition / derivation model and the Architectural Element Identification metamodel remain controlling. This final disposition supersedes the prerequisite's incomplete-K9 position without changing its identification rules or retrospectively validating legacy classifications. Approved K1–K8, including K5, remain unchanged.

11.1 Approved findings and explicit uncertainty

Architectural dimension	Established finding	Unresolved boundary
Architectural identity	Business Enabling Capability identities are established for the majority of affected elements.	The architectural-element status of every named catalogue decomposition entry is not established.
Semantic responsibility	The relevant responsibilities of those established Capabilities are preserved. Named responsibility contexts for Functions, Services and Processes are substantially established.	Established responsibility does not prove numerical Capability Tier or complete structural ancestry.
Feature type	All explicitly catalogued Features are FT; existing Feature identifiers are preserved. `IdentifierResolution` is FT / `FEAT-EM-01`.	Unsupported Domain03 Feature headings remain governed by approved K5; presentation alone creates no Feature identity.
Capability Tier	CT1 / CT2 / CT3 are the approved decomposition terminology.	No affected legacy L1 / L2 / L3 owner assignment is independently validated sufficiently to establish a CT1 / CT2 / CT3 assignment.
Capability ancestry and root status	Selected explicit parent/ responsibility relationships are established, including `ClientAdministration→PersonIdentity`.	Complete Capability ancestry and root status for the affected Business Enabling Capabilities remain unresolved. The explicit relationship does not establish either element's tier or Client Administration's root status.

Architectural dimension	Established finding	Unresolved boundary
Structural Canonical ID	The approved identification grammar and established Feature identifiers remain valid.	Complete structural Canonical IDs cannot yet be established where required tier, ancestry or identifier allocation is unresolved.

The Business Enabling Capability catalogue, Domain03 behaviour ownership, Process ownership, Information Responsibility and cross-capability dependencies establish the relevant identities and responsibility contexts within their evidenced boundaries. The explicit `PersonIdentity` parent relationship is stated in Governed Person Identity Correction, §2.1; its disputed L2 label does not establish the tier.

The historical K9 evidence baseline remains evidence of the source assertions requiring reconciliation. Established owner identity, established responsibility and established Feature type SHALL remain distinct from unresolved Capability Tier, ancestry and structural identification. A typing defect SHALL NOT invalidate an otherwise established Capability identity or change its semantic responsibility.

11.2 Legacy classification disposition

No affected legacy L1 / L2 / L3 assignment has been independently validated sufficiently to authorise mechanical translation to CT1 / CT2 / CT3. The investigation categories remain:

Existing assertion	Investigation category	Approved treatment
Affected legacy owner L1 / L2 assignments	C — unsupported / insufficiently evidenced	Preserve the established owning element and responsibility; retain its Capability Tier and complete ancestry as unresolved.
Former `IdentifierResolution` L3 example	D — established Feature incorrectly represented as a Capability Tier	Preserve FT / `FEAT-EM-01`; do not classify it as CT3 or use the former label to establish ancestry.
Former `Closed-LoopOrderProgression` L3 and `DiagnosticOrders` L2 examples	C — unsupported / insufficiently evidenced	The former examples establish no validated Capability Tier or ancestry. Preserve independently evidenced Process responsibility without manufacturing a Capability identity or parent.
Unsupported Domain03 Feature headings	Approved K5 controls	Preserve valid Functions and Services in their evidenced owning context; create no Feature, Feature ID or alias to obtain traceability symmetry.

No affected legacy assignment is established as category A — a semantically validated hierarchy requiring terminology migration alone. Unsupported hierarchy assertions SHALL NOT be treated as mere deprecated wording. `L1→CT1`, `L2→CT2` and `L3→CT3` SHALL NOT be performed as a mechanical migration.

11.3 Downstream derivation rule

> Unresolved Capability Tier or ancestry SHALL NOT prevent downstream architectural derivation where the owning Capability or Feature and its relevant responsibility are otherwise established. Downstream traceability SHALL reference the established architectural element and SHALL NOT manufacture unresolved structural ancestry.

Domain04 Package 2 may therefore derive Information Concepts from established Business Architecture responsibility without waiting for complete CT1 / CT2 / CT3 classification. This permission does not establish missing responsibility or architectural identity where those are themselves unresolved.

Where a full structural Canonical ID cannot yet be established, traceability SHALL use the established architectural type, canonical name and existing identifier where one exists, while

retaining the structural Canonical ID as unresolved. Placeholder structural IDs SHALL NOT be manufactured. An existing Feature identifier SHALL NOT be represented as proof of a complete structural Canonical ID.

11.4 Architectural Element identity and catalogue presentation

> Document structure, catalogue grouping, numbering, indentation and decomposition presentation SHALL NOT by themselves establish Architectural Element identity. Canonical Architectural IDs SHALL be assigned only to governed Architectural Elements whose identity and architectural context are established.

Named catalogue decomposition entries SHALL NOT automatically become CT2 or CT3 Capabilities because they appear beneath another Capability. Their architectural-element status may remain unresolved. Historical L1 / L2 / L3 labels and derivation-stage position likewise SHALL NOT serve as self-validating evidence of ancestry or tier.

The five contextual views — Entity Management, Service Administration, Service Delivery, Health Service Operations and Intrinsic / Shared Enablement — remain organisational views. They SHALL NOT automatically become CT1 elements or identifier ancestors. Established Capability contexts beneath a view do not by themselves prove root CT1 status.

11.5 Behavioural ownership and Feature preservation

Named responsibility contexts for affected Domain03 Functions, Services and Processes are substantially established even though their complete structural paths are not. Those behaviours may continue to trace to their established owning Capability or Feature without a complete structural Canonical ID. The identification prerequisite permits direct anchoring at an established Capability Tier or FT context; missing intermediate Capabilities or Features SHALL NOT be manufactured.

All explicitly catalogued Features SHALL remain FT, and all established Feature identifiers SHALL be preserved. The control case is:

Identifier Resolution property	Approved finding
Type	FT
Existing Feature identifier	`FEAT-EM-01`
Owning context	Person Identity
Complete structural Canonical ID	Unresolved

The explicit `ClientAdministration→PersonIdentity` relationship and Identifier Resolution's owning context establish only a partial responsibility chain. They SHALL NOT supply missing Capability Tier ancestry or a complete structural Canonical ID.

Approved K5 remains controlling for unsupported Feature headings, existing Feature IDs and canonical naming. Valid Functions and Services do not establish a new Feature merely through a heading or similarity to a broader Feature. No FN / SV / PR identifiers are allocated during K9 persistence.

11.6 Clinical Qualification / Credential distinction

`ClinicalQualificationManagement≠CredentialManagement`.

The approved rename `ClinicalCredentialManagement→ClinicalQualificationManagement` preserves professional / clinical qualification and competency responsibility. Domain03 `CredentialManagement` retains its separately evidenced operational badge / physical-access responsibility. The elements SHALL NOT be merged or aliased. Neither element's Capability Tier or ancestry is established by the rename or by lexical similarity.

The lexical similarity of Feature or behaviour names SHALL NOT establish semantic identity. In particular, the badge/access-context `OperationalCredentialCheck` behaviour SHALL NOT be

bound to `FEAT-HS0-23` solely by name. That catalogued Feature's qualification/competency responsibility remains distinct from the operational badge/access behaviour.

11.7 Later, separately authorised G1 reconciliation

The minimum later reconciliation SHALL distinguish:

- **Terminology and unsupported hierarchy assertions:** remove or qualify unsupported numerical Capability Tier assertions in the affected sources; use approved terminology only where tier is independently established. Preserve established Capability identities and responsibilities; otherwise retain tier, root status and complete ancestry as unresolved.
- **Evidenced relationship:** preserve `ClientAdministration→PersonIdentity` without inventing tiers, intermediate parents or root status. Completion of the hierarchy may occur through later Business Architecture refinement and is not required to close K9.
- **Feature correction:** preserve catalogued FT identities and existing Feature IDs; correct the former Identifier Resolution Capability Tier representation; apply approved K5 to unsupported Feature headings without manufacturing Features or identifiers.
- **Canonical naming correction:** apply approved K5 naming corrections, including Service Delivery naming and Clinical Logistics Coordination, and the approved Clinical Qualification Management rename. Preserve the distinct operational Credential Management element and its responsibility.
- **Downstream traceability correction:** reconcile affected Package 1 traceability labels against established architectural types and owning responsibilities, retaining unresolved structural context explicitly. Preserve the separately approved K3/K6 constraints; do not change Function, Service, Process, Information Responsibility or dependency semantics to repair a type label.
- **Historical evidence and identification:** preserve the evidence baseline, including its historical quotations and fingerprints; allocate no speculative structural Canonical IDs. Catalogue presentation SHALL NOT create governed Architectural Elements or substitute for established context.

Approval of this disposition and authorisation to perform source reconciliation remain separate. This persistence task changes only this durable G1 review record; frozen Domain01–03, Domain04 Foundation and Package 1 sources remain unchanged.

11.8 Closure and stop boundary

2026-10-13 reconciliation: The fixed-decomposition decision changes the target metamodel, not K9's approved evidence, closure or unresolved catalogue status. Complete CT1 → CT2 → CT3 ancestry is now required for a fully reconciled Feature. Existing Features remain valid evidence requiring ancestry reconciliation where their CT3 frame is unestablished. Legacy labels, document structure, contextual views, local Feature IDs and partial ownership SHALL NOT establish missing tiers, complete Canonical IDs or new ancestors. K9's permission to derive from established responsibility remains; it does not declare that responsibility structurally reconciled. The original closure text follows unchanged.

K9 is RESOLVED on the approved interpretation above. Unresolved Capability Tier, ancestry, root status, decomposition-entry identity and structural Canonical ID information remain explicit architectural uncertainties; they do not reopen K9 or require completion of the full Business Enabling Capability hierarchy before downstream derivation from established responsibility.

This disposition preserves AX-01/AX-04 business meaning, AX-14 semantic distinctions and AX-15 uncertainty. It neither changes the frozen Capability Model nor authorises missing identity, ancestry or identifier allocation to be invented. It preserves the approved identification prerequisite and K1–K8 unchanged, completes no G1 reconciliation, derives no Package 2 Information Concept and does not investigate K10 or approve G1 as a whole. Work stops after persistence of this approved K9 disposition and review of the resulting diff.

25. Harmonia Architectural Axioms

RAD source: `docs/markdown/governance/architectural-axioms.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

AX-01 — Harmonia Is Health-Information Centric

Axiom

Harmonia is a distributed health-information management and interoperability framework.

Its primary purpose is the ingestion, validation, transformation, governance, processing, persistence, distribution and exposure of health-related information.

Harmonia SHALL support applicable health-information standards and models while retaining the ability to manage information according to Harmonia's own internal operational requirements.

Rationale

Harmonia is not intended to become a generic distributed application framework to which healthcare happens to be attached.

Its architecture exists to solve health-information problems: interoperability, information authority, provenance, security, terminology, workflow, persistence, distribution, resilience and longitudinal information management.

Consequences

Health-information semantics take precedence over generic infrastructure convenience.

Standards such as HL7 FHIR and HL7 v2 are first-class architectural concerns.

Generic infrastructure capabilities SHOULD be used where useful, but SHALL NOT dictate Harmonia's health-information semantics.

This does not mean

- Harmonia is a FHIR server.
- Harmonia is restricted to FHIR information.
- Every internal Harmonia object must correspond to a healthcare interoperability standard.

AX-04 — Harmonia Owns the Semantics; Engines Provide the Machinery

Axiom

Harmonia owns the architectural semantics governing information authority, security, lifecycle, concurrency, provenance, auditability, resilience and operational integrity.

Harmonia SHALL preferentially use capabilities supplied by underlying technology engines where those capabilities satisfy Harmonia's architectural invariants rather than reproduce equivalent functionality.

Rationale

Harmonia should own its meaning without unnecessarily owning commodity infrastructure machinery.

Consequences

HAPI FHIR MAY provide FHIR persistence, indexing, versioning and search.

Infinispan MAY provide distributed caching, entry metadata and atomic coordination primitives.

ActiveMQ Artemis MAY provide durable messaging.

PostgreSQL MAY provide transactional relational persistence.

These technologies implement Harmonia capabilities. They do not define Harmonia's architectural semantics.

Maxim

Use the machinery. Own the semantics.

This does not mean:

- Harmonia should wrap every engine capability behind a bespoke implementation.
- An engine's native behaviour automatically satisfies a Harmonia invariant.
- Using HAPI FHIR makes Harmonia a HAPI FHIR application.

AX-14 — Semantic Distinctions Are Preserved

Axiom

Harmonia SHALL preserve meaningful distinctions between states, outcomes and concepts throughout its internal processing and across subsystem boundaries.

A distinction that is significant to information meaning, authority, security, concurrency, persistence, processing outcome or operational correctness SHALL NOT be collapsed merely because an underlying technology, transport, API or implementation abstraction does not represent that distinction directly.

Where an external interface contract intentionally presents a simpler or different semantic model, Harmonia MAY project its internal semantics into that contract explicitly at the applicable boundary.

Rationale

Loss of semantic distinction causes information about what Harmonia knows, what occurred and what may safely happen next to be lost.

Two conditions may produce similar implementation behaviour while having materially different meanings. Resource absence, access denial, persistence failure and indeterminate outcome may all prevent information from being returned, but they do not describe the same state.

Likewise, persistence version, active-state generation, interoperability version and information authority describe different properties even where an implementation happens to correlate them.

Implementation convenience SHALL NOT redefine those meanings.

Consequences

Semantically distinct outcomes SHALL remain distinguishable through Harmonia's internal contracts.

Security denial SHALL NOT be represented as resource absence merely because both prevent access to information.

A failure to establish authoritative state SHALL NOT be represented as authoritative absence.

An indeterminate outcome SHALL NOT be represented as either success or failure unless subsequent processing establishes that outcome.

ActiveStateToken, AuthoritativeVersion, externally meaningful version identifiers and standards-defined version metadata SHALL remain distinct where they represent different semantic domains.

Where an underlying engine or protocol lacks a distinction required by Harmonia, Harmonia SHALL introduce an appropriate semantic representation rather than infer meaning from incidental implementation details such as exception classes, message text or correlated version values.

Boundary projection MAY deliberately reduce or transform semantic distinctions where required by an external contract or policy, but such projection SHALL be explicit and SHALL NOT redefine Harmonia's internal semantics.

Maxim

Preserve semantics internally; project deliberately at boundaries.

This does not mean: Harmonia must expose every internal distinction externally. Similar outcomes can never share implementation machinery. Every semantic distinction requires a separate Java type. External standards or interface contracts may be ignored in favour of Harmonia's internal model.

AX-17 — Architectural Authority and Explicit Uncertainty

Axiom

Documented architectural decisions, definitions, relationships, boundaries and constraints are authoritative and SHALL NOT be silently reinterpreted, replaced, bypassed or contradicted by downstream architecture or implementation.

Where the architecture does not establish a fact, relationship, responsibility or decision, that absence SHALL be preserved explicitly rather than completed through inference, convention, structural convenience, lexical similarity or anticipated implementation.

Unresolved architecture MAY be accompanied by clearly identified candidates, suggestions or recommendations, but these SHALL remain explicitly non-authoritative until explicitly accepted into the architecture.

Where evidence indicates that authoritative architecture may be incorrect, incomplete or internally inconsistent, the conflict SHALL be raised explicitly for architectural review. The existing architecture remains authoritative until an approved architectural change is made. That approved change becomes the new authoritative architecture.

Rationale

Downstream derivation must distinguish established architectural knowledge from gaps and proposed resolutions. A plausible relationship supplies no authority merely because it makes a model or implementation appear complete. Absence of an established relationship is architecturally meaningful information.

Repeated architectural review during Domain04 Package2 G1 confirmed this need across different traceability relationships:

- K9: unresolved Capability hierarchy must not be completed through invented ancestry.
- K10: Behaviour / Process traceability must not be completed through invented stage equivalence.
- K11: Function / Service traceability must not be completed through unsupported Feature association.
- K12: cross-capability dependency traceability must not be completed through invented Service ownership or consumption.
- K13: downstream representation traceability must not be completed through anticipated FHIR, application or technology mappings.

These examples justify an architecture-wide rule; its application does not depend on Domain04 or G1 terminology.

Consequences

Human architects, human developers and AI development agents SHALL apply this axiom to architecture derivation, documentation generation and implementation.

Architectural traceability SHALL represent established relationships and explicitly preserve unresolved or unestablished relationships. Completeness SHALL NOT be manufactured through:

- inferred hierarchy;
- structural convenience;
- lexical similarity;
- conventional modelling patterns;
- assumed equivalence;
- anticipated implementation;
- familiar standards;
- probable application design.

Candidates, suggestions and recommendations SHALL be distinguishable from documented, approved architecture. A proposed resolution or apparently better alternative SHALL NOT acquire authority through downstream use.

AI development agents SHOULD challenge architecture when evidence warrants architectural review. They SHALL NOT silently bypass it because they recognise a familiar destination.

This axiom applies the authority and explicit-review obligations in Purpose, Relationship to Architecture Decisions and Architectural Review Rule to downstream derivation and implementation.

AX-14 preserves semantic distinctions; this axiom preserves the distinction between established, unresolved and proposed architecture. AX-15 governs uncertainty about operational state, outcome or effect; this axiom governs uncertainty about the architecture itself. Neither form of uncertainty may be silently resolved by assumption.

AX-02 and AX-04 preserve Harmonia's internal semantics from external standards and engine assumptions. Familiar standards or technology patterns do not establish otherwise undocumented architectural relationships. AX-06 governs the authority of managed information; architectural authority remains a distinct concern.

Maxim

Traceability must be truthful, not artificially complete.

AX-18 — Integration and Coordination Do Not Confer Domain Authority

Axiom

> Harmonia MAY integrate information concerning, and coordinate operational activity associated with, an externally governed domain activity without thereby acquiring authority to determine or manage that domain activity.
>
> Authority to determine or manage a domain activity SHALL remain with the responsible actor, system or governed capability unless that authority is explicitly assigned to Harmonia by the architecture.
>
> Harmonia's observation, processing, persistence, distribution, presentation or use of information concerning a domain activity SHALL NOT, by itself, establish authority over that activity.

Rationale

Participation in a domain activity and authority over that activity are different architectural concerns. Information possession, observation, integration, coordination and processing do not, by themselves, confer domain authority. Authority requires explicit architectural assignment.

This new human-approved axiom generalises an already-established class of architectural boundary. It is established at the 2026-10-09 migration boundary; it is not a claim that AX-01 through AX-17 always contained this axiom.

Consequences and qualifications

AX-18 SHALL NOT be interpreted as preventing Harmonia from performing operational activity for which architecture has assigned Harmonia responsibility. Explicit architectural assignment may establish authority to determine or manage a domain activity.

In particular, AX-18 does not mean:

- Harmonia cannot execute Praxis/Ergo activity;
- Harmonia cannot coordinate entity-specific operational activity through a Digital Twin;
- Harmonia cannot process or transform clinical information;
- Harmonia cannot perform workflow/activity coordination;
- Harmonia cannot manage responsibilities explicitly assigned to Harmonia;
- Harmonia cannot enforce policy, security, information governance or operational controls assigned to it.

Participation, information, integration and coordination do not, by themselves, establish authority to determine or manage the underlying domain activity. Assigned Harmonia operational responsibility remains applicable.

Examples and established specialisations (non-normative)

These examples explain the boundary; they do not expand the axiom or establish Application Components, Information Concepts, Services, Processes, AI allocation or implementation mechanisms.

- **Clinical work:** Harmonia may integrate and coordinate around clinical work without thereby acquiring authority to determine clinical work, manage clinical handover, own clinical worklists, allocate clinical work, make clinical decisions or become the EMR workflow authority. The detailed Domain03 clinical-work integration boundary remains the Business specialisation.
- **Digital Twins:** A Twin may coordinate operational activity associated with its real-world entity without thereby acquiring authority over originating clinical facts or domain decisions. The Strategy entity-specific coordination rule remains controlling for that coordination.

- **Capacity:** Higher-order Service Capacity Management may consume context-specific capacity without acquiring the originating responsibility for Clinic, Ward, Theatre, ED, Outpatient or Mobile Service Capacity Management. The detailed Business capacity responsibility boundary remains in Business Architecture.
- **Assurance:** Observation, access to evidence or establishment of an assurance conclusion does not confer management authority over the activity being assured. Independent Assurance remains governed by REQ-FND-005 and its established Strategy and Business derivation.
- **Presentation / UI:** Presentation or collection of information does not, by itself, confer originating information or domain-decision authority on the presenting component. The existing Iris contextual human interaction responsibility remains distinct.
- **AI:** Assistance with interpretation, pattern recognition, criteria evaluation or decision support does not, by itself, confer the authority of the governed actor, role or capability using that assistance. This example assigns no AI responsibility or capability.

Related architectural authority

- AX-06 governs information authority and credibility.
- AX-13 governs the Harmonia management boundary at ingress/egress.
- AX-14 preserves meaningful semantic distinctions.
- AX-16 governs coordinated operational activity/entity progression.
- AX-17 governs architectural authority and explicit uncertainty.
- AX-18 governs inference of domain authority from participation, information, integration or coordination.

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